

Universal Catastrophic Coverage with Health Savings Empowerment: A Structural Replacement for the ACA

A Market-Oriented Framework for Restoring Efficiency and Fiscal Sustainability in American Health Care

May 2026

Bennett K. Alphson MSc

(Independent Policy Designer and Health Systems Analyst)

Author and Principal Developer of UCC + HSE Model

EXECUTIVE BRIEF

American health care does not suffer from a lack of spending. It suffers from a misallocation of spending. Public programs, employer plans, and households continue to finance a system that routes both catastrophic events and routine care through comprehensive insurance. That structure weakens price discipline, inflates administrative costs, suppresses transparent routine-care markets, and ties federal subsidy growth to premium inflation rather than to household protection.

The Affordable Care Act expanded coverage, but it did not alter this underlying economic structure. Premium tax credits rise with benchmark premiums. Essential health benefit mandates preserve comprehensive-plan inflation. Exchange administration adds complexity without restoring price discovery. The result is a subsidy model that protects many households in the short run while leaving the federal government exposed to long-term premium growth.

Universal Catastrophic Coverage with Health Savings Empowerment replaces that structure with a simpler framework: insure catastrophic risk, finance routine care directly, and target federal support where it has the strongest economic justification.

Under UCC/HSE, every eligible citizen and statutorily eligible long-term lawful resident receives standardized catastrophic protection for large, financially ruinous medical events. Routine, preventive, pediatric, behavioral-health, and chronic-care needs are financed through portable Health Savings Empowerment accounts, with additional federal support for low-income households, children, and chronic-condition patients. Medicare remains unchanged. Medicaid remains focused on long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity.

The model is not repeal-only, not a pure HSA expansion, and not a reduction in catastrophic protection. **It is a replacement architecture: universal catastrophic insurance, funded routine-care accounts, federal reinsurance, employer-market reform, chronic-condition safeguards, rural stabilization, supplemental-coverage guardrails, and statutory fiscal controls.**

Core Design

UCC/HSE separates American health financing into two distinct layers.

First, the catastrophic layer covers major medical events above a national catastrophic threshold. This includes hospitalization, trauma, major surgery, intensive care, severe

acute illness, and other high-cost episodes. A federal reinsurance layer absorbs 80 percent of claims above \$50,000, reducing insurer exposure to extreme cases and stabilizing catastrophic premiums.

Second, the routine-care layer is financed through Health Savings Empowerment accounts. Primary care, routine labs, imaging, common outpatient services, behavioral-health visits, chronic-care monitoring, and many predictable services are paid through HSAs, direct-pay arrangements, or transparent routine-care markets. Federal HSA deposits are prefunded at the beginning of the benefit year for low-income and chronic-condition households, preventing predictable care needs from being pushed into emergency departments, uncompensated care, or medical debt.

This structure restores the proper function of insurance. Catastrophic risk is pooled. Routine care is purchased transparently. Federal support is bounded, formula-based, and directed toward financial protection rather than open-ended comprehensive-plan inflation.

Key Model Estimates

The fiscal estimates below are internal CBO-style model estimates, not official Congressional Budget Office scores.

Table ES-1. UCC/HSE Key Numbers at a Glance

Metric	Estimate
10-year net federal transition cost (Annex E / CBO-style scoring workbook)	+\$763.09B
20-year cumulative federal savings (Annex E / CBO-style scoring workbook)	\$7.462T
30-year cumulative federal savings (Annex E / CBO-style scoring workbook)	\$32.93T
10-year direct consumer savings (Annex E Table 6 / consumer-savings schedule)	\$2.203T
10-year Medicaid consumer savings (Annex E Table 7 / Medicaid transition population)	\$127.92B
10-year federal catastrophic credits (Annex E / HC-251 2023 excess-spending method)	\$6.013T
10-year federal HSA deposits (Annex E / formula-based HSE deposits)	\$5.283T
10-year federal reinsurance (Annex E / 80% above \$50,000 high-cost layer)	\$264.9B
Provider impact before / after stabilization (FY2024 HCRIS provider-impact model)	-\$11.30B → -\$11.08B
Rural impact before / after access protection (FY2024 HCRIS rural / CAH / IHS providers)	-\$0.341B → -\$0.123B

Source note: Federal fiscal estimates are drawn from the UCC/HSE CBO-style scoring workbook and Annex E. Catastrophic-credit estimates use MEPS HC-251 2023 to model expected spending above the deductible rather than treating the deductible itself as the cost unit. Provider-impact estimates are derived from the FY2024 HCRIS provider-impact model.

Federal fiscal totals are drawn from the UCC/HSE scoring workbook and Annex E. Provider-impact and rural-access estimates are derived from the FY2024 HCRIS provider-impact model.

Why the Model Saves Money

UCC/HSE does not rely on consumer shopping alone. Routine-care price transparency is important, but the long-term fiscal effect comes from the interaction of several structural changes.

The first is the replacement of open-ended ACA premium subsidies with bounded catastrophic credits and formula-based HSA deposits. Under the ACA, federal subsidies rise as benchmark premiums rise. Under UCC/HSE, federal support is tied to a defined catastrophic benefit and explicit deposit formulas rather than to comprehensive-plan inflation.

The second is the removal of routine care from comprehensive insurance. Routine services are the portion of the system most capable of price discovery. When primary care, imaging, laboratory testing, common outpatient services, and routine specialist visits are financed directly, providers face greater pressure to disclose prices and compete.

The third is employer-market re-composition. The federal tax preference for comprehensive employer-sponsored insurance is phased down. Employers remain involved, but federal tax policy no longer rewards opaque premium growth. Employers may contribute to HSAs, facilitate catastrophic-plan enrollment, offer narrow supplemental benefits, or convert former premium compensation into wages.

The fourth is federal reinsurance. Reinsurance is not the entire model. It is the high-cost-claim stabilizer. The experience of Alaska, Maine, and Wisconsin supports the narrower claim that high-cost-claim reinsurance can reduce premiums and stabilize risk pools. The national UCC/HSE architecture rests on broader modeling: expenditure-concentration data, federal subsidy baselines, HSA deposit formulas, employer-exclusion reform, and provider-impact analysis.

The fifth is provider-sector re-composition. UCC/HSE preserves payment for catastrophic and high-acuity care while applying price discipline to routine and shoppable outpatient services. The model shifts revenue away from opaque facility-fee billing and high-markup routine claims toward transparent routine-care markets, direct primary care, ambulatory surgery centers, independent imaging, laboratories, and efficient specialist practices.

Protection for Vulnerable Populations

The strongest objection to catastrophic-first reform is that high deductibles can harm low-income and chronically ill patients. That objection is valid for poorly designed high-deductible systems. UCC/HSE addresses it through financing rather than by recreating comprehensive insurance.

Low-income households receive prefunded HSA deposits. Children receive pediatric routine-care support. Certified chronic-condition patients receive additional HSA deposits, income-protective routine-care caps, risk-adjusted support, and optional condition-based bundles. The catastrophic plan remains uniform; what varies is the financing beneath the deductible.

Medicaid is not abolished. UCC/HSE transitions non-disabled adult acute-care populations into catastrophic coverage and HSE financing while preserving Medicaid for the populations Medicaid is structurally best suited to serve: LTSS, ABD populations, medically complex children, HCBS, and CHIP continuity.

Medicare is not altered. Medicare eligibility, traditional Medicare benefits, Medicare Advantage, Part D, and Medigap remain outside the UCC/HSE replacement population.

Rural and Provider Stability

A national catastrophic-and-HSA model must account for rural health systems. Rural hospitals face low volume, fixed standby costs, workforce constraints, long transfer distances, and limited substitutes. UCC/HSE therefore includes targeted provider-side stabilization without changing the uniform catastrophic benefit.

The rural framework includes a permanent rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider. It also includes a temporary implementation-period floor of -1.5 percent of net patient revenue. The permanent payment recognizes structural rural access constraints. The temporary floor protects against transition shock while providers adapt.

The FY2024 HCRIS provider-impact model shows rural impact improving from approximately -\$0.341 billion before access protection to approximately -\$0.123 billion after the preferred rural access package. The framework preserves rural access without using rural policy as a back door to restore comprehensive-plan inflation.

Guardrails Against Benefit Creep

UCC/HSE allows supplemental coverage but prevents it from recreating ACA-style comprehensive insurance under another label. Supplemental products are limited to

narrow ancillary, elective, or targeted coverage. They may not functionally reduce the catastrophic deductible, duplicate major-medical coverage, or receive federal support for first-dollar routine care.

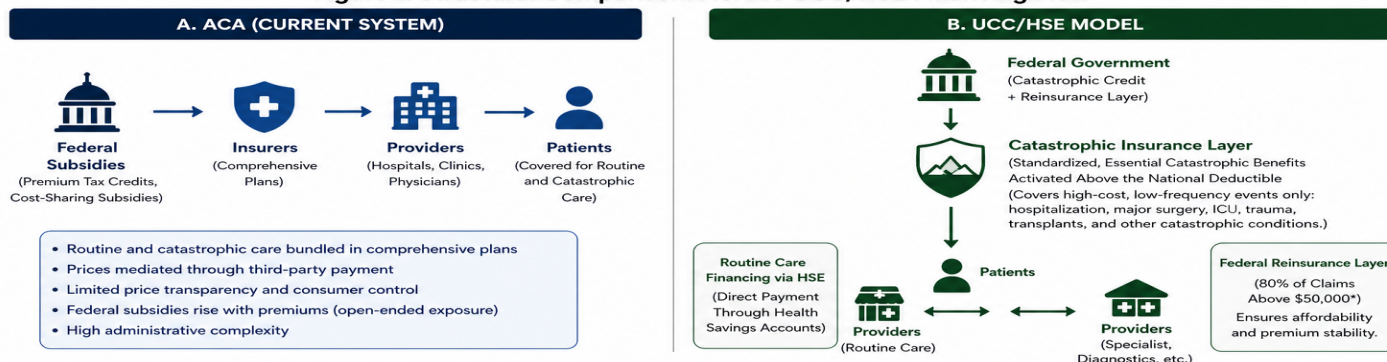
This guardrail is essential. Without it, insurers and large employers would gradually rebuild comprehensive first-dollar coverage through layered riders. UCC/HSE prevents that drift through actuarial-value limits, standardized disclosures, and prohibitions on federally subsidized wraparound major-medical coverage.

Conclusion

UCC/HSE rebuilds American health financing around a clearer division of responsibility. Insurance protects against catastrophic risk. HSAs finance routine care. Federal support is directed toward catastrophic protection, routine-care liquidity, chronic disease burden, pediatric care, rural access, and vulnerable populations. Medicaid remains focused on disability and long-term care. Medicare remains untouched. Employers remain involved, but federal tax policy no longer subsidizes comprehensive premium inflation.

The result is a universal system without single payer, a market-oriented system without laissez-faire exposure, and a fiscally bounded system without abandoning low-income or chronically ill households. UCC/HSE provides universal financial protection while addressing the central failure of the current system: the use of open-ended federal subsidies to finance comprehensive insurance inflation.

Figure 1. Structural Comparison: ACA vs UCC/HSE Financing Flow



Note: Diagram simplifies financial flows for conceptual clarity. UCC represents catastrophic-only insurance. HSE represents Health Savings Empowerment accounts used for routine and predictable medical expenditures.

*Policy parameter illustrative; see Annex B.

Table 1. Structural Comparison: ACA vs UCC/HSE

Component	ACA (Current Law)	UCC/HSE Model
Insurance Scope	Comprehensive (routine + catastrophic bundled)	Catastrophic-only coverage
Routine Care Financing	Insurance-mediated, third-party payment	Direct payment via HSA (HSE)
Catastrophic Protection	Included within comprehensive plans	Standardized national catastrophic layer
Federal Subsidies	Premium tax credits (open-ended, indexed to premiums)	Fixed catastrophic credit (bounded exposure)
Price Transparency	Low (prices obscured by insurer-provider contracts)	High (cash-pay and posted pricing)
Administrative Complexity	High (exchanges, AV tiers, EHB mandates, risk adjustment)	Reduced (simplified catastrophic plans + HSA infrastructure)
Employer Role	Central (tax-advantaged ESI system)	Reduced (shift to wages + optional HSA contributions)
Incentive Structure	Encourages benefit expansion and premium growth	Encourages price discipline and cost minimization
Chronic Condition Support	Subsidized via plan design and cost-sharing reductions	Targeted HSA funding + risk-adjusted catastrophic protection
Federal Fiscal Exposure	Expands with healthcare cost growth	Constrained via benchmark-linked catastrophic support
Market Competition	Fragmented, state-based exchange markets	Interstate catastrophic-plan competition
Risk Pooling	Mixed across comprehensive plans	Concentrated in catastrophic layer

Note: HSE (Health Savings Empowerment) refers to portable, tax-advantaged accounts used to finance routine and predictable medical expenditures outside the insurance system.

FULL EXECUTIVE SUMMARY

American health care absorbs rising levels of public and private spending while producing weak price discipline, limited consumer autonomy, and persistent market instability. Federal subsidy structures tied to premium inflation, employer-based benefit distortions, and the spread of comprehensive first-dollar coverage have insulated routine medical spending from ordinary competitive pressure. The United States spends nearly 17 percent of GDP on health care, almost twice the OECD average, yet continues to face predictable cost escalation, administrative complexity, uneven financial protection, and limited transparency in routine care markets **(1,2)**.

The Affordable Care Act (ACA) expanded coverage, but it did not change the economic engine driving medical inflation. Premium tax credits remain tied to benchmark premiums; as premiums rise, federal outlays rise with them. Essential health benefit mandates preserve comprehensive benefit inflation. Exchange infrastructure adds administrative complexity without restoring meaningful price discovery. As the Congressional Budget Office projects approximately \$1.4 trillion in premium-tax-credit outlays over the next decade **(3)**, the limits of subsidizing comprehensive insurance through open-ended federal commitments have become increasingly clear.

This white paper proposes a structural replacement: Universal Catastrophic Coverage (UCC) paired with Health Savings Empowerment (HSE). UCC/HSE reorganizes American health financing around a simple distinction: catastrophic medical risk should be insured, pooled, and federally stabilized; routine and predictable care should be financed through transparent, portable, consumer-controlled accounts, with targeted federal liquidity for low-income households, children, and chronic-condition patients.

The proposal is not a reduction in financial protection. It is a change in the form of protection. UCC/HSE replaces the ACA's comprehensive-insurance subsidy model with a two-part architecture: universal catastrophic coverage for large, financially ruinous medical expenses, and Health Savings Empowerment accounts for routine, preventive, pediatric, behavioral-health, and chronic-care needs. The objective is to protect households against medical bankruptcy while reducing the federal government's exposure to premium-linked subsidy inflation.

Under UCC/HSE, a typical household would receive:

1. A single catastrophic insurance plan with a fixed national deductible;
2. Annual HSA deposits for routine, preventive, pediatric, and chronic care;

3. Direct payment for routine services at transparent prices;
4. Full protection against high-cost medical events; and
5. No change to Medicare benefits, Medicare eligibility, Medicare Advantage, Part D, or Medigap.

Under UCC, every U.S. citizen and statutorily eligible long-term lawful resident receives standardized catastrophic-only protection through private carriers or a public option offering a federally defined catastrophic benefit. Coverage activates above the national catastrophic threshold. A federal reinsurance layer, modeled on the Section 1332 waiver experience in states such as Alaska, Maine, and Wisconsin, absorbs 80 percent of claims above \$50,000 (4). This mechanism stabilizes catastrophic premiums, reduces insurer exposure to high-cost outliers, and replaces the ACA's dependence on rising comprehensive-plan premiums.

The Alaska, Maine, and Wisconsin experience is used here as evidence for high-cost-claim reinsurance, not as a stand-alone validation of the entire UCC/HSE model. Those state programs show that reinsurance can reduce premiums and stabilize individual-market risk pools. The broader UCC/HSE architecture rests on additional modeling layers: MEPS expenditure-concentration data, CBO/JCT federal subsidy baselines, the Annex E scoring workbook, international catastrophic/savings-based analogues, and the FY2024 HCRIS provider-impact model.

Federal financing under UCC/HSE flows through three channels: catastrophic credits for UCC-compliant plans, formula-based HSA deposits for routine and chronic care, and federal reinsurance for high-cost claims. These replace ACA premium tax credits, cost-sharing reductions, exchange subsidies, and the federal tax preference for comprehensive employer-sponsored insurance. The model does not abolish employer participation. Employers remain free to contribute to HSAs, facilitate catastrophic-plan enrollment, offer narrow supplemental benefits, or convert former premium compensation into wages as the federal tax preference for comprehensive employer insurance phases down.

The fiscal engine of UCC/HSE is not consumer shopping alone. Routine-care price transparency contributes savings, but long-term solvency depends on the combined effect of benchmark-linked catastrophic reimbursement, fixed federal HSA formulas, income-calibrated support, federal reinsurance, employer-exclusion reform, and statutory guardrails against benefit creep. Enforceable benchmark-linked pricing in the catastrophic layer prevents universal catastrophic protection from becoming a conduit for unlimited

hospital and specialist price transmission. Fixed HSA formulas prevent routine-care subsidies from expanding mechanically with comprehensive-plan premiums.

Under HSE, routine and predictable medical care is financed through portable, tax-advantaged accounts. Primary care, mental-health services, imaging, laboratory testing, routine specialist visits, chronic-condition monitoring, and many preventive services would be purchased through HSAs, direct-pay arrangements, and transparent routine-care markets. FAIR Health price-index data document substantial price variation across routine and outpatient services, while RAND research on price transparency and direct-payment arrangements shows that transparent purchasing and patient steering can reduce prices for selected shoppable services **(5,20,30,31,35,36)**. HSE applies that logic systemwide while preserving catastrophic protection for severe events.

The model also includes a routine-care affordability floor. Federal HSA deposits are prefunded at the beginning of the benefit year for low-income and chronic-condition households. This ensures that routine-care liquidity exists before bills arise. The design is intended to prevent predictable care needs from shifting into emergency departments, uncompensated care, or medical debt, a known failure mode of poorly designed high-deductible systems.

Quantified federal budget effects are reported in the accompanying scoring workbook, which serves as the internal CBO-style scoring schedule for this paper. These estimates are not official CBO scores. As summarized in **Annex E Table 1**, UCC/HSE produces a net federal transition cost of approximately \$763.09 billion over the first ten years relative to the ACA baseline. The model then turns strongly positive over longer horizons, generating approximately \$7.462 trillion in cumulative federal savings over twenty years and approximately \$32.93 trillion over thirty years as benchmark-linked catastrophic pricing, fixed HSA formulas, employer-exclusion reform, administrative simplification, routine-care price discipline, and provider-market re-composition compound over time. The underlying outlay and savings components are reported in **Annex E Table 4** and **Annex E Table 5**.

The state-level premium effects observed under reinsurance programs should not be confused with UCC/HSE's twenty- and thirty-year federal savings estimates. Reinsurance programs measure premium effects from high-cost-claim stabilization in state individual markets. UCC/HSE's long-run federal estimates measure the cumulative budgetary effect of a full replacement architecture: catastrophic credits, HSA deposits, federal reinsurance, ACA subsidy replacement, employer-exclusion reform, administrative simplification, routine-care repricing, and provider-market re-composition.

The model also estimates substantial household savings. As reported in **Annex E Table 6**, UCC/HSE is projected to generate approximately \$2.203 trillion in direct consumer savings over 2026–2035 relative to the ACA baseline, driven by lower routine-care prices, reduced administrative overhead, lower comprehensive-premium exposure, and HSA-financed routine-care purchasing. As reported in **Annex E Table 7**, the affected Medicaid transition population is projected to realize approximately \$127.92 billion in Medicaid consumer savings over the same ten-year period.

Several design features are central to the model.

Interstate catastrophic-plan competition. Because catastrophic benefits are federally standardized, insurers licensed in one state may sell UCC-compliant catastrophic plans nationally. This expands consumer choice, reduces local insurance-market fragmentation, and limits the ability of state-specific benefit mandates to obstruct catastrophic-plan competition.

Chronic-support financing without insurance-tier creation. Individuals with certified chronic conditions receive additional HSA deposits, income-protective routine-care caps, risk-adjusted support, and optional condition-based bundles without changing the catastrophic benefit itself. The catastrophic plan remains uniform. What varies is the financing beneath the deductible.

Routine-care affordability floor. HSE funding is calibrated to preserve access to essential routine, preventive, pediatric, behavioral-health, and chronic-care services. This reduces the risk that low-income or medically vulnerable households defer care until conditions deteriorate.

Medicaid LTSS and ABD preservation. UCC/HSE reforms the non-disabled adult acute-care market while preserving Medicaid’s core responsibilities for long-term services and supports, home- and community-based care, Aged/Blind/Disabled populations, medically complex children, and CHIP continuity. Medicaid is not abolished. It is refocused on the populations and services for which Medicaid’s institutional structure is most necessary.

Medicare noninterference. Medicare remains outside the UCC/HSE replacement population. The framework does not alter Medicare eligibility, traditional Medicare benefits, Medicare Advantage, Part D, Medigap, or the statutory structure governing older Americans.

Employer-market re-composition. Employer-sponsored insurance is not eliminated. The federal tax preference for comprehensive employer insurance is phased down, while employers may continue to contribute to HSAs, facilitate catastrophic-plan enrollment,

offer narrow supplemental benefits, or convert former premium compensation into wages. Existing collective-bargaining agreements are preserved through expiration to prevent abrupt disruption.

Provider-sector re-composition. UCC/HSE preserves insured payment for catastrophic and high-acuity care while applying price discipline to routine and shoppable outpatient services. The model shifts revenue away from opaque facility-fee billing and high-markup routine claims toward transparent routine-care markets, direct primary care, ambulatory surgery centers, independent imaging, laboratories, and efficient specialist practices.

Rural stabilization. Rural and frontier providers receive targeted provider-side stabilization through a permanent rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider, paired with a temporary implementation-period floor of -1.5 percent of net patient revenue. The access-capacity payment is durable because rural geography, fixed standby capacity, and low patient volume are structural constraints. The implementation-period floor is temporary or reviewable because it protects against transition shock while providers adapt. Rural support is reinforced through Critical Access Hospital coordination, rural chronic-care bundles, virtual specialty networks, rural household HSA access supplements, transparency-linked operational flexibility, and time-limited transformation assistance. These mechanisms preserve access in low-density areas without altering the uniform catastrophic benefit or reintroducing ACA-style benefit inflation.

Supplemental-coverage guardrails. To prevent the re-creation of comprehensive first-dollar insurance, supplemental plans are limited to narrowly defined ancillary, elective, or targeted coverage. Actuarial-value caps, standardized disclosures, and prohibitions on wraparound major-medical duplication prevent the emergence of ACA-style silver, gold, or platinum equivalents under a different label.

High-cost drug and gene-therapy carve-out. Extremely high-cost biologics, cell therapies, and gene therapies remain outside the ordinary catastrophic pool unless administered as part of an acute inpatient episode. These therapies continue to be managed through Medicare, Medicaid ABD, orphan-drug pathways, rare-disease mechanisms, or specialized carve-outs. The purpose is to preserve actuarial integrity and prevent a small number of ultra-high-cost therapies from destabilizing the catastrophic pool.

Under full implementation, UCC/HSE is designed to:

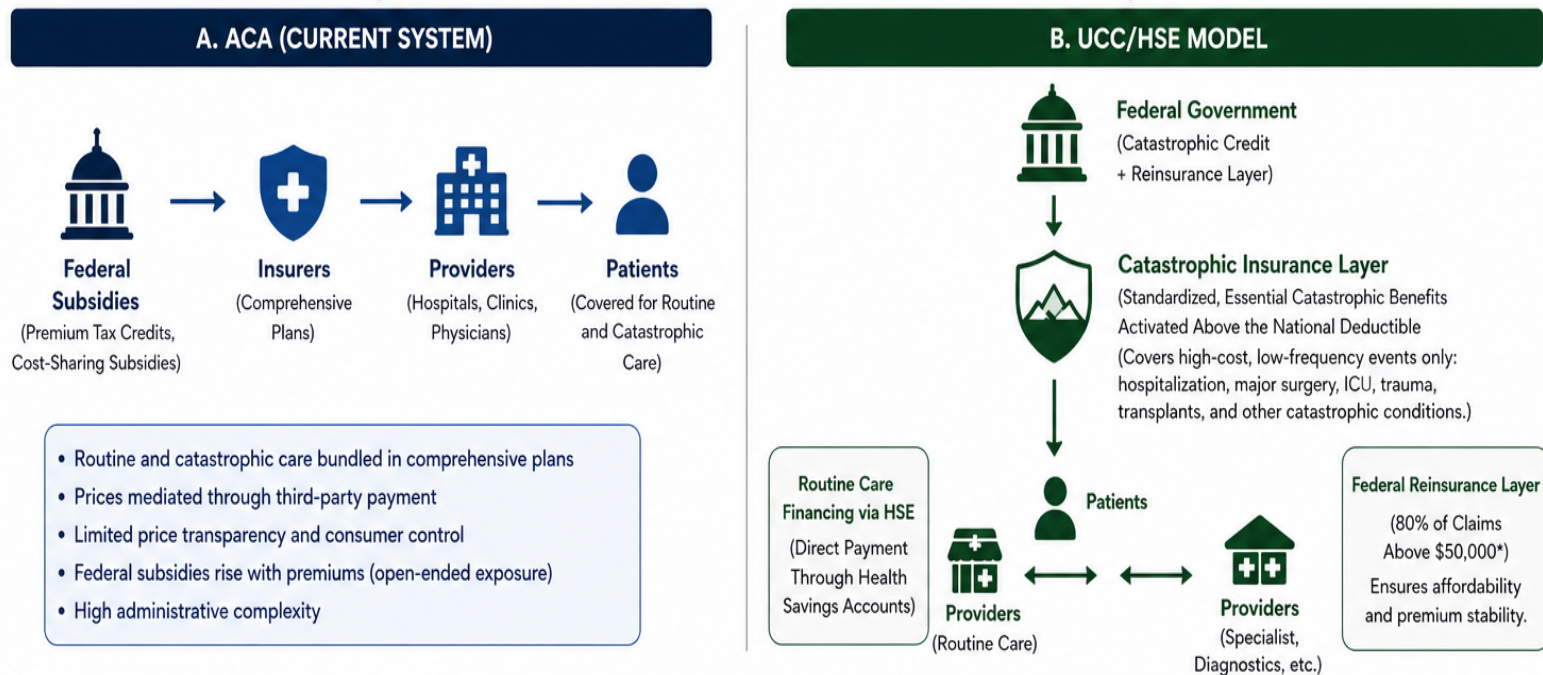
1. Reduce national health-expenditure growth by an estimated 8–12 percent through benchmark-linked catastrophic pricing, routine-care price transparency, reduced administrative overhead, and removal of routine-care insurance inflation;
2. Replace the ACA's open-ended subsidy architecture with bounded, actuarially predictable federal obligations;
3. Stabilize catastrophic premiums through federal reinsurance, simplified actuarial design, and standardized national benefit rules;
4. Recompose employer compensation by ending the federal bias toward comprehensive employer-plan inflation and shifting support toward wages, HSAs, and portable catastrophic protection;
5. Improve equity by preserving Medicaid LTSS/ABD functions, maintaining Medicare unchanged, and funding routine-care liquidity for low-income, pediatric, and chronic-condition populations;
6. Protect rural access through provider-side stabilization mechanisms that do not alter the uniform catastrophic insurance structure;
7. Prevent benefit creep through statutory guardrails that prohibit federally subsidized reconstruction of comprehensive first-dollar coverage; and
8. Strengthen federalism by reducing state administrative burdens while preserving state roles in solvency regulation, consumer protection, Medicaid coordination, rural supplements, and implementation flexibility.

The UCC/HSE framework rebuilds American health financing from first principles. Insurance protects against catastrophic risk. Routine care is purchased transparently. Federal support is targeted to catastrophic protection, routine-care liquidity, chronic disease burden, pediatric care, rural access, and vulnerable populations. Medicaid remains focused on disability and long-term care. Medicare remains untouched. Employers remain involved, but federal tax policy no longer favors opaque premium inflation. Federal spending becomes bounded rather than mechanically indexed to rising benchmark premiums.

UCC/HSE is therefore not repeal-only, not a pure HSA expansion, and not a reduction in catastrophic protection. It is a replacement architecture: universal catastrophic insurance, funded routine-care accounts, chronic-condition safeguards, Medicaid preservation where Medicaid is structurally necessary, rural stabilization, employer-market re-composition, and enforceable fiscal guardrails. Its objective is to provide universal financial protection

without perpetuating the open-ended subsidy dynamics, administrative complexity, and routine-care price inflation that define the current system. The structural distinction is shown in **Figure 1 and Table 1**: the ACA routes routine and catastrophic care through comprehensive insurance, while UCC/HSE separates catastrophic risk pooling from routine-care financing through HSAs.

Figure 1. Structural Comparison: ACA vs UCC/HSE Financing Flow



Note: Diagram simplifies financial flows for conceptual clarity. UCC represents catastrophic-only insurance. HSE represents Health Savings Empowerment accounts used for routine and predictable medical expenditures.

*Policy parameter illustrative; see Annex B.

Table 1. Structural Comparison: ACA vs UCC/HSE

Component	ACA (Current Law)	UCC/HSE Model
Insurance Scope	Comprehensive (routine + catastrophic bundled)	Catastrophic-only coverage
Routine Care Financing	Insurance-mediated, third-party payment	Direct payment via HSA (HSE)
Catastrophic Protection	Included within comprehensive plans	Standardized national catastrophic layer
Federal Subsidies	Premium tax credits (open-ended, indexed to premiums)	Fixed catastrophic credit (bounded exposure)
Price Transparency	Low (prices obscured by insurer-provider contracts)	High (cash-pay and posted pricing)
Administrative Complexity	High (exchanges, AV tiers, EHB mandates, risk adjustment)	Reduced (simplified catastrophic plans + HSA infrastructure)
Employer Role	Central (tax-advantaged ESI system)	Reduced (shift to wages + optional HSA contributions)
Incentive Structure	Encourages benefit expansion and premium growth	Encourages price discipline and cost minimization
Chronic Condition Support	Subsidized via plan design and cost-sharing reductions	Targeted HSA funding + risk-adjusted catastrophic protection
Federal Fiscal Exposure	Expands with healthcare cost growth	Constrained via benchmark-linked catastrophic support
Market Competition	Fragmented, state-based exchange markets	Interstate catastrophic-plan competition
Risk Pooling	Mixed across comprehensive plans	Concentrated in catastrophic layer

Note: HSE (Health Savings Empowerment) refers to portable, tax-advantaged accounts used to finance routine and predictable medical expenditures outside the insurance system.

TABLE OF CONTENTS

Executive Brief.....	(2)
Full Executive Summary.....	(7)
Table of contents.....	(14)
List of Tables and Figures.....	(15)
I. Introduction.....	(18)
II. Background and Literature Review.....	(18)
III. The Case for Structural Health-Care Reform.....	(23)
IV. Universal Catastrophic Coverage (UCC).....	(33)
V. Health Savings Empowerment (HSE).....	(41)
VI. How UCC/HSE Replaces the ACA.....	(46)
VII. Why UCC/HSE Lowers Costs for Everyone.....	(55)
VIII. Fiscal Modeling and Federal Budget Analysis.....	(61)
IX. Equity and Chronic-Condition Safeguards.....	(70)
X. Federalism and the Role of States.....	(83)
XI. RURAL HEALTH STABILITY UNDER UCC/HSE.....	(88)
XII. Implementation Pathway.....	(98)
XIII. Risks and Limitations.....	(102)
XIV. Cross-Ideological Foundations.....	(106)
XV. Legal and Constitutional Considerations.....	(109)
XVI. Comparative Cost Projections of Medicare for All vs ACA vs UCC + HSE.....	(110)
XVII. Provider Impact Analysis with Numeric Tables	(113)
XVIII. Employer Market Sector Analysis	(128)
XIX. Political Risk Mitigation.....	(132)
XX. Political Coalition Roadmap.....	(142)
XXI. Conclusion.....	(146)
Appendix A: Methodology	(149)
Appendix B: Reproducible HCRIS-Based Provider-Impact Scoring Appendix.....	(153)
Appendix C: References.....	(173)

ANNEX A: Draft Section-by-Section Legislative Outline.....	(183)
U.S. CODE INTERACTION SUMMARY.....	(197)
ANNEX B: FEDERAL CATASTROPHIC CREDIT AND HSA DEPOSIT FORMULAS.....	(208)
ANNEX C: Formula Section.....	(210)
Aggregate Implementation for Federal Scoring.....	(231)
Illustrative Parameter Set (Modeling Baseline).....	(233)
Annex D: TEN-YEAR FEDERAL SCORING FRAMEWORK.....	(237)
ANNEX E: 10-year CBO-style Scoring Table with Historical CBO Data.....	(250)
Supplementary Data and Analysis Repository	(250)
Supplementary Tables and Figures.....	(256)

List of Tables and Figures:

FIGURES:

Figure 1. Structural Comparison: ACA vs UCC/HSE Financing Flow.....	(6,13)
Figure 2. MEPS-Based Concentration Curve of Healthcare Expenditures, Non-Medicare Population.....	(28)
Figure 3. AHRQ Expenditure-Concentration Evidence: Medical Costs Concentrated Among Top Spenders.....	(30)
Supplementary Figure 1. AHRQ Evidence on Health-Care Expenditures Concentrated in the Elderly Population.....	(259)
Supplementary Figure 2. AHRQ Evidence on Service Categories Used by the Bottom 50 Percent of Health-Care Spenders.....	(260)
Supplementary Figure 3. AHRQ Evidence on Out-of-Pocket and Private-Insurance Shares Among the Bottom 50 Percent of Health-Care Spenders.....	(261)

TABLES:

Table ES-1. UCC/HSE Key Numbers at a Glance.....	(3)
Table 1. Structural Comparison: ACA vs UCC/HSE.....	(6,13)
Table 2. MEPS HC-251 Catastrophic-Threshold Calculations: Non-Medicare Spending Above \$7,500.....	(29)
Table 3. AHRQ Expenditure-Concentration Evidence: Medical Costs Concentrated Among Top Spenders.....	(30)
Table 4. Comparative Cost Projections of Medicare for All vs ACA vs UCC/HSE.....	(112)

Table 5. System-Level Provider-Facing Financial Flows Under UCC/HSE.....	(114)
Table 6. Provider Impact Allocation Framework Under UCC/HSE.....	(116)
Table 7. Stabilized Provider-Impact Model: Before and After Targeted Access.....	(118)
Table 8. Provider-Segment Effects Under Stabilized UCC/HSE.....	(121)
Table 9. Employer-Sector Federal Budget Interface Under UCC/HSE.....	(130)
Table 10. Employer Compensation Re-composition Under UCC/HSE.....	(131)
Table 11. Political Risk Mitigation Matrix.....	(138)
Table 12. Special Statutory Vulnerabilities.....	(141)
Table 13. Potential Congressional Coalition Entry Points for UCC/HSE.....	(146)

ANNEX E TABLES:

Annex E Table 1. Summary Table: Outlays–Savings, 2026–2056.....	(251)
Annex E Table 2. Beta Calculation.....	(251)
Annex E Table 3. Gamma Calculation.....	(251)
Annex E Table 4. Outlays: UCC/HSE, 2026–2056.....	(252)
Annex E Table 5. Savings: UCC/HSE, 2026–2056.....	(253)
Annex E Table 6. Consumer Savings, 2026–2035.....	(254)
Annex E Table 7. Medicaid Consumer Savings.....	(255)

SUPPLEMENTARY TABLES:

Supplementary Table 1. 2021 CBO Ten-Year Scoring of Net Federal Subsidies for Health Insurance.....	(256)
Supplementary Table 1 continued. 2021 CBO Ten-Year Scoring of Net Federal Subsidies for Health Insurance.....	(257)
Supplementary Table 2. 2021 CBO Ten-Year Projection of Health Insurance Coverage.....	(257)
Supplementary Table 3. Future Projections of Medicare Enrollment.....	(258)

Digital supplements

provider-impact summaries in Appendix B, pp. 166–168;
federal scoring workbook summarized in Annex E, pp. 251–255;

<https://github.com/Galtman-droid/Universal-Catastrophic-Coverage-with-Health-Savings-Empowerment-A-Structural-Replacement-for-the-ACA>

Supplementary Table 7A. Provider Impact 3×3 Scenario Matrix

Supplementary Table 7B. Provider Impact by Major Provider Group

Supplementary Table 7C. Provider Impact by Clean Provider Class

Supplementary Table 7D. Distribution of Provider-Level Effects

Supplementary Table 7E. Model Coverage and Data Quality Audit

Supplementary Table 7F. Exposure-Anomaly Sensitivity

Supplementary Table 7G. Access-Protection Policy Options

Supplementary Table 7J. Provider Transition-Protection Sensitivity

Supplementary Table 7K. Enhanced Rural Access Sensitivity

Supplementary Table 7L. Rural Provider-Level Enhanced Access Output

Supplementary Table 7M. Psychiatric / Behavioral Provider Sensitivity

Supplementary Table 7N. Psychiatric / Behavioral Provider-Level Review

Supplementary Table 7O. Remaining Severe Psychiatric / Behavioral Effects

Supplementary Table 7P. Behavioral Review Universe

Supplementary Table 8. Ten-year CBO table UCC_HSE_OUTLAYS_exceedShare_fix with full HC-251

I. INTRODUCTION

The central problem of American health care is not a lack of spending; it is the *misallocation* of spending. While federal, state, employer, and household outlays have risen continuously, the structure of American health insurance has remained rooted in a model that treats nearly all care, routine and catastrophic alike, as an insurable event. This encourages price inflation, fosters administrative proliferation, and suppresses the competitive mechanisms that discipline costs in most other sectors of the economy.

The ACA brought millions of Americans into coverage but did not reform this underlying structure. Instead, it layered additional subsidies and regulatory mechanisms onto a system already suffering from pervasive inefficiencies. The exchanges, premium tax credits, essential health benefits, and cost-sharing reductions were all designed to stabilize the individual market, but none addressed the fundamental fact that most U.S. health spending is channeled through benefit designs that are both overly comprehensive and excessively intermediated.

The purpose of this paper is to articulate a framework that restores health insurance to its proper economic function: protection against large, unpredictable losses. By removing routine care from insurance and reorienting federal support toward transparent catastrophic coverage, the UCC/HSE model offers a path toward long-term fiscal sustainability and improved consumer welfare.

II. BACKGROUND AND LITERATURE REVIEW

II.A. Administrative Overhead in the U.S. System

Administrative overhead is one of the central structural weaknesses of the American health-care system. The problem is not merely that the United States spends more on health care than peer countries; it is that a large share of that spending is absorbed by the machinery required to finance care through fragmented comprehensive insurance. When routine care, catastrophic care, employer benefits, public subsidies, provider networks, cost-sharing rules, prior authorization, negotiated rates, and claims adjudication are routed through multiple third-party payment systems, administrative cost becomes a structural feature of the financing model rather than an incidental expense.

The comparative literature establishes the scale of the problem. Papanicolas et al. find that administrative costs account for 8 percent of U.S. health spending, compared with 1–3 percent in the other high-income countries studied **(9)**. Broader billing-and-insurance-related estimates place the burden higher. Gee and Spiro estimate total billing-and-insurance-related administrative costs at approximately \$496 billion in 2019, including

roughly \$282 billion in provider-side billing-and-insurance-related costs and \$158 billion in private-insurance overhead; they further estimate approximately \$248 billion in excess administrative cost relative to more efficient systems **(10)**. Baranoff et al., using national health expenditure data, report that private insurers paid approximately \$1.24 trillion in health-care expenses in 2018 while incurring \$164.3 billion in administrative expenses, equal to 13.22 percent of private-insurance health-care expenses **(11)**. These estimates differ in scope, but they point in the same direction: a substantial share of U.S. health spending is consumed by financing and billing architecture rather than clinical care itself.

Provider-side evidence shows how this burden appears operationally. Tseng et al. estimate billing and insurance-related costs of \$20.49 for a primary-care visit, \$61.54 for an emergency-department visit, \$124.26 for a general inpatient stay, \$170.40 for an ambulatory surgical procedure, and \$215.10 for an inpatient surgical procedure. More important for routine-care reform, those costs represented 14.5 percent of professional revenue for primary-care visits and 25.2 percent of professional revenue for emergency-department visits **(12)**. Cross-national work by Richman et al. similarly finds that billing-and-insurance-related costs for an inpatient surgical bill were approximately \$6 in Canada compared with \$215 in the United States **(19)**. These figures matter because UCC/HSE does not treat administrative overhead as an abstract national accounting category; it treats it as a concrete operating cost generated by payer-specific billing rules, coding systems, benefit determinations, prior authorization, documentation requirements, denial management, and payment reconciliation.

The literature also suggests that administrative savings do not require a single-reform architecture. Cutler estimates that administrative costs account for roughly one-quarter to one-third of U.S. health-care spending and argues that targeted simplification of billing, prior authorization, quality reporting, data exchange, and payment administration could save at least \$50 billion annually, or roughly \$150 per person **(13)**. Scheinker et al. estimate that billing-and-insurance-related costs could fall by 33–53 percent under single-payer models and by 27–63 percent under alternative multi-payer administrative-simplification models **(14)**. The relevance for UCC/HSE is not that these models are identical to UCC/HSE, but that large administrative reductions are possible when benefit administration, payer contracting, and claims-processing complexity are reduced.

Private-plan complexity is also embedded in network and contract design. GAO reports that provider-network oversight varies across state and federal regulators and that network adequacy depends on provider availability, geography, plan type, and issuer contracting challenges **(15)**. Federal transparency rules reveal the same complexity from another angle. The Transparency in Coverage final rule requires plans and issuers to disclose in-

network negotiated rates, out-of-network allowed amounts and billed charges, and prescription-drug pricing information; CMS also required consumer cost-sharing tools for 500 shoppable services beginning in 2023 and for all covered items and services beginning in 2024 **(16,17)**. These requirements may improve price visibility, but they also demonstrate how administratively complex plan-specific negotiated-rate insurance has become.

Medicare Advantage provides a related example of private-plan administration within a public-financing framework. MedPAC reports that Medicare Advantage bids include administrative costs and profit, and that rebates are calculated only after those costs are accounted for. In 2022, Medicare Advantage bids averaged 85 percent of fee-for-service Medicare spending, while benchmarks averaged 108 percent of fee-for-service Medicare spending; plans generally returned 65 or 70 percent of the difference between bid and benchmark through lower cost sharing, supplemental benefits, or lower premiums **(18)**. This evidence should not be mechanically generalized to employer coverage or ACA marketplace plans, but it confirms that administrative expense, benefit design, and private-plan financing remain material even inside a publicly funded program.

The ACA expanded coverage, but it did not simplify this administrative architecture. State and federal exchanges, metal tiers, actuarial-value calculations, essential-health-benefit compliance, cost-sharing rules, risk adjustment, eligibility reconciliation, plan-level reporting, and network oversight preserved comprehensive insurance while adding new regulatory and operational layers. UCC/HSE responds to that structural problem by separating catastrophic insurance from routine-care financing. Catastrophic risk remains insured and federally stabilized. Routine care is financed through portable Health Savings Empowerment accounts, direct-pay arrangements, and transparent routine-care markets. The objective is not to eliminate administration entirely, but to reduce the share of health spending consumed by claims processing, benefit management, network contracting, and routine-care insurance administration that exists only because predictable medical expenses are routed through comprehensive insurance.

II.B. Price Variability and Provider Market Concentration

Significant empirical evidence confirms that U.S. health-care prices are higher than those of peer countries and vary substantially across payers, providers, sites of care, and contracting arrangements. OECD data show that the United States devotes a much larger share of national income to health care than other high-income countries, while comparative research finds that the U.S. spending gap reflects prices and administrative structure more than unusually high utilization alone. OECD price level indices indicate that hospital and specialist services in the United States are priced **60–80 percent above** the

OECD median **(2,9)**. CBO similarly reports large differences between commercial insurance prices and Medicare payment rates for hospital and physician services, underscoring the pricing power embedded in private-plan contracting and provider market concentration **(8)**.

Provider consolidation and third-party payment reinforce this price structure. RAND's hospital-price transparency work documents wide variation in prices paid by private health plans, while FAIR Health price-index data show substantial variation across medical services, places of service, and payer channels **(6,20)**. Cooper et al. further show that hospital prices for the privately insured vary substantially and are shaped by provider market structure and bargaining leverage **(21,35)**. These findings support a central premise of UCC/HSE: when routine and shoppable care is routed through opaque negotiated contracts, prices are less visible to patients and less disciplined by ordinary comparison shopping.

The price-dispersion problem is especially clear in shoppable imaging. Jiang, Makary, and Bai examined commercial negotiated prices for 13 CMS-specified shoppable radiology services using hospital price-transparency data. CMS defines such services as nonurgent services that can be scheduled in advance, and the study found substantial variation in commercial negotiated prices across hospitals and plans **(100)**. For example, CT examination of the head or brain had a median commercial negotiated price of \$813 compared with a median Medicare rate of \$137, and the study found broad dispersion across other CT, MRI, mammography, and ultrasound services **(100)**. A related summary of the same research reported that, within the same hospital, a brain CT scan could range from \$134 to \$4,065 depending on the negotiated payer arrangement, illustrating that price dispersion is not only a cross-hospital problem but can also occur within the same facility and payer environment **(100)**.

Consumer-facing price examples point in the same direction, but they should be interpreted as illustrations rather than national averages. TheStreet reports routine MRI examples of \$1,500–\$3,000 through insurance-linked billing compared with \$300–\$500 at direct-pay imaging centers, and gives a stylized example of a \$2,400 MRI bill applied to a deductible versus a \$350 cash price at a direct-pay center **(101)**. The point is not that every MRI, CT scan, laboratory panel, or outpatient procedure has a fixed national cash discount. The point is narrower and better supported: many routine, schedulable, and outpatient services exhibit large differences across cash prices, chargemaster rates, and negotiated commercial rates, while patients often lack a simple mechanism to compare prices before receiving care.

This distinction is central to UCC/HSE. Catastrophic events (trauma, emergency hospitalization, intensive care, major surgery, transplants, and severe acute illness) are not meaningfully shoppable at the point of need and should remain insured. Routine and schedulable services, i.e., primary care, imaging, laboratory testing, routine specialist visits, selected behavioral-health visits, and common outpatient procedures, are more suitable for transparent, HSA-financed purchasing. UCC/HSE therefore preserves pooled catastrophic protection while moving routine-care financing away from opaque comprehensive-insurance billing and toward direct payment, posted prices, and portable household-controlled accounts.

II.C. The Role and Limitations of the ACA Subsidy Model

The ACA's premium tax credits were designed to make individual-market coverage affordable, but because they are indexed to the cost of local benchmark silver plans, federal outlays automatically grow as premiums rise. The CBO projects that premium-tax-credit spending will reach \$1.4 trillion between 2025 and 2034 under current law **(3)**. There is no mechanism within the ACA to counteract provider price inflation, insurer consolidation, or the administrative overhead embedded in comprehensive benefit designs.

While the ACA significantly reduced the uninsured rate, it achieved this primarily through fiscal transfers rather than by addressing cost fundamentals. As a result, health spending continues to rise faster than income, and federal subsidy exposure expands accordingly.

II.D. Evidence on High-Deductible Plans and Consumer Behavior

The evidence on high-deductible plans and consumer cost-sharing is mixed but highly relevant to UCC/HSE. The RAND Health Insurance Experiment established that higher cost-sharing reduces medical utilization, including routine and discretionary care, and that average health outcomes did not worsen for most participants. However, the same evidence also showed that cost-sharing can reduce both lower-value and higher-value care, with greater risk for low-income and medically vulnerable patients **(37)**. Later studies of high-deductible health plans and health savings accounts reach a similar conclusion: consumers do respond to out-of-pocket prices, but poorly designed deductibles can also discourage preventive care, medication adherence, and guideline-concordant chronic-disease management **(38–41)**.

This distinction is central. UCC/HSE is not a conventional high-deductible health plan. Conventional HDHPs often expose households to first-dollar costs without giving them adequate liquidity, price information, or protection against predictable chronic-care

expenses. UCC/HSE is designed to preserve the useful price-sensitivity mechanism while correcting those failures. Routine-care spending is financed through prefunded Health Savings Empowerment accounts, with larger deposits for low-income households, children, and certified chronic-condition patients. Catastrophic exposure remains insured above the national threshold, and chronic-care supports are provided beneath the deductible through targeted HSA deposits, income-protective caps, and condition-based bundles.

Price transparency strengthens this architecture but does not replace the need for liquidity safeguards. RAND price-transparency research, direct-payment evidence, and studies of shoppable-service price availability show that patients and purchasers can obtain lower prices for selected routine and schedulable services when prices are visible and providers compete on posted or comparable rates (5,7,36). The UCC/HSE framework applies that lesson to routine care while avoiding the central failure of ordinary HDHPs: asking patients to behave like price-sensitive consumers without first ensuring that they have usable funds, transparent prices, and protection against catastrophic or chronic financial risk.

The policy implication is therefore not that high deductibles alone are sufficient. They are not. The evidence supports a narrower and more defensible conclusion: consumer cost-sharing can reduce routine-care spending when paired with price visibility, but it must be combined with prefunded accounts, chronic-condition safeguards, and catastrophic insurance to avoid underuse of necessary care. UCC/HSE is built around that distinction.

III. THE CASE FOR STRUCTURAL HEALTH-CARE REFORM

The deficiencies of the current U.S. health-financing structure are not temporary or circumstantial. They arise from economic distortions embedded in employer-sponsored insurance, federal subsidy mechanisms, and comprehensive benefit mandates. For decades, policymakers have attempted to control costs through regulatory interventions, coverage expansions, benefit mandates, and payment adjustments. Yet the core financing logic has remained largely unchanged: the United States continues to route much routine and predictable care through third-party payment, weakening consumer cost-consciousness and limiting price discipline.

In this environment, cost control becomes extraordinarily difficult. When routine services are financed through benefit designs that obscure real prices, households cannot meaningfully compare costs, and providers face weaker incentives to compete on price. The result is a system in which increasing financial inputs—through employer premiums, federal subsidies, or tax expenditures—do not reliably produce proportional gains in efficiency, affordability, or transparency.

The case for structural reform rests on three propositions.

First, **insurance should primarily cover catastrophic risk, not routine expenses.** Routine expenditures are more foreseeable at the population level and are more suitable for transparent, direct-payment competition than for opaque third-party billing. However, routine costs can still impose severe hardship on low-income households and medically complex families if liquidity is not addressed. UCC/HSE therefore separates the insurance function from the access-protection function: catastrophic events are insured, while routine-care access is supported through prefunded HSE deposits, pediatric support, chronic-condition supplements, and income-sensitive guardrails.

Second, **households benefit from transparent, competitive pricing when services are routine and schedulable.** Price transparency does not make emergency care shoppable, and it cannot substitute for insurance in catastrophic cases. But for primary care, imaging, laboratory testing, routine specialist visits, and scheduled outpatient services, evidence from price-transparency and direct-payment research shows that visible prices can support lower-cost purchasing and stronger provider competition (5–7,36).

Third, **federal support should protect against financial ruin rather than subsidize comprehensive-plan inflation.** Catastrophic expenditures impose severe financial risk and justify broad pooling. Routine expenditures are more manageable in aggregate, but they are not equally manageable for households with low cash reserves, children, or chronic disease. UCC/HSE addresses that problem through income-sensitive HSE support and the safeguards described in Section IX, rather than by routing all predictable care through comprehensive insurance.

Taken together, these propositions point toward a reform model that redefines insurance, restores household control over routine-care purchasing, and targets federal support where it has the strongest economic justification: catastrophic financial protection, routine-care liquidity for vulnerable households, and bounded support for chronic and pediatric needs.

III.A. Why Comprehensive First-Dollar Coverage Fails

Comprehensive first-dollar coverage weakens price sensitivity by insulating patients from the marginal cost of routine services. The evidence on cost-sharing is clear but nuanced. The RAND Health Insurance Experiment found that higher cost-sharing reduced utilization, including routine and discretionary care, without worsening average health outcomes for most participants, but it also showed that cost-sharing can reduce both effective and less-effective care and can create greater risks for poorer and sicker patients (37). More recent

evidence on high-deductible plans reaches a similar conclusion: consumers respond to out-of-pocket prices, but poorly designed deductibles can discourage preventive care, medication adherence, and guideline-concordant chronic-disease management **(38–41)**.

The problem with comprehensive first-dollar coverage is therefore not simply “too much care.” The deeper problem is that routine care is financed through a mechanism that obscures prices, increases administrative complexity, and reduces the household’s ability to compare providers before care is delivered. Employers and insurers negotiate discounts, but those negotiations are opaque, vary across markets, and are often shaped by provider bargaining leverage rather than transparent service value.

The ACA expanded coverage but did not fundamentally alter this structure. Essential health-benefit rules, actuarial-value metal tiers, premium tax credits, cost-sharing reductions, and exchange regulation preserved comprehensive-plan financing. Because premium tax credits are tied to benchmark premiums, federal subsidy exposure rises as premiums rise. This structure protects many households in the short run, but it does not restore price discipline in routine-care markets.

UCC/HSE breaks this cycle by separating catastrophic insurance from routine-care financing. Catastrophic care remains insured and federally stabilized. Routine care is financed through HSE accounts, direct-payment arrangements, transparent prices, and targeted liquidity support. This preserves financial protection for severe medical events while reducing the incentive to pad routine benefits, inflate prices for schedulable services, or shift predictable expenses into tax-subsidized premium structures.

III.B. The Hidden Cost of Employer-Sponsored Insurance

Employer-sponsored insurance distorts labor markets, inflates premiums, and contributes to wage suppression by compensating workers through tax-preferred health benefits rather than fully transparent cash wages. The federal tax exclusion for employer-paid premiums is one of the largest health-related tax expenditures in the federal budget, and the Joint Committee on Taxation estimates its ten-year cost at approximately \$4.3 trillion **(22)**. This exclusion incentivizes employers to purchase increasingly comprehensive plans, even when those plans reduce wage transparency and weaken routine-care price discipline.

The employer market also illustrates the household-level burden of comprehensive-plan inflation. KFF reports that average annual family premiums for employer-sponsored coverage rose from roughly \$13,000 in 2009 to more than \$22,000 in 2023 **(23)**. These premiums are formally paid through employer benefit structures, but economically they

are part of total worker compensation. As premiums rise, less compensation is available for wages, savings, or direct household control.

The ACA did not reform this structure. It created a parallel subsidy system for non-employer markets while leaving the employer tax exclusion largely intact. UCC/HSE addresses this distortion by replacing the tax preference for comprehensive employer-sponsored insurance with a universal catastrophic credit and allowing employers to redirect compensation into HSE deposits, cash wages, catastrophic-plan facilitation, or narrow supplemental benefits. The goal is not to abolish employer participation, but to end the federal bias toward opaque comprehensive-plan inflation.

III.C. The Failures of the ACA's Market Architecture

The ACA brought genuine expansions in coverage, but it relied on a design that implicitly subsidizes cost growth. Premium tax credits rise with premiums; cost-sharing reductions compensate for the rising actuarial value of silver plans; essential health benefits mandate broad coverage; and risk-adjustment programs attempt to smooth insurer incentives while preserving cross-market uniformity **(3)**.

These mechanisms succeeded in expanding access but simultaneously entrenched a structure that:

- Shields consumers from real prices
- Encourages providers to raise charges
- Preserves incentives for broader benefit design and premium-linked subsidy growth
- Only partially mitigates adverse selection
- Places federal finances on an escalating cost trajectory

Moreover, the ACA's administrative architecture (exchanges, metal tiers, actuarial-value rules, network adequacy testing, EHB determinations, marketing standards) has introduced layers of bureaucracy that have not been offset by corresponding gains in efficiency.

A reform model that simply expands ACA subsidies or prescribes additional coverage mandates will expend vastly more public resources without addressing these underlying distortions. A structurally different model is required.

III.D. Returning Insurance to Its Proper Economic Role

Insurance exists to protect individuals from **low-probability, high-cost events**.

Economists have long recognized that insuring predictable expenses is neither efficient nor necessary; doing so introduces administrative burdens, encourages overutilization, and reduces price discipline.

Under a properly designed system:

- Households would manage routine expenses directly.
- Prices for routine services would be transparent.
- Insurance would intervene only when expenditures exceed a catastrophic threshold.
- Federal support would focus on catastrophic risk pooling rather than subsidizing routine care.

This is conceptually similar to selected catastrophic and cost-sharing features found in countries such as Singapore and, to a partial extent, Switzerland, i.e., systems where universal protection coexists with household cost exposure and more constrained routine-care financing **(24,25,28)**.

The UCC/HSE framework operationalizes these principles within an American context.

III.D. (addendum) Empirical Distribution of Healthcare Spending

Healthcare spending in the United States is not evenly distributed across the population. As shown in **Figure 3 and Table 3**, healthcare spending is highly concentrated among a relatively small share of individuals with acute or chronic medical needs.

Using nationally representative data from the Medical Expenditure Panel Survey (MEPS HC-251), weighted to reflect the full U.S. population, individuals can be ordered by total annual healthcare expenditures and aggregated into cumulative population and spending shares. This produces a standard concentration curve illustrating the distribution of healthcare costs.

The resulting distribution is sharply skewed. Approximately 30 percent of the non-Medicare population accounts for roughly 90 percent of total healthcare spending, while the bottom half of the population accounts for only a small fraction of aggregate expenditures. **Figure 2** illustrates this MEPS-based concentration curve for the non-Medicare population, showing that a substantial share of individuals incur minimal or near-zero medical costs in a given year while a smaller group accounts for most aggregate spending.

This concentration reflects the underlying structure of healthcare risk. Most individuals require routine, low-cost services, while a minority experience high-cost events that drive the majority of system-wide expenditures. These high-cost cases are often associated with serious illness, hospitalization, or chronic conditions rather than predictable or discretionary care.

The implication is that healthcare spending is fundamentally “catastrophic” in nature. The primary economic function of insurance, therefore, is not to prepay routine services, but to pool risk against infrequent, high-cost events that individuals cannot reasonably finance on their own.

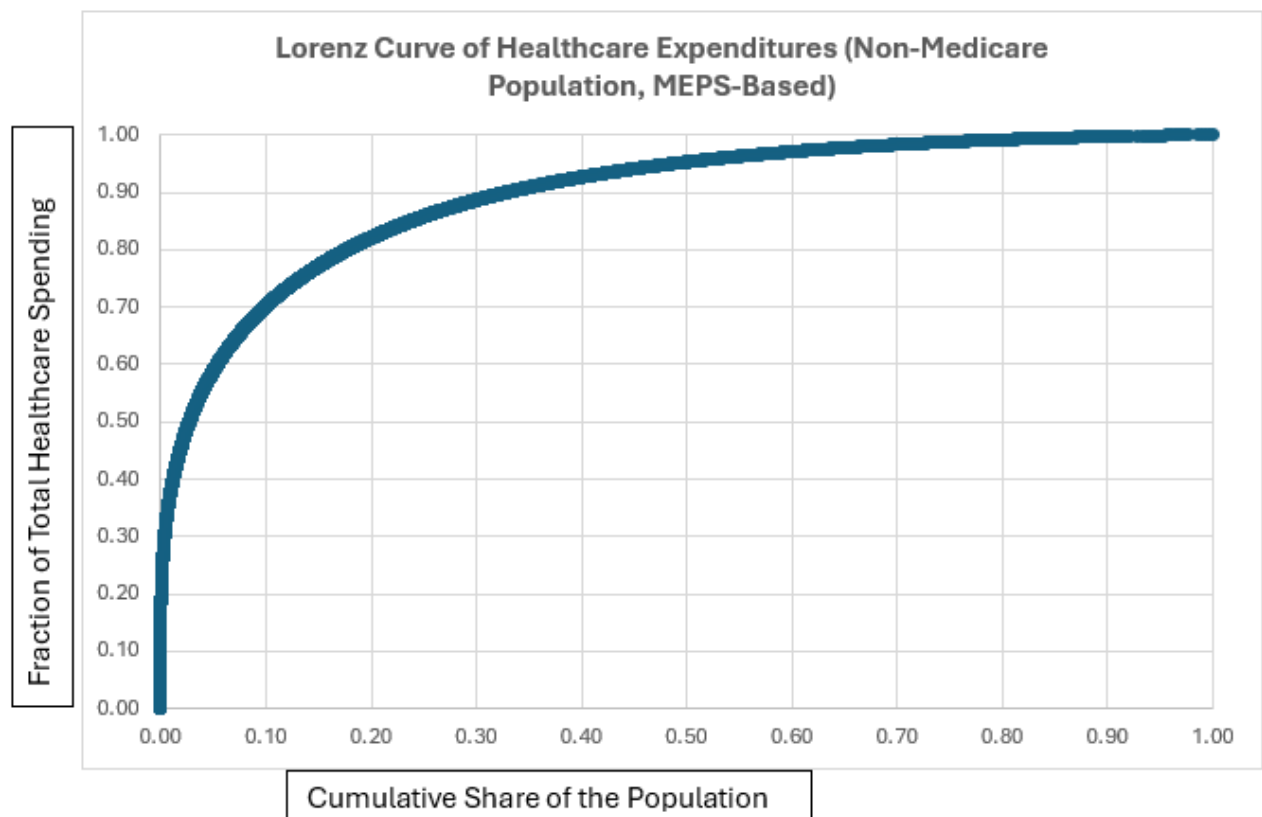


Figure 2: This figure displays the cumulative distribution of healthcare spending across the non-Medicare population using MEPS HC-251 data. Individuals are weighted to reflect national totals and ordered by annual expenditures. The curve illustrates the extreme concentration of healthcare costs, where a minority of individuals account for most of the total spending.

This empirical distribution directly informs the design of the catastrophic deductible in the UCC framework. Analysis of the MEPS expenditure distribution indicates that a deductible in the range of approximately \$7,500 to \$10,000 for individuals captures the transition

point between routine and catastrophic spending. Below this threshold, expenditures are widely distributed across the population and are more responsive to price transparency and consumer decision-making. **Table 2** reports the MEPS HC-251 threshold calculations underlying this breakpoint, showing the share of the non-Medicare population exceeding \$7,500 in annual spending and the corresponding concentration of spending above that threshold.

By setting the deductible at this empirically derived breakpoint, the system separates routine care from catastrophic risk in a manner consistent with observed spending patterns. Routine expenditures remain subject to consumer choice and market pricing dynamics, while catastrophic costs, representing the majority of aggregate spending, are pooled and insured at the national level.

Current insurance structures, including the ACA, do not align with this distribution. By covering routine services on a first-dollar basis, they obscure price signals for the majority of care while failing to efficiently isolate catastrophic risk. A system structured around universal catastrophic coverage, paired with direct financing of routine care, aligns more closely with the empirical distribution of healthcare spending and restores the core economic function of insurance. The supplementary AHRQ figures extend the same distributional logic. **Supplementary Figure 1** shows why Medicare beneficiaries should

Calculations	Total Population	% of overall population	fraction
All persons with ≥ \$7500:	70,600,430.72	20.71	0.207100119
Adults (18+) with ≥ \$7500:	63,576,692.31	23.82	0.238204168
Non-Medicare persons with ≥ \$7500 (MCARE23X=2 means NO):	40,731,495.08	15.10	0.150969778
Adults (18+) AND non-Medicare with ≥ \$7500:	34,987,724.34	17.87	0.178692048
Band	Adult pop (weighted)	Triggered pop (adult, >=7500) (weighted)	Trigger share (of trigger)
Low (<150)	44,423,803.62	11,438,238.03	0.179912443
Lower-mid (150–300)	57,021,458.56	11,876,625.46	0.186807854
Upper-mid (300–500)	66,595,495.33	15,374,373.83	0.24182406
High (>500)	91,640,308.16	24,887,454.99	0.391455643
Total (triggered)		63,576,692.31	
		proportion of cost	
Total Spending	1,414,052,602,236.69	1	
NonMedicarePop_weighted	2.66E+08	n/a	
nonmedicareExcessAboveTrigger_weightedDollars	778,913,128,805.54	0.550837449	
Non-Medicare persons with ≥ \$7500 (MCARE23X=2 means NO):	19,123.12	n/a	
Adults (18+) AND non-Medicare with ≥ \$7500:	22,262.47	n/a	
nonmedicareBelowTrigger_weightedDollars	635,139,473,431.15	0.449162551	

Table 2 shows that, in the MEPS HC-251 reconstruction, approximately 40.73 million non-Medicare individuals, or 15.10 percent of the non-Medicare population, had annual health-care expenditures above \$7,500 in 2023. This group accounted for approximately 55 percent of total non-Medicare health-care spending, while the remaining 85 percent of the non-Medicare population accounted for approximately 45 percent of the roughly \$1.4 trillion in total non-Medicare spending.

remain outside the UCC/HSE replacement population; **Supplementary Figure 2** shows that lower-spending populations rely heavily on routine services suited to HSA financing; and **Supplementary Figure 3** shows why routine-care liquidity matters even for relatively low-spending households.

Table 3. Percentile of population ranked by healthcare expenditures by year

Percentile of population	Annual healthcare expenditures (inflated to 2021 dollars)			
	2021	2020	2019	2018
Top 1%	\$86,783 or more	\$86,733 or more	\$82,731 or more	\$77,827 or more
Top 5%	\$29,921 or more	\$28,609 or more	\$29,185 or more	\$28,404 or more
Top 10%	\$16,113 or more	\$15,098 or more	\$16,068 or more	\$15,790 or more
Bottom 50%	Less than \$1,374	Less than \$1,202	Less than \$1,390	Less than \$1,419

Figure 2. Average expenditure per person by expenditure percentile, 2018-2021

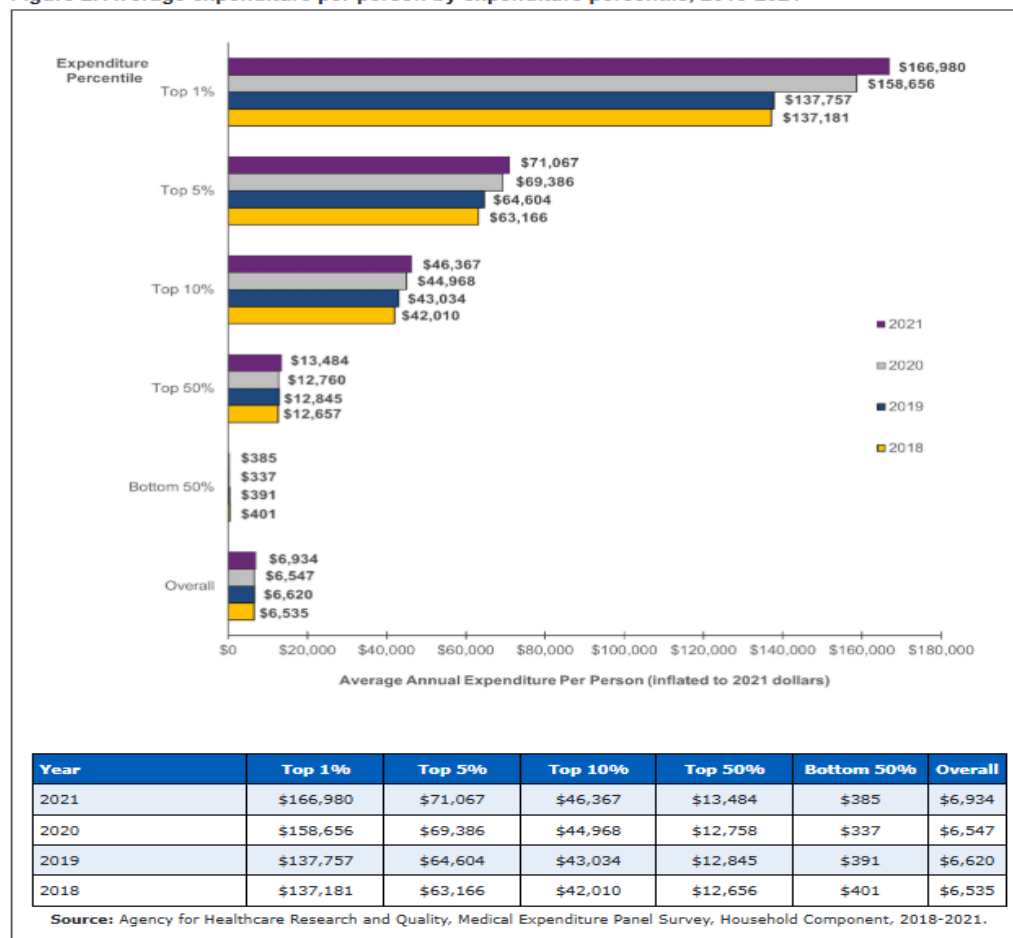


Figure 3: The Agency for Healthcare Research and Quality also suggest that most medical costs in the USA are concentrated among the top 5% of spenders.

III.E. International Evidence for Catastrophic and Savings-Based Systems

International experience demonstrates that universal health protection does not require first-dollar comprehensive insurance for all routine care. While no foreign model fully replicates the UCC/HSE framework, several advanced health systems contain structural elements relevant to it: broad protection against high-cost medical events, meaningful household responsibility for routine costs, standardized insurance rules, and risk equalization. Singapore is the closest analogue to the savings-based logic of UCC/HSE,

while the Netherlands and Switzerland provide narrower examples of regulated universal coverage paired with cost sharing, standardized benefits, and mechanisms to stabilize risk pools.

1. Singapore: Catastrophic Insurance Paired With Mandatory Medical Savings

Singapore's system, centered on MediShield Life and MediSave, is the most direct international analogue to the logic underlying UCC/HSE. MediShield Life provides broad protection against large hospital and selected outpatient bills, while MediSave requires workers to set aside income in individual medical savings accounts that can be used for approved health expenses. Catastrophic risks are pooled broadly, while many routine or predictable expenditures are financed through prefunded household accounts rather than first-dollar comprehensive insurance **(24,25)**.

Empirical data show that Singapore's health spending remains substantially below that of the United States and most other high-income nations **(24)**. Although demographic, geographic, wage, governance, and public-hospital factors affect Singapore's cost levels, the system demonstrates that universal protection can coexist with medical savings, household cost awareness, and a limited role for first-dollar insurance **(24,25)**. The relevant lesson for UCC/HSE is not that the United States should copy Singapore's administrative model, but that catastrophic protection and routine-care savings accounts can operate together within a universal framework.

2. The Netherlands: Standardized Universal Coverage, Regulated Competition, and Risk Equalization

The Netherlands operates a regulated universal insurance system in which individuals purchase a standard benefits package from private insurers. Unlike Singapore, routine care is more heavily insurance-financed. Nevertheless, the Dutch system incorporates several structural principles relevant to UCC/HSE: nationally standardized coverage rules, regulated competition among insurers, risk equalization, and consumer cost sharing.

The Dutch approach demonstrates that universal coverage can be standardized at the national level while private insurers compete on service quality, administration, and network management. Risk equalization reduces incentives for insurers to avoid high-cost enrollees while preserving a role for private-plan competition **(26,27)**. This is conceptually related to UCC/HSE's use of federal reinsurance and risk-adjusted support to stabilize catastrophic markets. The Netherlands should not be treated as a direct catastrophic-savings model, but it does support the narrower claim that standardized national insurance rules and risk-stabilization mechanisms can coexist with private insurer competition.

3. Switzerland: Universal Coverage With Deductibles and Household Cost Sharing

Switzerland similarly demonstrates that universal coverage can be paired with meaningful household cost sharing. Swiss residents are required to purchase basic health insurance, but households face deductibles and coinsurance before insurance pays for many services. Deductible options range from relatively low to substantially higher levels, preserving a greater degree of household cost exposure than typical U.S. first-dollar employer coverage **(28)**.

Switzerland is not a direct analogue to UCC/HSE, and its overall health spending remains high by international standards. Its relevance is narrower: it shows that universal coverage and consumer cost exposure are not mutually exclusive. The Swiss case therefore supports the proposition that universal protection can operate alongside deductibles, standardized insurance rules, and household responsibility for a portion of routine costs .

Across these systems, three principles are relevant to UCC/HSE:

1. High-cost medical risk can be pooled broadly.
2. Routine or predictable care can retain some degree of household cost exposure.
3. Standardized rules, risk equalization, and transparent benefit definitions can support universal protection without relying entirely on first-dollar comprehensive insurance.

These principles parallel the UCC/HSE structure. Catastrophic risk is pooled through universal catastrophic coverage and federal reinsurance. Routine care is financed through Health Savings Empowerment accounts, direct payment, and transparent markets. Low-income households, children, and chronic-condition patients receive targeted support beneath the deductible so that consumer-directed routine-care financing does not become medical underuse or emergency-department backflow.

III. F. Summary of the Economic Rationale

In sum, the structure of American health care fails because:

1. It insures too much.
2. It obscures prices.
3. It misallocates federal resources.
4. It inflates employer-based premiums.
5. It treats routine care as a financial risk rather than a household expense.
6. It subsidizes cost growth rather than cost discipline.

The UCC/HSE model aims to correct these structural failures by establishing a clear division of responsibilities: government protects against catastrophic risk, while households, armed with transparent prices and tax-advantaged HSAs, control routine spending.

IV. UNIVERSAL CATASTROPHIC COVERAGE (UCC)

IV.A. Overview

Universal Catastrophic Coverage (UCC) restores insurance to its foundational purpose: protecting citizens/eligible individuals from medical expenses large enough to cause financial ruin. UCC is **not** a comprehensive health plan and does **not** cover routine care, preventive services, or predictable medical expenses. Instead, it creates a universal safety net that caps each citizen/eligible individual annual medical liability at a manageable, predictable, and insurable level.

This framework eliminates the need for the ACA's metal-tier architecture, actuarial-value mandates, and extensive subsidy structures. It is designed to be actuarially simple, administratively lightweight, and economically transparent.

IV.B. Eligibility and Enrollment

UCC applies to **U.S. citizens and eligible individuals only**, reflecting both fiscal responsibility and political feasibility. Eligibility is automatic, enrollment is automatic, and there are no application forms or household self-attestation; eligibility verification and any income calibration for deposits or caps are performed automatically using existing DHS/SSA/IRS data systems, without manual recertification. Citizen-anchored and lawful-presence-verified eligibility ensures predictable fiscal exposure and aligns federal obligations with statutory eligibility rules.

Citizens and eligible individuals retain the freedom to choose among:

- **Private catastrophic carriers** meeting federal standards, or
- **A public catastrophic option** administered by CMS.

Default enrollment applies if no selection is made, ensuring universal catastrophic protection without imposing additional administrative burdens on households.

IV.B.1(Addendum) Eligibility and Enrollment

Eligibility for federal catastrophic credits, federal HSA deposits, and federal reinsurance under UCC/HSE is limited to individuals with verified, long-term lawful presence in the United States.

“Eligible individuals” means:

- 1. U.S. citizens;**
- 2. Lawful permanent residents (LPRs);**
- 3. Nonimmigrant residents who have:**
 - **(a) maintained continuous lawful presence in the United States for at least five consecutive years, and**
 - **(b) filed federal income tax returns for at least three of those five years under a valid SSN or ITIN.**
- 4. Any additional lawfully present categories expressly designated by Congress in statute.**

“Ineligible individuals” includes:

- 1. Persons present in the United States without lawful status;**
- 2. Pending asylum applicants, defensive asylum claimants, or individuals in humanitarian or discretionary parole categories;**
- 3. Individuals whose lawful presence cannot be verified through DHS, SSA, and IRS records;**
- 4. Any category of temporary protection or humanitarian relief established solely by executive discretion rather than statute.**

Eligibility determinations shall rely on DHS/SSA/IRS cross-matching.

Self-attestation is prohibited.

Individuals present in the United States without lawful status are categorically ineligible for federal catastrophic credits, federal HSA deposits, and federal reinsurance payments under this title.

Eligibility determinations shall rely exclusively on IRS, SSA, and DHS verification; self-attestation is not permitted.

Eligibility Verification and Immigration Compliance

To maintain program integrity and ensure that federal catastrophic credits and HSA deposits serve the intended beneficiary population, eligibility for UCC/HSE is restricted to **U.S. citizens and lawful permanent residents (LPRs) who have satisfied a minimum five-year residency period**, and **other eligible individuals** defined in this section. This is consistent with existing federal standards for means-tested benefits. Individuals present in violation of federal immigration law are **not eligible** for catastrophic credit, federal HSA seeding, reinsurance-funded cost protection, or any other subsidy created under UCC/HSE.

Eligibility determinations are verified through **IRS, SSA, and DHS data-matching**, eliminating reliance on self-attestation and reducing administrative error. Emergency medical treatment obligations under EMTALA remain unchanged and continue to operate independently of UCC/HSE. This clarification ensures that the program is both fiscally disciplined and aligned with long-standing federal eligibility norms.

IV. C. Catastrophic Deductible Levels

The deductible defines the threshold at which catastrophic coverage activates:

- **\$7,500–\$10,000 per adult**
- **\$15,000–\$20,000 per couple**

These thresholds reflect empirical distributions of medical spending. CMS expenditure data and MEPS HC-251 data show that **90% of U.S. adults incur less than \$8,500 in annual medical costs**, while only 5–7% exceed \$20,000 **(29,78)**. Catastrophic insurance targets precisely this small, high-cost tail of the distribution.

The central modeled baseline uses a \$7,500 adult catastrophic threshold. The statutory range is \$7,500–\$10,000 per adult and \$15,000–\$20,000 per couple. Children receive separate pediatric HSE deposits calculated by age band and coordinated with CHIP, Medicaid pediatric coverage, and medically complex children's protections.

Deductible parameters can be indexed annually to medical inflation (CPI-M) or a blend of CPI-M and overall CPI to prevent bracket erosion.

The deductible threshold defines catastrophic activation, while **income-based HSE support protects routine-care access for households for whom predictable costs are not financially trivial.**

These thresholds define catastrophic activation, not routine-care affordability. Routine-care liquidity for low-income households is protected through the HSE prefunding mechanism (federal HSA deposits are delivered at the start of each benefit year so households have immediate funds for predictable services rather than deferring care or relying on medical debt).

IV. D. Covered Services

UCC covers:

- Emergency hospitalization
- Major surgery
- Inpatient and outpatient procedures exceeding the deductible
- Trauma care
- Catastrophic disease events
- Organ transplants
- ICU-level interventions

It does **not** cover:

- Primary care visits
- Standard laboratory tests
- Imaging below the deductible
- Dental, vision, or hearing services
- Elective outpatient procedures
- Preventive medicine
- High-cost prescription therapies (e.g., gene therapies)

Routine and mid-level care are financed entirely through HSAs (see Section V).

IV.D.1 (Addendum). Treatment of High-Cost Chronic Biologics and Gene Therapies Under UCC/HSE

UCC does not cover predictable pharmaceutical expenditures, including chronic biologics or gene therapies.

High-cost specialty drugs are not included within universal catastrophic coverage unless administered during acute, inpatient episodes.

The exclusion of ultra-high-cost biologics and one-time gene therapies reflects a fundamental principle of catastrophic insurance design: the risk pool must be composed of *stochastic* and *unpredictable* medical shocks, not guaranteed multi-million-dollar liabilities concentrated in narrowly defined patient populations. International systems such as Singapore, Switzerland, and the Netherlands treat universal coverage, cost sharing, and high-cost pharmaceutical financing through distinct statutory and administrative channels rather than relying on a single undifferentiated catastrophic pool for all recurring high-cost therapies. Incorporation of million-dollar therapies into the UCC pool would destabilize premiums, distort actuarial neutrality, and undermine the model's budget predictability. By maintaining these therapies under Medicare, Medicaid, and specialized federal or state programs, UCC/HSE preserves its fiscal integrity while ensuring that high-need patients remain covered through established mechanisms explicitly designed for long-term and rare-disease treatment innovation.

Clarification on Ultra-High-Cost Therapies

Excluded ultra-high-cost genetic, cellular, and biologic therapies remain eligible for funding through existing federal mechanisms, including Medicare, Medicaid ABD, and orphan-drug carve-out pathways. Nothing in UCC/HSE shall be construed to restrict or diminish federal or state authority to finance such therapies under existing programs. The purpose of this exclusion is to preserve the actuarial integrity of the catastrophic risk pool, not to alter the availability of these therapies through established federal benefit structures.

IV.D.2 (Addendum). Medicaid Managed-Care Transition

States currently relying on Medicaid managed-care organizations (MCOs) for expansion populations may phase out these arrangements as beneficiaries transition into UCC/HSE. States would maintain MCO contracts for ABD populations only, while non-disabled adults shift from capitated Medicaid plans into catastrophic coverage plus HSA-funded routine

care. This reduces administrative duplication, eliminates reimbursement rate disparities across populations, and simplifies Medicaid’s mission around disability-related and long-term-care services.

IV.D.3 (Addendum). Federal Funding Limitations and Conscience Protections

Federal catastrophic credits, federal HSA seeding, and federal reinsurance payments may **not** be used to pay for abortion services except in the limited circumstances permitted under current federal law—**rape, incest, or when the life of the mother is endangered**. Nothing in UCC/HSE may be construed to require insurers, employers, or medical providers to furnish, reimburse, or participate in abortion services as a condition of program compliance. These provisions maintain alignment with longstanding federal appropriations restrictions governing the use of federal health-care funds.

Byrd Compliant language:

“No funds appropriated or made available under this title, including federal catastrophic credits, federal HSA contributions, federal reinsurance payments, or any other federal outlays established under this Act, may be used to pay for abortion services except in cases of rape, incest, or where a physician certifies that the life of the mother would be endangered if the pregnancy were carried to term.”

“Nothing in this title shall be construed to obligate any insurer, provider, or plan administrator receiving federal funds pursuant to this title to furnish, arrange, or pay for abortion services beyond the limitations above.”

By ensuring routine-care funds are liquid on day one, the model reduces incentives to substitute emergency departments for predictable outpatient care.

IV. E. Benefit Design Simplicity

Unlike the ACA, UCC does not require metal tiers, actuarial calculations, essential health-benefit mandates, cost-sharing reduction plans, network-adequacy testing for routine care, or extensive regulatory reporting.

Every catastrophic plan has:

- **One deductible**
- **One out-of-pocket maximum**
- **No coinsurance**
- **No multi-tier formularies**

- **No provider-network micromanagement beyond catastrophic care**

This simplicity yields substantial administrative savings and improves consumer understanding.

IV. F. Federal Reinsurance Layer

Reinsurance is the central stabilizing mechanism of UCC.

Reinsurance Structure

- The federal government reimburses **80% of claims above \$50,000 per enrollee per year.**
- This mirrors real Section 1332 reinsurance programs that cut premiums **19–43%** in participating states (4).

Why Reinsurance Works

Reinsurance decouples high-cost claims from premiums. Under ACA markets, a small number of high-cost patients can dramatically raise premiums for everyone. Under UCC, the federal reinsurance pool absorbs the tail risk, allowing insurers to price catastrophic plans predictably.

This eliminates the need for:

- risk corridors
- cost-sharing reductions
- silver-loading strategies
- exchange-level premium equalization
- complex risk-adjustment transfers

The federal government assumes catastrophic volatility; insurers compete on administrative efficiency and consumer service.

IV. G. Actuarial Logic and Premium Estimates

- Under the central illustrative parameter set, catastrophic-only premiums are modeled in the range of:
- **\$1,200–\$1,800 per adult annually**
- **\$2,400–\$3,200 per family**

These figures reflect three structural advantages of UCC:

1. **Elimination of comprehensive benefits** lowers the actuarial value dramatically.
2. **Federal reinsurance** suppresses variability.
3. **Reduced administrative overhead** sits closer to Medicare's 2–5% than private insurance's 12–18%.

These premiums can be paid out-of-pocket, through employer contributions, or through the universal refundable credit replacing the employer exclusion.

IV.H. Employer Transition Pathway

UCC/HSE preserves employer-sponsored insurance and allows firms to adopt the new system on a voluntary, staged basis. Employers may continue offering their existing coverage without penalty, offer UCC-compliant catastrophic plans paired with employer HSA contributions, or allow employees to apply the universal catastrophic credit independently.

To avoid disruption in large multi-state ERISA groups, the employer tax exclusion is phased down gradually over several years rather than removed abruptly. This ensures continuity for workers while allowing firms to migrate toward the simpler UCC/HSE architecture as administrative and payroll systems adapt.

No employer is required to discontinue current health benefits. The universal catastrophic credit attaches to individuals regardless of whether coverage is obtained through an employer or independently.

IV.I. Optional State-Administered Public Catastrophic Plan

To provide additional choice and enhance public confidence in the transition to UCC/HSE, states may offer a standardized public catastrophic plan that mirrors the nationally defined deductible, covered services, and reinsurance parameters. The public option is funded solely by the universal catastrophic credit and adheres to actuarially fair premiums. It does not include supplemental benefits, expanded formularies, or cost-sharing reductions beyond those defined in the national catastrophic standard.

This option serves as a benchmark for private catastrophic plans and offers households a transparent, non-complex alternative. Because its design cannot deviate from national catastrophic rules, the public option strengthens (not weakens) the competitive routine-care market built around HSAs and cash-priced services.

V. HEALTH SAVINGS EMPOWERMENT (HSE)

V.A. Overview

Health Savings Empowerment (HSE) is the counterpart to UCC. While UCC protects against large, unpredictable expenses, HSE empowers citizens and eligible individuals to manage routine and predictable medical spending with transparency and autonomy.

HSE is built around portable, tax-advantaged HSAs, which become the central mechanism for everyday care. Unlike current HSAs, which require enrollment in specific IRS-defined high-deductible plans, HSE decouples HSAs from plan type, allowing every citizen and eligible individual to maintain an account.

V.B. Household Control and Transparency

Routine care is predictable: primary-care visits, laboratory testing, imaging, generic prescriptions, telemedicine, minor procedures, and many scheduled outpatient services. These encounters represent the majority of household interactions with the health system, but they account for a smaller share of total health spending than catastrophic events, hospitalizations, complex specialty care, and advanced therapies.

UCC/HSE shifts routine and shoppable care into a more transparent purchasing environment. Under this structure, providers have stronger incentives to post prospective, all-inclusive prices; households can compare providers before receiving routine or scheduled care; employers and third-party administrators can steer patients toward high-value providers; price dispersion is exposed rather than hidden inside negotiated insurance contracts; and routine-care spending is financed through HSE accounts rather than opaque comprehensive premiums.

Evidence from transparent cash-pay and direct-payment surgical markets supports this mechanism. The Surgery Center of Oklahoma demonstrates the operational feasibility of posting all-inclusive surgery prices prospectively for common procedures **(30)**. In congressional testimony, Dr. Ron Piniecki reported that Wellbridge Surgical's transparent bundled-price model produced nearly 55 percent average savings and approached 70 percent savings for some complex procedures, while identifying the Surgery Center of Oklahoma as an established predecessor in transparent-priced surgical care **(31)**. Peer-reviewed evidence from Whaley et al. reinforces the same mechanism in a commercially insured employer population: a direct-payment bundled program for joint replacement, spinal fusion, and bariatric surgery reduced total surgery costs by **\$4,229 per episode**, a

10.7 percent relative reduction, while lowering patient cost-sharing by **\$498 per episode**, or **27.7 percent**. Employers captured approximately **85 percent** of the savings, equal to **\$3,582 per episode**, and RAND summarized the employer return as roughly **\$7 saved for every \$1 in waived patient copayments (7)**.

The point is not that every service can be shopped for in the same way. Emergency care, trauma, advanced oncology, intensive care, and complex inpatient episodes remain poorly suited to ordinary consumer shopping. UCC/HSE therefore keeps catastrophic risk insured while moving predictable routine care into a financing structure where prices can be seen, compared, and disciplined before care is delivered.

V.C. HSA Funding Structure

HSA contributions consist of three sources:

1. Federal HSA seeding:

- **\$750-4,150 per year**
- Targeted to low-income citizens/ eligible individuals and chronic-condition patients

2. Optional employer contributions:

- Employers may contribute to an employee's HSA tax-free
- Replaces traditional premium-payment obligations

3. Voluntary citizen/eligible individual contributions:

- With higher annual limits
- Fully tax-deductible

Balances roll over indefinitely and compound tax-free.

V.D. Annual Prefunding and Liquidity Protection for Low-Income Households

A common limitation of high-deductible designs is the liquidity burden they impose at the beginning of the year, when households may face medical bills before adequate savings accumulate. UCC/HSE eliminates this risk through an annual prefunding mechanism administered by the Treasury. Federal HSA deposits for low-income and chronic-condition citizens/eligible individuals are made at the start of each benefit year, ensuring households have immediate access to funds for routine or disease-management services. This prefunding structure also stabilizes provider cash flow and minimizes reliance on credit or

medical debt. Because deposits are automatic, income-verified through existing IRS data, and delivered uniformly on January 1, the liquidity constraint that often undermines consumer-directed models is substantially mitigated.

This annual prefunding is the core guardrail against emergency-department backflow: by making routine-care funds liquid at the start of the year for low-income and chronic-care households, it reduces deferred care, medical debt accumulation, and avoidable ER substitution for predictable outpatient needs.

V.D.1 (Addendum). Automatic Stabilizers for Household Liquidity During Economic Downturns

A catastrophic-first system must preserve household liquidity even during macroeconomic stress. Because recessions depress household HSA contributions precisely when cash-on-hand is needed most, UCC/HSE incorporates an automatic, formula-based stabilizer to maintain access to routine care without reintroducing comprehensive insurance.

1. Recession Trigger Mechanism

Federal HSA deposits increase automatically when either of the following conditions is met:

- the national unemployment rate rises **1.0 percentage point** above its trailing twelve-month average, or
- the U.S. economy experiences **two consecutive quarters of negative real GDP growth**.

This trigger mirrors widely accepted recession indicators and eliminates the need for discretionary congressional action.

2. Automatic HSA Augmentation

When activated, stabilizers increase federal HSA seeding by:

- **\$300 per adult enrollee**, and
- **\$150 per dependent child**,
with proportional increases for chronic-condition bundles.

The augmentation offsets reduced household contributions and ensures continuity of routine primary care, labs, and maintenance medications during periods of labor-market stress.

3. Sunset and Return-to-Baseline

Enhanced seeding phases out automatically when either:

- unemployment returns to within **0.5 percentage points** of its pre-trigger average, or
 - real GDP growth exceeds **1 percent** for two consecutive quarters.
- The mechanism is fully countercyclical: it turns on when needed and off when conditions normalize.

4. Fiscal Guardrail for CBO Scoring

To maintain budget discipline, total recession-triggered supplementation is capped at **0.05 percent of GDP** in any fiscal year. This ceiling preserves fiscal predictability while offering meaningful household support during economic downturns.

By adopting a rules-based stabilizer, UCC/HSE protects households in recessions without reintroducing inflationary comprehensive benefits or relying on emergency appropriations. The mechanism reinforces liquidity, preserves utilization of essential routine care, and keeps the catastrophic core intact.

V.E. Chronic-Condition Support

Chronic conditions require predictable, ongoing spending. To ensure adequate financial support, HSE includes:

- **Higher annual HSA seeding for diagnosed chronic conditions**
- **Condition-based care bundles**, in which provider groups offer annual fixed-price packages covering labs, medications, and disease management

Medicare Advantage and risk-adjusted payment literature demonstrates the operational feasibility of adjusting payments for predictable differences in enrollee risk, although UCC/HSE applies this logic to HSA supplementation and condition-based routine-care support rather than to comprehensive-plan enrollment **(18)**.

The chronic-condition HSA supplement is fixed by diagnosis category and does not vary by household income. All individuals qualifying under the statutory chronic-condition criteria receive the same chronic supplement regardless of income level. Any variation in total HSA funding across chronic households reflects differences in the base, income-calibrated deposit structure and household composition, not adjustments to the chronic-condition supplement itself.

V.F. Direct Primary Care Integration

Direct Primary Care (DPC) is an optional but naturally compatible feature of HSE. For a flat monthly fee (typically **\$60–\$120**), patients receive unlimited primary care, extended visits, same-day communication, and chronic-condition monitoring.

DPC practices report:

- reduced emergency-department use
- lower specialist referrals
- greater patient satisfaction

Under HSE, citizens/eligible individuals may pay DPC membership fees directly from HSAs.

V.G. Routine Care Out of Insurance

A central aim of HSE is to eliminate the economically irrational practice of routing routine expenses through insurance. Insurance is not necessary for:

- Primary-care visits
- Most imaging
- Outpatient labs
- Urgent care
- Mental-health counseling
- Physical therapy
- Telehealth

Removing these services from insurance:

- eliminates billing costs
- reduces administrative overhead
- lowers provider compliance costs
- increases price transparency
- encourages market entry of new low-cost providers

Provider-side billing and insurance-related administrative costs are substantial, with estimates varying by setting and method; Tseng et al. estimate that billing and insurance-related costs represented 14.5 percent of professional revenue for primary-care visits and 25.2 percent for emergency-department visits **(12)**. HSE is designed to reduce these costs by moving routine-care payment away from insurer-mediated claims processing and toward direct HSA-financed payment.

V.H. Statutory Definition of Routine Care

For purposes of UCC/HSE, “routine care” refers to medical services characterized by predictable utilization patterns and low financial variance, including but not limited to:

- primary care visits;
- routine behavioral and mental health services;
- standard laboratory testing;
- imaging services priced below the catastrophic deductible;
- physical therapy and rehabilitative care;
- preventive pediatric and adult screenings; and
- non-emergent outpatient consultations.

Routine care, as defined herein, is not an insurable event under UCC/HSE and shall be financed through Health Savings Empowerment accounts, out-of-pocket cash markets, or direct primary care arrangements. This definition prevents reclassification of routine services as catastrophic events and preserves the actuarial function of the national deductible.

VI. HOW UCC/HSE REPLACES THE ACA

The UCC/HSE framework is not merely a modification of the ACA; it is a structural replacement of the ACA’s subsidy-driven architecture with a system that is simpler, more fiscally disciplined, and more aligned with correct economic incentives. While the ACA expanded insurance coverage, it did so through mechanisms that perpetuated administrative complexity, suppressed price transparency, and allowed provider pricing power to grow unchecked.

Replacing the ACA does not mean reducing coverage. Rather, UCC/HSE achieves universal catastrophic protection, a stronger, more stable guarantee against medical bankruptcy than the ACA ever provided, while reducing federal exposure to routine medical spending and redirecting resources toward catastrophic risk pooling.

VI. Key Insight: No Changes to Medicare Programs

Nothing in UCC/HSE alters Medicare eligibility, benefit design, Medigap policies, Medicare Advantage contracts, or any Part A, B, D, or C administrative structures. All provisions apply solely to pre-Medicare populations. Medicare remains unchanged and operates under existing law.

This explicit separation ensures a stable transition for older Americans and prevents cross-program interference during implementation.

VI. A. ACA Subsidies vs. UCC Catastrophic Protection

The ACA's subsidy architecture consists of four major components:

1. **Premium Tax Credits (PTCs)**
2. **Cost-Sharing Reductions (CSRs)**
3. **Essential Health Benefit (EHB) requirements**
4. **Actuarial Value (AV) standards tied to metal tiers**

Together, these create a system where federal obligations automatically scale with premium inflation, primarily driven by comprehensive benefit design, hospital monopoly pricing, and insurer administrative loads. The CBO projects that PTC spending alone will reach **\$150–\$180 billion annually** by 2032 (3).

UCC replaces:

- PTCs → **Universal refundable catastrophic credit**
- CSRs → **Eliminated (no comprehensive plans)**
- Metal tiers → **Single catastrophic benefit**
- EHB mandates → **Removed for routine care**
- Marketplace administration → **Dissolved**

Under UCC/HSE:

- Catastrophic deductibles anchor the system
- HSAs replace cost-sharing subsidies
- Federal resources are redirected from routine coverage to catastrophic protection
- Administrative layers collapse entirely

This redesign sharply reduces federal outlays, removes the open-ended subsidy exposure associated with rising premiums, and simplifies households' decision-making.

VI.B. The Dissolution of ACA Exchanges

ACA exchanges serve three administrative purposes:

1. Income-based subsidy eligibility
2. Marketplaces for ACA-compliant plans
3. Risk-adjustment and actuarial reporting infrastructure

Under UCC/HSE, all three become unnecessary.

1. Subsidy eligibility disappears

UCC/HSE uses:

- A universal refundable credit (not means-tested)
- Universal catastrophic plan eligibility
- HSA seeding distributed automatically based on income verified through IRS data

There is no need for application mechanics, documentation uploads, re-verification, or exchange determinations.

2. Exchanges as marketplaces become redundant

Instead of each state managing a complex insurance marketplace, UCC catastrophic plans are:

- Standardized
- Easily comparable
- Not differentiated by benefit structures
- Priced with federal reinsurance already included

This reduces the role of marketplaces to something closer to simple listings (akin to Part D or Medigap comparison tools).

3. Risk-adjustment infrastructure simplifies

Current ACA risk adjustment transfers billions of dollars annually between insurers based on intricate statistical models.

Under UCC, federal reinsurance absorbs most cost volatility, eliminating the need for complex transfers. Private insurers handle a narrow catastrophic band, not comprehensive risk stratification.

VI.C. Essential Health Benefits and Actuarial-Value Mandates

The ACA requires qualified health plans to cover a broad package of essential health benefits, including maternity care, mental-health services, prescription drugs, habilitative services, preventive services, and pediatric dental and vision coverage. Those requirements are compatible with comprehensive insurance, but they are structurally incompatible with a catastrophic-only insurance model.

UCC/HSE therefore eliminates the ACA's essential-health-benefit structure for the catastrophic layer. This does not mean households lose access to routine, preventive, pediatric, behavioral-health, or chronic-care services. It means those services are financed differently. Instead of being embedded inside comprehensive premiums, routine and predictable services are financed through HSE accounts, direct-pay arrangements, transparent routine-care markets, and targeted supplemental support for low-income, pediatric, and chronic-condition populations.

The policy rationale is that many routine and scheduled services are better suited to transparent pricing than to comprehensive first-dollar insurance. Evidence from transparent surgical-pricing models shows that providers can publish bundled, prospective prices for scheduled procedures **(30)**. Congressional testimony from Wellbridge Surgical reports large savings under a bundled direct-pay outpatient surgery model, including nearly 55 percent average savings and approaching 70 percent savings for some complex procedures **(31)**. In peer-reviewed commercial-insurance evidence, Whaley et al. found that a direct-payment bundled program covering the procedure and related care for 30 days reduced episode prices for selected surgical procedures by **\$4,229, or 10.7 percent**, and reduced patient cost-sharing by **\$498, or 27.7 percent (7)**. The study's scope was narrower than UCC/HSE (it focused on selected orthopedic, spine, and bariatric procedures) but it directly supports the proposition that prospective bundled payment and patient steering can reduce shoppable episode costs without relying on opaque comprehensive insurance billing.

Actuarial-value mandates likewise disappear in the catastrophic layer. The ACA's 60, 70, 80, and 90 percent actuarial-value tiers are designed for comprehensive insurance products that blend routine and catastrophic expenses into one premium. UCC/HSE replaces those tiers with a single catastrophic structure: coverage begins above the deductible, with federal reinsurance stabilizing extreme claims. What varies beneath the deductible is not the insurance product, but the financing support available through HSE deposits, chronic-condition supplements, pediatric support, and income-based protections.

Catastrophic coverage has only one AV: **coverage above the deductible.**

VI.D. Medicaid Expansion Populations

The ACA's Medicaid expansions enrolled millions of low-income adults. Under UCC/HSE:

- Medicaid expansion enrollees convert to **citizen/eligible individual catastrophic protection**
- Routine care is financed through **federally seeded HSAs**
- States retain Medicaid for the aged, blind, and disabled
- States may optionally offer supplemental assistance for low-income citizens/eligible individuals.

This transition modernizes Medicaid's role, reducing state and federal fiscal exposure while improving patient autonomy.

VI.D.1 (Addendum) Medicaid No-Harm Rule

UCC/HSE does not alter Medicaid's long-term services and supports (LTSS), Aged-and-Disabled (ABD) programs, medically fragile categories, or any federal matching rules that apply to those populations. States do not lose federal Medicaid funding for LTSS, ABD, or medically complex beneficiaries under UCC/HSE. The reform applies solely to the non-disabled adult population currently enrolled through ACA expansion or traditional eligibility pathways.

VI.E. Employer-Sponsored Insurance Transitions

The employer tax exclusion is phased out and replaced by a universal, portable credit. This equalizes tax treatment between employer-sponsored and individually purchased catastrophic plans.

Employers may:

- Contribute directly to employees' HSAs
- Offer supplemental (non-catastrophic) benefits
- Facilitate catastrophic plan enrollment
- Offer cash wages instead of insurance

This removes distortions that historically encouraged oversized employer plans and suppressed wages.

VI.F. Employer-Market Transformation Under UCC/HSE

The employer market undergoes a structural shift under UCC/HSE, but not a disruptive one. By replacing the tax exclusion for employer-sponsored insurance with a universal, portable credit, the framework eliminates the incentive for firms to purchase ever-larger comprehensive plans while preserving their ability to support employees' health-care needs.

Employers may still contribute pre-tax dollars, but those contributions now flow into employees' HSAs. This change reduces administrative overhead, increases wage transparency, and enhances portability, especially valuable for lower-wage workers who experience more frequent job transitions. Evidence from labor-economics studies shows that the employer exclusion suppresses wage growth and can anchor workers to jobs they might otherwise leave **(22,32,33)**.

UCC/HSE transitions employers away from benefit-inflation competition and toward direct wage or HSA support. Firms can still offer supplemental benefits, but these are no longer subsidized more favorably than cash wages. Over time, this shift is expected to increase wage transparency, reduce job lock, and limit the distortions created by comprehensive employer plans **(34)**.

VI.F.1 (Addendum) Union-Plan Continuity

Existing collective-bargaining agreements may maintain their current benefit structures until the end of the negotiated contract term. At renewal, employers and unions may transition to combinations of UCC catastrophic coverage, employer HSA contributions, and narrowly targeted supplemental riders, provided the employer's existing fiscal commitment is preserved. This ensures stability for unionized workforces during the transition and maintains the integrity of collectively bargained agreements

VI.G. Ten-Year Employer Exclusion Transition Path

Although the current employer-sponsored insurance exclusion is economically distortionary and regressive, its abrupt removal would be unnecessarily disruptive for firms and workers accustomed to wage-benefit bundling. To avoid destabilizing employer compensation models, UCC/HSE phases out the exclusion over a ten-year schedule, with predictable annual reductions that allow firms to adjust wage structures, renegotiate plan contracts, and redesign benefit offerings.

Each year, the allowable exclusion is reduced by a fixed percentage, shifting compensation gradually from non-transparent benefits to taxable wages and HSA contributions. During this period, employers may contribute directly to employees' HSAs without adverse tax consequences, easing the transition to portable, worker-centered coverage.

A ten-year timeline preserves the economic logic of ending the distortion while providing the political and operational stability necessary for adoption

VI.G.1. Collective Bargaining and Multi-Employer Plan Transition

UCC/HSE's employer transition provisions apply prospectively and do not invalidate existing collective bargaining agreements. Multi-year union contracts may maintain their current benefit structures through their natural expiration dates, after which UCC/HSE transition rules apply. Employers sponsoring collectively bargained or multi-employer plans may convert existing benefit value into combinations of: (a) UCC catastrophic coverage, (b) employer HSA contributions, and (c) narrowly defined supplemental riders, provided total employer fiscal commitment is preserved.

This approach prevents abrupt disruption in unionized sectors while ensuring a predictable glide path toward the wage-transparent, portable coverage structure of UCC/HSE.

Byrd Compliant language:

“Amounts excludable from income under Section 106 of the Internal Revenue Code, as in effect on the date of enactment, shall remain excludable for the duration of any collective-bargaining agreement in force on such date. Upon expiration of the agreement, the phase-down schedule in subsection (G) shall apply.”

VI.H. Supplemental Coverage Guardrails and Market Integrity

A universal catastrophic structure can only maintain long-term cost discipline if the boundary between catastrophic insurance and supplemental coverage remains unambiguous. Historical precedent from employer-sponsored markets, Medigap markets, and ACA exchange designs shows that insurers expand benefits whenever regulatory definitions leave room for interpretation. Without clear statutory guardrails, supplemental plans risk becoming de facto comprehensive insurance, eroding transparency and reintroducing benefit inflation.

1. Permissible Scope of Supplemental Coverage

Supplemental products may finance only:

- Non-catastrophic elective or ancillary services (dental, vision, short-term disability)
- Defined service bundles (e.g., maternity episodes, musculoskeletal packages)

- Pre-deductible primary-care arrangements such as DPC or integrated physician models
- Catastrophic-deductible financing mechanisms (voluntary savings or fixed indemnity)

They may **not** cover:

- routine care on a claim-by-claim basis
- any service below the catastrophic deductible
- generalized “wraparound” major-medical coverage
- benefits that replicate ACA gold/platinum actuarial values

These limits ensure that supplemental offerings support targeted consumer preferences without reconstructing comprehensive first-dollar insurance through incremental expansion.

2. Actuarial Integrity Requirement

To prevent benefit creep, supplemental plans must demonstrate that:

- Actuarial value (AV) $\leq 15\%$
- The plan does not raise pre-deductible coverage levels above federal thresholds
- Coverage remains strictly outside the catastrophic structure

MedPAC and CBO analyses consistently show that even small incremental increases in AV materially weaken cost discipline. Clear actuarial boundaries keep supplemental plans from drifting into quasi-comprehensive products.

3. Transparency and Standardized Disclosures

All supplemental plans must:

- provide a standardized one-page disclosure summarizing benefits and limits
- publish annual premiums, claims ratios, and AV calculations
- explicitly state what percentage of the catastrophic deductible they offset

This creates a transparent supplemental marketplace that supports consumer choice without undermining the catastrophic architecture.

4. Employer-Side Guardrails

To prevent employers from reconstituting comprehensive plans through tax-preferred channels:

- employer contributions are restricted to HSAs only
- supplemental plans must be purchased with after-tax dollars
- employers may not condition employment on purchasing supplemental coverage

These provisions ensure tax neutrality and preserve the shift toward portable, wage-transparent compensation models.

VI.I. Political Feasibility and Transition Timeline

Core coverage transition would occur over 3–5 years, while the employer-exclusion phase-down would proceed separately over the ten-year schedule described in Section VI.G:

1. Year 1: UCC catastrophic plans become available
2. Year 2: ACA exchanges close to new enrollment
3. Year 3: Subsidies phase out; UCC credits phase in
4. Year 4–5: Medicaid expansion populations transfer to UCC/HSE
5. Employer-exclusion phase-down continues under the ten-year transition schedule described in Section VI.G.

This staged approach follows the general logic of phased federal benefit implementation: coverage architecture changes first, while employer compensation and tax-treatment changes transition more gradually.

VI. J. State Flexibility and Innovation Within Federal Standards

Byrd Compliant Language:

“A state may administer a state-based catastrophic plan provided that deductible levels, covered catastrophic services, and reinsurance cost-sharing parameters meet or exceed the federal minimum standards established under this title. States meeting these standards shall receive federal catastrophic credits and federal reinsurance payments.”

“States may redirect existing federal waiver funds (including 1332 and 1115 waiver pass-through funds) to enhance HSA deposits or to support rural stabilization payments, provided such expenditures remain within federal allotments.”

VI.K. Continued Availability of Comprehensive Private Insurance

UCC/HSE does not eliminate or restrict the private insurance market. While the reform shifts routine and predictable care into transparent, HSA-funded cash markets, insurers remain fully permitted to offer comprehensive health plans, PPOs, HMOs, employer group plans, and other coverage models.

Under UCC/HSE, comprehensive plans simply receive **no federal subsidy** and **cannot be financed with the universal catastrophic credit** or federal HSA deposits. This ensures the catastrophic system maintains actuarial cohesion while preserving consumer choice in the private market.

Employers may continue to offer comprehensive coverage during the transition period and after the employer tax exclusion is phased out. Plans purchased with post-tax income or through collectively bargained agreements remain legal and are unaffected by federal catastrophic standards.

The UCC/HSE architecture restructures the federal financing of high-severity medical risk; it does **not** prohibit individuals or firms from purchasing more expansive private coverage if they prefer to do so.

VII. WHY UCC/HSE LOWERS COSTS FOR EVERYONE

The UCC/HSE framework reduces costs for four distinct reasons, each supported by empirical research and economic theory.

VII.A. It removes routine care from insurance (the biggest driver of cost inflation)

Insurance intermediaries impose administrative loads, billing overhead, utilization review, network contracting, and coding requirements. Routine care accounts for the majority of these interactions. Removing routine care from insurance eliminates:

- prior authorization
- billing departments
- CPT/ICD coding complexity
- network negotiations
- contract disputes
- insurer overhead on routine claims

A 20–30 percent reduction in affected provider billing and insurance-related overhead is a central modeling assumption informed by comparative administrative-cost analyses, GAO findings, and provider-side billing-cost estimates **(9,12,15,19)**.

VII.A.1 (Addendum): Deductible Indexing Rule

To prevent upward drift in the catastrophic deductible and preserve long-term predictability for households, the national deductible may increase only at the rate of CPI-U unless

Congress explicitly authorizes an adjustment by statute. This protects the integrity of the catastrophic model while ensuring that deductible growth does not outpace household income.

VII.B. It eliminates comprehensive benefit inflation

Comprehensive plans expand over time because employers compete through benefits rather than wages, ACA subsidies are tied to premiums, and hospital/provider pricing power is embedded in third-party payment structures **(3,8,32–35)**. Catastrophic-only plans cannot inflate in this manner. They cover only major events, so there is nothing to pad.

VII.C. It collapses price opacity and fosters competitive routine-care markets

When consumers spend from HSAs:

- Providers publish prices
- High-cost providers lose business
- Low-cost providers gain market share
- Entrepreneurs enter transparent markets

Evidence from cash-pay surgical centers and imaging markets shows:

- **MRI:** \$350 cash vs. \$1,200–\$3,000 billed
- **CT:** \$200–\$400 cash vs. \$1,000+ billed
- **Colonoscopy:** \$900–\$1,400 cash vs. \$2,500–\$5,000 billed

This is not theory; it is currently observable in U.S. markets not dominated by insurer-mediated pricing **(7,20,30,35,36,100,101)**.

VII.D. Structural Price Discipline as the Core Fiscal Engine

The United States does not suffer from a scarcity of medical innovation, nor from a deficiency of professional capacity. It suffers, rather, from a systematic failure to impose rational restraints on the pricing architecture of catastrophic care (a segment of the market uniquely insulated from competitive discipline, consumer oversight, and meaningful price sensitivity). In this context, the fiscal credibility of the Universal Catastrophic Coverage with Health Savings Empowerment (UCC/HSE) framework rests not on the hope of spontaneous market correction, but on the deliberate reintroduction of pricing structure where no functioning market presently exists.

UCC/HSE therefore establishes explicit, enforceable commercial payment corridors in the catastrophic layer, tied directly to Medicare’s existing national benchmark schedule. Under the central fiscal scenario, commercial reimbursement for catastrophic hospital services shall not exceed 150–160 percent of the applicable Medicare Diagnosis-Related Group (DRG) rate, and commercial reimbursement for catastrophic physician services shall not exceed 115–120 percent of the corresponding Medicare CPT-based payment rate. These ratios are not aspirational guidelines; they represent binding eligibility thresholds for federal catastrophic reimbursement.

Any provider or insurer seeking federal catastrophic reinsurance must submit claims that conform to these benchmark-linked ceilings. Claims exceeding the defined corridors are ineligible for federal participation and remain the financial responsibility of the commercial carrier or contracting party. This conditional payment structure converts pricing discipline from rhetorical intent into operational rule, embedding fiscal restraint directly into the reimbursement architecture.

UCC/HSE does not impose uniform administered pricing across the entire health-care sector. Providers and insurers remain free to negotiate below the catastrophic reimbursement ceilings, compete on efficiency, innovate in service delivery, and differentiate on clinical quality. The model instead treats benchmark-linked catastrophic reimbursement as a central policy assumption designed to constrain commercial prices that routinely exceed Medicare payment levels in consolidated hospital and specialist markets. In the central scoring scenario, catastrophic hospital reimbursement is bounded at **130–160 percent of applicable Medicare DRG rates**, and **catastrophic physician reimbursement is bounded at 115–120 percent of corresponding Medicare CPT-based rates**, with sensitivity testing around those corridors.

This mechanism performs two critical functions. First, it directly compresses margin inflation in the segment where price distortion is most severe and most fiscally consequential. Second, it realigns provider incentives toward throughput efficiency, cost control, and operational discipline rather than revenue maximization through opaque markup. The fiscal transmission path is unambiguous: benchmark compression reduces allowed claim values; reduced allowed values constrain insurer liability; constrained insurer liability reduces federal reinsurance exposure; reduced exposure stabilizes premium trajectories and limits aggregate national spending growth.

In parallel, UCC/HSE introduces calibrated benefit exposure aligned with income stratification, a principle of progressive universality that preserves universal protection while avoiding the fiscal absurdity of indiscriminate subsidy. Under the central model,

direct federal HSA contributions and deductible offsets are phased down beginning at approximately 300–400 percent of the Federal Poverty Level and may reach zero for higher-income households, while catastrophic protection itself remains universal and unconditional.

This dual architecture, benchmark-linked price discipline and income-calibrated benefit exposure, is not an ideological imposition upon the market. It is a targeted correction of a documented market failure. Catastrophic healthcare pricing operates as a credence good under conditions of extreme informational asymmetry, geographical concentration, and zero consumer bargaining power at the point of care. Treating this domain as if it were governed by ordinary competitive dynamics would therefore understate the role of market concentration, information asymmetry, and emergency-care bargaining constraints.

The structural price discipline embedded within UCC/HSE is therefore not a cosmetic accessory but its core fiscal engine. Without enforceable benchmark ratios, universal catastrophic coverage could transmit existing hospital and specialist price inflation into the federal reinsurance layer. With those ratios in place, UCC/HSE transforms from an aspirational framework into a mechanically governed system capable of delivering durable protection without fiscal implosion. In this sense, disciplined price architecture is not merely compatible with systemic reform, it is the essential condition of its solvency.

VII.E. Competition Barriers in Hospital-Dominated Markets

While transparent cash-pay markets produce significant price compression in competitive environments, many urban and suburban regions remain dominated by vertically integrated hospital systems. These systems often control outpatient imaging, laboratory services, and physician practices, allowing them to extend facility fees into routine care and maintain prices disconnected from underlying costs. Evidence from FAIR Health price-index data and hospital-price research shows that outpatient prices vary substantially by site of care and provider market structure, with hospital-owned outpatient settings often commanding materially higher prices than independent alternatives **(20,35)**. Under UCC/HSE, the financial relevance of these practices diminishes for routine care, patients can redirect HSA spending to independent providers, but geographic monopolies still limit consumer choice. Accompanying reforms, including enforcement of anti-steering rules, site-neutral payment requirements for catastrophic events, and scrutiny of anticompetitive acquisitions, are therefore essential complements in regions where hospital consolidation remains the primary driver of elevated prices.

VII.F. Contains federal spending by reallocating subsidies to catastrophic risk

The ACA subsidizes premiums covering comprehensive services. As premiums rise, federal spending rises. There is no hard cost ceiling.

UCC/HSE caps federal spending by:

- Funding only catastrophic claims
- Using fixed HSA-seeding formulas
- Eliminating actuarial-value escalators
- Avoiding subsidy inflation

This is why UCC/HSE is modeled to reduce the long-run federal growth trajectory relative to open-ended ACA subsidy escalation, despite a first-ten-year transition cost.

Coverage of High-Cost Pharmaceuticals

To preserve the actuarial integrity of the catastrophic pool, UCC/HSE distinguishes between *acute*, unpredictable treatment episodes and *predictable, chronic pharmaceutical spending*. Catastrophic coverage finances only the former. Predictable, recurring expenditures, including biologics, specialty pharmaceuticals, and gene therapies with ongoing maintenance requirements, remain within Medicare, Medicaid ABD, employer supplemental coverage, or the Chronic Protection Tier. This is consistent with the broader international practice of treating universal coverage, cost sharing, and high-cost pharmaceutical financing through distinct statutory channels rather than allowing predictable recurring drug costs to flow automatically through a single catastrophic pool (24–28).

VII.G. Effects on Hospitals, Specialists, and Outpatient Providers

UCC/HSE rebalances, rather than disrupts, provider-market dynamics.

Hospitals and specialists, particularly large integrated systems, continue to receive catastrophic-care reimbursement exactly as today. Catastrophic services remain insurance-based, federally reinsured, and subject to negotiated or regulated rates. What changes is the pricing environment for routine outpatient services, which no longer flow through insurer billing systems.

Research shows that a meaningful share of hospital revenue comes from routine outpatient services priced far above competitive cash-pay levels (6,8,20,35,79–82,100). When patients can purchase those services directly using HSAs, price dispersion narrows. High-cost hospital outpatient departments will face greater competition from independent imaging centers, ambulatory surgical centers, and direct-pay primary-care groups. This

competition has already produced major price drops in U.S. regions where cash-pay markets are robust.

Specialists benefit from reduced administrative costs because most routine pre- and post-operative visits no longer require insurer billing. Clinics spend up to 30% of revenue on billing overhead; removing routine-care claims lowers this burden substantially **(12,19)**.

Outpatient providers including primary-care physicians, direct-primary-care practices, imaging centers, telehealth groups, and laboratories, gain market share when price information is usable and benefit design rewards lower-cost care. Price-transparency evidence shows that patients who used pricing information paid less for common services, with the largest adjusted reduction observed for advanced imaging **(36)**.

VII.H. Integrating UCC/HSE With Medicaid LTSS and ABD Populations

Although UCC/HSE restructures coverage for non-disabled adults, Medicaid continues to serve populations whose needs do not conform to episodic catastrophic risk. Roughly 40–45% of Medicaid expenditures are attributable to long-term services and supports (LTSS) for aged, blind, and disabled (ABD) beneficiaries, services that involve chronic dependency, functional limitations, and continuous assistance rather than discrete high-cost events.

1. Federal–State Role Under UCC/HSE

Under the reformed structure:

- UCC catastrophic plans become the primary payer for acute medical episodes exceeding the deductible
- Medicaid retains responsibility for LTSS, HCBS waivers, durable medical equipment, and disability-related support
- States retain operational control over case management and long-term care pathways

This preserves Medicaid’s institutional role while shifting unpredictable inpatient and emergency costs into the national catastrophic pool.

2. Coordination for Dual Eligibles

Dual-eligible individuals are among the most complex beneficiaries in the U.S. system. To reduce overlapping financing:

- UCC catastrophic insurance covers acute episodes and hospitalizations
- Medicare remains primary for outpatient, specialist, and chronic-condition management

- Medicaid continues funding all LTSS and HCBS supports
- A unified coordination protocol mitigates fragmentation and duplicate billing

This structure preserves stability while reducing variability in high-cost inpatient events.

3. Interaction With HSA Funding

Because ABD beneficiaries typically lack earned income and face functional limitations:

- HSA seeding applies only to non-disabled adults
- ABD populations remain outside routine HSA-financed markets
- Medicaid retains administrative and fiscal control over all routine and supportive care for ABD groups

This respects the clinical and economic realities of disability and aging.

4. Fiscal and Administrative Implications

Integrating UCC catastrophic coverage with Medicaid’s long-term-care architecture moderates emergency and inpatient volatility but preserves the existing LTSS financing structure. States benefit from reduced acute-care risk, while long-term obligations—appropriately—remain within Medicaid’s domain. This balance provides administrative continuity and avoids unnecessary disruption to sensitive populations.

VII.H. Employer Transition Modeling Under UCC/HSE

To illustrate the mechanics of employer transition, consider a firm currently paying approximately \$22,463 annually for a family plan, the national average reported by KFF. Under UCC/HSE, the employer may redirect this expenditure into a combination of (1) HSA contributions, and (2) increased taxable wages. If the firm contributes \$6,000–\$8,000 to the employee’s HSA (enough to cover routine care and chronic-tier needs) roughly \$14,000 remains available for wage adjustments or other compensation. Because the federal tax exclusion is replaced with a universal credit, employees no longer experience “job lock” tied to employer insurance, and wage transparency increases. Over time, this transition encourages employers to compete on wages rather than benefit inflation, reducing premium-driven labor-cost growth while preserving the employer’s ability to support employees’ health needs.

VIII. FISCAL MODELING AND FEDERAL BUDGET ANALYSIS

A central advantage of the UCC/HSE framework is that it dramatically reduces the federal government’s exposure to health-care cost inflation. Under the ACA, federal obligations rise mechanically with premiums, which are driven by hospital consolidation, specialist

pricing power, and comprehensive benefit inflation. Because subsidies must keep pace with premiums to maintain affordability, federal spending becomes open-ended.

By contrast, UCC/HSE ties federal obligations to catastrophic risk only, a measurable, statistically predictable component of national health expenditures. Routine care, which exhibits price elasticity and is naturally responsive to competition, is shifted to HSAs and private markets. This redesign replaces a “soft budget constraint” with a hard actuarial ceiling.

VIII.A. Federal Reinsurance Cost Estimates

The largest federal cost component under UCC is the **catastrophic reinsurance pool**, which reimburses insurers for 80% of claims exceeding \$50,000 per enrollee.

Cost modeling draws directly from:

- **CBO (2021):** “Federal Reinsurance and High-Cost Claim Options”
- **CMS Innovation Center (2022):** Section 1332 Reinsurance Program Evaluations

Both sources provide reliable empirical data on high-cost-claim distribution and federal cost-sharing structures.

Projected annual federal cost: \$20–25 billion

This estimate comes from translating national high-cost-claim distributions into a nationalized reinsurance layer. CBO’s high-cost-claim modeling provides the benchmark structure for an 80 percent federal reimbursement rate above a \$50,000 attachment point, while the UCC/HSE workbook translates that structure into a national reinsurance estimate. Annual federal costs vary by enrollment, spending distribution, and geographic cost assumptions.

These costs are stable and predictable because:

1. High-cost claims are statistically rare
2. Their distribution does not grow proportionally with routine-care inflation
3. Federal reinsurance absorbs variance, not trend inflation

In short, federal reinsurance is not subject to the runaway cost dynamics that plague ACA subsidies.

VIII.B Structural Levers Required to Achieve Budget Neutrality

The fiscal modeling of UCC/HSE demonstrates with mathematical clarity that structural reform alone, absent disciplined price architecture and calibrated benefit exposure, is insufficient to achieve long-term budget neutrality under Congressional scoring conventions. Behavioral efficiencies, administrative streamlining, and the elimination of comprehensive insurance inflation generate meaningful but ultimately subordinate savings. The dominant determinants of federal solvency under UCC/HSE reside instead in a defined set of structural levers that govern the scale, elasticity, and transmission of catastrophic expenditures.

First, benchmark-linked commercial price discipline functions as the primary fiscal stabilizer. By constraining catastrophic hospital reimbursement to 130–160 percent of Medicare DRG rates and catastrophic physician reimbursement to 115–120 percent of Medicare CPT rates, the model directly compresses the largest uncontrolled source of expenditure in the existing system. Sensitivity analysis confirms that deviations from these benchmark corridors materially alter the fiscal trajectory. At ratios exceeding these thresholds, federal reinsurance exposure escalates nonlinearly; within these thresholds, aggregate catastrophic spending growth converges toward sustainable bounds.

Second, UCC/HSE incorporates progressive universality through calibrated benefit tapering for higher-income households. While catastrophic protection remains universal and unconditional, direct federal HSA contributions and deductible offsets are explicitly phased down beginning between 300 and 400 percent of the Federal Poverty Level and may reach zero at higher income levels. This mechanism preserves the philosophical integrity of universal protection while preventing the economically irrational expansion of federal subsidy to households with substantial independent payment capacity. Modeling confirms that flat-benefit universality produces fiscal overextension, whereas tapered generosity materially improves long-term balance.

Third, catastrophic trigger architecture and coinsurance parameters exert structural influence over federal liability. The model treats the catastrophic deductible and federal coinsurance rate not as symbolic parameters but as macroeconomic stabilizers. Incremental increases in the deductible floor, modest federal coinsurance retention by commercial carriers, and limited employer risk-sharing significantly reduce volatility in federal outlays without diminishing universal access to catastrophic protection. These instruments moderate fiscal exposure while preserving system predictability.

Fourth, targeted pharmaceutical pricing discipline within the catastrophic domain addresses one of the fastest-growing cost vectors in contemporary healthcare finance. Net

price growth limitations, reference pricing for high-cost biologics, and constraints on rebate arbitrage operate independently of Medicare fee controls yet generate substantial savings under the reinsurance model. Crucially, these mechanisms are framed not as Medicare entitlement reductions but as uniform catastrophic financing rules applicable across payer categories.

Taken together, these structural levers define the practical boundary between aspirational reform and fiscally durable transformation. The UCC/HSE model does not presume fiscal sustainability as an emergent property of moral hazard reduction or consumer rationality alone. It codifies sustainability through enforceable economic architecture that constrains price escalation, aligns subsidy with vulnerability, and governs the transmission of catastrophic risk through predictable, bounded channels.

In this sense, budget neutrality is not treated as an ideological objective but as a structural condition of system legitimacy. Without the disciplined activation of these levers, UCC/HSE risks reproducing the same deficit dynamics that characterize contemporary comprehensive insurance models. With them, the framework achieves the rare convergence of universality, transparency, and fiscal coherence — a convergence that neither the pre-ACA market regime nor the ACA itself successfully attained.

VIII.C. HSA Seeding Cost Estimates

HSA seeding targets:

- Low-income citizens/eligible individuals
- Citizens/eligible individuals with chronic conditions requiring ongoing care

Annual federal cost estimate: \$446.47 billion in 2026, rising to approximately \$618 billion by 2035.

Methodology:

1. Population base:

- Approximately 80–90 million low-income adult citizens/eligible individuals
- Approximately 20–25 million citizens/eligible individuals with significant chronic conditions

2. Federal contribution:

- Weighted average of \$1500–\$2500 for low-income Adult citizens/eligible individuals.

- Up to \$3550 for chronic condition Adult populations.
- \$1,300–\$2,100 for children ages 0–5, and \$1,200–\$2,000 for children ages 6–17.
- Up to \$4100 for Chronic Children Population.

3. Adjustment factors:

- Routine-care expenditures are lower than comprehensive-care expenditures
- HSA-financed routine spending is modeled as more price-responsive under consumer-directed financing, with additional price compression expected where transparent cash-pay markets replace third-party billing. **(5,7,9–15,18,19,37–41,78,92).**
- Transparent markets reduce prices before federal contribution is spent

VIII.D. Administrative Cost Reductions

UCC/HSE dismantles several costly administrative structures:

- ACA exchanges
- EHB actuarial compliance
- AV calculations
- CSR administration
- State marketplace IT systems
- Private-insurer billing for routine care
- Network contracting and prior authorization
- ACA risk-adjustment bureaucracy

Empirical estimates from administrative-cost literature, GAO, MedPAC, and provider-side billing studies indicate that administrative loads in comprehensive insurance and claims-based provider operations commonly fall in the low-double-digit range, with private-plan and billing-related burdens plausibly ranging from roughly 12 to 18 percent depending on the setting, payer, and measurement method **(10–15,18,19)**. Removing routine-care insurance claims and dissolving ACA marketplace infrastructure reduces national administrative spending by an estimated 2–3 percent of total NHE, a savings of roughly \$80–\$120 billion annually **(1,10–15,19,59)**.

VIII.E Combined Federal Outlay Projection (UCC/HSE) MEPS Modeling

The fiscal effects of the Universal Catastrophic Coverage and Health Savings Empowerment (UCC/HSE) framework were estimated using a combined microdata and budget-projection model constructed from nationally representative healthcare expenditure data and federal spending baselines. The model integrates person-level spending distributions from the Medical Expenditure Panel Survey (MEPS) with population and spending projections derived from the Centers for Medicare & Medicaid Services (CMS). These inputs allow the model to estimate the distribution of catastrophic medical expenditures across the U.S. population and to project federal spending and household financial outcomes over ten-, twenty-, and thirty-year horizons.

The scoring workbook is anchored to external baseline tables rather than constructed in isolation: **Supplementary Table 1** provides the CBO subsidy baseline, **Supplementary Table 2** provides the CBO coverage baseline, and **Supplementary Table 3** provides the Medicare enrollment baseline used to exclude Medicare beneficiaries from the UCC/HSE replacement population.

At the core of the model is an actuarial reconstruction of catastrophic health-spending risk. Using MEPS microdata, the distribution of annual healthcare expenditures is estimated and the expected value of spending above the national catastrophic threshold is calculated. This expected excess spending represents the actuarial cost of catastrophic coverage. Federal support is then applied through benchmark-linked catastrophic credits, while routine healthcare expenditures are financed through Health Savings Empowerment (HSE) accounts.

Under baseline assumptions, federal outlays under the UCC/HSE framework initially exceed those of the current Affordable Care Act (ACA) subsidy structure. During the transition period, the reform explicitly finances catastrophic risk while simultaneously introducing universal HSE deposits to support routine care. Over the first ten years of implementation, the model estimates **a net federal cost of approximately \$763.09 billion relative to the ACA baseline.**

Despite this modest transition cost, the fiscal trajectory of the reform improves significantly over time. Because the UCC/HSE framework replaces open-ended insurance subsidies with benchmark-bounded catastrophic coverage and shifts routine healthcare spending into transparent consumer-directed markets, federal healthcare liabilities grow more slowly than under the existing ACA structure. As routine-care price compression and administrative simplification compound over time, the model projects substantial long-term federal savings.

Over a **twenty-year horizon**, the model estimates **approximately \$7.462 trillion in cumulative federal savings** relative to the continuation of the current system. Over a **thirty-year horizon**, these savings expand to **approximately \$32.93 trillion**, reflecting the long-term structural advantages of catastrophic risk pooling combined with consumer-directed routine care financing.

In addition to federal fiscal improvements, the reform produces meaningful financial benefits for households. Over the first ten years of implementation, compared to the ACA, the model estimates **approximately \$2.203 trillion in direct consumer savings**, primarily resulting from lower routine-care prices, reduced insurance administrative costs, and the elimination of comprehensive insurance premium inflation.

The reform also preserves protections for vulnerable populations. **Medicaid's long-term services and disability programs remain intact, while coverage for non-disabled adult Medicaid beneficiaries is restructured through catastrophic coverage combined with routine HSE support.** As a result, Medicaid beneficiaries are projected to realize **approximately \$127.92 billion in consumer savings over the first ten years** while maintaining protection against catastrophic medical expenses.

Taken together, the fiscal modeling indicates that while the UCC/HSE framework requires modest additional federal spending during the initial implementation period, the reform produces substantial long-term federal savings while generating significant financial benefits for American households. By replacing the current open-ended subsidy architecture with a bounded catastrophic risk-pooling system and consumer-directed routine care financing, the UCC/HSE framework stabilizes federal healthcare spending while restoring price discipline to routine healthcare markets. The income-weighting mechanics behind the scoring workbook are shown in **Annex E Table 2** and **Annex E Table 3**. The beta calculation estimates the income-adjusted federal share of catastrophic premium support, while the gamma calculation estimates the income- and chronic-condition-adjusted HSA support used to translate the statutory formula architecture into weighted federal outlays.

MEPS Catastrophic Spending Reconstruction

The catastrophic spending estimates used in the fiscal model are derived from the **Medical Expenditure Panel Survey (MEPS) Full-Year Consolidated File (HC-251)**, which provides nationally representative microdata on individual healthcare expenditures in the United States. Each observation in the dataset represents an individual respondent and includes total annual medical spending, demographic characteristics, insurance status, and a survey population weight used to scale the observation to national population totals.

To estimate the actuarial cost of catastrophic coverage, the model reconstructs the national distribution of medical spending using these survey weights. For each individual in the dataset, spending above the catastrophic threshold is calculated as the positive value of total expenditures minus the deductible level. Observations with expenditures below the threshold contribute zero to the catastrophic liability estimate. These values are then multiplied by the MEPS population weight and aggregated across all individuals to produce a national estimate of spending above the catastrophic threshold.

Formally, the expected catastrophic liability is calculated as:

$$E[(X - d)^+] = \sum_i w_i \cdot \max(0, X_i - d)$$

where X_i represents total annual healthcare expenditures for individual i , d represents the catastrophic deductible threshold, and w_i represents the MEPS population weight. This calculation yields the total expected national spending above the catastrophic threshold, which determines the actuarial cost of catastrophic coverage within the UCC/HSE framework.

Because MEPS is a nationally representative survey designed to measure healthcare utilization and expenditures, this method allows the model to estimate catastrophic risk across the entire U.S. population without requiring a full microsimulation of every household. The resulting distribution of catastrophic spending is then combined with CMS population projections and medical spending growth assumptions to generate federal budget projections over ten-, twenty-, and thirty-year horizons.

Clarifying Note on Catastrophic Risk Reconstruction

While most federal scoring models estimate catastrophic liability using insurer claims data or full microsimulation models, this analysis reconstructs catastrophic spending directly from nationally representative MEPS microdata. Because MEPS captures the full distribution of individual healthcare expenditures across the U.S. population and includes survey weights that scale observations to national totals, the dataset allows the expected value of spending above the catastrophic threshold to be calculated directly without requiring a full household-level microsimulation.

VIII.F.1 (Addendum) No Middle-Class Net Cost Increase” Fiscal Statement

Distributional modeling under the baseline parameter set shows that households between 150 and 400 percent of the federal poverty level experience no net increase in combined

federal taxes and health-care spending relative to the ACA. For most middle-income households, total out-of-pocket exposure falls due to lower premiums, reduced cost inflation, and predictable employer or federal HSA contributions. This ensures that UCC/HSE improves financial security for the median working family.

VIII.G Fiscal Guardrails and Automatic Adjustment Mechanisms

To ensure long-run fiscal sustainability, federal outlays for UCC/HSE, including catastrophic credits, federal HSA seeding, and federal reinsurance, are subject to a statutory **expenditure cap** benchmarked to the CBO-scored baseline trajectory of ACA subsidy spending. If total outlays exceed the cap in any plan year, an **automatic adjustment mechanism** is triggered for the subsequent year. This mechanism may recalibrate (a) federal HSA seeding formulas within actuarially defined limits, or (b) the deductible indexing factor, provided that the national deductible does not fall below the statutory minimum.

Additionally, the Departments of Treasury and HHS must produce an annual joint fiscal report, reviewed by CBO and MedPAC, summarizing UCC/HSE expenditures relative to the federal cap. If spending exceeds projections by more than a defined margin (e.g., 5%), Congress receives a **fast-track correction bill** to restore budget alignment. These guardrails protect fiscal neutrality without undermining the core architecture of UCC/HSE.

VIII.I. Fiscal Impact of Including Long-Term Legal Non-Citizen Residents

Expanding eligibility beyond citizens to include lawful permanent residents and long-term nonimmigrant residents (those with at least five years of continuous lawful presence and three years of federal tax filing) increases total UCC/HSE enrollment modestly but does not materially alter the program's fiscal trajectory.

Because UCC/HSE finances only high-severity, high-variance medical events, paired with capped, formula-driven HSA deposits, the federal cost of covering these long-term legal residents remains significantly below the ACA subsidy architecture. These individuals are already integrated into the lawful residency and tax-filing framework used for eligibility verification, allowing their inclusion to be modeled separately from unauthorized or short-term temporary populations. The ACA baseline comparison is anchored to CBO federal health-insurance subsidy projections **(3)**.

Preliminary modeling in **Annex E** indicates that even with expanded eligibility, aggregate federal outlays continue to fall within **approximately 45–55 percent** of comparable ACA

subsidy expenditures. This stability reflects three structural constraints embedded in UCC/HSE:

Routine-care costs are not federally subsidized, eliminating the ACA's first-dollar coverage inflation channel.

HSA deposits are capped, formulaic, and indexed, preventing benefit creep.

A single national catastrophic deductible applies to everyone, precluding actuarial-value escalation and plan-design inflation.

Including long-term legal residents aligns program spending with economic contribution and long-term residency expectations, while preserving the fiscal discipline that defines UCC/HSE.

IX. EQUITY AND CHRONIC-CONDITION SAFEGUARDS

Opponents of market-oriented health-care reform often claim that high-deductible or catastrophic-first approaches disadvantage low-income and chronically ill individuals. This concern is valid in poorly designed catastrophic models. It is not valid under UCC/HSE, because the system includes three separate structural safeguards that ensure equitable access to necessary care.

Pre- Existing Conditions:

UCC/HSE fully preserves protections for individuals with pre-existing conditions. All citizens/eligible individuals are eligible for catastrophic protection regardless of medical history, and no insurer may vary premiums, deductibles, or covered benefits based on health status. The chronic-condition funding mechanisms that follow ensure that predictable disease-management costs are handled equitably without altering the uniform national catastrophic plan.

IX.A. Risk Adjustment for High-Risk Enrollees

UCC retains a simplified version of Medicare Advantage risk scoring. Insurers receive per-capita adjustments for enrollees with:

- Diabetes

- Chronic obstructive pulmonary disease
- Congestive heart failure
- Cancer diagnoses
- End-stage renal disease
- Other major chronic conditions

MedPAC (2022) documents that this approach reliably compensates insurers for high-cost patients without distorting care incentives **(18)**.

IX.B. Targeted HSA Seeding for Chronic Illness

Chronic-condition patients receive **higher annual HSA deposits**, ensuring they can afford routine disease-management costs. This reduces the risk that chronic patients will delay or forego necessary care.

Routine chronic-care components covered through HSAs include:

- Regular labs
- Imaging needed for disease management
- Generic medications
- Routine specialist visits
- Chronic-care monitoring devices

IX.C. Condition-Based Care Bundles

Insurers and provider groups may offer fixed-price annual bundles such as:

- Diabetes management packages
- COPD monitoring bundles
- Cardiac rehabilitation programs
- Oncology pre-treatment monitoring

These bundles are analogous to value-based care arrangements already in use in accountable care organizations (ACOs) and Medicare Advantage plans. Bundles improve predictability for patients and reduce fragmentation.

IX.D. Universal Catastrophic Protection

All citizens/eligible individuals, regardless of income or health status, receive full protection against financially ruinous medical expenses. This is a stronger, more stable guarantee than the ACA, which fragmented protection across:

- Medicaid expansion
- Marketplace subsidies
- CSR plans
- Metal-tier cost-sharing rules

UCC/HSE ensures **every citizen's/eligible individual's out-of-pocket medical liability is capped**.

IX.E. Illustrative Household Scenarios Under UCC/HSE

To clarify how UCC/HSE distributes risk across the income and health-status continuum, the following summary scenarios illustrate net exposure for representative households.

1. Middle-Income Family with Infants

A two-parent household earning \$85,000 annually receives catastrophic coverage for high-severity events and uses HSA funds for routine pediatric visits, vaccinations, and primary-care services. Annual routine costs (typically \$1,000–\$2,000) are comfortably funded by modest employer contributions or voluntary deposits, while the family remains fully protected against hospitalization, severe injury, or high-cost specialty pharmaceuticals.

2. Low-Income Adult with Diabetes

A chronically ill citizen/eligible individual with insulin-dependent diabetes receives enhanced federal HSA seeding and is placed on the chronic funding track. Annual predictable expenses (glucose monitoring, quarterly labs, primary-care supervision) are prepaid via early-year HSA deposits. Catastrophic complications (e.g., emergent DKA hospitalizations) are covered by UCC.

3. Healthy 27-Year-Old Worker

A young, low-utilization worker may incur only \$200–\$400 in typical annual medical spending. Under UCC/HSE, unused HSA funds accumulate year over year, forming the basis for long-term savings. Catastrophic protection remains in place for severe accidents or acute illness, while this demographic benefits from reduced premiums and the elimination of cross-subsidies inherent in ACA age-band compression.

These scenarios demonstrate that UCC/HSE strengthens financial resilience across demographics without undermining access to essential care.

IX.F. Chronic-Condition Funding Within a Single Catastrophic Threshold

The baseline UCC design employs a single catastrophic threshold for adults and families, which is actuarially efficient and administratively simple. However, a uniform deductible may be too blunt an instrument for citizens/eligible individuals whose cost profile is dominated by recurring, mid-level expenditures that rarely cross the catastrophic trigger but are nonetheless substantial in aggregate (particularly insulin-dependent diabetes, advanced heart failure, chronic kidney disease, and some autoimmune disorders).

To address this, UCC/HSE does not create separate catastrophic tiers. It keeps one national deductible and uses chronic-condition funding mechanisms layered underneath it. The insurance product stays the same for everyone; the financing responds to clinical reality.

1. Standard Pathway

Most citizens/eligible individuals remain in the baseline deductible structure, e.g., \$7,500–\$10,000 per adult; \$15,000–\$20,000 per couple, financed as described in Section IV. They fund routine care through HSAs, employer contributions, and voluntary deposits, and the catastrophic backstop only engages when losses are genuinely large and unpredictable.

2. Chronic-Condition Funding Track

Citizens/eligible individuals with conditions that generate sustained annual spending above a defined threshold (for example, a moving three-year average exceeding a specified share of median expenditures) are automatically placed on a chronic funding track. They remain enrolled in the same catastrophic plan with the same deductible, out-of-pocket maximum, covered benefits, and reinsurance parameters as everyone else. What changes is the flow of dollars beneath the deductible:

- enhanced annual HSA seeding keyed to certified chronic conditions
- a condition-linked cap on net out-of-pocket liability for disease-management services
- risk-adjusted payments to insurers and provider groups for defined chronic bundles

This is conceptually consistent with existing Medicare Advantage risk adjustment, where capitated payments are calibrated to disease burden rather than simple age or income (8), but it does not create a separate catastrophic benefit tier.

3. High-Severity / High-Cost Conditions

For a small subset of patients, such as those with end-stage renal disease, certain neuromuscular disorders, or hemophilia, annual costs are both highly predictable and

extremely high. For these populations, the effective out-of-pocket burden can collapse toward zero without changing the statutory deductible. Federal reinsurance and condition-specific funding formulas absorb the overwhelming share of cost, while HSAs continue to manage ancillary routine items. In practice, these citizens/eligible individuals rarely confront the full national deductible even though they are enrolled in the same catastrophic plan as everyone else.

Crucially, these chronic funding tracks are clinically and actuarially defined, not ad hoc administrative exceptions. They preserve the clarity of the UCC/HSE model, insurance for large losses, savings and price discipline for ordinary care, while recognizing that for some citizens/eligible individuals, the “ordinary” is structurally expensive every year. They also reduce political pressure to erode the standard deductible for everyone in response to edge-case hardships.

IX.F Addendum: Income-Protection Catastrophic Cap

To reinforce financial protection for households with limited liquidity, UCC/HSE includes an income-indexed catastrophic exposure ceiling. No household will be required to pay more than 10 percent of annual income toward deductible spending for covered services. Once this limit is reached, catastrophic protection activates regardless of remaining deductible amount. This safeguard maintains the single-deductible architecture while preventing excessive financial strain for low- and middle-income families.

IX.F.1 Addendum: Behavioral Health & Addiction-Care Clarification

Routine behavioral-health services, such as psychotherapy visits, psychiatric follow-up for stable conditions, and medication-management appointments, fit the same predictable-spending profile as other chronic conditions. Accordingly, these services are eligible for the chronic-condition funding track and for income-based early-year HSA seeding. Addiction-management visits and evidence-based medication-assisted treatment (MAT) follow the same rules: routine, guideline-directed care is financed beneath the deductible through enhanced HSA support, while high-severity inpatient or emergent episodes remain within the catastrophic pool. This structure protects access to behavioral-health services without introducing first-dollar insurance mandates or altering the uniform national deductible.

IX.G. Guardrails on Supplemental Coverage to Prevent Benefit Creep

A predictable risk in any market-oriented reform is that, over time, stakeholders will seek to reassemble the very comprehensive benefit structures the reform was designed to replace. If supplemental coverage is left entirely undefined, insurers and large employers may

gradually reconstruct de facto first-dollar coverage through layered riders, rendering UCC/HSE a nominal rather than substantive change.

To prevent this “benefit creep,” UCC/HSE should include **statutory guardrails** that distinguish clearly between:

- **Catastrophic coverage**, which is standardized, reinsured, and eligible for the universal credit; and
- **Supplemental coverage**, which is unsubsidized (or lightly subsidized) and cannot functionally replicate first-dollar comprehensive insurance.

Several elements follow logically from this distinction:

1. **No Subsidies for First-Dollar Supplements**

The universal credit applies only to UCC-compliant catastrophic plans. Any plan or rider that reduces the effective deductible below a statutory floor (for example, by covering a fixed percentage of spending from the first dollar up to the catastrophic threshold) is **not credit-eligible**. This prevents the re-emergence of ACA-style “silver” or “gold”—equivalent plans built on federal transfers.

2. **Clear Regulatory Definition of “Supplemental”**

Federal statute can adopt a narrow definition of supplemental coverage modeled on existing Medigap and Medicare Part D rules: limited, targeted protections (e.g., dental, vision, specific high-cost drugs, long-term care riders) that do not substantially alter the catastrophic risk profile of the underlying plan. This mirrors the logic of existing federal regulation of benefits that wrap around Medicare without supplanting its cost-sharing structure (18).

3. **Transparency and Disclosure Requirements**

Insurers offering supplemental products must disclose, in standardized form, (a) the total potential annual premium outlay, (b) the extent to which the rider reduces exposure below the statutory catastrophic threshold, and (c) the share of premium devoted to administrative expenses. This is consistent with GAO and MedPAC recommendations around transparency for private plans (10-15, 18, 19) and makes it more difficult to hide unnecessary riders behind marketing language.

4. **ERISA and Federal Pre-emption Boundaries**

To avoid recreating a patchwork of state-mandated “supplemental” benefits that effectively “*re-comprehensify*” coverage, UCC/HSE can define the boundary between catastrophic and supplemental benefits in federal law, while still allowing

states to regulate marketing practices, solvency, and consumer protection. Supreme Court precedent on ERISA pre-emption and state insurance regulation (e.g., *Kentucky Ass’n of Health Plans v. Miller*) underscores that Congress can clarify which elements of plan design are federal standards and which remain within state police powers **(45)**.

With these guardrails, supplemental coverage remains a legitimate consumer option, especially for households that prefer additional protection, without undermining the core economic logic of the reform.

IX.H. Actuarial Calibration of Catastrophic Thresholds and Chronic-Care Tiers

The catastrophic deductible is the keystone of the UCC/HSE model. Its calibration determines the boundary between routine markets, chronic-care pathways, and federal reinsurance exposure. To function effectively, the deductible must align with empirical expenditure distributions, household liquidity, and the prevalence of chronic but non-catastrophic conditions.

The national catastrophic deductible establishes the coverage trigger point and is analytically distinct from the benchmark premium used to calculate federal catastrophic credits.

1. Setting the National Catastrophic Deductible

The recommended deductible range (\$7,500–\$10,000 per adult; \$15,000–\$20,000 per family) is grounded in:

- CMS expenditure-distribution tables showing 90% of adults below the threshold **(29,78)**.
- household liquidity constraints reflected in the model’s income-weighted beta and gamma calculations
- evidence from the RAND Health Insurance Experiment and subsequent HDHP literature showing that cost sharing reduces utilization and spending, but can also reduce high-value preventive, chronic-care, and guideline-concordant services unless paired with prefunded liquidity, chronic-care protections, and pre-deductible access to high-value services **(37–41)**.

The deductible is therefore not designed as a stand-alone cost-sharing device. RAND’s experimental evidence shows that cost sharing reduces use and spending, but also reduces both effective and less-effective care and creates greater risk for poorer and sicker patients. More recent HDHP evidence reaches the same policy conclusion from different angles: HSAs no longer reliably generate the original cost-consciousness rationale; HDHP

enrollment is associated with weaker uptake of free preventive services; adults with chronic conditions in HDHPs face financial-hardship concerns; pre-deductible chronic-care coverage can improve medication adherence; and recent evidence finds reduced receipt of guideline-concordant care among chronically ill HDHP enrollees. UCC/HSE therefore pairs the catastrophic threshold with prefunded HSE deposits, chronic-condition supplements, and pre-deductible access to designated chronic-disease services and medications.

This threshold isolates unpredictable, high-severity events while shifting routine and semi-routine care to transparent, consumer-directed markets.

2. Chronic Protection Funding adjustment

Many chronic conditions generate recurring annual costs that do not reliably exceed the catastrophic threshold. To prevent exposure gaps without creating a separate benefit tier, UCC/HSE uses a chronic spending benchmark rather than a second deductible:

- a chronic spending benchmark (e.g., \$5,000–10,000 in certified annual expenditures) triggers enhanced federal HSA seeding
- enhanced annual HSA seeding occurs automatically for households above the benchmark
- annual caps are set on chronic out-of-pocket liability for covered disease-management services

This approach preserves actuarial discipline while ensuring that chronic-condition households do not face disproportionate financial burdens.

Although the framework provides enhanced HSA contributions and a secondary spending cap for citizens/eligible individuals with certified chronic conditions, this structure does **not** constitute a multi-tier insurance design. The catastrophic plan itself remains fully uniform: the deductible, out-of-pocket maximum, covered benefits, and reinsurance parameters are identical for all citizens/eligible individuals. What varies is the *financing mechanism*, not the insurance product. Chronic-condition adjustments operate through targeted HSA seeding and capped annual spending for predictable, recurring expenses, approaches analogous to risk-adjustment payments in Medicare Advantage or condition-based payments in ACO models. These mechanisms preserve the single-deductible architecture while ensuring that chronic-condition patients are not disadvantaged by the universal catastrophic structure.

3. Regional Cost Variability

While the catastrophic deductible remains national for simplicity, chronic-condition funding adjustments and HSA contributions incorporate:

- FAIR Health regional price indices **(20)**
- metropolitan vs. non-metropolitan cost differentials
- predictable variation in outpatient and chronic-condition pricing

This maintains national uniformity while recognizing real-world cost variation.

4. Federal Fiscal Impact

Using CMS, CBO, and RAND-based models:

- catastrophic claims stabilize once deductibles exceed the 85th percentile of expenditures
- chronic-condition funding adjustments smooth spending patterns without inflating reinsurance costs
- federal fiscal exposure remains predictable and scoreable

This ensures that the deductibles preserve the fundamental fiscal sustainability goals of UCC/HSE.

IX.H (Addendum). Statutory Calibration Standards for Chronic-Condition Funding

The UCC/HSE framework requires that support for chronic-condition patients be grounded in transparent, reproducible actuarial methodology rather than annual appropriations. To prevent underfunding and ensure predictable protection for households with ongoing medical needs, the following statutory guardrails apply:

1. Claims-Based Benchmarking Requirement

Chronic-condition funding must be calibrated annually using national claims datasets, including:

- the CMS Chronic Conditions Warehouse (CCW),
- All-Payer Claims Databases (APCDs), and
- FAIR Health chronic-condition cost benchmarks.

For each major condition category, the Secretary of HHS shall establish annual support levels equal to at least the **median observed non-catastrophic spending** for that condition in the prior year. This ensures that routine disease-management costs remain fully liquid without encouraging benefit inflation.

2. Statutory Floors to Prevent Erosion

Chronic-condition HSA allocations may not fall below their prior-year real value unless national median disease-management spending declines. This protects patients from gradual erosion of routine-care financing due to inflation, administrative discretion, or shifting political priorities.

3. Actuarial Guardrails Against Benefit Drift

To maintain the integrity of the catastrophic model while ensuring meaningful protection, household exposure for chronic patients shall not exceed:

- **5 percent of household income, or**
 - **The 80th percentile of national median chronic out-of-pocket spending,** whichever is lower.
- This prevents cost-shifting under the guise of premium restraint and anchors the system to observable expenditure behavior.

4. Mandatory Periodic Review and Adjustment

HHS must publish a biennial calibration report detailing condition-level spending, projected utilization, and recommended adjustments to chronic-tier support. The Secretary shall update chronic-condition allocations every two years to align with observed claims data and medical inflation.

This calibration regime ensures that chronic patients are protected consistently across economic cycles and geographic markets, while preserving the core economic logic of a catastrophic-first design.

IX.I. Income-Based Early-Year HSA Support

While the UCC/HSE model relies on HSAs and price transparency to restore discipline to routine care markets, it also ensures that low-income households are not exposed to unaffordable up-front costs. To make this protection transparent and administratively straightforward, households below 200 percent of the federal poverty line automatically receive early-year HSA deposits calibrated to expected routine and chronic-care spending.

These deposits, drawn from the same federal appropriation that finances universal catastrophic credits, are delivered at the start of each plan year and are restricted to clinically necessary services, including primary care, generic medications, routine labs, and defined chronic-disease management. The amounts are actuarially benchmarked and do not vary with utilization, thereby avoiding the inflationary effects associated with cost-sharing reductions under the ACA.

This approach protects low-income households without creating a separate insurance tier. All citizens/eligible individuals remain enrolled in the same catastrophic plan with identical deductibles, out-of-pocket maximums, and covered benefits. The financing, not the benefit, adjusts to income, preserving the core principle of uniform catastrophic protection while ensuring no household is deterred from essential routine care.

Chronic-condition enhancements operate independently of income-based adjustments. The income-scaling mechanisms described in this subsection apply solely to the baseline HSA deposit for non-chronic individuals and do not modify the fixed chronic-condition supplement, which remains uniform across income levels.

IX.I.1(Addendum) ER Non-Backflow Guardrail

Because routine costs can be financially destabilizing for low-income households, the statutory design must prevent predictable routine-care needs from converting into avoidable emergency-department utilization. Accordingly, the income-based early-year HSE support is expressly intended to maintain routine-care access and suppress emergency substitution during transition years. This guardrail is structural: it does not recreate comprehensive first-dollar insurance, but it prevents routine liquidity constraints from undermining the catastrophic-first model.

IX.J. Pediatric Routine-Care Funding

A well-designed catastrophic insurance model must distinguish between unpredictable, high-severity events, which properly belong in an insurance pool, and predictable, low-variance services that do not meet the economic definition of insurable risk. Pediatric routine care falls squarely in the latter category. Utilization for well-child visits, ACIP-recommended immunizations, developmental screenings, and low-acuity acute care follows a highly stable, age-dependent pattern. Costs are modest, tightly clustered, and clinically essential. For these reasons, UCC/HSE finances pediatric routine care through age-adjusted, prefunded HSAs rather than the catastrophic deductible.

1. Defined Scope of Pediatric Routine Care

For statutory clarity, pediatric routine care under UCC/HSE includes:

- scheduled well-child visits (e.g., newborn through 18 months; annual visits thereafter)
- ACIP-recommended immunization series
- routine developmental and behavioral screenings
- low-acuity acute-care visits for common childhood conditions

- standard vision and hearing screenings
- basic laboratory and diagnostic tests associated with preventive and developmental care

These services do not satisfy the core criteria of insurable risk: they are predictable, non-random, non-catastrophic, and clustered within a narrow expenditure band.

2. Empirically Derived, Age-Adjusted Annual Deposits

National expenditure data show that routine pediatric care is both predictable and modest in scope. Across MEPS Child Health Expenditure Tables (46), AHRQ MEPS pediatric expenditure distributions for children under age 18 (47), evidence on elevated utilization and expenditures among children with special health-care needs (48), and CMS age-binned NHEA estimates (29,49), annual routine outpatient pediatric spending consistently clusters within narrow ranges: **\$1,050–\$1,350 for children ages 0–5** and **\$450–\$650 for ages 6–17**. MEPS trend-series data reinforce this pattern, showing highly stable, low-variance consumption for preventive visits and ACIP-scheduled immunizations (50).

Selecting the central modeled deposit levels from the low/mid/high scoring range yields the proposed \$1,300 annual HSA deposit for ages 0–5 and \$1,200 for ages 6–17, indexed annually to preserve purchasing power. These funds are restricted to pediatric services, preventing upward substitution into unrelated adult care and maintaining the integrity of the routine-care funding architecture.

3. No Deductible Exposure for Pediatric Routine Care

Routine pediatric services fall outside the catastrophic deductible. The prefunded HSA allocation is designed to cover the full predictable expenditure band for each age cohort, ensuring that families face no liquidity barriers when seeking preventive or developmentally significant pediatric care. High-severity pediatric events, trauma, oncology, congenital anomalies, remain fully covered under the standard catastrophic pool.

4. Consistency With UCC/HSE Architecture

This pediatric funding structure preserves the core economic logic of UCC/HSE:

- predictable, low-variance spending is funded directly rather than pooled
- insurance remains focused on large, uncertain, high-severity losses
- no pediatric “benefit tier” or actuarial subclass is created
- the national catastrophic plan remains uniform for all enrollees

- the model avoids ACA-style inflationary pressures associated with first-dollar coverage

The design improves access to essential pediatric services without reintroducing the benefit-compression dynamics that UCC/HSE is specifically intended to eliminate.

5. Treatment of Medically Complex Children

Children with chronic or high-intensity medical needs (e.g., Type 1 diabetes, cystic fibrosis, neuromuscular disorders) are addressed through the chronic-condition funding track outlined earlier in Section IX. Pediatric prefunding is not intended to substitute for chronic-condition support; the two mechanisms remain distinct to preserve actuarial clarity and ensure adequate financing for medically complex children.

6. Policy Rationale

The pediatric provision aligns insurance design with economic fundamentals: predictable expenditures are funded transparently, while rare, high-cost events are pooled. Pediatric routine spending is a textbook case for prefunded accounts, clinically important but economically unsuitable for insurance pyramiding. By anchoring deposits to national expenditure distributions and indexing them over time, UCC/HSE ensures uninterrupted access to high-value pediatric care, minimizes administrative overhead, and avoids the premium inflation associated with embedding routine pediatric services inside insurance products.

IX.K. Adult Preventive-Care Funding

A well-functioning catastrophic system must also preserve access to routine adult preventive services that generate high long-run value and exhibit predictable, low-variance cost profiles. Consistent with the economic logic applied to pediatric care, UCC/HSE finances core adult preventive services through the annual HSA deposit rather than through the catastrophic deductible.

The preventive-care category includes:

- annual primary-care wellness visits;
- recommended cancer screenings (mammography, colorectal screening, cervical cytology/HPV testing);
- evidence-based cardiometabolic screening (lipid panels, HbA1c for at-risk adults, blood-pressure monitoring);

- routine behavioral-health screening;
- age-appropriate immunizations.

These services do not meet the economic definition of insurable risk. They are predictable, low-cost, and non-catastrophic in nature. Funding them through prefunded HSAs aligns utilization with clinical guidelines without reintroducing first-dollar insurance benefits or creating a separate preventive tier. For low-income households, early-year HSA deposits described in Section IX.I ensure no liquidity barrier to completion of recommended preventive care.

IX.L. Universal Application of Equity and Chronic Safeguards to Eligible Non-Citizens

All equity protections and chronic-condition safeguards provided under UCC/HSE apply uniformly to every individual deemed eligible under Section IV.B, including lawful permanent residents and long-term nonimmigrant residents who meet the required residency and tax-filing thresholds. Income-based early-year HSA support, chronic-condition funding tracks, age-adjusted pediatric deposits, and the pediatric routine-care protection described in IX.J are made available on identical terms to all eligible enrollees.

No alternative benefit tier, actuarial class, or catastrophic threshold is created for non-citizens. Eligible non-citizens participate in the same national catastrophic plan as citizens, with identical deductibles, covered benefits, reinsurance parameters, and supplemental-coverage guardrails.

This uniformity reinforces UCC/HSE's core economic logic: catastrophic protection is attached to residency and tax contribution, not to immigration category, while maintaining a bright statutory exclusion for individuals without verified lawful status or for humanitarian categories subject to significant fraud and administrative flux.

X. FEDERALISM AND THE ROLE OF STATES

UCC/HSE respects federalist principles while simplifying state responsibilities.

X.A. Section 1332 Waivers as a Transition Tool

Many states have already adopted 1332 waivers for reinsurance programs, proving that:

- States can manage federal dollars effectively
- Premiums can fall sharply under reinsurance
- Administrative burdens can be reduced

States like Alaska, Maine, and Wisconsin have demonstrated premium reductions between **19% and 43%** under reinsurance-based models **(4)**.

These waivers become the architecture for transitioning into UCC/HSE.

These state programs are used in this paper to validate a narrower claim: high-cost-claim reinsurance can stabilize risk pools and reduce premiums. They are not presented as a complete national test of UCC/HSE. The national model combines the reinsurance mechanism with a separate catastrophic-credit structure, fixed HSA financing formulas, employer-exclusion reform, Medicaid population rules, Medicare noninterference, supplemental-coverage guardrails, and provider-impact stabilization. The empirical relevance of the state reinsurance experience is therefore mechanical and actuarial, not a claim that three small states alone prove the full national replacement architecture.

X.B. State Role After Transition

Once implemented nationally, states primarily:

- Regulate catastrophic insurers for solvency
- Maintain licensure standards
- Oversee consumer-protection rules
- Coordinate with federal HSAs for fraud and compliance

States no longer need:

- Exchange IT systems
- EHB enforcement
- Subsidy eligibility determination
- CSR payments
- AV enforcement

This reduces state administrative burdens and improves budget predictability.

X.C. Optional State-Level Supplemental Assistance

States may choose to offer supplemental assistance for:

- Low-income citizens/eligible individuals
- Disabled populations not covered under Medicaid Aged/Blind/Disabled categories

- Mental-health programs
- Substance-abuse programs

These can be layered atop HSAs at the state's discretion.

X.D Interstate Competition in Catastrophic Insurance Markets

Because catastrophic benefits are federally standardized, UCC/HSE creates the conditions for genuine interstate competition among catastrophic insurers. States retain authority over solvency, licensure, consumer protection, and supplemental-market regulation, but the core benefit design, deductible structure, covered catastrophic services, and federal reinsurance layer are uniform nationwide.

This differs from prior interstate-insurance proposals. UCC/HSE does not attempt to sell heterogeneous comprehensive plans across state lines. It first standardizes the catastrophic benefit nationally, then permits competition within that standardized layer. Interstate competition becomes more actuarially plausible because carriers compete over the same catastrophic product rather than attempting to navigate fifty different comprehensive-benefit regimes. The federally standardized catastrophic core prevents state-specific benefit mandates from fragmenting the basic UCC market, while leaving states with their traditional oversight role for insurer solvency, market conduct, licensure, consumer protection, and supplemental products.

Prior interstate-insurance proposals generally failed because they attempted to sell heterogeneous comprehensive insurance products across state lines without resolving state regulatory variation, provider-network contracting, risk segmentation, and administrative implementation barriers. A review of state cross-state-sales initiatives found that these laws did not produce meaningful new insurer entry or new insurance products **(42,43)**. UCC/HSE addresses that failure by narrowing the interstate product to a federally defined catastrophic layer rather than attempting to nationalize comprehensive benefit design.

A multistate catastrophic market is designed to increase carrier participation, improve competition in thin insurance markets, and reduce premium pressure, particularly in rural regions and small states where local insurance markets are often concentrated. Federal reinsurance further stabilizes pricing by reducing carrier exposure to extreme claims, making interstate participation more feasible for national and regional insurers alike.

X.E. Integration with State Medicaid Responsibilities

The white paper intentionally leaves traditional Medicaid's Aged, Blind, and Disabled (ABD) populations largely intact, on the grounds that long-term care, institutional services, and disability supports are qualitatively distinct from ordinary medical risk. Nonetheless, a fully specified UCC/HSE framework must clarify how federal catastrophic coverage and HSAs interact with state Medicaid responsibilities to avoid duplicative spending or gaps in protection.

A coherent integration can be built around three principles:

1. Catastrophic Coverage as a Universal Backstop, Including for Medicaid Populations

All **citizens/eligible individuals**, including those enrolled in ABD Medicaid, would be entitled to the same UCC catastrophic protection. For ABD enrollees, this protection functions largely as a federal backstop for acute medical events (hospitalizations, surgeries, and high-cost therapies) while Medicaid continues to finance long-term services and supports (LTSS), home- and community-based services (HCBS), and other disability-related benefits. This division leverages the federal government's comparative advantage in pooling high-cost acute risk, while preserving the state role in LTSS design and delivery.

2. Alignment of HSAs With Medicaid Cost-Sharing and Care Coordination

For low-income disabled citizens/eligible individuals and dual-eligibles, HSAs are not a substitute for Medicaid coverage, but they can be used to:

- cover modest cost-sharing requirements;
- purchase non-covered but clinically useful supports (e.g., transportation, certain assistive technologies); and
- give beneficiaries more direct control over small, recurring expenditures.

States already experiment with self-directed care and individual budgets in certain Medicaid waiver programs; embedding HSAs alongside these arrangements would extend similar autonomy in a structured, transparent way, while retaining the core LTSS guarantees.

3. Avoiding Double Payment and Misaligned Incentives

Integration requires clear data-sharing and payment-coordination rules to prevent the federal government from paying twice (once through UCC and again through Medicaid) for the same acute event. CMS already administers complex payment-coordination

frameworks for dual-eligibles in Medicare and Medicaid; a similar, but simpler, protocol can be applied to UCC and Medicaid acute-care claims **(1,18)**. In practice, this means:

- UCC functions as the **primary** payer for qualifying acute claims above the catastrophic threshold.
- Medicaid remains the **primary** payer for LTSS, HCBS, and non-acute disability services.
- States receive explicit guidance on which services are federally reinsured catastrophic events and which remain within the traditional Medicaid rubric.

This division preserves the essential safety-net role of Medicaid while preventing it from being used as an open-ended acute-care financing mechanism for populations who are, under UCC/HSE, already protected against catastrophic risk.

X.F. Alignment of UCC With Medicare Coverage for Older Adults

Individuals aged 65 and older remain enrolled in Medicare, and UCC does not replace any Medicare component. Instead, UCC serves as a complement for the small subset of older adults encountering catastrophic events not fully covered under Medicare Parts A or B. Because catastrophic expenditures often exceed Medicare's deductibles and co-insurance, UCC provides a standardized backstop, reducing exposure to medical debt while avoiding duplication of Medicare benefits. This alignment preserves Medicare's statutory structure while ensuring catastrophic protections are uniform across the entire population.

X.G. State Innovation and Flexibility Options

UCC/HSE establishes a national catastrophic framework while preserving substantial room for state innovation. States may administer their own UCC-compliant catastrophic plans so long as they meet federal minimum standards for deductible levels, covered services, and reinsurance eligibility. States may also redirect resources from existing **1332 waivers, 1115 demonstrations**, or state-funded uncompensated-care pools toward enhanced HSA seeding, rural stabilization efforts, or local market reforms consistent with UCC/HSE.

States retain the authority to pursue broader deregulatory measures—such as certificate-of-need (CON) repeal, scope-of-practice expansion, telehealth liberalization, or outpatient-market competition reforms, without jeopardizing UCC/HSE compatibility. This federalism structure promotes policy experimentation while ensuring national uniformity in catastrophic protection.

XI. RURAL HEALTH STABILITY UNDER UCC/HSE

The UCC/HSE framework is built around a national catastrophic insurance standard, HSA-financed routine care, transparent routine-market pricing, and bounded federal support. That architecture remains intact in rural markets. At the same time, rural health-care systems operate under structural constraints that differ materially from urban and suburban provider markets. Rural hospitals face lower patient volume, higher fixed standby costs, thinner labor markets, longer transfer distances, fewer nearby substitutes, and greater dependence on Medicare payment structures. These conditions limit the ability of rural providers to absorb routine outpatient repricing in the same manner as higher-volume metropolitan systems.

UCC/HSE therefore incorporates a rural-stability framework that is provider-side, targeted, formula-based, and fiscally bounded. The framework does not exempt rural providers from all price discipline and does not create a separate rural insurance product. It preserves essential geographic access while allowing the broader reform to shift routine care toward transparent HSA-financed markets. Rural stabilization operates outside the consumer-facing benefit design. It does not alter the national catastrophic deductible, the universal catastrophic credit, HSA deposit formulas, supplemental-coverage guardrails, or the separation between catastrophic insurance and routine-care financing.

The rural policy in UCC/HSE is anchored in the FY2024 HCRIS provider-impact model. The model identifies rural / CAH / IHS-classified providers, evaluates their net outpatient exposure and modeled transition effects, and applies a preferred rural access-protection option based on net patient revenue. In the baseline rural result, rural / CAH / IHS providers show approximately **-\$0.341 billion** in net impact, equal to **-3.1 percent of net patient revenue**, with **10 rural providers below -5 percent of NPR**. Under the preferred rural access protection, a **permanent rural access-capacity payment equal to 2 percent of NPR, capped at \$5 million per qualifying rural provider**, paired with a **temporary implementation-period floor at -1.5 percent of NPR**, rural net impact improves to approximately **-\$0.123 billion**, or **-1.1 percent of NPR**, and modeled rural providers below -5 percent and -10 percent of NPR are eliminated.

This two-part structure defines the rural-health strategy in UCC/HSE. The first component is durable: a permanent rural access-capacity payment that recognizes structural geography, low volume, fixed standby capacity, workforce constraints, and the absence of nearby substitute providers. The second component is temporary: an implementation-period floor that prevents transition shock while rural providers adapt to the UCC/HSE routine-care market. The framework does not rely on a separate rural bailout, an uncapped

rural fund, or a broad reinsurance multiplier. It uses a bounded permanent access-capacity payment and a temporary transition floor, then reinforces rural access through chronic-care bundles, patient-side HSA access supplements, virtual specialty networks, transparency-linked regulatory flexibility, and time-limited transformation assistance.

XI.A. Rural Access Risk and the Need for Targeted Provider-Side Stabilization

Rural hospital vulnerability is primarily geographic and structural. Many rural facilities maintain emergency stabilization, inpatient beds, obstetric readiness, basic diagnostic capacity, and transfer relationships for populations too small to support the same revenue base available to urban systems. The fixed cost of standby capacity is therefore spread over fewer patients. A rural hospital may be economically fragile even when it is clinically necessary and reasonably managed.

This creates a distinct problem for health-financing reform. A model that relies entirely on consumer shopping and outpatient price discipline can function effectively in competitive metropolitan markets while creating access risk in low-density areas. Conversely, exempting rural providers from all price discipline weakens the structural logic of UCC/HSE and invites the same inflationary dynamics the reform is designed to correct.

The rural-stability framework resolves this tension by separating **access-capacity support** from **consumer-facing insurance design**. Rural providers remain within the same catastrophic and routine-care architecture as other providers. Routine and shoppable care remains subject to transparent HSA-financed purchasing. Catastrophic events remain insured. The rural difference is that qualifying low-volume access providers receive a permanent capped provider-side access-capacity payment, with temporary implementation-period floor protection layered on top during the transition.

This distinction is central. Rural stabilization is not a general subsidy for high charges, excess utilization, or opaque outpatient pricing. It is a limited access policy for providers whose geographic role cannot be replicated by ordinary market substitution.

XI.B. HCRIS-Based Rural Access Protection

The primary rural stabilization mechanism under UCC/HSE is the HCRIS-derived rural access guarantee and implementation floor. The model identifies qualifying rural / CAH / IHS providers and applies a provider-side access-capacity payment tied to net patient revenue.

The permanent rural access-capacity payment is:

$$\text{Rural Access Guarantee}_j = \min(0.02 \times \text{NPR}_j, \$5,000,000)$$

where NPR_j is net patient revenue for qualifying rural provider j .

This formula has three important features. First, it is proportional to provider size, preventing a one-size-fits-all grant from overpaying very small facilities or underpaying larger rural access points. Second, it is capped at \$5 million per provider, preventing rural-labeled systems from receiving uncapped federal support. Third, it is provider-side and does not alter household deductibles, HSA formulas, premiums, or covered benefits.

The rural access guarantee is the **permanent rural health stabilization factor** in the model. It addresses structural rural access constraints that persist beyond the implementation period: low volume, fixed standby costs, broad service areas, workforce limitations, and lack of nearby substitutes. It is not a temporary margin guarantee and does not compensate providers for ordinary business-model losses unrelated to rural access capacity. It is a durable access-capacity payment that recognizes that some rural providers must maintain essential emergency and inpatient readiness even when local patient volume cannot fully finance that standby capacity.

The implementation-period floor is separate from the permanent rural access guarantee. During the transition period, no qualifying rural provider falls below **-1.5 percent of net patient revenue** after application of the rural access guarantee. The floor functions as temporary or reviewable transition protection layered on top of the permanent access-capacity payment. It prevents access-threatening implementation losses while providers adapt to the UCC/HSE routine-care market.

Together, the permanent rural access-capacity payment and the temporary implementation-period floor produce the preferred HCRIS rural result: rural net impact improves from approximately **-\$0.341 billion** to approximately **-\$0.123 billion**, while modeled rural providers below -5 percent and -10 percent of NPR are eliminated. Rural access is protected without erasing the broader provider-side price-discipline effect of UCC/HSE.

XI.C. Preservation of Critical Access Hospital and Medicare Rural Payment Rules

UCC/HSE does not modify Medicare's cost-based reimbursement for Critical Access Hospitals, rural DRG add-ons, Sole Community Hospital rules, Medicare-dependent hospital protections, or other Medicare rural payment formulas. Medicare remains outside

the UCC/HSE replacement population, and existing Medicare rural payment structures remain intact.

This preservation rule is essential. Medicare is the financial backbone of many rural hospitals. A reform that appeared to reopen CAH reimbursement or destabilize Medicare rural payment formulas would generate opposition independent of the actual UCC/HSE design. UCC/HSE avoids that problem by preserving the Medicare rural payment architecture and applying UCC/HSE reforms to the non-Medicare catastrophic and routine-care market.

The permanent rural access-capacity payment complements Medicare rural protections rather than replacing them. Medicare continues to finance Medicare beneficiaries under existing law. UCC/HSE governs the non-Medicare replacement population. Provider-side rural stabilization addresses the incremental access risk created by routine-care repricing and market re-composition.

XI.D. Rural Condition-Based Chronic-Care Bundle Mechanism

Rural chronic-care delivery requires a defined payment mechanism rather than a general encouragement of value-based care. UCC/HSE permits qualifying rural providers to offer **Rural Condition-Based Chronic-Care Bundles** as HSA-purchasable service contracts.

Eligible providers include rural hospitals, Critical Access Hospitals, rural health clinics, federally qualified health centers, independent rural physician practices, and integrated rural provider networks. Eligible bundles cover predictable, recurring, non-catastrophic services for chronic conditions such as diabetes, COPD, congestive heart failure, hypertension with complications, stable coronary disease, chronic kidney disease monitoring, behavioral-health continuity, addiction-treatment follow-up, and oncology surveillance before or after acute treatment episodes.

A qualifying rural bundle has a defined annual or quarterly price and a defined service schedule. Covered services include routine labs, medication reconciliation, remote monitoring review, care-plan updates, primary-care follow-up, nurse care-management contacts, telehealth-supported specialist oversight, and patient education. High-severity complications, hospitalizations, acute decompensation, trauma, ICU care, or other catastrophic episodes remain inside the UCC catastrophic layer and are not converted into routine bundles.

Payment treatment remains simple. HSA funds, including chronic-condition supplements and rural household access supplements, are available for qualifying bundles. Bundles may be priced by condition and intensity level, but they do not function as insurance. They

do not cover open-ended claims, reduce the national catastrophic deductible, wrap around the catastrophic plan, or operate as comprehensive coverage.

Each bundle is subject to basic transparency and quality requirements. Providers disclose the bundle price, included services, excluded services, renewal terms, escalation rules, patient responsibilities, and referral triggers. Quality metrics include follow-up completion, avoidable emergency-department use, hospitalization for ambulatory-care-sensitive conditions, medication adherence where measurable, and patient access intervals.

The mechanism has two functions. For patients, rural bundles convert predictable chronic-care needs into transparent, HSA-financed service contracts. For rural providers, bundles create predictable outpatient revenue without relying on opaque facility fees or avoidable inpatient admissions. This supports rural access while preserving the UCC/HSE distinction between predictable routine care and insurable catastrophic risk.

XI.E. Rural Household Access Supplement

Provider-side stabilization protects essential rural institutions. It does not, by itself, solve the household-level access barriers that rural patients face. Rural households often bear higher travel time, fewer local specialist options, thinner outpatient availability, and higher indirect costs of obtaining routine care. UCC/HSE therefore includes a **Rural Household Access Supplement** delivered through HSAs.

The supplement is patient-side liquidity, not provider stabilization. Eligibility is based on objective geographic criteria, such as residence in a low-density county, frontier designation, medically underserved rural area, excessive travel-time threshold, or other statutory rural-access definition. The supplement may be structured per adult, per child, or per eligible household, with additional adjustment where chronic-condition or pediatric HSA deposits already apply.

Permitted uses are limited to access-related routine and chronic-care expenses. These include local routine labs, monitoring devices, telehealth-supported specialist consultations, primary-care visits, behavioral-health follow-up, chronic-care bundles, medication-management visits, and other HSE-eligible services. The supplement is not used to purchase insurance riders, reduce the catastrophic deductible, finance comprehensive supplemental coverage, or subsidize nonmedical consumption.

The Rural Household Access Supplement is coordinated with pediatric and chronic-condition deposits. A rural child retains pediatric routine-care support when the household also qualifies for a rural access supplement. A rural adult with a certified chronic condition may combine chronic-condition HSA support with rural access support, provided funds are

used for eligible services. This layered design addresses rural access barriers without converting the catastrophic plan into a broader insurance product.

XI.F. Rural Virtual Access Network

Telehealth is not treated merely as a convenience feature under UCC/HSE. In rural markets, it functions as a defined access mechanism that expands the effective supply of specialty, behavioral-health, and chronic-care services without requiring every low-volume community to maintain a full local specialist base. UCC/HSE establishes a **Rural Virtual Access Network** for qualifying rural providers and eligible rural households.

The Rural Virtual Access Network has three purposes. First, it allows rural hospitals, rural health clinics, Critical Access Hospitals, federally qualified health centers, and independent rural physician practices to contract with regional specialists, academic medical centers, behavioral-health providers, and chronic-care teams for scheduled telehealth-supported services. Second, it allows rural patients to use HSA funds, including Rural Household Access Supplement deposits, for qualifying virtual routine-care, chronic-care, and behavioral-health encounters. Third, it gives rural providers a defined role in the HSE routine-care market by allowing them to serve as the local originating site, care coordinator, laboratory access point, monitoring hub, or post-visit follow-up provider.

Covered telehealth-supported services include chronic-disease management, medication reconciliation, post-discharge follow-up, behavioral-health continuity, addiction-treatment follow-up, diabetes and COPD monitoring, congestive-heart-failure monitoring, oncology surveillance, specialist consultation, remote interpretation of diagnostic results, and care-plan review for patients whose condition does not require immediate in-person specialist examination. Emergency stabilization, acute inpatient care, trauma, obstetric emergencies, and other high-acuity services remain governed by the catastrophic-care layer and are not converted into telehealth substitutes.

Participation in the Rural Virtual Access Network is conditioned on basic operational standards. A participating rural provider maintains secure telehealth capacity, integrates virtual visits into the local patient record, provides local follow-up when clinically necessary, maintains referral and transfer protocols for high-acuity cases, discloses patient prices in advance for HSA-financed services, and reports basic utilization and access metrics.

Payment treatment remains consistent with UCC/HSE. HSA funds are available for qualifying routine and chronic telehealth services. Rural chronic-care bundles may include telehealth-supported specialist oversight as part of a fixed annual or quarterly price.

Provider-side stabilization may support one-time infrastructure, care-redesign, or network-integration costs, but not open-ended per-visit subsidies. Catastrophic reinsurance does not apply merely because a service is delivered virtually; it applies only when the underlying episode qualifies as catastrophic under the UCC standard.

The mechanism also protects rural providers from being bypassed entirely by distant virtual-only platforms. UCC/HSE permits telehealth competition, but rural stabilization and rural bundle treatment favor models that integrate local providers rather than drain the remaining viable routine-care volume from rural communities. A rural hospital or clinic may serve as the local diagnostic, laboratory, monitoring, or care-navigation site while a remote specialist supplies expertise. This preserves local access capacity, improves continuity, and strengthens rural participation in the routine-care market.

XI.G. Rural Transparency Compact

UCC/HSE connects rural stabilization to transparency and access obligations. Qualifying rural providers that receive access-capacity support participate in a **Rural Transparency Compact**.

The compact requires participating providers to:

1. Publish cash prices for routine HSE-eligible services;
2. Disclose bundle prices and included services;
3. Maintain basic cost reporting compatible with HCRIS or successor reporting systems;
4. Participate in rural access monitoring;
5. Maintain referral and transfer protocols for high-acuity cases;
6. Comply with no-surprise and no-balance-billing rules for UCC-covered catastrophic episodes;
7. Report use of rural stabilization payments; and
8. Demonstrate telehealth readiness or participate in a recognized virtual access network.

In exchange, participating rural providers receive targeted operational flexibilities. These include streamlined approval for outpatient service-line redesign, simplified participation in telehealth networks, technical support for transparent price posting, expedited

contracting templates for condition-based bundles, and limited regulatory relief where state or federal rules obstruct efficient rural access models.

These flexibilities are budget-neutral and conditioned on compliance. The purpose is not to shield rural providers from competition. It is to exchange transparency, access obligations, and cost reporting for operational flexibility needed to survive under a more price-disciplined routine-care environment. Rural providers that refuse transparency, use stabilization to preserve opaque pricing, or redirect support into unrelated expansion lose access to rural stabilization benefits.

XI.H. Transitional Stability and Rural Transformation Track

The permanent rural access-capacity payment and temporary implementation-period floor address near-term financial risk and long-term geographic access capacity. Some rural providers also require operational support to adapt their service lines to the UCC/HSE environment. UCC/HSE therefore includes a voluntary **Rural Transformation Track**.

Participation is available to qualifying rural providers receiving stabilization support or identified as transition-sensitive in the provider-impact model. A participating facility submits a time-limited transformation plan describing how it will adapt to HSE routine-care markets, maintain essential access, improve price transparency, integrate telehealth, redesign outpatient services, develop chronic-care bundles, and manage low-volume inpatient capacity.

Eligible uses of transformation assistance include:

1. Financial and operational redesign;
2. HCRIS-compatible reporting improvements;
3. Transparent price-posting infrastructure;
4. Development of rural chronic-care bundles;
5. Telehealth network integration;
6. Care-management redesign;
7. Diagnostic and laboratory access coordination;
8. Workforce cross-training; and
9. Service-line planning for low-volume inpatient units.

Prohibited uses are explicit. Transformation assistance is not used to reduce patient deductibles, finance insurance riders, subsidize commercial rate increases, support unrelated capital expansion, fund executive compensation, or preserve opaque facility-fee billing. Assistance is time-limited and subject to milestones.

The transformation track is paired with the implementation-period floor. During the transition period, qualifying rural providers are protected from access-threatening modeled losses, but that protection is not a permanent guarantee of existing business models. The permanent rural access-capacity payment supports durable geographic access needs; the temporary floor gives rural providers time to adapt; transformation assistance helps them use that time productively.

XI.I. Closure Review and Rural Access Monitoring

During the implementation period, rural and frontier providers receiving stabilization support are subject to access-monitoring procedures. The objective is not federal control over every rural hospital closure. The objective is to prevent UCC/HSE implementation from creating avoidable access deserts in communities where no reasonable substitute exists.

A closure or service-line reduction attributed to UCC/HSE implementation triggers state and federal access review when the facility receives rural stabilization support, serves a low-density area, is the only acute-care facility within a defined travel-time radius, or provides essential emergency stabilization, obstetric, inpatient, or diagnostic capacity.

The review considers:

1. Whether the provider is geographically essential;
2. Whether closure risk is driven by low volume rather than avoidable inefficiency;
3. Whether alternative access points exist within clinically reasonable travel times;
4. Whether telehealth, transfer protocols, or service-line redesign preserve access;
5. Whether rural access support or transformation assistance was used appropriately;
and
6. Whether state or federal policy changes are needed to prevent an access gap.

This review is tied to objective rural-access criteria and participation in stabilization support. It is not an open-ended federal guarantee against all rural restructuring. Some service-line consolidation remains appropriate where access can be preserved more efficiently. The purpose is to distinguish necessary transformation from access-threatening collapse.

XI.J. Interaction With Section XVII Provider-Impact Analysis

Section XVII reports the main provider-impact results and is read together with this rural-health section. The rural section describes the policy architecture; **Section XVII** quantifies the provider-impact effect.

The HCRIS model establishes the baseline provider-impact result as a diagnostic reference case, not the final policy design. The final model includes bounded stabilization tiers, pediatric treatment, rural access protection, behavioral-health institutional adjustment, and severe transition-risk review. Rural access protection is therefore part of the stabilized provider-transition model.

Full rural sensitivity results are reported in **Supplementary Section 2**. Supplementary Table 7K presents the enhanced rural access sensitivity analysis, and **Supplementary Table 7L** presents the provider-level rural access output. **Appendix B** provides the reproducible HCRIS methodology and script mapping underlying these estimates. In the preferred rural specification, the permanent rural access-capacity payment equals 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider, and is paired with a temporary implementation-period floor of -1.5 percent of net patient revenue. This specification improves modeled rural provider impact from approximately **-\$0.341 billion** to approximately **-\$0.123 billion** and eliminates modeled severe rural tail risk below both the -5 percent and -10 percent net-patient-revenue thresholds.

XI.K. Preserving UCC/HSE's Core Economic Logic

Rural stabilization under UCC/HSE is not a retreat from catastrophic-first reform. It recognizes that geographic access capacity cannot always be financed through ordinary outpatient volume. The rural framework preserves the core UCC/HSE structure while addressing a specific access problem.

None of the rural-support mechanisms alter:

1. The single national catastrophic deductible;
2. The universal catastrophic credit;
3. Federal HSA deposit formulas;
4. The HSA-centered approach to routine care;
5. The guardrails on supplemental coverage;
6. Medicare's existing rural payment rules; or

7. The federal reinsurance architecture.

The rural access guarantee is the permanent rural health stabilization factor. It provides durable, capped access-capacity support for qualifying rural providers whose structural constraints persist beyond the transition period. The implementation-period floor is temporary protection against transition shock. The Rural Household Access Supplement is patient-side routine-care liquidity. Chronic-care bundles convert predictable rural care needs into transparent HSA-purchasable service contracts. The Rural Virtual Access Network expands access without turning telehealth into a new insurance entitlement. The Rural Transparency Compact exchanges support and flexibility for price disclosure, access obligations, and reporting discipline. The Rural Transformation Track provides temporary operational assistance, not permanent subsidy.

Under the preferred HCRIS-based rural option, rural providers are not exempted from all repricing, but access-threatening modeled losses are eliminated. This is the rural-health equilibrium for UCC/HSE: preserve catastrophic protection, discipline routine-care prices, maintain Medicare rural payment stability, establish a permanent rural access-capacity payment for structural rural access needs, and use temporary implementation protection only to manage transition shock.

XII. IMPLEMENTATION PATHWAY

Transforming the U.S. health-financing structure requires a carefully phased strategy that minimizes disruption, preserves consumer protections, and ensures states and insurers can transition smoothly. The UCC/HSE model is deliberately designed so that its components can be introduced sequentially, allowing households and employers to adjust behavior gradually.

XII.A (Addendum). HSA Administrative Infrastructure

Treasury will administer HSAs through a national custodial platform, similar in structure to the federal Thrift Savings Plan. Each citizen/eligible individual automatically receives an account at birth or naturalization, with optional transfer to private custodians at any time. Federal HSA deposits are delivered through standardized ACH operations, and account statements are integrated into annual IRS filings. This centralized administrative backbone minimizes compliance costs, prevents fragmentation, and supports seamless nationwide portability.

XII. A. Year 1: Legislative Authorization and Infrastructure Creation

Congress passes the **Universal Catastrophic Coverage and Health Savings Empowerment Act**, which:

1. Establishes UCC as a federally regulated catastrophic-insurance category
2. Creates the national reinsurance pool
3. Creates the universal refundable catastrophic credit
4. Authorizes automatic HSA issuance through Treasury
5. Sets the phaseout schedule for ACA subsidies
6. Allows states to consolidate existing 1332 waivers into the national model

In Year 1, CMS and Treasury develop:

- Standardized catastrophic-plan templates
- Actuarial guidance
- HSA distribution and verification systems
- Reinsurance disbursement protocols

Private insurers prepare to offer catastrophic-only plans in Year 2.

XII.B. Year 2: UCC Plans Become Available

Catastrophic plans are offered on simplified federal and private platforms. ACA exchanges remain operational **for one final enrollment season**, but new enrollment shifts toward UCC plans.

States convert existing 1332 waivers into UCC-compatible reinsurance structures.

Employers begin shifting from premium-based benefits to:

- HSA contributions
- Supplemental benefits
- Cash-based wage adjustments

Medicaid expansion enrollees are notified of their transition to citizen/eligible individual catastrophic plans.

XII.C. Year 3: ACA Subsidies Phase Out

Premium tax credits and cost-sharing reductions are replaced by:

- The universal catastrophic refundable credit
- Automatic HSA deposits

ACA exchanges cease operations. Their administrative functions are retired or absorbed by CMS.

Medicaid expansion adults formally transition to UCC + HSE. States continue to manage ABD (Aged, Blind, Disabled) categories.

XII.D. Years 4–5: Employer Transition and Continued Exclusion Phase-Down

The employer tax exclusion is phased out over ten years and replaced by a universal credit that applies equally to all citizens/eligible individuals. This eliminates coverage distortion based on employment status and allows a fully portable, individually owned insurance structure.

At the end of Year 5:

- The ACA's market architecture is fully dissolved
- UCC/HSE is the national system of catastrophic protection and routine-care financing
- Federal spending is stabilized at a predictable, actuarially bounded level

XII.E. Federal Health Programs Unaffected by UCC/HSE

UCC/HSE does not alter, replace, or supersede existing federal health programs operating under separate statutory authority. **TRICARE**, **Veterans Health Administration (VA) health services**, and the **Federal Employees Health Benefits (FEHB) Program** remain intact with their current eligibility and benefit structures. Any optional future alignment, such as pilot programs incorporating catastrophic overlays or HSA-based enhancements, may occur only through separate statutory authorization or agency-led demonstration projects. This separation preserves continuity for military families, veterans, and federal employees, and prevents cross-program disruption during UCC/HSE implementation.

XII.F. Program Integrity and Anti-Fraud Enforcement

Given the introduction of a universal catastrophic credit and federally seeded HSAs, UCC/HSE incorporates a robust program integrity framework. Eligibility determinations rely on cross-matching data from **Treasury (IRS)**, **Social Security Administration**, and **Department of Homeland Security** to prevent duplication, ineligible enrollment, and

improper payments. HSA custodians must comply with federal reporting standards and undergo periodic **GAO** and **HHS Office of Inspector General (OIG)** audits.

Federal reinsurance payouts are subject to audit verification and standardized reporting to prevent misclassification or inappropriate billing behavior. Civil and criminal penalties apply to fraudulent claims, improper HSA spending, or deliberate misrepresentation by insurers, providers, or households. This structure ensures program integrity while minimizing administrative burden.

Byrd Compliant Language:

“The Secretary of the Treasury shall not disburse federal catastrophic credits or federal HSA deposits unless eligibility has been verified through IRS taxpayer records and, where appropriate, DHS lawful-presence databases.”

“Treasury shall recoup improper payments made under this title in accordance with section 6402 of the Internal Revenue Code.”

“As a condition of receiving federal reinsurance payments, insurers must submit encounter data sufficient for verification of allowed claims. Failure to provide such data shall bar further payments until resolved.”

XII.G. No Changes to Medicare Programs: Restated

Nothing in UCC/HSE alters Medicare eligibility, benefit design, Medigap policies, Medicare Advantage contracts, or any Part A, B, D, or C administrative structures. All provisions apply solely to pre-Medicare populations. Medicare remains unchanged and operates under existing law.

XII.H. State Innovation and Pilot Authority

To encourage bipartisan experimentation and allow states to tailor implementation to local needs, UCC/HSE authorizes a structured innovation lane aligned with existing 1332 and 1115 waiver frameworks. States may redirect waiver-aligned federal dollars toward enhanced HSA seeding, rural-provider stabilization, maternal-health pilots, behavioral-health integration initiatives, or telehealth modernization programs, provided they maintain compliance with national catastrophic-plan standards. These pilots allow states to test delivery-system reforms that complement the catastrophic-plus-savings architecture without expanding federal benefit mandates or compromising actuarial discipline.

XII.I. Continuity of Coverage for Children and Transitional Medicaid Populations

To ensure that no child experiences a disruption in medical coverage during the transition to UCC/HSE, all children enrolled in Medicaid or the Children's Health Insurance Program (CHIP) at the time of enactment shall receive continuous eligibility for a period of twelve months following their next scheduled redetermination. States may elect to extend continuous eligibility to twenty-four months using existing administrative funding streams.

Children who transition from CHIP or Medicaid into UCC/HSE remain eligible for pediatric HSA allocations established under Section IX.J, and catastrophic protection applies without interruption. States shall coordinate verification and enrollment processes to ensure that a child's UCC/HSE catastrophic enrollment begins no later than thirty days prior to the end of their Medicaid or CHIP continuous-eligibility period.

For parents in transitional Medicaid eligibility groups, including postpartum coverage extensions and income-fluctuation households, states shall apply a unified redetermination process such that UCC/HSE enrollment is automatically triggered when income thresholds are crossed, preventing any lapse in catastrophic protection. Nothing in this subsection modifies state authority over Medicaid eligibility rules or benefits for LTSS, ABD, or medically complex children.

This continuity framework ensures that children and transitional Medicaid populations remain fully protected under either Medicaid/CHIP or UCC/HSE during the implementation period, without duplicative enrollment or gaps in coverage.

XIII. RISKS AND LIMITATIONS

Structural reforms of this magnitude inevitably confront uncertainty. While the UCC/HSE architecture corrects many of the incentive misalignments embedded in comprehensive insurance models, several residual risks remain. These risks do not undermine the feasibility of the framework but identify areas where ongoing calibration, empirical monitoring, and policy discipline are essential.

XIII.A. Behavioral Adjustment in the Transition to Cash-Priced Routine Care

The shift from comprehensive first-dollar insurance to cash-priced routine care represents a significant behavioral change for households accustomed to opaque cost structures. Some degree of initial underutilization of routine services is possible as consumers adjust to transparent pricing and HSA-based payments. Evidence from direct primary care practices, ambulatory surgery centers, and cash-based imaging markets shows that

consumers adapt relatively quickly once liquidity barriers are removed and prices become predictable.

Early-year HSA funding, standardized cash-price disclosure, and phased implementation are designed to reduce potential friction. Nevertheless, transitional behavioral uncertainty remains an inherent feature of moving to a more price-responsive system.

XIII.B. Persistent Price Pressures in Consolidated Provider Markets

In regions dominated by highly consolidated hospital systems, cash prices for certain services may remain elevated despite the removal of routine care from insurance. Federal reinsurance will continue to face higher catastrophic claims in these markets, and price competition for outpatient services may emerge more slowly than in markets with a more diversified provider base.

Competition policy, DOJ/FTC enforcement, the unwinding of anti-competitive contracting practices, and state-level removal of entry barriers such as certificate-of-need laws, remains an essential complement to UCC/HSE. The reform strengthens the environment in which competition can occur but cannot independently dismantle entrenched provider monopolies.

XIII.C. Institutional Resistance and Long-Horizon Policy Drift

Programs built around comprehensive benefit design have created durable constituencies, large carriers, hospital systems, and administrative vendors, who benefit materially from the ACA-era architecture. Structural reform will encounter organized resistance from these actors. Moreover, long-run political pressures may encourage efforts to reintroduce comprehensive-style benefits through supplemental riders, mandated coverage expansions, or erosion of deductible integrity.

The UCC/HSE framework includes statutory guardrails against such re-expansion: uniform catastrophic benefit design, restrictions on subsidized supplemental coverage, and fiscal caps that constrain federal exposure. Nonetheless, policy drift remains an inherent risk whenever long-standing institutional arrangements are disrupted.

XIII.D. Adequacy of Chronic-Condition Funding Mechanisms

Chronic-condition households are especially sensitive to misalignment between statutory HSA deposits and real-world disease-management expenditures. If deposits fall below median clinically necessary outlays, liquidity pressure could emerge even though catastrophic protection remains intact.

The framework incorporates multiple stabilizers:

- expenditure-triggered chronic-condition funding pathways,
- statutory minimum HSA deposits indexed to median disease-management costs derived from CMS CCW, MEPS, and FAIR Health datasets, and
- periodic recalibration requirements tied to claims-level analysis.

These mechanisms substantially reduce the likelihood of systematic underfunding. However, variability across clinical profiles and regional practice patterns necessitates ongoing monitoring to maintain alignment between statutory funding rules and observed disease-management costs.

XIII.E. Macroeconomic Shocks and Countercyclical Funding Needs

During economic downturns, household HSA contributions and employer deposits may weaken precisely when routine health-care needs rise. Without explicit stabilizers, the cash-pay environment could impose liquidity strain on some households despite stable catastrophic coverage.

Automatic adjustments to federal HSA deposits, tied to unemployment rates or wage-income indicators, and predictable reinsurance parameters mitigate pro-cyclical stress. Yet, in the event of a severe or prolonged recession, additional temporary federal support may still be warranted. No catastrophic-first framework can fully neutralize the impact of a major macroeconomic shock.

XIII.F. Empirical Uncertainty and Implementation Complexity

Implementation of the UCC/HSE model will require sustained empirical evaluation in several domains:

- **Reinsurance calibration:** Adjustment of attachment points and cost-sharing parameters to maintain predictable federal exposure across diverse hospital-price environments.
- **Chronic-condition benchmarking:** Continuous updating of chronic-condition funding thresholds and pediatric allocations as clinical practice patterns and service prices evolve.
- **Consumer response:** Observation of HSA utilization patterns, cash-price trajectories, and substitution effects during the market transition.

- **Administrative performance:** Assessment of eligibility verification, cross-agency data integration, and program-integrity safeguards as the system scales.

These uncertainties are characteristic of any large-scale replacement of a legacy insurance architecture. The UCC/HSE framework incorporates automatic adjustment mechanisms and reporting requirements precisely so that empirical feedback can be incorporated without compromising its core economic logic.

XIII.G. Supplemental Benefit Creep and Insurance Gaming

Even with statutory guardrails limiting subsidized coverage to a uniform catastrophic design, pressure to reassemble de facto comprehensive benefits remains a structural vulnerability. Insurers, large employers, and supplemental-plan vendors may attempt to introduce riders that, while nominally “supplemental,” functionally reconstruct first-dollar coverage through mechanisms such as fixed indemnity add-ons, non-deductible outpatient bundles, or narrowly designed cost-sharing offsets.

Such benefit creep would reintroduce the core inflationary pressures that UCC/HSE seeks to eliminate: opaque pricing, moral hazard in routine consumption, and escalating actuarial-value expectations. It would also risk shifting federal catastrophic credits toward an implicit subsidy of quasi-comprehensive arrangements, undermining both the fiscal logic and market-discipline objectives of the framework.

Mitigation depends on strict enforcement of statutory definitions distinguishing catastrophic coverage from supplemental products, clear disclosure requirements, and regular auditing of plan design by federal oversight agencies. These measures reduce, but do not eliminate, the long-run risk of supplemental-market drift toward comprehensive coverage.

XIII.H. Rural Provider Revenue Compression and Cross-Subsidy Decline

Rural hospitals often rely on cross-subsidization from elevated outpatient commercial prices to maintain solvency despite low patient volumes and high fixed costs. By shifting a substantial portion of routine care into transparent cash markets and reducing the role of insurance in outpatient pricing, UCC/HSE may gradually compress these implicit revenue transfers. In markets where alternative outpatient providers enter quickly, revenue reallocation may accelerate the need for operational restructuring among rural facilities.

The framework includes a dedicated Rural Hospital Stability Pool and access guarantees designed to offset foreseeable revenue declines, particularly during the transition period. Nevertheless, the long-run financial position of rural hospitals will depend on local

demographics, cost structures, and the pace at which outpatient volume migrates toward lower-cost competitors. Transparent cash markets promote efficiency but also expose existing inefficiencies that cross-subsidies previously concealed.

XIII.I. Political and Administrative Transition Risk

Replacing a decade-old subsidy and regulatory architecture entails operational, political, and administrative uncertainty. Federal and state agencies must coordinate major changes in eligibility determination, HSA distribution, reinsurance administration, and data-sharing processes. Transition misalignment, whether in IRS reporting, CMS eligibility systems, or state-level enrollment infrastructure, could generate temporary friction or public confusion.

Politically, any transition away from ACA exchanges carries narrative risk. Stakeholders accustomed to the current architecture may frame administrative changes as disruptions even when coverage continuity is fully protected. Large carriers, state-based exchange administrators, and existing vendors may resist the new structure, creating potential for litigation or regulatory delay.

The UCC/HSE transition schedule, continuity guarantees, and multi-year glide path substantially mitigate these concerns, but they cannot fully eliminate them. Successful implementation will require coordinated federal guidance, stable statutory interpretation, and active oversight during the first several enrollment cycles. Administrative transition risk is inherent in any structural reform and must be acknowledged as such.

This dynamic does not undermine the economic rationale for UCC/HSE, but it represents a genuine implementation risk that requires continuous monitoring, targeted stabilization mechanisms, and state-level engagement to ensure rural access is preserved without reintroducing comprehensive-price inflation through indirect subsidies.

XIV. Cross-Ideological Foundations and Areas of Convergence

Although U.S. health-care debates are often framed as zero-sum conflicts between competing political traditions, the structural elements of UCC/HSE rest on concepts with unusually broad ideological lineage. The framework does not attempt to reconcile philosophical differences; rather, it aligns with several major currents of American political thought because it respects each tradition's core assumptions about the proper roles of households, markets, and government.

XIV. A. Social Democratic and Progressive Perspectives

Progressive health-policy frameworks typically emphasize universal protection, redistribution toward low-income households, and insulation from medical bankruptcy. The UCC/HSE structure meets these objectives through:

- 1. Universal catastrophic protection**

Catastrophic coverage applies to all citizens/eligible individuals under uniform terms, eliminating discontinuities between Medicaid expansion, exchange plans, and employer coverage. Universalism is achieved without means-testing or administrative gateways.

- 2. Targeted support for vulnerable groups**

Higher HSA seeding for low-income and chronically ill citizens/eligible individuals channels federal resources toward populations that face the steepest routine-care burdens.

- 3. Structural constraints on cost escalation**

Transparent routine-care markets and the removal of comprehensive benefit inflation create downward pressure on provider pricing, an objective shared by progressive critics of hospital and insurer concentration.

XIV.B. Center-Left and Technocratic Perspectives

Centrists within the Democratic coalition typically prioritize fiscal sustainability, continuity of protection, and administrative coherence. The UCC/HSE model aligns with this perspective insofar as it:

- 1. Replaces an open-ended subsidy mechanism**

Federal obligations are no longer indexed to fluctuating premiums but instead tied to actuarially predictable catastrophic-claim distributions.

- 2. Preserves regulatory guardrails**

Solvency standards, licensure, and federal catastrophic-benefit definitions remain intact, ensuring continuity of oversight while reducing administrative complexity.

- 3. Simplifies enrollment architecture**

Automatic catastrophic enrollment and standardized plan structures reduce the administrative overhead associated with ACA exchange determinations.

XIV.C. Market-Oriented Conservative Perspectives

Conservative frameworks prioritize market competition, consumer responsibility, and the reduction of federal fiscal commitments. UCC/HSE advances these ends by:

- 1. Restoring routine-care price signals**

The shift to direct payment for routine services imposes discipline on provider pricing and reduces opportunities for cross-subsidization.

- 2. Expanding consumer agency**

Portable HSAs allow households, not employers or insurers, to determine routine-care purchasing, enhancing mobility and reducing labor-market distortions.

- 3. Limiting the federal role to catastrophic risk**

Federal reinsurance and standardized catastrophic benefits address the segment of health risk most appropriately managed at national scale, without extending federal authority into routine service markets.

XIV.D. Populist and Economic-Nationalist Perspectives

Populist approaches emphasize the interests of working households, skepticism of concentrated corporate power, and citizen-oriented benefit design. UCC/HSE aligns in several respects:

- 1. Increased wage transparency**

Replacement of the employer tax exclusion with a portable credit shifts compensation toward wages and HSAs rather than opaque benefit structures.

- 2. Reduction of insurer and hospital market power**

Disintermediation of routine care weakens dominant actors whose pricing strategies rely on insurer reimbursement systems rather than market competition.

- 3. Citizen-based eligibility**

Catastrophic protection is linked to citizenship, consistent with frameworks emphasizing national community over administrative universality.

XIV. E. Libertarian and Classical-Liberal Perspectives

This tradition values decentralization, minimal state involvement in routine economic decisions, and strong consumer choice. UCC/HSE reflects these themes:

- 1. Minimization of third-party payment**

Routine services are removed from the insurance framework, reducing administrative layers and allowing suppliers to compete on posted prices.

2. **Neutral tax treatment of individual decision-making**

HSA become universally available without the current regulatory constraints tied to plan type.

3. **Narrow, clearly defined federal function**

Catastrophic reinsurance and standardized catastrophic benefits are the sole federal commitments; routine care remains governed by decentralized market dynamics.

XV. Legal and Constitutional Considerations

The UCC/HSE framework fits squarely within Congress’s constitutional authority under both the Commerce Clause and the Taxing and Spending Clause. Catastrophic insurance is a national market, and federal reinsurance constitutes a lawful spending program analogous to Medicare Part D risk-sharing arrangements. Courts have repeatedly upheld federal authority to regulate health-insurance markets that substantially affect interstate commerce **(44)**.

Replacing the employer tax exclusion with a universal credit is likewise well-established under Congress’s taxing authority; the Supreme Court has consistently affirmed broad discretion in structuring federal tax expenditures and credits. HSAs administered via Treasury raise no constitutional issues beyond routine tax administration.

States retain traditional police-power functions, including licensure, solvency monitoring, and consumer-protection enforcement. Their authority is not pre-empted, because UCC sets minimum federal catastrophic standards while permitting additional state oversight. Supreme Court precedent on ERISA pre-emption and state insurance regulation supports the distinction between federally defined plan standards and preserved state insurance-regulatory functions **(45)**.

No federalism or Tenth Amendment conflicts arise because participation is universal and benefits are federally funded. States remain free to supplement routine-care assistance at their discretion, maintaining sovereign flexibility.

Byrd Compliant language:

“As a condition of receiving federal catastrophic credit payments, federal HSA deposits, or federal reinsurance transfers under this title, no participating insurer or provider shall be required as a condition of such participation to furnish, arrange, or subsidize any service for which federal funds are prohibited under this title.”

XV.A. Conscience Protections and Non-Compulsion Standards

UCC/HSE operates within the existing federal framework of conscience protections, including the Church Amendments, the Weldon Amendment, and related statutory provisions. The program does not compel any physician, hospital, insurer, or employer to provide, pay for, or facilitate medical services that conflict with religious or moral convictions. UCC/HSE's catastrophic benefit and HSA structures remain strictly neutral with respect to such services, and program participation cannot be conditioned on the provision or subsidization of services beyond the federal minimum standard. These protections ensure constitutional durability and reduce litigation risk under the First Amendment and federal conscience statutes.

XVI. Comparative Cost Projections of Medicare for All vs ACA vs UCC + HSE

A substantial body of economic literature has attempted to estimate the fiscal and systemic effects of Medicare-for-All-style reforms. While these analyses vary widely in methodology and assumptions, they converge on a central conclusion: the proposal would require a large reallocation of health-care financing from private and state sources to the federal government, with total national health expenditures highly sensitive to key structural parameters.

The Urban Institute provides one of the most widely cited baseline estimates. In its 2019 analysis of comprehensive single-payer reform, Urban projects that federal health-care spending would increase by approximately **\$34 trillion over a ten-year period**, driven primarily by the elimination of private insurance premiums, expansion of covered benefits, and increased utilization resulting from reduced cost-sharing **(83)**. The model assumes only partial reductions in provider payment rates relative to current private insurance levels, which contributes to its finding that total national health expenditures would increase modestly despite administrative simplification.

Similarly, the RAND Corporation emphasizes that cost outcomes under Medicare for All are highly dependent on provider reimbursement assumptions and benefit design. RAND's modeling framework incorporates variations in payment rates, utilization effects, and the inclusion of long-term services and supports, demonstrating that total national health spending may either increase or decrease depending on these inputs **(84)**. In particular, scenarios that maintain provider payments above Medicare levels tend to produce net spending increases, while more aggressive payment reductions generate potential savings.

The Mercatus Center analysis by Blahous provides a detailed estimate of the federal budgetary impact of a Sanders-style Medicare for All proposal. The study projects

approximately **\$32.6 trillion in additional federal expenditures over ten years (85)**.

Importantly, this estimate reflects a shift in financing rather than a direct measure of new health-care consumption, as it incorporates the replacement of private insurance and out-of-pocket spending with federal outlays. The analysis assumes substantial administrative savings and lower provider payment rates, yet still concludes that federal fiscal exposure would increase significantly.

In contrast, earlier work by Gerald Friedman presents a more optimistic projection of single-payer financing. Friedman's analysis of the Sanders proposal estimated total costs at approximately **\$13.8 trillion over ten years**, with projected savings from reduced administrative overhead, negotiated pharmaceutical pricing, and global budgeting sufficient to offset new federal spending **(86)**. This estimate has been widely debated in subsequent literature, with critics arguing that it relies on aggressive assumptions regarding the magnitude and speed of achievable system-wide cost reductions.

Further supportive analysis from the Political Economy Research Institute reinforces the view that single-payer systems can generate substantial savings under favorable conditions. PERI's work argues that administrative efficiencies and centralized price negotiation could significantly reduce overall expenditures, while also acknowledging that these outcomes depend on effective implementation and sustained policy discipline **(87)**.

Synthesizing these findings, the Committee for a Responsible Federal Budget reports that most credible estimates of Medicare for All fall within a broad range of approximately **\$30 trillion to \$40 trillion** in additional federal spending over a ten-year horizon **(88)**. This range reflects variation in assumptions regarding provider reimbursement, utilization increases, benefit expansion, and the treatment of long-term care services. The organization emphasizes that, regardless of the precise estimate, the proposal entails a historically large expansion of federal fiscal responsibility.

Finally, the Kaiser Family Foundation highlights that differences across proposals and estimates are driven less by modeling technique than by underlying policy design choices. Key variables include whether cost-sharing is eliminated, the scope of covered benefits, the degree of provider payment reduction, and the integration of existing public programs such as Medicaid and long-term care **(89)**. These factors ultimately determine whether a Medicare-for-All system produces net savings or increased total spending.

Taken together, the literature demonstrates that while Medicare for All may simplify financing and expand coverage, its fiscal and economic outcomes are not predetermined. Instead, they depend critically on policy choices regarding pricing, utilization, and benefit structure. This variability contrasts with the UCC/HSE framework, which constrains federal exposure through explicit catastrophic thresholds and bounded subsidy mechanisms rather than relying on system-wide cost compression assumptions; **Table 4** places those estimates beside the ACA baseline and the UCC/HSE scoring model to show the structural difference between federalizing comprehensive care and limiting federal exposure to catastrophic credits, HSA deposits, and reinsurance. The key distinction is structural rather than merely numerical. Medicare for All largely federalizes the financing of comprehensive care, and its fiscal outcome depends heavily on aggressive assumptions about provider-payment compression and administrative savings. UCC/HSE instead limits federal exposure to catastrophic protection, targeted HSA support, and reinsurance, allowing routine care to move into price-transparent markets rather than expanding first-dollar public financing.

Federal Budgetary Effects of Major Health Reform Frameworks

(Billions of dollars, nominal)

Framework	Source	10-Year (2026–2035)	20-Year (2036–2045)	30-Year (2046–2055)	Metric Definition
Current Law (ACA baseline)	CBO (current law projections)	13,300	—	—	Federal subsidies (non-Medicare population)
Medicare for All (Comprehensive Single-Payer)	Urban Institute [70]	34,000	—	—	Additional federal spending
	RAND Corporation [71]	32,000–34,000	—	—	Total/federal spending (scenario-dependent)
	Mercatus Center (Blahous) [72]	32,600	—	—	Additional federal spending
	Friedman [73]	13,800	—	—	Total federal cost (high-savings scenario)
	PERI (Pollin et al.) [74]	~30,000	—	—	Total national spending (net savings possible)
	CRFB [75]	25,000–35,000	—	—	Additional federal financing (range)
	KFF [76]	—	—	—	No single estimate (design comparison)
UCC + HSE (Universal Catastrophic Coverage + HSE)	Author model (CBO-style scoring workbook)	763	–7,463	–32,934	Net federal impact
		(11,570 outlays / 10,807 savings)	—	—	Gross outlays and savings (10-year)

- Notes: 1. Estimates for Medicare for All reflect published analyses of comprehensive single-payer proposals and are not directly comparable due to differences in benefit design, provider payment assumptions, and treatment of long-term care.
2. “Additional federal spending” refers to increases relative to current law, not total national health expenditures.
3. UCC + HSE estimates are derived from the accompanying scoring workbook and reflect a structural reallocation of health spending, with federal exposure limited to catastrophic coverage, targeted HSA support, and reinsurance.
4. Negative values indicate net federal savings relative to current-law baseline projections.
5. All figures are presented in nominal dollars and reflect model-specific assumptions regarding utilization, pricing, and population eligibility.

Table 4: Existing Medicare-for-All estimates generally imply **\$25 trillion to \$35 trillion in additional federal financing over ten years**, with leading estimates from Urban Institute and Mercatus at roughly **\$34.0 trillion** and **\$32.6 trillion**, respectively. By contrast, the UCC/HSE scoring model used in this paper projects roughly **\$11.57 trillion in gross federal outlays** over 2026–2035 and a **net federal impact of approximately +\$763.1 billion** over the same window, followed by substantial long-run savings.

XVII. Provider Impact Analysis with Numeric Tables

The provider-sector effect of UCC/HSE is best understood as **revenue re-composition with targeted access stabilization**, not as an across-the-board provider payment reduction. The framework does not impose a single administered reimbursement schedule, does not adopt a Medicare-for-All-style rate conversion, and does not rely on direct statutory cuts to all hospital or physician payments. Its effect arises from a change in financing channels. Catastrophic and high-acuity care remains insured, pooled, and federally stabilized. Routine, shoppable, and lower-acuity outpatient care is moved toward HSA-financed, transparent payment markets. The resulting provider impact is therefore not a uniform reduction in revenue, but a reallocation of revenue pressure away from opaque, high-markup routine billing and toward a system in which catastrophic care remains insured, routine care becomes more price-disciplined, and vulnerable access providers receive targeted stabilization.

This distinction is central to the provider-impact analysis. UCC/HSE preserves insured payment for high-severity medical events: inpatient hospitalization, trauma, ICU care, major surgery, transplant-related hospitalization, high-acuity oncology admissions, catastrophic disease episodes, and true emergency care. At the same time, it applies price discipline to routine office visits, standard imaging, laboratory testing, chronic-care monitoring, low-complexity outpatient procedures, routine specialist follow-up, and other predictable services that are currently routed through comprehensive insurance. The provider-sector effect is therefore a structural shift in the site, price, and channel of payment for routine care, not a withdrawal of catastrophic medical protection.

The system-level provider-facing flows are shown in **Table 5**. These flows establish the financing environment within which provider-segment effects occur. They include federal catastrophic credits, federal HSA deposits, federal reinsurance, household catastrophic premium payments, routine-care spending before and after HSE price discipline, uncompensated-care baseline exposure, avoided emergency-department overutilization, bad-debt reduction, and charity-care tax effects.

Table 5: System-Level Provider-Facing Financial Flows Under UCC/HSE

Metric	2026	2030	2035	10-Year Total	Meaning
Federal catastrophic credits	486.4	582.9	730.9	6,012.9	Federal catastrophic financing stream supporting high-cost claims
Federal HSA deposits	446.5	516.1	618.3	5,283.1	Federal routine-care purchasing support moving below-deductible spending into HSE
Federal reinsurance	21.4	25.7	32.2	264.9	Direct federal support to stabilize catastrophic claims above attachment thresholds
Total federal transfers to households	932.8	1,099.0	1,349.2	11,296.0	Combined catastrophic credits plus HSA deposits
Household catastrophic premium payments	304.6	365.1	457.8	3,766.2	Household-financed share of catastrophic premium volume
Baseline routine spend	598.7	695.0	837.5	7,121.8	Routine-care spending before HSE price-discipline adjustment
HSE routine spend	588.6	683.4	823.4	7,002.2	Routine-care spending after HSE assumptions
Routine savings	10.1	11.7	14.1	119.6	Modeled routine-care savings from HSE
Uncompensated care baseline	53.0	66.9	89.5	698.6	Provider-facing uncompensated-care burden before modeled reductions
Avoided ER overutilization	2.0	2.5	3.4	26.4	Modeled savings from reduced avoidable ED use
Bad debt reduction effect	6.0	7.5	10.1	78.6	Modeled decline in provider bad debt
Charity-care tax offset	2.0	2.5	3.4	26.4	Indirect federal offset associated with reduced charity-care burden

Table 5 shows that UCC/HSE does not remove financing from the health-care system in a simple linear fashion. In 2026, the model includes \$486.4 billion in federal catastrophic credits, \$446.5 billion in federal HSA deposits, \$21.4 billion in federal reinsurance, and \$304.6 billion in household catastrophic premium payments. These flows preserve a large insured and federally supported payment channel for high-cost claims while shifting routine-care purchasing into HSA-financed markets. Over the ten-year period, the table reports \$6.013 trillion in federal catastrophic credits, \$5.283 trillion in federal HSA

deposits, \$264.9 billion in federal reinsurance, and \$3.766 trillion in household catastrophic premium payments.

At the routine-care level, **Table 5** reports baseline routine spending of \$598.7 billion in 2026 and HSE routine spending of \$588.6 billion, producing modeled routine savings of \$10.1 billion in the first year and \$119.6 billion over the ten-year period. The table also reports a provider-facing uncompensated-care baseline of \$53.0 billion in 2026, rising to \$89.5 billion by 2035, with a ten-year baseline of \$698.6 billion. Avoided emergency-department overutilization, bad-debt reduction, and charity-care tax effects are modeled as separate channels because they operate differently from routine-care repricing. Avoided emergency-department overutilization reduces pressure on non-emergency hospital use. Bad-debt reduction improves provider collections. Charity-care tax effects operate indirectly through the reduced charity-care burden.

These national flows are necessary but not sufficient to evaluate provider impact. They describe the size and direction of the financing transition, but they do not by themselves allocate gains and losses across provider classes, service lines, rural status, institutional vulnerability, or site of care. That allocation requires a provider-segment framework and a provider-level scoring model.

The provider impact allocation framework is shown in **Table 6**. This table identifies the principal provider categories affected by UCC/HSE, the channel through which each category is affected, the calculation structure needed to quantify the effect, and the data inputs required for full scoring.

Table 6 separates provider effects by service type and institutional role. Hospital inpatient and catastrophic-care providers are evaluated through catastrophic case volume and reimbursement corridor assumptions. Hospital outpatient departments are evaluated through outpatient revenue, the routine/shoppable share of services, migration assumptions, and price-reset factors. Independent clinics, ambulatory surgery centers, imaging centers, laboratories, and direct primary care practices are evaluated through routine-care migration and capture share. Physicians and specialists are evaluated through routine specialist revenue, billing simplification, and administrative overhead reduction. Emergency departments are evaluated through uncompensated-care relief, avoided emergency-department use, and bad-debt reduction. Rural and critical access hospitals are evaluated through catastrophic stabilization, rural access support, uncompensated-care relief, and outpatient repricing loss. Safety-net hospitals are evaluated through payer mix, uncompensated-care exposure, bad-debt relief, and stabilization eligibility.

Table 6: Provider Impact Allocation Framework Under UCC/HSE

Provider segment	Impact channel	Calculation structure	Data inputs needed	Output
Hospital inpatient / catastrophic	Catastrophic reimbursement preserved or compressed	catastrophic case volume × UCC catastrophic reimbursement corridor	CMS cost reports, inpatient service mix, corridor assumptions	catastrophic hospital revenue under UCC
Hospital outpatient departments	Routine revenue compression	outpatient revenue × routine/shoppable share × migration share × price reset factor	HCRIS/AHA revenue mix + HCCI/FAIR site-of-service ratios	outpatient revenue at risk
Independent clinics / ASCs	Revenue gain from migration	routine spend shifted × migration share × capture share	workbook routine spend + service-mix / site-of-service data	independent-provider gain
Physicians / specialists	Billing/admin simplification	routine specialist revenue shifted out of insurer billing × removable admin overhead share	physician revenue mix + admin cost estimates	admin savings
Hospitals	Bad debt reduction	system bad debt reduction × hospital allocation share	workbook + HCRIS/AHA	hospital bad debt relief
Hospitals / EDs	Uncompensated care and ED pressure relief	uncompensated-care relief + avoided ER use × hospital/ED share	workbook + AHA/CMS operational data	provider relief
Rural / CAH hospitals	Net rural effect	catastrophic stabilization gain + rural support + uncompensated-care relief – outpatient repricing loss	rural/CAH identifiers + hospital baseline file	rural net impact
Employer market	Compensation shift	employer health-spend reduction × wage pass-through + HSA substitution	workbook + employer segmentation assumptions	wage/benefit transition effect

The framework also clarifies the boundary between directly modeled and directionally expected effects. The HCRIS model developed for this paper directly estimates hospital and institutional provider exposure. It does not fully quantify the offsetting revenue gains that may accrue to independent clinics, ambulatory surgery centers, imaging centers, laboratories, direct primary care practices, and independent physician groups as routine care migrates out of hospital outpatient departments. Those gains are directionally expected but require additional claims-level and site-of-service data to quantify precisely.

The provider-impact model uses FY2024 CMS HCRIS hospital cost-report data. The model begins with a selected provider backbone of 912 provider records, of which 886 have usable financial and exposure data for scoring. Twenty-six providers are excluded because required financial or exposure variables are missing, unusable, or fail validation. Provider-level exposure is measured as **net outpatient exposure**, not total hospital revenue. Net outpatient exposure is derived from gross outpatient revenue, gross total patient revenue,

and net patient revenue, and is intended to estimate the portion of routine and outpatient revenue plausibly exposed to UCC/HSE price discipline.

This exposure construction is important. The model does not apply a blanket haircut to all hospital revenue. It does not assume that inpatient, catastrophic, trauma, ICU, transplant, or major high-acuity service revenue is repriced through consumer shopping. Instead, it isolates outpatient and routine-care exposure and applies scenario-based repricing assumptions to that exposure base. This approach is consistent with the core design of UCC/HSE: catastrophic risk remains insured, while routine and shoppable care becomes more transparent and price-disciplined.

The model also corrects provider classification before scoring. Raw HCRIS labels are not sufficient for policy analysis because some providers appearing in rural or CAH-adjacent administrative categories may function analytically as large general hospitals, academic medical centers, children's hospitals, psychiatric institutions, or specialty providers. The model therefore uses cleaned analytic provider groups and classes, including general acute/academic, children's/pediatric, rural/CAH/IHS, and specialty/behavioral/rehab providers. This correction prevents stabilization policy from being directed by raw administrative labels alone.

The baseline HCRIS model is not the final provider policy advanced by this paper. It is the diagnostic reference case used to identify where routine/outpatient repricing pressure appears before the full stabilization package is applied. Under the base/base reference case, the model estimates approximately \$13.52 billion in outpatient repricing pressure. That pressure is partially offset by approximately \$1.91 billion in uncompensated-care and liquidity relief and approximately \$0.30 billion in bounded stabilization support already included in the base case. The resulting net provider impact before the full access-protection package is approximately -\$11.30 billion, equal to approximately -4.3 percent of modeled net patient revenue.

The final provider policy advanced by UCC/HSE is the **stabilized provider-transition model**. That model preserves routine-care price discipline while applying targeted protections for access-sensitive and transition-sensitive institutions. Stabilization is not universal. It is structured through bounded stabilization tiers, pediatric treatment, rural access protection, behavioral-health institutional adjustment, and enhanced review for severe transition-risk providers.

Table 7: Stabilized Provider-Impact Model: Before and After Targeted Access

Table 7. Stabilized Provider-Impact Model: Before and After Targeted Access Protection

Model component / provider group	Baseline exposure before full access protection	Stabilization or access-protection treatment	Result after targeted stabilization	Policy interpretation
All modeled institutional providers	-\$11.300B net impact; -4.3% of modeled NPR	Existing bounded stabilization plus preferred targeted rural and behavioral access package	-\$11.081B net impact; -4.2% of modeled NPR	Final stabilized provider-impact model preserves routine-care price discipline while reducing access-risk tail exposure
Modeled provider universe	912 HCRIS backbone providers; 886 modeled providers; 26 excluded for missing, unusable, or failed validation data	Provider-level HCRIS scoring using cleaned provider classes and net outpatient exposure	886 modeled providers retained in central scoring	Model is hospital/institutional-provider focused; non-hospital gains are directional unless separately modeled
Stabilization-tier architecture	589 Tier 0 providers not eligible for stabilization; 323 providers eligible for bounded stabilization	Tier 1–4 support based on protected class and uncompensated-care burden, not universal hold-harmless	Tier 1: 2; Tier 2: 266; Tier 3: 40; Tier 4: 15	Stabilization is targeted and bounded; most providers remain outside the stabilization tiers
Children's / pediatric providers	83 backbone providers; 73 modeled providers; approximately -\$0.235B net impact; -3.8% of NPR before full supplementary stabilization sensitivities	Pediatric classification, pediatric revenue-fallback logic, stabilization tiering, and pediatric hold-harmless sensitivity	Pediatric stabilization outputs reported in Supplementary Section 2	Pediatric institutions are explicitly modeled as access-sensitive providers, not treated as ordinary general acute hospitals
Rural / CAH / IHS providers	Approximately -\$0.341B net impact; -3.1% of NPR; 10 providers below -5% NPR	Preferred rural access guarantee: 2% of NPR, capped at \$5M per rural provider, plus implementation floor at -1.5% NPR	Approximately -\$0.123B net impact; -1.1% of NPR; modeled rural providers below -5% and -10% eliminated	Rural access risk is materially reduced without changing the uniform catastrophic benefit
Psychiatric / behavioral hospitals	Approximately -\$0.063B net impact; 24 providers below -5% NPR; 11 providers below -10% NPR	Behavioral institutional treatment: 25% of standard repricing pressure plus implementation floor at -5% NPR	Approximately -\$0.015B net impact; providers below -10% NPR eliminated	Corrects institutional behavioral exposure without creating an open-ended behavioral-health subsidy
Tier 3 / Tier 4 severe transition-risk providers	55 providers identified as high or critical transition-support cases	Enhanced Tier 3 / Tier 4 sensitivity and provider-level review in Supplementary Section 2	Severe-risk stabilization assessed outside the main-body table	Highest-risk tail is reviewed separately without expanding support to all providers
Preferred access-protection package	Baseline institutional-provider impact of -\$11.300B	Approximately +\$0.219B in targeted rural and behavioral support	Final stabilized impact of approximately -\$11.081B	Small targeted support materially improves rural and behavioral tail risk while preserving aggregate provider-side price discipline

Notes: NPR = net patient revenue. CAH = Critical Access Hospital. IHS = Indian Health Service-like or rural/IHS-classified provider group within the cleaned HCRIS classification. Dollar amounts are static first-order provider-impact estimates from the FY2024 HCRIS provider-impact model. The baseline HCRIS result is a diagnostic reference case; the stabilized model is the policy design advanced in the paper. Full scenario matrices, provider-group outputs, provider-class outputs, pediatric hold-harmless sensitivity, rural access sensitivity, behavioral-health sensitivity, exposure-anomaly audits, and provider-level review tables are reported in Supplementary Section 2. The model's Tier 1–4 support is based on protected provider class and uncompensated-care burden; the scripts explicitly define the final stabilization tier using baseline class stabilization and safety-net uncompensated-care burden rather than diagnostic margin stress alone.

Protection

Table 7 reports the main before-and-after provider-impact result. Before the full access-protection package, the base HCRIS model estimates approximately -\$11.30 billion in net provider impact, or -4.3 percent of modeled net patient revenue. After the preferred access-protection package is applied, the final stabilized model estimates approximately -\$11.08 billion in net provider impact, or approximately -4.2 percent of modeled net patient revenue. The incremental targeted support is approximately \$0.219 billion. The aggregate provider-side price-discipline effect is therefore largely preserved, while targeted support reduces access-threatening tail risk.

The stabilization-tier architecture is an important part of this result. The model identifies 589 providers as Tier 0, meaning not eligible for stabilization. Two providers are Tier 1, reflecting monitoring or low support. Two hundred sixty-six providers are Tier 2, reflecting moderate transition support. Forty providers are Tier 3, reflecting high transition support. Fifteen providers are Tier 4, reflecting critical transition support. In total, 323 providers receive some level of bounded stabilization eligibility, while most providers remain outside the stabilization tiers. This structure is designed to protect access-sensitive providers without converting UCC/HSE into a broad provider hold-harmless program.

Children's and pediatric providers are explicitly included in the model. The HCRIS pipeline includes pediatric classification, pediatric revenue-failure diagnostics, children's fallback exposure assumptions, provider-group results for children's/pediatric hospitals, and pediatric hold-harmless sensitivity analysis. In the baseline provider-group result, children's and pediatric providers show approximately -\$235 million in net impact, equal to about -3.8 percent of net patient revenue, before full supplementary stabilization sensitivities. Pediatric institutions are concentrated primarily in Tier 2, with one provider in Tier 3. This reflects the access-sensitive nature of pediatric institutional capacity without exempting pediatric providers wholesale from the UCC/HSE financing structure.

Rural and CAH/IHS-like providers receive specific access protection because their economics differ from those of urban hospital systems. Rural hospitals face low patient volume, high fixed standby costs, workforce scarcity, long travel distances, thin local markets, and limited substitute capacity. Before added rural access protection, the rural/CAH/IHS group shows approximately -\$341 million in net impact, or approximately -3.1 percent of net patient revenue, with ten rural providers below the -5 percent threshold. Under the preferred rural access model, qualifying rural providers receive a permanent rural access guarantee equal to 2 percent of net patient revenue, capped at \$5 million per provider, plus an implementation-period floor ensuring that no qualifying rural provider falls

below -1.5 percent of net patient revenue during the transition. After this protection, rural net impact improves to approximately -\$123 million, or approximately -1.1 percent of net patient revenue, and modeled rural providers below -5 percent and -10 percent of net patient revenue are eliminated.

Behavioral and psychiatric institutions receive a separate exposure correction. Institutional psychiatric care does not function like ordinary shoppable outpatient care. Many psychiatric hospitals, state hospitals, forensic facilities, inpatient behavioral institutions, addiction-treatment providers, and specialized behavioral-health facilities operate under access obligations, staffing constraints, payer-mix limitations, regulatory requirements, and substitute-capacity limits that differ from routine office-based care. Before behavioral adjustment, psychiatric and behavioral hospitals show a relatively small aggregate net impact of approximately -\$63 million, but the provider-level tail risk is severe: 24 providers fall below -5 percent of net patient revenue and 11 fall below -10 percent. Under the preferred behavioral treatment, qualifying behavioral providers are assigned 25 percent of standard modeled repricing pressure and receive an implementation-period floor ensuring that no qualifying behavioral provider falls below -5 percent of net patient revenue. After this treatment, behavioral net impact improves to approximately -\$15 million, and modeled behavioral providers below -10 percent of net patient revenue are eliminated.

Tier 3 and Tier 4 providers represent the high-risk tail of the transition model. These providers are identified for enhanced review because of severe exposure, access sensitivity, uncompensated-care burden, pediatric or rural status, behavioral-health institutional role, or other transition-risk characteristics. The tiered structure is essential. A model with no stabilization would be operationally vulnerable. A model that stabilizes all providers would erase the intended price-discipline mechanism. The tiered approach creates a middle course: most providers absorb ordinary routine-care repricing, while access-sensitive and severe transition-risk providers receive targeted, bounded protection.

The provider-segment effects under the stabilized model are summarized in **Table 8**.

Table 8. Provider-Segment Effects Under Stabilized UCC/HSE

Provider segment	Primary impact channel	Expected direction under UCC/HSE	Directly quantified in HCRIS model?	Stabilization / policy treatment	Main-body interpretation
Inpatient and catastrophic-care hospitals	Catastrophic reimbursement, high-acuity payment, reinsurance-backed claims	Generally preserved payment channel for true catastrophic and high-acuity episodes	Partly; HCRIS captures institutional revenue, but not a full service-line catastrophic case model	Catastrophic coverage remains insured; federal reinsurance stabilizes high-cost claims	Main risk is not loss of insured catastrophic payment, but reduced ability to cross-subsidize from high-margin outpatient departments
Hospital outpatient departments	Routine outpatient revenue compression; facility-fee and commercial-rate exposure	Most exposed hospital segment	Yes, through net outpatient exposure	No broad hold-harmless; ordinary routine-care price discipline applies	Principal intended site of provider-side price pressure, especially for imaging, labs, low-complexity procedures, routine specialist services, and other shoppable services
General acute / academic hospitals	Large outpatient exposure, commercial routine-care revenue, facility-based billing	Largest absolute dollar exposure	Yes	Tiered stabilization only if provider meets class, uncompensated-care, pediatric, rural, behavioral, or severe-risk criteria	Largest modeled dollar impact because these systems dominate NPR and outpatient exposure; not categorically exempt from repricing
Independent clinics, ASCs, imaging centers, laboratories, urgent-care clinics, and DPC practices	Routine-care migration from hospital-owned settings into transparent HSA-funded markets	Likely relative beneficiaries	Not directly in HCRIS hospital model	No provider-side stabilization assumed; market gain expected through transparent routine-care purchasing	Gains are directionally expected but require claims-level site-of-service and non-hospital provider data for full quantification
Physicians and specialists	Mix of catastrophic episode payment, routine specialist repricing, administrative simplification	Mixed	Not fully	High-acuity specialists remain tied to insured catastrophic episodes; routine specialist services face price transparency	Independent specialists may benefit from direct-pay arrangements; hospital-affiliated or facility-fee-dependent specialists may face pricing pressure
Emergency departments	Avoidable ED use, uncompensated-care relief, bad-debt relief, true emergency coverage	Mixed	Partly	True emergencies remain insured; lower avoidable ED use and lower bad debt expected where routine-care liquidity improves	Net effect depends on payer mix, avoidable ED volume, uncompensated-care burden, and hospital reliance on ED throughput
Rural / CAH / IHS providers	Low-volume fixed costs, outpatient repricing exposure, uncompensated-care relief, rural access need	Stabilized	Yes	Preferred rural access guarantee: 2% of NPR capped at \$5M per provider plus -1.5% NPR implementation floor	Rural severe-tail risk is eliminated in the model while preserving the uniform catastrophic benefit
Behavioral and psychiatric institutions	Institutional behavioral repricing exposure, fragile margins, limited substitutes	Stabilized through exposure correction	Yes for psychiatric / behavioral hospitals in HCRIS institutional model	25% of standard modeled repricing pressure plus -5% NPR implementation floor	Behavioral adjustment is an exposure correction for institutional psychiatric providers, not a general behavioral-health subsidy
Children's / pediatric hospitals	Pediatric institutional capacity, specialized service lines, nonstandard reporting, routine/outpatient exposure	Access-sensitive and explicitly modeled	Yes	Pediatric classification, pediatric fallback logic, stabilization tiering, and pediatric hold-harmless sensitivity	Pediatric providers are included in stabilization architecture; pediatric access risk is not deferred or ignored
Safety-net hospitals	Uncompensated-care relief, bad-debt reduction, payer-mix transition, outpatient repricing exposure	Mixed	Partly, through S-10 uncompensated-care burden and stabilization eligibility	UC/liquidity offset plus safety-net stabilization triggers where uncompensated-care burden is high	Safety-net hospitals may benefit from reduced uncompensated care, but can still face outpatient repricing pressure
Tier 3 / Tier 4 severe transition-risk providers	High or critical transition-support status from protected class and/or uncompensated-care burden	Stabilized / reviewed	Yes	Enhanced Tier 3 / Tier 4 sensitivity and provider-level review in Supplementary Section 2	Severe-risk tail is handled through bounded support rather than systemwide provider hold-harmless
Employer-market interface	Employer premium reduction, wage pass-through, HSA substitution	Indirect provider effect	Not a provider-level HCRIS output	Employer-sector analysis reported separately	Relevant to system financing, but not a direct hospital/provider payment category

Notes: ASC = ambulatory surgery center. DPC = direct primary care. HCRIS directly supports hospital and institutional-provider scoring; independent outpatient providers, ASCs, laboratories, imaging centers, DPC practices, and physician-practice revenue capture require additional claims-level and site-of-service modeling. The older provider-impact framework already identified the segment-specific channels for inpatient/catastrophic hospitals, hospital outpatient departments, independent clinics/ASCs, physicians, emergency departments, rural/CAH hospitals, and safety-net hospitals; this updated table preserves that structure while adding the HCRIS-stabilization treatment for pediatric, rural, behavioral, safety-net, and severe transition-risk providers.

Table 8 is the main-body guide to who is affected, through what channel, and with what policy treatment. It distinguishes directly quantified HCRIS hospital and institutional effects from directional effects on non-hospital outpatient providers. The segment-level interpretation is as follows.

Inpatient and catastrophic-care hospitals. Hospitals providing trauma care, ICU services, major surgery, transplant-related hospitalization, oncology hospitalization, high-acuity emergency care, and other catastrophic services should experience relative payment stability compared with routine outpatient departments. Those services remain inside the insured catastrophic layer. UCC/HSE does not move major inpatient events into cash-pay markets or HSAs. Catastrophic coverage remains the core insurance function, and federal reinsurance stabilizes the high-cost claim tail.

The main risk for inpatient and catastrophic-care hospitals is not the loss of insured payment for catastrophic care. It is the loss of cross-subsidy from high-margin outpatient departments. Under the current system, large hospital systems often use outpatient facility fees, commercial markups, and insurer-negotiated rates on routine services to support broader institutional revenue. UCC/HSE reduces the ability to rely on that outpatient margin. Hospitals whose inpatient or catastrophic service lines are financially dependent on routine outpatient overpayment may therefore face margin pressure even if high-acuity care remains insured.

Hospital outpatient departments. Hospital outpatient departments are the most directly exposed provider segment under UCC/HSE. Many routine services are currently billed through hospital-owned outpatient departments at prices substantially above independent-site equivalents. Imaging, laboratory testing, low-complexity procedures, routine specialist visits, infusion services, outpatient diagnostics, and facility-fee-associated services are often billed through insurer contracts at rates that reflect hospital market power rather than production cost.

UCC/HSE intentionally moves routine and semi-routine services into transparent HSA-funded markets. That creates revenue-compression risk for hospital outpatient departments, especially where services can be safely provided in lower-cost settings. This is not an accidental side effect of the model; it is one of the central economic mechanisms of the reform. The HCRIS model captures this pressure through net outpatient exposure and applies repricing assumptions to that exposure base.

General acute and academic hospitals. General acute and academic providers carry the largest absolute dollar exposure in the modeled hospital sector. In the base/base reference case, this group accounts for approximately -\$10.57 billion of the -\$11.30 billion total

before the full access-protection package, equal to about -4.4 percent of net patient revenue. This result is expected because general acute and academic systems dominate net patient revenue and outpatient exposure in the HCRIS model.

This is where much of the routine-care price discipline is intended to fall. Large hospital systems and academic medical centers are not categorically exempted from repricing pressure simply because they provide important high-acuity services. Their catastrophic and high-acuity services remain insured, but their routine and outpatient revenue channels face the transparency and HSA-funded purchasing pressure that defines UCC/HSE. Stabilization applies only where providers meet defined access-risk, pediatric, rural, uncompensated-care, safety-net, behavioral, or severe transition-risk criteria.

Independent clinics, ambulatory surgery centers, imaging centers, laboratories, urgent-care clinics, and direct primary care practices. These providers are likely relative beneficiaries of UCC/HSE. They are better positioned to compete in transparent HSA-funded markets because they can post prices, reduce administrative friction, offer predictable cash-pay bundles, and compete on convenience, access, and value. Routine-care migration away from hospital outpatient departments is likely to create opportunities for lower-cost independent providers.

This segment is not fully quantified in the HCRIS provider-impact model. HCRIS directly supports hospital and institutional provider analysis, not a complete market-wide allocation of routine-care gains to independent outpatient providers. Therefore, hospital-sector revenue compression should not be misread as a net loss to the entire provider economy. Some revenue compressed in hospital outpatient departments is likely to migrate toward lower-cost independent outpatient sites. Future modeling should integrate claims-level site-of-service data, ambulatory surgery center utilization, independent imaging and laboratory data, direct primary care data, and physician-practice revenue data to estimate those gains.

Physicians and specialists. Physician effects are mixed. Specialists involved in catastrophic episodes, major surgery, trauma care, oncology hospitalization, transplant care, ICU care, and other high-acuity episodes remain attached to insured payment channels. Their revenue is not expected to collapse simply because routine care is moved out of comprehensive insurance. Catastrophic and high-acuity care remains the proper domain of insurance.

Routine specialist visits, follow-up visits, chronic monitoring, outpatient diagnostics, and lower-acuity procedures are more exposed to price transparency. Physicians and specialists who currently benefit from hospital-owned facility billing or opaque insurer-

negotiated reimbursement may face pricing pressure. By contrast, independent specialists who can offer transparent prices, bundled episodes, cash-pay follow-up, or direct contracts with patients and employers may gain volume. The physician-sector effect is therefore likely to vary by specialty, site of service, ownership structure, and dependence on hospital outpatient billing.

Emergency departments. Emergency departments face offsetting effects. Avoidable emergency-department utilization should decline as routine-care liquidity improves through HSA prefunding, direct primary care access, chronic-condition support, pediatric deposits, and transparent alternatives to non-emergency care. At the same time, true emergency and catastrophic events remain insured. UCC/HSE does not require patients to cash-pay for trauma, acute inpatient emergencies, ICU-level care, or other catastrophic episodes.

The financial effect on emergency departments therefore depends on payer mix, uncompensated-care burden, avoidable ED volume, and hospital reliance on emergency-department throughput. Hospitals with high avoidable ED volume may see lower non-emergency utilization. Those same hospitals may also experience lower bad debt and uncompensated care if patients have catastrophic protection and routine-care liquidity. The net result is therefore mixed rather than uniformly negative. The present model captures part of this through uncompensated-care and liquidity offsets, but it does not yet isolate emergency-department margins as a separate service-line model.

Rural and critical access hospitals. Rural and critical access hospitals are stabilized in the final model. Before added rural protection, rural/CAH/IHS providers show approximately -\$341 million in net impact, or approximately -3.1 percent of net patient revenue. After the preferred rural access guarantee and implementation floor, rural impact improves to approximately -\$123 million, or approximately -1.1 percent of net patient revenue, and modeled severe rural tail risk is eliminated.

This is not an exemption from UCC/HSE. Rural providers remain within the catastrophic/HSA framework. The rural access guarantee is provider-side, capped, and access-based. It preserves rural access without altering the uniform catastrophic benefit, without lowering the national deductible, and without recreating comprehensive first-dollar insurance.

Behavioral and psychiatric institutions. Behavioral and psychiatric institutions are stabilized through a distinct exposure correction. Institutional psychiatric care is not equivalent to ordinary routine shopping. The preferred model therefore assigns qualifying behavioral providers 25 percent of standard modeled repricing pressure and applies a -5

percent net-patient-revenue implementation floor. Before this treatment, behavioral institutions show approximately -\$63 million in net impact, with 11 providers below -10 percent of net patient revenue. After the preferred treatment, behavioral impact improves to approximately -\$15 million, and modeled behavioral providers below -10 percent are eliminated.

This adjustment is not a general behavioral-health subsidy. It is a repricing-exposure correction for institutional psychiatric providers whose services are not ordinary consumer-directed outpatient purchases. UCC/HSE can still support transparent outpatient behavioral care, tele-behavioral care, direct-pay therapy, and HSA-funded mental-health services. Institutional psychiatric providers require separate treatment because their access role and economics differ from ordinary routine outpatient care.

Children's and pediatric hospitals. Children's and pediatric hospitals are explicitly modeled and access-sensitive. In the baseline provider-group result, children's/pediatric providers show approximately -\$235 million in net impact, or approximately -3.8 percent of net patient revenue. Pediatric providers are also incorporated into the stabilization-tier model, with most pediatric providers concentrated in Tier 2 and one in Tier 3. The model includes children's revenue-failure diagnostics and pediatric fallback exposure assumptions, reflecting the fact that pediatric institutions often report differently and cannot simply be treated as ordinary general acute hospitals.

Pediatric hospitals serve specialized populations, medically complex children, neonatal intensive-care needs, pediatric oncology, congenital disease, rare disease, and children requiring coordinated specialty care. UCC/HSE preserves Medicaid responsibilities for medically complex children and CHIP continuity, but pediatric hospitals may still experience routine/outpatient repricing pressure. The appropriate policy treatment is targeted pediatric stabilization and hold-harmless sensitivity where needed, not a blanket exemption from the UCC/HSE framework and not neglect of pediatric access risk.

Safety-net hospitals. Safety-net hospitals have mixed exposure under UCC/HSE. They may benefit from lower uncompensated care, lower bad debt, and more reliable catastrophic coverage. At the same time, many safety-net hospitals operate with fragile margins, high Medicaid or low-income patient shares, and limited commercial revenue cushion. Their exposure depends heavily on local payer mix, uncompensated-care burden, Medicaid ABD/LTSS preservation, state Medicaid coordination, and the transition design for non-disabled adults.

The HCRIS model captures part of this issue through the uncompensated-care and liquidity offset and through stabilization eligibility. In the base/base reference case, the

model includes approximately \$1.91 billion in UC/liquidity offset, reflecting provider benefit from reduced uncompensated-care exposure. Safety-net providers with high uncompensated-care burden should benefit more from this offset than providers with little uncompensated-care exposure. However, safety-net hospitals should not be treated as automatically protected simply because uncompensated care falls. If a safety-net hospital also depends heavily on outpatient facility-fee revenue, it may still face routine-care repricing pressure. Conversely, if its primary problem is uncompensated catastrophic exposure, UCC/HSE may improve its position. The net effect is provider-specific.

Tier 3 and Tier 4 severe transition-risk providers. Tier 3 and Tier 4 providers represent the high-risk tail of the transition model. These providers are identified for enhanced review because of severe exposure, access sensitivity, uncompensated-care burden, pediatric or rural status, behavioral-health institutional role, or other transition-risk characteristics. The purpose of enhanced Tier 3/Tier 4 sensitivity analysis is to test whether severe transition-risk providers can be protected without undoing the broader outpatient price-discipline mechanism.

This tiered structure is essential to the credibility of the provider-impact framework. A model with no stabilization would be operationally vulnerable. A model that stabilizes every provider would erase the intended price-discipline mechanism. The tiered architecture creates a middle path: most providers absorb ordinary routine-care repricing, while pediatric, rural, safety-net, behavioral, and severe transition-risk providers receive targeted, bounded protection.

Full supporting outputs are reported in Supplementary Section 2. That supplementary section includes the full HCRIS scenario matrices, provider-group results, provider-class results, stabilization-tier outputs, pediatric hold-harmless sensitivity, rural access sensitivity, behavioral-health sensitivity, exposure-anomaly audits, and provider-level review tables. Appendix B provides the reproducible HCRIS methodology and script mapping.

The audit results support the reliability of the model. The provider-impact pipeline begins with 912 backbone providers and includes 886 modeled providers, producing a modeled share of approximately 97.1 percent. The model tracks standard G-2 split records, rural fallback records, children's fallback records, nonstandard fallback records, total-only records, missing/unusable records, failed-plausibility records, net-to-gross outliers, and exposure-anomaly records. Exposure-anomaly sensitivity shows that excluding flagged exposure records does not materially change the central result: the base/base result

remains approximately -4.3 percent of net patient revenue. This indicates that the core provider-impact result is not driven by a small number of anomalous HCRIS records.

The limitations of the model should also be clear. This is a static, first-order provider-impact analysis. It does not predict hospital closures, mergers, capital-market reactions, bargaining responses, service-line redesign, provider entry or exit, changes in local market concentration, or long-run dynamic adaptation. It does not fully quantify non-hospital provider gains from routine-care migration. It does not yet estimate independent ASC, imaging, laboratory, DPC, or physician-practice revenue capture using claims-level site-of-service data. It should therefore be read as a hospital and institutional provider-impact model, supplemented by directional provider-segment analysis.

Those limitations do not undermine the usefulness of the model. A serious provider-impact analysis should distinguish what is directly modeled from what is directionally expected. The HCRIS model directly estimates hospital and institutional provider exposure. The segment analysis explains how the broader provider market is likely to adjust. Future extensions should integrate AHA hospital financial data, MedPAC margin and payment-adequacy estimates, HCCI and FAIR Health site-of-service price ratios, HCUP emergency-department data, MACPAC/AHA uncompensated-care and bad-debt estimates, physician-practice revenue data, ASC utilization data, independent imaging data, laboratory claims data, and local market concentration measures.

The interpretation is straightforward. UCC/HSE produces real provider-side revenue pressure, but the pressure is targeted. The main exposure falls on routine and outpatient revenue channels, especially hospital outpatient departments and general acute or academic systems with significant outpatient exposure. Catastrophic and high-acuity care remains insured and federally stabilized. Rural access risk is addressed through a capped rural access guarantee and implementation floor. Institutional behavioral-health exposure is corrected through a reduced repricing factor and implementation floor. Children's and pediatric providers are explicitly included in the stabilization architecture. Safety-net providers are supported through uncompensated-care relief and stabilization eligibility. Tier 3 and Tier 4 providers are identified for enhanced transition-risk review. Independent outpatient providers, ASCs, imaging centers, laboratories, direct primary care practices, and efficient physician groups are likely relative beneficiaries of routine-care migration, although those gains are not fully quantified in the HCRIS hospital model.

The final policy conclusion is not that UCC/HSE is provider-neutral. It is not. The conclusion is that UCC/HSE creates **controlled provider-sector re-composition**. The baseline HCRIS reference model estimates approximately -\$11.30 billion in net provider

impact before the full access-protection package. The final stabilized model improves that estimate to approximately -\$11.08 billion while eliminating modeled severe rural tail risk and eliminating modeled behavioral providers below -10 percent of net patient revenue. The incremental targeted support required to achieve that stabilization is approximately \$0.219 billion.

That is the provider-sector bargain embedded in UCC/HSE. Routine and shoppable care becomes more price-disciplined. Catastrophic and high-acuity care remains insured. Rural access is stabilized. Institutional behavioral-health exposure is corrected. Children's and pediatric providers are explicitly protected within the stabilization framework. Safety-net and severe-risk providers are handled through bounded tiered support. Lower-cost independent outpatient providers gain room to compete. The federal fiscal structure remains bounded. The result is not provider austerity and not provider bailout, but a transition from opaque comprehensive-insurance revenue channels toward catastrophic protection, transparent routine-care markets, and targeted access preservation.

XVIII. Employer market sector analysis

The employer market under UCC/HSE is not abolished; it is structurally recomposed. Under current law, employer-sponsored insurance functions simultaneously as a coverage mechanism, a tax shelter, and a compensation vehicle. CBO and JCT identify employment-based coverage as one of the four major federal health-insurance subsidy channels, with employment-based coverage subsidies projected at approximately **\$5.3 trillion over 2024–2033, equal to roughly 21 percent of total federal health-insurance subsidies over that period (3)**. CBO further notes that employment-based coverage remains the predominant source of health insurance coverage in the United States. This makes the employer sector central to any serious ACA replacement framework: the model must specify not only how federal subsidies change, but also how employer-paid premium compensation is redirected.

UCC/HSE replaces the tax preference for comprehensive employer-sponsored insurance with a universal catastrophic credit and portable HSA-based routine-care financing. This does not imply that employer health spending disappears. Rather, employer premium commitments are redirected into three channels: taxable wages, employer HSA contributions, and narrowly defined supplemental benefits that cannot recreate first-dollar comprehensive coverage. This is consistent with the transition architecture already described in Section VI: employers may contribute directly to employee HSAs, offer supplemental non-catastrophic benefits, facilitate catastrophic-plan enrollment, or offer cash wages instead of insurance.

The core accounting distinction is that employer premiums are private compensation, while the employer exclusion is the federal subsidy. UCC/HSE reforms both, but they must be modeled separately. The federal score captures the phase-down of the employer exclusion as a revenue gain. The labor-market analysis captures the reallocation of former premium compensation into wages, HSAs, and supplemental benefits. Treating those two channels separately prevents double-counting: employer premium reductions are not counted as direct federal savings, while the loss of the tax exclusion is properly counted as increased taxable compensation.

In competitive labor markets, employer-paid premiums are not free compensation; they are part of total labor cost. Over time, reductions in employer health-benefit costs are expected to be reflected partly in higher taxable wages, partly in employer HSA contributions, and partly in retained labor-cost flexibility during transition. The model therefore does not assume full immediate wage pass-through. It treats wage effects as phased and partial, consistent with the workbook's employer-exclusion phase-down and the paper's transition provisions for firms, unions, and existing collective-bargaining agreements. Section VI already provides for union-plan continuity, allowing existing collective-bargaining agreements to maintain current benefit structures until expiration before transitioning to UCC catastrophic coverage, employer HSA contributions, and narrowly targeted supplemental riders.

The current scoring workbook models the employer-exclusion baseline at \$451.0 billion in 2026, \$578.0 billion in 2030, and \$759.6 billion in 2035, with a 2026–2035 total of approximately \$5.998 trillion. **Table 9** shows this employer-sector federal budget interface directly, separating the employer-exclusion baseline and phased revenue gain from the UCC/HSE outlay streams that replace the current subsidy architecture. The corresponding revenue gain from the phase-down is \$45.1 billion in 2026, \$289.0 billion in 2030, and \$759.6 billion in 2035, totaling approximately \$3.569 trillion over ten years. These figures represent the federal-budget effect of reducing the tax preference for comprehensive employer insurance. They do not imply that employers cease supporting worker health care. Instead, they reflect the conversion of tax-favored premium compensation into a more transparent wage-plus-HSA structure.

The Outlays tab provides the parallel federal architecture replacing the current subsidy system. In 2026, UCC/HSE provides \$486.4 billion in federal catastrophic credits, \$446.5 billion in federal HSA deposits, and \$21.4 billion in federal reinsurance. By 2035, those values rise to \$730.9 billion, \$618.3 billion, and \$32.2 billion, respectively. Over the full 2026–2035 window, the model projects \$6.013 trillion in catastrophic credits, \$5.283 trillion in federal HSA deposits, and \$264.9 billion in federal reinsurance. Total federal outlays under the Act are therefore \$958.3 billion in 2026, \$1.125 trillion in 2030, and \$1.381 trillion in 2035, with a ten-year total of \$11.570 trillion. These values are consistent

Table 9. Employer-Sector Federal Budget Interface Under UCC/HSE

Metric	2026	2030	2035	2026–2035 Total	Interpretation
Employer-exclusion baseline	451.0	578.0	759.6	5,998.1	Current-law federal tax expenditure baseline for employment-based coverage
Revenue gain from employer-exclusion phase-down	45.1	289.0	759.6	3,568.8	Federal revenue recovered as tax-favored comprehensive ESI is phased down
Federal catastrophic credits	486.4	582.9	730.9	6,012.9	Federal premium support for standardized catastrophic coverage
Federal HSA deposits	446.5	516.1	618.3	5,283.1	Federal routine-care liquidity support deposited into HSAs
Federal reinsurance	21.4	25.7	32.2	264.9	Federal stabilization of high-cost catastrophic claims
Total federal outlays under UCC/HSE	958.3	1,124.7	1,381.4	11,569.9	Gross UCC/HSE federal outlays before savings and revenue offsets
Household catastrophic premium payments	304.6	365.1	457.8	3,766.2	Residual catastrophic premium volume paid by households

Source: UCC/HSE scoring workbook, Outlays and Savings tabs; Section XVII provider-facing financial-flow table; Annex E.

with the paper’s Annex E scoring table and the provider-facing financial-flow table already included in Section XVII.

The economic purpose of this transition is not merely fiscal. It changes the form of labor compensation. Under the current system, workers often receive a large and rising share of compensation in the form of opaque health benefits. Under UCC/HSE, the same compensation space is divided more transparently: catastrophic protection is standardized, routine-care liquidity is deposited into HSAs, and remaining employer contributions can flow into wages or targeted supplemental benefits. This is consistent with the paper’s earlier finding that employer-sponsored insurance distorts labor markets,

inflates premiums, contributes to wage stagnation, and ties workers to jobs they might otherwise leave (22,23,32–34).

Table 10. Employer Compensation Recomposition Under UCC/HSE

Channel	Current Employer-Sponsored Insurance System	UCC/HSE Treatment
Employer-paid health premiums	Tax-preferred comprehensive insurance bundles routine and catastrophic care	Comprehensive premium channel phased down as privileged compensation vehicle
Worker wages	Suppressed by rising benefit costs and tax-favored premium substitution	Partial wage pass-through expected over transition as premium compensation is converted
Employer health contributions	Primarily directed to insurance premiums	Redirected toward employee HSAs and limited supplemental benefits
Routine care financing	Embedded inside opaque comprehensive insurance premiums	Financed through HSAs and direct payment in transparent routine-care markets
Catastrophic coverage	Bundled into employer major-medical plans	Standardized national catastrophic plan supported by federal credit and reinsurance
Supplemental benefits	Often embedded in comprehensive benefit design	Permitted only if they do not recreate first-dollar comprehensive coverage
Federal subsidy mechanism	Open-ended tax exclusion tied to employer premiums	Bounded catastrophic credit plus HSA deposits; exclusion phased down
Portability	Coverage tied to employment relationship	Catastrophic credit and HSA structure follow the individual

The model intentionally avoids treating employer premium reductions as direct federal savings. Employer-paid premiums are private compensation; the exclusion of those premiums from taxable income is the federal budget item. Accordingly, the scoring workbook captures the federal effect through the employer-exclusion phase-down, while the broader economic analysis treats former premium spending as re-allocable compensation. This distinction preserves the separation between fiscal savings, household purchasing power, and private labor-market incidence.

Over the ten-year transition, UCC/HSE therefore moves the employer sector away from benefit-inflation competition and toward a wage-plus-HSA structure. Employers retain a meaningful role in health financing, but that role becomes portable, transparent, and compatible with catastrophic-only insurance. The result is not an abandonment of employer support, but the removal of the federal bias that made comprehensive employer insurance the privileged form of worker compensation. **Table 10** illustrates that compensation re-composition, showing how employer-paid comprehensive premiums can

be redirected over time into wages, employer HSA contributions, standardized catastrophic coverage, and limited supplemental benefits.

XIX. Political Risk Mitigation

The political risk of UCC/HSE is not that the model lacks an affirmative theory of health reform. Its risk is that opponents will compress the proposal into familiar and politically damaging categories: “high deductibles,” “ACA repeal,” “Medicaid cuts,” “employer dumping,” “privatization,” or “hospital destabilization.” Those attacks are predictable because prior market-oriented health reforms often failed to distinguish between cost exposure and funded consumer control. UCC/HSE must therefore be evaluated not as a conventional high-deductible plan, but as a complete replacement architecture combining universal catastrophic protection, federally seeded routine-care accounts, variable HSE funding allocations, chronic-condition support, Medicaid LTSS/ABD preservation, rural stabilization, and enforceable fiscal guardrails.

The political strategy must begin with a basic concession: poorly designed catastrophic systems can harm low-income and chronically ill households. If routine care is pushed out of insurance without adequate liquidity, patients may defer care, rely more heavily on emergency departments, or accumulate unpaid medical debt. UCC/HSE is designed specifically to avoid that failure mode. The model does not merely expose households to higher deductibles. It pairs catastrophic protection with prefunded HSA support, income-sensitive routine-care assistance, chronic-condition funding, and a no-lapse implementation sequence. Its political defense should therefore rest on the structure of the design, not on abstract appeals to consumer choice.

The **first major political risk** is the accusation that UCC/HSE is simply a national high-deductible plan. That framing is incomplete. A high-deductible plan leaves the household exposed below the deductible with limited liquidity. UCC/HSE instead separates the insurance function from the routine-care financing function. Catastrophic coverage protects against large, unpredictable losses; HSE deposits finance routine and chronic care before the deductible is reached. The policy should therefore be described as universal catastrophic protection plus funded routine-care liquidity, not as deductible expansion. The distinction is central: the deductible defines where insurance begins, while the HSA system determines whether routine care is practically accessible.

The **second risk** is the low-income access critique. Progressive policy organizations, Medicaid advocates, and patient groups will argue that low-income households cannot be expected to manage routine care through consumer-directed accounts. This concern is credible if HSA deposits are too small, delayed, or poorly targeted. UCC/HSE mitigates this

through early-year prefunding, income-indexed deposits, routine-care affordability protections, and additional support for chronic-condition households. The policy should avoid any implication that routine care is “minor” for low-income families. The correct argument is that routine care should be financed differently, not ignored. UCC/HSE removes routine care from insurance claims administration while preserving access through direct, funded purchasing power.

The **third risk** is chronic-condition backlash. Disease-specific advocacy organizations will focus on patients with diabetes, congestive heart failure, chronic obstructive pulmonary disease, cancer, end-stage renal disease, hemophilia, autoimmune disease, and other conditions that generate recurring costs below the catastrophic threshold. This is one of the strongest political vulnerabilities in any catastrophic-first model. UCC/HSE mitigates it through targeted HSA seeding, condition-based care bundles, risk adjustment, and income-protective routine-care caps. The communication must be disciplined: chronic-condition support should be described as a financing adjustment beneath the deductible, not as a separate insurance tier. The catastrophic plan remains uniform; what varies is the level of HSE funding support provided to households with predictable disease burden.

The **fourth risk** is the accusation that UCC/HSE creates “tiers.” That charge should be answered directly. UCC/HSE does not create separate catastrophic insurance tiers. The catastrophic structure remains uniform: the same national deductible, the same out-of-pocket maximum, the same covered catastrophic benefits, and the same reinsurance architecture. What differs is the flow of funding beneath that uniform insurance structure. HSA deposits, chronic-condition supplements, pediatric allocations, income-indexed support, and rural access enhancements vary because household liquidity needs and predictable routine-care burdens vary. The correct formulation is therefore: no insurance tiers; differentiated HSE funding allocations within a uniform catastrophic structure.

The **fifth risk** is Medicaid disruption. State Medicaid agencies, hospitals, disability advocates, and low-income advocacy groups will argue that moving populations out of Medicaid threatens vulnerable beneficiaries. UCC/HSE should not answer this by minimizing Medicaid’s role. It should state clearly that Medicaid remains structurally necessary for LTSS, ABD populations, medically complex children, home- and community-based services, and other disability-related supports. The reform applies primarily to the non-disabled adult acute-care coverage architecture that the ACA expanded. Medicaid is not abolished; it is refocused on the populations for whom Medicaid’s service model is most essential. This distinction is critical to neutralizing the claim that UCC/HSE is a blanket Medicaid cut.

The sixth risk is employer dumping. Labor unions, benefit consultants, and employer-plan stakeholders will argue that firms may use UCC/HSE to terminate coverage without passing savings through to workers. This concern is politically potent because employer health benefits are embedded in compensation expectations, collective bargaining agreements, and worker security. The proper mitigation is not to claim that all premium savings immediately become wages. The defensible position is more precise: employer-paid premiums are private compensation, while the employer exclusion is the federal subsidy. UCC/HSE phases down the federal tax preference for comprehensive employer insurance while allowing compensation to be reallocated into taxable wages, employer HSA contributions, and limited supplemental benefits. Existing collective-bargaining agreements should be protected through expiration, and the transition should be gradual enough to allow compensation renegotiation rather than sudden benefit termination.

The **seventh risk** is provider destabilization. Hospitals, especially rural and safety-net institutions, will argue that routine-care price compression threatens their operating margins. This concern is credible for hospital outpatient departments that rely heavily on commercial markups, facility fees, and opaque insurer-negotiated rates. UCC/HSE should not deny that some provider revenue streams will be compressed. The policy goal is precisely to discipline inflated routine outpatient prices. The mitigation is that catastrophic and high-acuity care remain insured and federally stabilized; uncompensated care and bad debt are expected to decline; and rural and safety-net providers receive targeted support. The provider message should be: UCC/HSE does not impose blanket reimbursement cuts, but it does shift revenue away from opaque, high-markup routine billing toward transparent routine-care markets and protected catastrophic payment streams.

The **eighth risk** is rural access and rural provider stability. Rural hospitals operate under different economics than large urban systems: lower volume, higher fixed costs, thinner labor markets, longer travel distances, and fewer substitute providers. A purely consumer-directed routine-care model may not be sufficient to preserve access in rural regions. UCC/HSE addresses this through its rural health framework, including a Rural Access Guarantee, Rural Stability Pool, rural reinsurance adjustments, Critical Access Hospital coordination, rural chronic-care support, and rural HSA access enhancements. The political point is straightforward: rural hospitals should not be asked to survive solely on urban-style price competition. Rural support is not a deviation from the model; it is a necessary provider-side stabilizer in markets where ordinary competition is structurally limited.

The **ninth risk** is benefit creep. Insurers, brokers, large employers, and state regulators may gradually rebuild comprehensive first-dollar coverage through supplemental products,

wraparound riders, and employer-sponsored add-ons. If that occurs, UCC/HSE would become nominal rather than substantive reform. The mitigation is statutory clarity. The universal catastrophic credit must apply only to UCC-compliant catastrophic plans. Federal HSA deposits must not be used to subsidize comprehensive first-dollar coverage. Supplemental coverage must be limited, unsubsidized or lightly subsidized, transparent, and subject to actuarial-value caps. The model's fiscal sustainability depends on preventing the re-emergence of ACA-style silver, gold, and platinum equivalents under a different label.

The **tenth risk** is incumbent industry opposition. Comprehensive-plan insurers, exchange contractors, brokers, consultants, and administrative intermediaries benefit from complexity. UCC/HSE reduces that complexity by standardizing catastrophic coverage, removing routine care from claims administration, and replacing exchange-based subsidy machinery with simpler federal credits and HSA deposits. Some industry opposition is therefore unavoidable. The mitigation is to preserve a legitimate role for private insurers in catastrophic coverage, reinsurance administration, supplemental products, direct enrollment, fraud control, and claims management for high-cost events while denying them the ability to recapture federal subsidies through comprehensive-plan inflation. The objective is not to eliminate private insurance, but to confine subsidized insurance to its proper economic function.

The **eleventh risk** is ideological misclassification. Progressives may describe UCC/HSE as privatization. Conservatives may describe universal catastrophic credits and HSA deposits as a new entitlement. Libertarians may object to federal standardization. Moderates may fear disruption. The answer is to avoid ideological overclaiming. UCC/HSE should be presented as a bounded universal guarantee: universal protection against financial ruin, routine-care purchasing power controlled by households, and federal obligations capped by transparent statutory formulas. The proposal is not single-payer, not ACA expansion, and not laissez-faire deregulation. It is a structured catastrophic guarantee paired with consumer-directed routine-care financing.

The **twelfth risk** is fiscal credibility. Any universal federal health benefit will be attacked as another open-ended entitlement unless the scoring architecture is explicit. UCC/HSE mitigates this through benchmark-linked catastrophic pricing, federal reinsurance, income-sensitive credit calibration, HSA deposit formulas, and automatic correction mechanisms. The political defense must emphasize that the ACA's core fiscal weakness is its open-ended link to benchmark premiums. UCC/HSE replaces that architecture with defined federal exposure. The credibility of the proposal therefore depends on maintaining

the statutory outlay cap, publishing the ten-year scoring workbook, and clearly separating structural savings from behavioral savings.

The **thirteenth risk** is implementation disruption. Exchanges, Medicaid systems, employer plans, tax rules, HSA custodians, insurers, providers, and state regulators cannot be reorganized overnight. UCC/HSE therefore requires a staged implementation sequence. ACA premium tax credits and cost-sharing reductions should phase down only as catastrophic credits and HSA deposits become operational. Medicaid transitions should occur only after state coordination systems are in place. Employer tax-exclusion reform should be phased over multiple years, with collective-bargaining protections. No eligible person should lose coverage before catastrophic enrollment and HSA account activation are available. The operational rule should be simple: no repeal without replacement infrastructure already functioning.

The central political defense of UCC/HSE is that it makes tradeoffs explicit. The current system hides costs through employer premiums, federal tax exclusions, ACA subsidies, provider markups, administrative complexity, and suppressed wages. UCC/HSE replaces hidden cross-subsidies with visible federal support, household-controlled routine-care dollars, and catastrophic protection. That transparency will create political resistance because some incumbent revenue streams become visible and contestable. But it also creates the possibility of a broader coalition: taxpayers gain bounded federal exposure, workers gain wage transparency, households gain portable routine-care funds, and patients retain protection against financial ruin.

Political mitigation therefore requires three disciplines. First, UCC/HSE must never be described as merely a high-deductible system. Second, it must never imply that low-income or chronic-condition households are expected to self-finance predictable care without support. Third, it must never promise that all stakeholders are held harmless. Some provider, insurer, broker, and administrative revenue streams are intentionally compressed. The honest claim is not that UCC/HSE has no losers; it is that the current system imposes larger hidden losses through premium inflation, wage suppression, administrative waste, and open-ended federal subsidy growth.

Properly framed, UCC/HSE offers a politically defensible alternative to both ACA expansion and Medicare for All. It preserves universal financial protection without nationalizing provider payment. It preserves private insurance without subsidizing comprehensive first-dollar coverage. It preserves Medicaid where Medicaid is structurally necessary while moving non-disabled adults into a portable catastrophic-and-HSA framework. It preserves

employer participation while ending the federal bias toward opaque premium compensation. It preserves federalism while defining a national catastrophic floor.

The political viability of UCC/HSE depends on presenting these safeguards as integral design elements rather than later concessions. Universal catastrophic coverage, HSA prefunding, chronic-condition support, Medicaid LTSS/ABD preservation, rural stabilization, employer transition rules, supplemental-coverage guardrails, and statutory fiscal caps are not optional add-ons. They are the mechanisms that convert a market-oriented reform into a politically durable replacement architecture.

In summary, UCC/HSE should be defended not as a reduction in coverage, but as a reallocation of health financing toward the functions that government and insurance perform best. Government protects against financial ruin and provides targeted liquidity. Insurance pools catastrophic risk. Households purchase routine care with transparent, portable funds. Providers compete for routine services while remaining protected for high-acuity care. Federal spending is bounded rather than indexed to uncontrolled premium growth. That is the political center of gravity for the reform, and the principal basis on which it can be defended before Congress, state officials, employers, providers, and the public.

Table 11 organizes the principal political and stakeholder attack lines against UCC/HSE, explains why each concern is credible, identifies the mitigation already embedded in the model, and gives the appropriate framing for public and legislative defense. Some political vulnerabilities require more than messaging; they require statutory insulation. **Table 12** isolates those special statutory vulnerabilities, including abortion-funding objections, conscience protections, immigration eligibility, ERISA/federalism disputes, and concerns about state authority.

Table 11. Political Risk Mitigation Matrix

Part 1 of 3

Political Risk / Attack Line	Why the Concern Is Credible	Existing UCC/HSE Mitigation	Correct Framing
1. This is just high deductibles.	Poorly designed high-deductible plans can cause deferred routine care, especially for low-income and chronically ill households.	UCC/HSE pairs a uniform catastrophic plan with prefunded HSA deposits, income-sensitive support, chronic-condition allocations, and early-year liquidity.	Universal catastrophic protection plus funded routine-care liquidity.
2. Low-income people will skip care.	Routine care can be unaffordable if households lack cash liquidity, even when catastrophic coverage exists.	Income-indexed HSA deposits, early-year HSA prefunding, income-protective exposure caps, and routine-care affordability supports reduce deferred care and ER substitution risk.	Routine care is not abandoned; it is financed directly through targeted HSE support.
3. Common chronic patients will be harmed.	Diabetes, COPD, CHF, behavioral health, oncology monitoring, and similar conditions create recurring spending below the catastrophic threshold.	Certified chronic conditions receive enhanced HSA seeding, disease-management caps, risk-adjusted insurer/provider payments, and condition-based bundles.	Same catastrophic plan; stronger chronic-care financing underneath it.
4. High-severity chronic patients still face impossible costs.	ESRD, hemophilia, severe neuromuscular disease, and similar conditions can generate structurally catastrophic annual spending.	For narrow high-severity categories, the effective out-of-pocket burden can collapse toward zero through condition-specific funding formulas, chronic support, and reinsurance without changing the statutory deductible.	The deductible stays uniform; funding absorbs structurally catastrophic routine burden.
5. The plan creates tiers.	If different households receive different support levels, critics may argue the system is tiered.	The catastrophic insurance product remains uniform: same deductible, out-of-pocket maximum, covered benefits, and reinsurance parameters. Variation occurs only through HSE funding allocations based on chronic status, age, pediatric status, income, and access factors.	No insurance tiers; variable HSE funding allocations within a uniform catastrophic structure.
6. Medicaid is being gutted.	Medicaid covers vulnerable populations whose needs extend beyond acute medical insurance.	UCC/HSE preserves LTSS, ABD populations, medically complex children, CHIP, HCBS-related functions, and disability-linked supports. The transition focuses on non-disabled adult acute-care coverage.	Non-disabled adult acute-care coverage transitions; structurally necessary Medicaid functions remain.

Table 11. Political Risk Mitigation Matrix

Part 2 of 3

Political Risk / Attack Line	Why the Concern Is Credible	Existing UCC/HSE Mitigation	Correct Framing
7. Employers will dump coverage and pocket savings.	Employer coverage is embedded in compensation, and workers may fear benefit loss without wage replacement.	Employer tax-exclusion phase-down is paired with wage/HSA transition logic, employer HSA contributions, supplemental-benefit guardrails, and collective-bargaining continuity through contract expiration.	Employer health spending shifts toward wages and HSAs, not disappearance.
8. Unions will lose negotiated benefits.	Collective-bargaining agreements often contain explicit health-benefit promises.	Existing CBAs are preserved through expiration, then transition into UCC catastrophic coverage, employer HSA contributions, and narrow supplemental riders.	No forced mid-contract benefit disruption.
9. Hospital outpatient revenue compression will destabilize provider systems.	Many hospitals rely on commercial outpatient markups, facility fees, and routine-service cross-subsidies.	Catastrophic/high-acuity care remains insured and reinsured; uncompensated care and bad debt are modeled to fall; rural and safety-net institutions receive targeted stabilization. Routine outpatient markups are intentionally disciplined.	Provider revenue is recomposed, not blindly cut.
10. Emergency departments will be flooded.	If routine care becomes unaffordable or inaccessible, patients may substitute ER use.	HSE deposits, routine-care affordability protections, chronic-condition support, and continuity of catastrophic coverage are designed to keep predictable care out of uncompensated emergency channels.	The model funds routine access specifically to prevent ER backflow.
11. Rural patients will lose access.	Sparse provider networks, long travel times, workforce shortages, and weak local infrastructure create distinct access barriers.	The paper includes rural HSA access enhancement, telehealth support, rural chronic-care bundles, and continuity protections designed to preserve access in geographically sparse markets.	Rural access protection is built into the HSE support layer.
12. Rural hospitals and CAHs will collapse.	Rural providers face low volume, high fixed costs, and dependence on specialized rural payment rules.	The paper already includes a rural reinsurance multiplier, Rural Hospital Stability Pool, CAH and Medicare rural payment protection, rural chronic-care support, and transition continuity.	Rural provider stability is built into the design, not added later.

Table 11. Political Risk Mitigation Matrix
Part 3 of 3

Political Risk / Attack Line	Why the Concern Is Credible	Existing UCC/HSE Mitigation	Correct Framing
13. Insurers will recreate comprehensive coverage through riders.	Supplemental products could rebuild first-dollar coverage and destroy fiscal discipline.	Supplemental-coverage guardrails prohibit wraparound major-medical duplication, cap actuarial value, restrict federal subsidy eligibility, and require standardized disclosures.	Supplemental coverage is allowed only if it cannot recreate ACA-style comprehensive insurance.
14. This is ACA repeal without replacement.	Prior repeal efforts often lacked a complete coverage architecture.	UCC/HSE replaces ACA PTCs, CSRs, exchange subsidies, and metal tiers with universal catastrophic credits, HSA deposits, reinsurance, default enrollment, no-lapse transition, and preserved Medicaid functions where structurally necessary.	Replacement architecture, not repeal vacuum.
15. This creates a new open-ended entitlement.	Universal credits and HSA deposits can create federal spending risk if not bounded.	The model uses benchmark-linked catastrophic pricing, calibrated federal benefit exposure, HSA formulas, reinsurance, and automatic correction mechanisms to bound liability.	Universal protection with bounded federal exposure.
16. Implementation will be chaotic.	Exchanges, Medicaid systems, employers, insurers, IRS, CMS, HSA custodians, and providers require coordinated transition.	UCC/HSE uses a staged transition, exchange wind-down, no-lapse coverage rule, catastrophic default enrollment, state coordination, and CBA protections.	No repeal before replacement infrastructure is operational.
17. States will lose flexibility.	States regulate insurance and administer large parts of Medicaid and provider oversight.	States retain roles in solvency, consumer protection, Medicaid coordination, waiver use, rural stabilization, and optional state supplements, while the catastrophic floor remains national.	National catastrophic floor; state implementation flexibility.
18. The plan is ideologically unstable.	Critics may frame it as privatization, entitlement expansion, or deregulation depending on audience.	UCC/HSE combines universal catastrophic protection, public or private catastrophic delivery options, HSA-based routine care, preserved Medicaid functions, rural stabilization, and federal fiscal caps.	A bounded universal guarantee, not single-payer or laissez-faire deregulation.

Table 12. Special Statutory Vulnerabilities

Statutory / Political Vulnerability	Why It Matters	Existing UCC/HSE Mitigation	Correct Framing
Abortion-funding objections	Federal health financing triggers Hyde-related and pro-life objections.	The paper includes Hyde-consistent restrictions and related statutory language applying abortion-funding limits to catastrophic credits, HSA deposits, and reinsurance-linked transfers.	Federal financing remains Hyde-consistent.
Conscience-protection objections	Providers, plans, and employers may seek protection from mandated participation in objectionable services.	The paper includes conscience-protection language alongside federal financing restrictions and implementation guardrails.	Conscience protections are preserved within the financing framework.
Immigration / eligibility objections	Benefit eligibility can become politically vulnerable if verification is weak or self-attested.	Eligibility is restricted to citizens, LPRs, and specified lawfully present groups; DHS/SSA/IRS verification is required; self-attestation is not sufficient.	Verified eligibility; no self-attestation loophole.
Federal preemption / state authority objections	States may object if a federal catastrophic floor is seen as displacing too much state regulatory authority.	The model preserves state roles in insurance oversight, consumer protection, waiver implementation, Medicaid coordination, and optional supplemental policies, while standardizing only the catastrophic floor.	Federal floor with preserved state implementation authority.

XX. Political Coalition Roadmap

UCC/HSE cannot advance as a conventional partisan health-care proposal. Its structure cuts across existing ideological categories: it is universal but not single-payer, market-oriented but not laissez-faire, fiscally bounded but not austerity, and federal in its catastrophic guarantee while preserving state implementation roles. The coalition strategy must therefore be built around functional interests rather than party labels alone.

The political coalition for UCC/HSE should begin with the committees that control the relevant statutory architecture. In the House, the principal jurisdictional centers are Ways and Means for tax credits, HSAs, and the employer exclusion; Energy and Commerce for Medicaid, insurance standards, rural health, and federal health programs; and Education and Workforce for ERISA and employer-plan issues. The House Ways and Means Committee identifies Jason Smith as chairman and Richard Neal as ranking member. The House Energy and Commerce Committee is the relevant House jurisdictional center for Medicaid, insurance-market structure, rural health, and federal health-program design. In the Senate, Finance is the principal jurisdictional center for tax credits, Medicaid, CHIP, federal health financing, and the reinsurance architecture; the Senate Finance Committee identifies Mike Crapo as chairman and Ron Wyden as ranking member. Senate HELP remains relevant for insurance-market standards, consumer protections, and public-health implementation **(97-99)**.

The initial congressional audience should not be the full House or Senate. It should be a narrower group of members and staff whose institutional responsibilities match the design of the proposal: tax writers focused on HSAs and the employer exclusion; health-committee members focused on Medicaid and insurance markets; rural-state members concerned with hospital stability; and fiscal conservatives concerned with open-ended federal subsidy growth. UCC/HSE should first be treated as a committee-level architecture question, not as a floor-level partisan health-care bill.

The **first coalition** target is the fiscal-sustainability bloc. These members are not necessarily the most ideological critics of the ACA, but they are sensitive to open-ended subsidy growth, federal deficit pressure, and entitlement expansion. UCC/HSE should be presented to this bloc as a way to replace benchmark-premium escalation with bounded catastrophic credits, HSA formulas, federal reinsurance, and automatic correction mechanisms. The argument is not that federal health spending disappears; it is that federal exposure becomes defined, scoreable, and less dependent on comprehensive-plan inflation.

The **second coalition** target is the market-reform bloc. This group should be approached through price transparency, HSA ownership, direct primary care, interstate catastrophic-plan competition, and the removal of routine care from third-party claims administration. For this bloc, the political appeal is the restoration of insurance to its proper economic function: protection against large, unpredictable loss. The strongest message is that UCC/HSE does not rely on federal price controls to discipline routine-care costs. It changes the financing channel so routine care becomes visible, priced, and contestable.

The **third coalition** target is the employer and wage-transparency bloc. Employer-sponsored insurance is one of the largest hidden compensation systems in the American economy. UCC/HSE gives employers a transition path out of premium inflation while allowing compensation to move into wages, HSAs, and limited supplemental benefits. The central message for this bloc is not that employers stop providing health benefits. It is that employers stop being forced by the tax code into comprehensive premium escalation. This frame is especially important for business groups, employer coalitions, and members concerned with wage stagnation and labor mobility.

The **fourth coalition** target is the rural-health bloc. Rural members will not support a reform that appears to expose low-volume hospitals to urban-style price competition without protection. UCC/HSE should therefore lead with its rural framework: Rural Access Guarantee, Rural Stability Pool, rural reinsurance adjustments, Critical Access Hospital coordination, rural chronic-care support, and rural HSA access enhancements. The argument is that rural markets require provider-side stabilization precisely because ordinary competitive assumptions do not fully apply in low-density regions.

The **fifth coalition** target is the Medicaid-refocus bloc. This group includes members concerned about Medicaid's growth, but also members who are protective of disability, LTSS, children, and medically complex populations. The coalition message should be narrow and disciplined: UCC/HSE transitions non-disabled adult acute-care coverage into a universal catastrophic-and-HSA structure while preserving Medicaid where Medicaid is structurally necessary. Medicaid is not abolished; it is refocused on ABD, LTSS, medically complex children, HCBS-related functions, and CHIP continuity.

The **sixth coalition** target is the chronic-condition and patient-protection bloc. This is a politically sensitive constituency because catastrophic-first systems are easily attacked as exposing sick patients to unaffordable recurring costs. UCC/HSE should approach this bloc through the distinction between insurance tiers and HSE funding allocations. The catastrophic plan remains uniform. Chronic patients receive additional financing beneath the deductible through enhanced HSA deposits, risk adjustment, disease-management

caps, and condition-based bundles. This makes the reform defensible to patient groups without sacrificing the single catastrophic architecture.

The **seventh coalition** target is the anti-consolidation and anti-middleman bloc. This group cuts across both parties. Progressives may focus on hospital consolidation, insurer complexity, and medical debt. Conservatives and populists may focus on opaque billing, employer-plan inflation, and the administrative class embedded in the ACA exchange structure. UCC/HSE can speak to both without adopting either side's full ideology. The common argument is that the current system hides costs through premiums, tax exclusions, provider markups, benefit mandates, and subsidy escalation. UCC/HSE makes those costs visible and redirects federal support toward catastrophic protection and household-controlled routine-care funds.

The **eighth coalition** target is the state-flexibility bloc. Governors, state legislators, and state insurance commissioners will be wary of a federal structure that appears to displace state authority. UCC/HSE should therefore emphasize federal standardization only where national uniformity is essential: catastrophic benefit design, eligibility, credits, reinsurance, and fiscal guardrails. States retain roles in solvency, consumer protection, Medicaid coordination, rural supplements, provider regulation, and waiver implementation.

The coalition should be developed in phases rather than through a single national launch.

Phase I: Technical Validation.

The first audience should be health economists, CBO-adjacent budget experts, tax-policy specialists, Medicaid experts, rural-health analysts, and employer-benefit economists. The objective is not endorsement; it is error correction. Before seeking political sponsorship, the paper should be stress-tested on population scope, Medicare exclusion, Medicaid transition, employer-exclusion scoring, catastrophic-claims modeling, and HSA deposit calibration.

Phase II: Committee Staff Engagement.

After technical review, the paper should be circulated to staff on the committees of jurisdiction. This should not begin as a request for bill sponsorship. It should begin as a request for technical feedback on statutory architecture and scoring assumptions. The goal is to make UCC/HSE legible to Ways and Means, Energy and Commerce, Finance, HELP, and Education and Workforce staff before it is framed as a member-driven proposal.

Phase III: Member-Level Champion Identification.

The first elected champions should not necessarily be national health-care ideologues. The strongest early sponsors are likely to be members with one or more of the following

profiles: tax-policy credibility, HSA familiarity, rural-hospital exposure, Medicaid-budget pressure, employer-market concerns, deficit sensitivity, or interest in price transparency and direct primary care. A proposal this large should begin with members who can defend one part of the architecture well, rather than members who want to own the entire reform immediately.

Phase IV: Cross-Ideological Pairing.

Once the model survives technical review, the ideal public launch would include members from different ideological entry points. One member can emphasize fiscal discipline and tax reform; another can emphasize universal catastrophic protection; another can emphasize rural stabilization; another can emphasize price transparency and anti-middleman reform. The goal is not artificial bipartisanship. It is to prevent the proposal from being trapped in a single ideological category before the architecture is understood.

Phase V: Outside Coalition Formation.

The outside coalition should include employer groups, direct primary care organizations, HSA administrators, independent physicians, ambulatory surgery centers, rural-health advocates, fiscal-policy organizations, and patient groups willing to engage on chronic-condition financing. Large insurers and hospital systems may resist parts of the proposal, but they should not be excluded from technical engagement. Their opposition will be more manageable if the catastrophic-insurance role, reinsurance architecture, and rural/safety-net protections are clear from the start.

The coalition strategy must avoid three mistakes. First, UCC/HSE should not be introduced as an ACA repeal slogan. That frame activates opposition before the replacement architecture is visible. Second, it should not be sold as a pure HSA expansion. That understates the universal catastrophic guarantee, Medicaid preservation, chronic-condition financing, and federal fiscal architecture. Third, it should not promise that every incumbent institution is held harmless. Some business models built on comprehensive-plan complexity, routine-care markups, and exchange administration will lose revenue. That is not a defect; it is part of the reform's cost-control mechanism.

The correct coalition frame is narrower and stronger: UCC/HSE replaces open-ended comprehensive-insurance subsidies with a bounded universal catastrophic guarantee and funded routine-care accounts. It preserves support where insurance and government are most justified: catastrophic risk, low-income liquidity, chronic disease burden, disability-related services, rural access, and medically complex populations. It restores price discipline where the current system is weakest: routine outpatient care, employer premium compensation, exchange administration, and opaque third-party billing. **Table 13**

translates that coalition frame into institutional entry points, linking each potential coalition bloc to its policy hook, committee or institutional venue, and strategic importance.

The coalition roadmap should therefore proceed from technical credibility to committee staff, from staff to targeted member champions, and from member champions to a broader outside coalition. A reform of this scale cannot be sold first and corrected later. It must be made technically durable before it becomes politically visible.

Table 13. Potential Congressional Coalition Entry Points for UCC/HSE

Coalition bloc	Policy hook	Example congressional entry points	Why this bloc matters
Tax / HSA reform bloc	HSAs, refundable catastrophic credits, employer exclusion phase-down	House Ways and Means; Senate Finance	Controls the tax-side architecture of UCC/HSE.
Medicaid sustainability bloc	Transition of non-disabled adults; preservation of LTSS, ABD, CHIP	House Energy & Commerce; Senate Finance	Determines whether the Medicaid transition is politically and operationally credible.
Rural-health bloc	Rural Stability Pool, CAH coordination, rural HSA access enhancements	Rural-state members; rural hospital caucus-aligned offices	Essential to neutralizing rural hospital and access opposition.
Employer-market bloc	Wage transparency, employer exclusion reform, employer HSA contributions	Ways and Means; Education & Workforce; employer-heavy districts	Critical for employer-sponsored insurance transition and labor-market credibility.
Price-transparency / anti-middleman bloc	Routine-care cash markets, DPC, site-of-service competition, anti-opacity reform	Members active on PBM reform, hospital consolidation, and price transparency	Provides a cross-party anti-opacity rationale for the reform.
Chronic-condition protection bloc	Enhanced HSA deposits, chronic bundles, risk adjustment	Members focused on diabetes, kidney disease, cancer, disability, and behavioral health	Needed to rebut the claim that catastrophic-first financing harms the sick.
State-flexibility bloc	1332 / 1115 alignment, state implementation roles, federalism	Governors' allies, state delegation leaders, federalism-oriented members	Helps make the reform administratively credible to states.
Fiscal-sustainability bloc	Bounded federal exposure, automatic correction, ACA subsidy replacement	Budget-focused members; deficit hawks; tax-policy members	Provides the core fiscal argument for UCC/HSE.

Illustrative coalition-entry map; not an exhaustive whip list.

XXI. CONCLUSION

The United States has reached the limit of incremental health-care reform. Additional subsidies, mandates, exchange adjustments, and benefit expansions cannot correct the structural incentives that drive health-care inflation. The ACA expanded coverage, but it preserved the core architecture of comprehensive insurance: open-ended premium-linked subsidies, employer-plan distortions, opaque provider pricing, routine-care intermediation, and administrative complexity. A system that subsidizes comprehensive coverage while insulating routine spending from price discipline cannot deliver durable affordability.

UCC/HSE offers a structural alternative. It restores insurance to its proper economic function: protection against large, unpredictable, financially ruinous medical events. Routine and predictable care are financed through portable, transparent, consumer-controlled Health Savings Empowerment accounts, with targeted support for low-income households, children, and chronic-condition patients. This division does not abandon routine care. It finances routine care differently, outside the high-cost machinery of comprehensive insurance, while preserving universal protection against catastrophic loss.

The central policy judgment of this paper is that health-care financing should be divided according to the nature of the risk. Catastrophic risk should be pooled. Routine care should be price-transparent. Chronic-condition burden should be supported through targeted financing beneath a uniform catastrophic structure. Medicaid should remain focused on long-term services, disability support, medically complex children, and other populations whose needs do not fit ordinary insurance logic. Medicare should remain untouched. Employers should remain involved, but the federal government should no longer privilege opaque premium compensation over wages, HSAs, and portable coverage.

The refined UCC/HSE framework incorporates the safeguards necessary to make this architecture durable. It excludes ultra-high-cost biologics, cell therapies, and gene therapies from the ordinary catastrophic pool unless administered as part of acute inpatient episodes. It maintains Medicaid LTSS, ABD, CHIP, and medically complex pediatric protections while transitioning non-disabled adult acute-care coverage into a catastrophic-and-HSE framework. It supports chronic-condition patients through enhanced HSA deposits, risk adjustment, condition-based bundles, and income-protective routine-care mechanisms without creating separate catastrophic insurance tiers. It prevents comprehensive-plan reconstitution through supplemental-coverage guardrails and protects rural access through targeted provider-side stabilization.

Fiscal sustainability cannot rest on consumer shopping alone. Routine-care transparency matters, but the core fiscal engine is broader: benchmark-linked catastrophic reimbursement, federal reinsurance, fixed HSA formulas, income-calibrated support, employer-exclusion reform, and statutory guardrails against open-ended subsidy growth. Under the scoring framework accompanying this paper, UCC/HSE entails a transition cost over the first ten years relative to the ACA baseline, but becomes strongly saving over longer horizons as premium-linked subsidy growth is replaced by bounded federal obligations and routine-care inflation is disciplined outside comprehensive insurance.

UCC/HSE is not a repeal-only proposal, a pure HSA expansion, or a retreat from universal protection. It is a replacement architecture. It provides universal catastrophic protection

without nationalizing the provider-payment system. It preserves private insurance while confining federally subsidized insurance to the domain where insurance is economically justified. It preserves Medicaid where Medicaid is structurally necessary. It preserves Medicare by leaving it outside the replacement population. It preserves employer participation while ending the federal bias toward comprehensive premium inflation. It preserves state authority over solvency, consumer protection, Medicaid coordination, and local implementation while establishing a national catastrophic floor.

The political durability of UCC/HSE depends on maintaining this full structure. If reduced to catastrophic coverage alone, the model would be vulnerable to the same criticism as conventional high-deductible plans. If allowed to drift back into comprehensive first-dollar insurance, it would reproduce the ACA's fiscal and pricing failures under a different name. The model succeeds only if its components remain linked: catastrophic protection, prefunded HSE accounts, chronic-condition safeguards, Medicaid LTSS/ABD preservation, rural stabilization, employer-market re-composition, supplemental-coverage limits, and enforceable fiscal discipline.

The case for UCC/HSE is ultimately a case for honesty in health financing. The current system hides costs in premiums, tax exclusions, subsidies, provider markups, administrative overhead, and suppressed wages. UCC/HSE makes those costs visible. It directs federal support toward catastrophic protection and targeted liquidity rather than comprehensive-plan inflation. It allows routine-care markets to function where price transparency is possible, while preserving collective financing where individual risk is too large to bear.

A successor to the ACA must do more than expand subsidies or rearrange existing coverage categories. It must correct the economic structure beneath them. The choice is no longer between the ACA and single payer. A third architecture is possible: universal catastrophic protection, transparent routine-care purchasing, targeted support for vulnerable households, preserved safety-net functions, and bounded federal liability.

APPENDIX A: MODELING METHODOLOGY

1. Catastrophic Reinsurance Cost Modeling

Primary source:

CBO (2021), *Federal Reinsurance and High-Cost Claim Options*.

The catastrophic reinsurance estimate models the federal cost of stabilizing the high-cost claim tail under UCC/HSE. The model applies an 80 percent federal reimbursement rate to allowed claims above a \$50,000 annual attachment point per enrollee. This reinsurance layer is not total UCC/HSE spending; it is the federal volatility-suppression mechanism for unusually high-cost claims.

Methodological steps:

1. Extract national high-cost-claim distributions from CBO high-cost-claim curves.
2. Apply an 80 percent federal reimbursement rate above \$50,000 per enrollee.
3. Apply the reinsurance structure to the UCC/HSE-covered population, including privately insured individuals and transition Medicaid adults, while excluding Medicare beneficiaries.
4. Adjust for the lower actuarial value and narrower catastrophic-plan structure relative to ACA silver plans.
5. Project annual federal reinsurance outlays across the scoring window.

Modeled result:

Federal reinsurance outlays are approximately \$20–25 billion annually in the early scoring window and total approximately **\$264.9 billion over 2026–2035** under the current scoring workbook.

2. Federal HSA Deposit Modeling

Federal HSA deposits are modeled as the routine-care financing layer under HSE. These deposits are not limited to a narrow chronic-condition or low-income subsidy. They include the full HSE deposit structure: base deposits, income-indexed support, pediatric deposits, chronic-condition supplements, and related routine-care liquidity protections.

Earlier narrow estimates of chronic and low-income seeding suggested a smaller annual cost range. Those estimates are superseded by the full HSE deposit schedule in Annex E.

Primary inputs:

- MEPS HC-251 2023 expenditure distributions for routine and below-threshold spending;
- population counts for eligible non-Medicare adults and children;
- income distribution assumptions used in the scoring workbook;
- pediatric deposit assumptions;
- chronic-condition supplement assumptions;
- gamma-based income-adjustment parameters;
- routine-care growth and inflation assumptions.

Methodological steps:

1. Define the UCC/HSE-eligible non-Medicare population.
2. Apply base federal HSA deposits to eligible adults.
3. Apply pediatric deposits to eligible children outside preserved CHIP or special-needs Medicaid categories.
4. Apply chronic-condition supplements using the model's chronic-share assumption.
5. Apply income-sensitive HSE adjustments through the gamma schedule.
6. Grow deposits over time according to the workbook's indexing assumptions.
7. Aggregate annual deposits across the scoring window.

Modeled result:

Federal HSA deposit outlays are modeled at approximately **\$446.5 billion in 2026**, rising to approximately **\$618.3 billion by 2035**, with a **2026–2035 total of \$5.283 trillion** under the current scoring workbook.

3. Catastrophic Credit Outlay Modeling

Catastrophic credit outlays are modeled from expected spending above the catastrophic threshold rather than by multiplying the deductible by the share of enrollees exceeding the threshold. This distinction is central to the model. The deductible is the activation threshold; it is not the expected federal cost per triggered enrollee.

Primary source:

MEPS HC-251 2023 Full-Year Consolidated file.

Core method:

$$E[(\textit{spend} - D)^+]$$

where D is the catastrophic deductible or trigger threshold.

Methodological steps:

1. Use MEPS HC-251 2023 person-level spending and survey weights.
2. Exclude Medicare beneficiaries from the UCC/HSE-covered population.
3. Identify individuals with annual spending above the catastrophic threshold.
4. Calculate weighted excess spending above the threshold:

$$(\textit{spend} - D)$$

for each person whose spending exceeds D .

5. Aggregate weighted excess spending across the eligible population.
6. Divide by the weighted eligible population to estimate expected excess spending per enrollee.
7. Scale the result to the UCC/HSE-covered population and apply federal catastrophic-credit assumptions.

Modeling principle:

Lowering the catastrophic threshold increases expected federal exposure because more spending lies above the threshold. The model therefore prices expected excess spending above the deductible, not the deductible amount itself.

4. Administrative Savings Modeling

Administrative savings are modeled from the reduction in claims processing, benefit administration, exchange infrastructure, actuarial-value compliance, essential-health-benefit administration, and routine-care billing complexity that occurs when routine care is moved out of comprehensive insurance and into HSE-financed direct payment.

Sources:

- Papanicolas et al. (JAMA 2018);
- GAO-23-105642;
- MedPAC 2022.

Methodological steps:

1. Estimate administrative overhead associated with comprehensive private insurance and ACA exchange infrastructure.
2. Identify administrative functions reduced or eliminated under UCC/HSE, including routine-care claims processing, exchange operations, actuarial-value tiering, essential-health-benefit compliance, and complex benefit adjudication.
3. Apply administrative-reduction assumptions only to the spending categories affected by UCC/HSE, rather than to total national health expenditure indiscriminately.
4. Preserve administrative costs associated with catastrophic claims, HSA administration, reinsurance, Medicaid LTSS/ABD functions, Medicare, and required program integrity systems.

Modeled effect:

Administrative simplification contributes to a projected **2–3 percent reduction in affected health-system spending**, with the exact federal budget effect captured in the savings schedules of Annex E.

5. Routine-Care and Total Expenditure Effects

The projected 8–12 percent reduction refers to the modeled reduction in the **national health-expenditure growth trajectory**, not an immediate across-the-board reduction in total spending. The estimate is derived by applying separate mechanisms to the spending bases they plausibly affect.

Components:

- Routine-care price discipline from HSE-financed direct payment and transparent cash markets;
- reduced administrative overhead from removing routine care from comprehensive insurance;
- catastrophic-premium stabilization from federal reinsurance;

- employer-market re-composition as the tax preference for comprehensive employer insurance phases down;
- reduced uncompensated care, bad debt, and avoidable emergency-department use;
- provider-sector re-composition away from opaque facility-fee billing and toward transparent outpatient markets.

Methodological steps:

1. Apply routine-care spending reductions only to the routine/outpatient share of spending affected by HSE.
2. Apply reinsurance premium effects only to the catastrophic-plan premium layer, not to total national health expenditure.
3. Apply administrative savings only to affected claims-processing and benefit-administration categories.
4. Apply provider-facing savings and stabilization effects using the FY2024 HCRIS provider-impact model.
5. Combine effects into a long-run growth-path adjustment rather than a one-time reduction in all health spending.

Modeled result:

The framework produces an estimated **8–12 percent reduction in the national health-expenditure growth trajectory** under full implementation, driven by routine-care price discipline, administrative simplification, catastrophic-premium stabilization, employer-market re-composition, and provider-sector repricing.

Appendix B: Reproducible HCRIS-Based Provider-Impact Scoring Appendix

The provider-impact analysis was developed as a reproducible, provider-level scoring appendix to estimate the first-order financial effects of Universal Catastrophic Coverage with Health Savings Empowerment on hospitals and institutional providers. The analysis uses FY2024 CMS HCRIS hospital cost-report data to move beyond aggregate national estimates and evaluate how UCC/HSE affects different provider classes through four linked mechanisms: routine/outpatient repricing pressure, coverage and liquidity offsets, bounded transition stabilization, and targeted access protection for providers whose role in the health system cannot be evaluated through routine-market exposure alone. The model is intentionally static and provider-level. It is not a dynamic hospital-

closure forecast, not a full general-equilibrium provider-market model, and not a claim that every provider is held harmless. Its purpose is to provide an auditable scoring framework for estimating near-term provider revenue exposure under the UCC/HSE transition while distinguishing ordinary market repricing from access-preservation policy.

This distinction is central to the provider-impact methodology. UCC/HSE deliberately imposes price discipline on routine and shoppable outpatient services, especially where current revenue depends on opaque facility-fee billing, high commercial markups, or insurer-mediated administrative complexity. At the same time, the model recognizes that not all providers are equally situated to absorb transition pressure. Rural hospitals, Critical Access Hospital-like institutions, pediatric hospitals, safety-net providers, and psychiatric or behavioral-health institutions often carry fixed standby costs, payer-mix constraints, thin local markets, workforce shortages, regulatory obligations, or essential-access responsibilities that ordinary outpatient price competition does not capture. For that reason, the provider-impact model separates five concepts rather than collapsing them into one net figure: market repricing exposure, coverage and liquidity offsets, implementation-period transition stabilization, permanent rural access protection, and behavioral institutional exposure correction. The current white paper's Annex C states this distinction directly, explaining that the **Table 7** framework preserves the UCC/HSE principle that routine-care markets should face price discipline while recognizing that providers differ in their ability to absorb transition pressure.

The model begins with raw FY2024 HCRIS hospital cost-report files rather than manually entered aggregate assumptions. The project environment initializes around the CMS HCRIS alpha, numeric, and report files: HOSP10_2024_alpha.csv, HOSP10_2024_nmrc.csv, and HOSP10_2024_rpt.csv. The setup script defines the project root, raw HCRIS folder, documentation folder, scripts folder, processed-data folder, output folder, and build year, while also verifying that required input files exist before the model proceeds.

The full provider-impact pipeline contains 27 R scripts, beginning with package installation and raw HCRIS loading and ending with publication-grade tables and compact figure outputs. The combined script manifest documents the complete sequence, including setup, HCRIS loading, provider-master construction, identity mapping, beds/capacity extraction, S-10 uncompensated-care extraction, G-2/G-3 revenue extraction, classification validation, stabilization eligibility, provider-impact scenario construction, coverage/exposure audits, transition-protection sensitivity, enhanced rural-access sensitivity, behavioral/psychiatric sensitivity, access-protection summary creation, publication-grade table creation, table-image generation, and compact figure generation.

Data component	Role in model	Reason included
<i>HCRIS alpha file</i>	Provider identity and alphanumeric worksheet fields	Recovers provider names, location fields, and identity information needed for classification validation
<i>HCRIS numeric file</i>	Worksheet-level numeric values	Supplies revenue, capacity, uncompensated-care, and exposure variables
<i>HCRIS report file</i>	Report metadata	Identifies provider number, report record number, fiscal year, report status, facility/control type, and report-day validity
<i>Provider master</i>	Consolidated provider-level backbone	Serves as the central join table for all provider characteristics and financial variables
<i>S-10 fields</i>	Uncompensated-care burden	Used for coverage/liquidity offsets, safety-net proxying, and stabilization eligibility
<i>G-2/G-3 fields</i>	Revenue and outpatient exposure	Used to calculate net patient revenue, gross outpatient revenue, gross total revenue, and net outpatient exposure
<i>Beds/capacity fields</i>	Scale and institutional-capacity checks	Used to validate provider class and identify implausible raw classifications
<i>Clean provider classification</i>	Corrected analytic grouping	Separates general acute, pediatric, rural/CAH/IHS, and specialty/behavioral providers
<i>Stabilization variables</i>	Transition-support eligibility and support caps	Used to estimate bounded safety-net, rural, pediatric, and negative-margin support
<i>Rural access variables</i>	Permanent rural access protection	Used to model rural access guarantees and implementation-period floors

<i>Behavioral sensitivity variables</i>	Psychiatric/behavioral exposure correction	Used to model reduced repricing exposure and floors for institutional behavioral providers
<i>Scenario outputs</i>	Table 7 results	Used to create the 3×3 provider-impact matrix and access-protection appendices

The model begins by loading and normalizing the raw HCRIS files. The loader reads the alpha, numeric, and report files without headers, assigns expected CMS-style column names, normalizes report record numbers, worksheet codes, line numbers, column numbers, numeric values, provider numbers, and date fields, and saves processed .rds versions for downstream scripts. This is necessary because HCRIS is a cost-report database, not an analysis-ready provider-impact file. The loader also produces basic validation summaries, including unique report-record counts, worksheet counts, report-status counts, provider-control counts, and report-day summaries.

The report-base and provider-master scripts convert raw report-level metadata into a provider-level analysis file. The report-base script deduplicates records at the report-record level, labels report status, assigns facility-type labels, creates initial provider model classes, flags full-year reports, and identifies providers eligible for the first-pass hospital-scoring universe. The provider-master scripts then add provider identity, names, city/state information, raw classification fields, and later capacity and financial fields.

The model starts from a broad HCRIS provider base but narrows to a defined backbone universe and then to a modeled universe:

<i>Universe</i>	<i>Count</i>	<i>Meaning</i>
<i>Provider base rows</i>	6,102	Full provider base loaded into the working HCRIS master
<i>Backbone providers</i>	912	Providers selected for the UCC/HSE provider-impact backbone
<i>Modeled providers</i>	886	Backbone providers with usable financial/exposure data
<i>Excluded providers</i>	26	Backbone providers excluded because required exposure or financial variables were missing or unusable
<i>Modeled share</i>	~97.1%	Share of backbone providers included in Table 7 modeling

The scenario script reports 912 backbone providers and 886 modeled providers. It also shows that the 886 modeled providers are distributed across the cleaned provider groups as 73 children's/pediatric providers, 604 general acute/academic providers, 68 rural/CAH/IHS providers, and 141 specialty/behavioral/rehab providers.

The most important financial concept in the model is net outpatient exposure, not total hospital revenue. UCC/HSE does not apply price discipline to all hospital revenue. Catastrophic and high-acuity care remains insured. The exposed revenue channel is the routine, outpatient, and shoppable-care component most likely to face cash-price competition, HSA-funded purchasing, site-neutral substitution, direct-pay alternatives, and reduced facility-fee opacity.

The G-2/G-3 revenue-extraction script constructs this exposure base using HCRIS revenue fields. In the model run, the revenue extraction script loaded 6,102 provider rows and 19,402,648 HCRIS numeric rows, then identified 912 backbone-included providers and profiled G-2/G-3 candidate revenue lines.

Variable	Definition	Role in the provider-impact model
<i>NPR</i>	Net patient revenue	Realized patient-care revenue; denominator for impact as % of NPR
<i>GOR</i>	Gross outpatient revenue	Gross outpatient/routine revenue channel
<i>GTR</i>	Gross total patient revenue	Gross-charge denominator
<i>NTG</i>	Net-to-gross revenue ratio	Converts gross charges into net-revenue-equivalent values
<i>OPEB</i>	Net outpatient exposure base	Final outpatient/routine exposure base used for repricing

Calculation	Formula	Interpretation
<i>Net-to-gross ratio</i>	$NTG = NPR / GTR$	Estimates how much gross charge revenue converts into net patient revenue
<i>Net outpatient exposure base</i>	$OPEB = GOR \times NTG$	Converts gross outpatient revenue into net-revenue-equivalent outpatient exposure
<i>Expanded form</i>	$OPEB = GOR \times (NPR / GTR)$	Prevents gross-charge values from overstating actual economic exposure

This method is deliberately conservative relative to a total-revenue haircut. It does not assume that inpatient, catastrophic, ICU, transplant, trauma, or other high-acuity revenue is repriced by the routine-care HSA market. It isolates the outpatient/routine portion of revenue and converts it into net-revenue-equivalent dollars before applying scenario assumptions.

The model does not mechanically accept every HCRIS revenue record. It records the method used to derive each provider's revenue exposure and flags unusable or anomalous records. This matters because some rural, pediatric, psychiatric, and specialty institutions do not report revenue in the same clean G-2/G-3 pattern as standard acute-care hospitals.

Revenue source method	Purpose
<i>standard_g2_l028_split</i>	Standard G-2 line 28 inpatient/outpatient/total split
<i>nonstandard_g200000_l04300_fallback</i>	Nonstandard fallback where standard split was not usable
<i>rural_g200000_l04300_fallback</i>	Rural-specific fallback for providers with nonstandard reporting
<i>children_total_only_fallback</i>	Pediatric fallback when only total revenue basis was usable
<i>g2_total_only_no_outpatient_split</i>	Total-only revenue record with no usable outpatient split
<i>missing_or_unusable</i>	No reliable modeled revenue basis
<i>nonstandard_fallback_failed_plausibility</i>	Fallback candidate rejected after plausibility checks

The publication audit confirms that revenue-source methods, fallback records, failed-plausibility records, net-to-gross checks, and exposure-anomaly flags were tracked explicitly rather than hidden. The audit reports standard G-2 split records, rural fallback records, children’s fallback records, nonstandard fallback records, failed-plausibility records, total-only records, missing/unusable records, and exposure-anomaly records.

Raw HCRIS provider labels were not accepted as final analytic classes. The classification-validation script corrects classification problems, especially cases where large academic hospitals, children’s hospitals, psychiatric institutions, specialty hospitals, or other large facilities appear in raw CAH-labeled categories. This step is critical because rural-stabilization logic should not be driven by large academic medical centers that happen to appear in a CAH-coded bucket.

<i>Clean provider group</i>	Purpose in analysis
<i>General acute / academic</i>	Captures ordinary short-term acute hospitals, major academic centers, and large hospital systems
<i>Children’s / pediatric</i>	Captures pediatric hospitals and children’s institutions requiring separate access treatment
<i>Rural / CAH / IHS</i>	Captures internally safe CAH candidates, rural primary-care/IHS-like providers, and rural access providers
<i>Specialty / behavioral / rehab</i>	Captures psychiatric, behavioral, rehab, LTCH, and specialty institutions

The stabilization stage is where the model stops being a simple revenue-loss calculation and becomes a policy-relevant provider-impact framework. Without stabilization, the model would estimate repricing pressure but would not distinguish between providers that can reasonably absorb a transition and providers whose access role requires special treatment. The stabilization script creates bounded transition-support eligibility before the scenario model is run. It assigns support based on provider characteristics and transition vulnerability rather than providing a blanket offset to all providers. This distinction is essential. UCC/HSE is designed to preserve price discipline in routine-care markets while preventing access loss in settings where the provider cannot be treated as an ordinary price-competitive outpatient supplier.

The general stabilization architecture has four main components:

Stabilization component	Formula concept	Policy function
<i>Safety-net stabilization</i>	$\text{NPR} \times \text{safety-net rate} \times \text{safety-net indicator}$	Protects providers with high uncompensated-care or low-income burden
<i>Rural stabilization</i>	$\text{NPR} \times \text{rural rate} \times \text{rural indicator}$	Protects geographically necessary low-volume providers
<i>Children's hospital stabilization</i>	$\text{NPR} \times \text{pediatric rate} \times \text{children's hospital indicator}$	Protects pediatric institutional capacity
<i>Negative-margin protection</i>	$\text{NPR} \times \text{margin-protection rate} \times \text{negative-margin indicator}$	Provides transition support for financially distressed providers

The white paper's provider-impact formula section defines transition stabilization as the sum of safety-net stabilization, rural stabilization, children's hospital stabilization, and negative-margin protection. It also states that safety-net eligibility can be proxied in the HCRIS-only model by high S-10 uncompensated-care burden, that rural eligibility can be tied to rural primary care, critical access, sole-community, Medicare-dependent rural, or low-volume rural status, and that the current HCRIS model used available rural primary-care classification.

The scenario model uses stabilization multipliers to test how strongly the bounded support layer is funded:

Stabilization scenario	Multiplier	Interpretation
<i>Low stabilization</i>	0.5×	Half of bounded transition support
<i>Base stabilization</i>	1.0×	Full bounded transition support
<i>High stabilization</i>	1.5×	Expanded transition support sensitivity

The stabilization-support line inside Table 7A is therefore not an ad hoc “plug.” It is the scenario-funded value of bounded eligibility created earlier in the pipeline. The central

base/base result includes approximately \$0.30 billion in stabilization support across the 886 modeled providers, which partially offsets the \$13.52 billion in outpatient repricing pressure and \$1.91 billion UC/liquidity offset.

The model then goes further by separating ordinary transition stabilization from **rural access protection**. This is a critical methodological distinction. Transition stabilization is implementation-period support for providers facing acute adjustment pressure. Rural access protection is treated as a durable access-capacity policy because rural providers face structural constraints that do not necessarily disappear after transition. Low volume, fixed standby capacity, broad geographic service areas, lack of nearby substitute providers, thin labor markets, and workforce constraints make rural hospitals different from ordinary outpatient competitors. The enhanced rural access script explicitly states that rural access support is treated as a permanent access-capacity payment rather than ordinary transition stabilization, because rural providers face low volume, fixed standby costs, broad geographic service areas, workforce constraints, and lack of nearby substitutes.

The rural sensitivity analysis isolates the rural / CAH / IHS provider group from the corrected base/base provider-impact file and tests a menu of rural-access designs:

<i>Rural access scenario</i>	Protection design	Policy purpose
<i>Current Base/Base</i>	No added rural access support beyond existing model	Reference case
<i>Current rural access guarantee</i>	1.0% of NPR, capped at \$3M per rural provider	Modest permanent rural capacity support
<i>Enhanced rural access guarantee</i>	2.0% of NPR, capped at \$5M per rural provider	Preferred rural protection option
<i>High rural access guarantee</i>	3.0% of NPR, capped at \$7.5M per rural provider	Stronger sensitivity case
<i>Enhanced guarantee + rural floor</i>	2.0% NPR / \$5M cap plus implementation floor	Protects against excessive provider-level losses
<i>High guarantee + stronger floor</i>	3.0% NPR / \$7.5M cap plus tighter floor	Upper-bound rural support case

The preferred rural access option is the enhanced rural access guarantee plus an implementation-period rural floor. In the access-protection summary, the preferred rural

layer is described as a 2 percent of NPR payment capped at \$5 million per rural provider, plus an implementation floor ensuring no qualifying rural provider falls below -1.5 percent of NPR during the implementation period. Under the rural baseline, rural providers show approximately -\$0.341 billion in net impact, or -3.1 percent of NPR, with 10 providers below -5 percent of NPR. Under the preferred rural access protection, added support of approximately \$0.218 billion improves rural net impact to approximately -\$0.123 billion, or -1.1 percent of NPR, and eliminates rural providers below -5 percent and -10 percent of NPR in the model.

This rural treatment is analytically important because it keeps UCC/HSE from becoming an urban-only price-discipline model. Rural providers are not exempted from all repricing, but they are protected from access-threatening transition effects. The preferred rural policy is capped, formula-based, and provider-side. It does not lower the national catastrophic deductible, does not create a separate rural insurance tier, does not reintroduce first-dollar comprehensive coverage, and does not alter the UCC/HSE benefit design. It functions as an access-capacity payment outside the routine-care price mechanism.

The model also includes a separate **behavioral and psychiatric institutional sensitivity**, because psychiatric and behavioral-health providers are not ordinary routine-care suppliers. Many institutional behavioral providers have fragile finances, state or county roles, forensic or involuntary-treatment obligations, limited substitute capacity, and service lines that do not fit neatly into a consumer-shopping model. The behavioral sensitivity script isolates providers whose clean model class is “Psychiatric / behavioral hospital” and also creates a broader behavioral review universe using provider-name flags such as psychiatric, behavioral, mental, state hospital, forensic, recovery, substance, addiction, and related terms.

The behavioral sensitivity tests whether standard outpatient repricing pressure overstates the appropriate exposure for institutional psychiatric and behavioral providers. It uses repricing scale factors rather than simply adding generic subsidies. The script defines a current repricing scale of 1.00, a reduced behavioral repricing scale of 0.50, and a minimal behavioral repricing scale of 0.25. It also defines behavioral access floors, including a moderate floor at -7.5 percent of NPR and a strong floor at -5.0 percent of NPR.

<i>Behavioral sensitivity element</i>	Treatment	Policy rationale
<i>Primary behavioral definition</i>	Clean class equals psychiatric / behavioral hospital	Main behavioral-institution universe
<i>Review-universe definition</i>	Primary flag plus provider-name behavioral terms	Captures possible missed behavioral institutions
<i>Current treatment</i>	100% of standard modeled repricing pressure	Baseline reference case
<i>Reduced repricing</i>	50% of standard modeled repricing pressure	Tests partial routine-market exposure
<i>Minimal repricing</i>	25% of standard modeled repricing pressure	Preferred institutional behavioral correction
<i>Behavioral floor</i>	No qualifying behavioral provider below -5% NPR in preferred case	Prevents severe access-threatening losses

The access-protection summary reports that the behavioral institutional baseline has a small aggregate impact but contains severe provider-level outliers. Under the behavioral baseline, psychiatric/behavioral hospitals show approximately -\$0.063 billion in net impact, with 24 providers below -5 percent of NPR and 11 below -10 percent. Under the preferred behavioral institutional treatment, qualifying behavioral providers are assigned 25 percent of standard modeled repricing pressure and receive an implementation floor so no qualifying behavioral provider falls below -5 percent of NPR. This adds approximately \$0.001 billion of support, improves behavioral net impact to approximately -\$0.015 billion, reduces providers below -5 percent of NPR to 6, and eliminates behavioral providers below -10 percent of NPR.

The behavioral adjustment is not a claim that behavioral providers should be insulated from all market discipline. It is a recognition that institutional psychiatric and behavioral-health facilities are poor candidates for full routine-market repricing assumptions. A cash-pay model may work for ordinary outpatient therapy, primary-care behavioral integration, or routine mental-health visits, but state psychiatric hospitals, forensic facilities, inpatient behavioral institutions, and specialized psychiatric providers often operate under different access obligations and payer structures. The model therefore treats behavioral institutional adjustment as a structural repricing-exposure correction, not merely as temporary stabilization.

The preferred access-protection package combines the rural and behavioral refinements while preserving the base provider-impact model. The access-protection summary defines the preferred package as rural access protection plus preferred behavioral institutional treatment. The rural component is permanent and capped; the implementation floors are temporary and reviewable; the behavioral treatment reduces modeled repricing exposure for qualifying institutional psychiatric providers and applies an implementation floor. In aggregate, the preferred package adds approximately \$0.219 billion of support and changes the overall base/base provider impact from approximately -\$11.300 billion to approximately -\$11.081 billion, with the net impact remaining approximately -4.2 percent of total NPR.

Access-protection layer	Provider group	Permanent component	Temporary / implementation component	Modeled effect
<i>Baseline model</i>	All modeled providers	None beyond existing model	Existing bounded stabilization only	-\$11.300B net impact; -4.3% NPR
<i>Rural baseline</i>	Rural / CAH / IHS	None beyond existing model	Existing bounded stabilization only	-\$0.341B net rural impact; -3.1% NPR
<i>Preferred rural access protection</i>	Rural / CAH / IHS	2% of NPR capped at \$5M per rural provider	Floor: no qualifying rural provider below -1.5% NPR	Improves rural net impact to -\$0.123B; eliminates rural providers below -5% NPR
<i>Behavioral baseline</i>	Psychiatric / behavioral hospitals	None	No behavioral-specific adjustment	-\$0.063B net impact; 24 below -5% NPR; 11 below -10% NPR
<i>Preferred behavioral treatment</i>	Psychiatric / behavioral hospitals	25% of standard modeled repricing pressure	Floor: no qualifying behavioral provider below -5% NPR	Improves behavioral net impact to -\$0.015B; eliminates behavioral providers below -10% NPR
<i>Preferred access-protection package</i>	Targeted rural and behavioral providers	Rural permanent access guarantee; behavioral repricing correction	Rural and behavioral implementation floors	Overall net impact improves to -\$11.081B while preserving main price-discipline effect

This preferred package is important because it shows that targeted stabilization does not overwhelm the provider-side savings mechanism. The model's main repricing pressure remains intact. The preferred access-protection package changes the aggregate net impact only modestly, from -\$11.300 billion to -\$11.081 billion, while materially improving the tail

risk for rural and behavioral providers. That is the policy logic of the stabilization architecture: protect essential access where the model identifies concentrated provider-level risk, but do not neutralize the broader outpatient price-discipline mechanism.

The Table 7 scenario script creates the main 3×3 matrix. Each provider is evaluated under low, base, and high repricing assumptions crossed with low, base, and high stabilization assumptions.

Scenario dimension	Low	Base	High
<i>Routine/outpatient repricing rate</i>	5%	10%	15%
<i>UC/liquidity offset rate</i>	15%	25%	35%
<i>Stabilization multiplier</i>	0.5×	1.0×	1.5×

The scenario output confirms the full 3×3 structure and reports, for each row of the matrix, providers in model, total net patient revenue, total gross patient revenue, total outpatient exposure used, outpatient repricing pressure, uncompensated-care offset, stabilization support, total net provider impact, net impact as a percentage of total NPR, provider counts with negative or positive impact, and provider counts with large negative impact.

The provider-level calculation is:

Component	Formula / interpretation
<i>Outpatient repricing pressure</i>	-1 × outpatient exposure used × repricing rate
<i>UC / liquidity offset</i>	uncompensated-care cost × UC offset rate
<i>Bounded transition stabilization</i>	stabilization cap × stabilization multiplier
<i>Rural access protection</i>	rural access payment + implementation floor, where applicable
<i>Behavioral institutional treatment</i>	reduced repricing exposure + behavioral floor, where applicable
<i>Net provider impact</i>	repricing pressure + UC/liquidity offset + stabilization/access support
<i>Impact as % of NPR</i>	net provider impact / net patient revenue

The current executable model reports the offset as UC / Liquidity Offset, using S-10 uncompensated-care cost as the operational proxy. The broader formula framework also contemplates bad-debt relief, but unless a separate bad-debt field is integrated into the **Table 7** implementation, the publication text should avoid implying that a distinct bad-debt line is already separately modeled in the central Table 7 output.

The scoring appendix produces a full publication package, not a single table.

Table	Content	Purpose
<i>Table 7A</i>	Provider Impact 3×3 Scenario Matrix	Shows low/base/high repricing and stabilization results
<i>Table 7B</i>	Provider Impact by Provider Group	Shows base/base results by major provider group
<i>Table 7C</i>	Provider Impact by Provider Class	Shows base/base results by cleaned provider class
<i>Table 7D</i>	Distribution of Provider-Level Effects	Shows how providers fall into impact-percentage buckets
<i>Table 7E</i>	Model Coverage and Data Quality Audit	Shows inclusion, exclusion, fallback, and data-quality status
<i>Table 7F</i>	Exposure-Anomaly Sensitivity	Tests whether flagged exposure anomalies drive the result
<i>Table 7G</i>	Access-Protection Policy Options	Summarizes targeted rural and behavioral protections
<i>Appendix Table 7K</i>	Enhanced Rural Access Sensitivity	Tests rural guarantee and floor options
<i>Appendix Table 7L</i>	Rural Provider-Level Enhanced Access	Shows provider-level rural access effects
<i>Appendix Table 7M</i>	Psychiatric / Behavioral Provider Sensitivity	Tests behavioral repricing and floor options
<i>Appendix Table 7N</i>	Psychiatric / Behavioral Provider-Level Review	Shows provider-level behavioral sensitivity

<i>Appendix Table 7O</i>	Remaining Severe Psychiatric / Behavioral Effects	Identifies behavioral tail-risk after treatments
<i>Appendix Table 7P</i>	Behavioral Review Universe	Expands review beyond primary behavioral class

The publication-grade table script defines these titles and placements, identifying Table 7G as “Access-Protection Policy Options,” Appendix Table 7K as “Enhanced Rural Access Sensitivity,” Appendix Table 7L as “Rural Provider-Level Enhanced Access,” Appendix Table 7M as “Psychiatric / Behavioral Provider Sensitivity,” Appendix Table 7N as “Psychiatric / Behavioral Provider-Level Review,” Appendix Table 7O as “Remaining Severe Psychiatric / Behavioral Effects,” and Appendix Table 7P as the “Behavioral Review Universe.”

The central result is the base repricing/base stabilization scenario.

Metric	Base/Base result
<i>Modeled providers</i>	886
<i>Total net patient revenue</i>	~\$264.9B
<i>Total gross patient revenue</i>	~\$1.030T
<i>Total outpatient/routine exposure</i>	~\$135.2B
<i>Outpatient repricing pressure</i>	-\$13.52B
<i>UC / liquidity offset</i>	+\$1.91B
<i>Bounded stabilization support</i>	+\$0.30B
<i>Net provider impact before added access package</i>	-\$11.30B
<i>Net impact as % of NPR</i>	-4.3%
<i>Median provider impact</i>	-4.2% of NPR
<i>Net provider impact after preferred rural/behavioral access package</i>	-\$11.081B
<i>Net impact after preferred access package as % of NPR</i>	-4.2%

The publication preview reports the base/base central estimate as approximately -\$13.52 billion in repricing pressure, +\$1.91 billion in UC/liquidity offset, +\$0.30 billion in stabilization support, and -\$11.30 billion in net provider impact before the added access-

protection package. The access-protection summary then shows that the preferred rural and behavioral package changes the overall net impact to approximately -\$11.081 billion while preserving the main provider-side price-discipline effect.

Provider group	Backbone providers	Modeled providers	Net patient revenue	Outpatient exposure	Net impact before added access package	Impact % of NPR
<i>Children's / pediatric</i>	83	73	~\$6.15B	~\$2.96B	-\$235M	-3.8%
<i>General acute / academic</i>	618	604	~\$239.97B	~\$126.23B	-\$10.57B	-4.4%
<i>Rural / CAH / IHS</i>	68	68	~\$11.12B	~\$4.11B	-\$341M	-3.1%
<i>Specialty / behavioral / rehab</i>	143	141	~\$7.68B	~\$1.86B	-\$151M	-2.0%

The group-level results show that most of the absolute dollar impact is concentrated in general acute and academic providers because they dominate total revenue and outpatient exposure. Rural/CAH/IHS providers have smaller aggregate dollar exposure, but they remain policy-sensitive because of low-volume economics, geographic access concerns, and fixed-cost fragility. Behavioral and psychiatric providers show lower average percentage exposure in the base/base table, but the separate behavioral sensitivity exists because institutional psychiatric providers can have severe provider-level outliers even when the aggregate behavioral impact is small.

The model includes a formal audit layer. The audit does not simply report final impacts; it verifies how much of the model is based on standard revenue extraction, how much uses fallback logic, how many providers are included or excluded, and whether anomalous exposure records materially affect the result.

Verification category	Result / function
<i>Raw file verification</i>	Required FY2024 HCRIS alpha, numeric, and report files must exist before processing
<i>Provider backbone verification</i>	912 backbone providers identified
<i>Model inclusion verification</i>	886 modeled providers; 26 excluded
<i>Group coverage verification</i>	Coverage reported by cleaned provider group
<i>Revenue-source verification</i>	Standard, rural fallback, children's fallback, nonstandard fallback, total-only, missing, and failed-plausibility records tracked
<i>Net-to-gross plausibility</i>	Model-used net-to-gross outliers ≤ 0 or > 1.50 reported as zero in the publication audit
<i>Exposure-anomaly audit</i>	79 exposure-anomaly records identified
<i>Exposure-anomaly sensitivity</i>	Base/base result recomputed excluding exposure anomalies
<i>Rural sensitivity</i>	Rural access guarantee and implementation-floor options tested
<i>Behavioral sensitivity</i>	Psychiatric/behavioral repricing-scale and floor options tested
<i>Publication freeze</i>	Table outputs, methodology notes, and data-state summary written to publication folders

The publication audit reports 912 backbone providers, 886 modeled providers, 26 excluded providers, a modeled share of approximately 97.1 percent, zero model-used net-to-gross outliers outside the stated range, and 79 modeled exposure-anomaly records. The exposure-anomaly sensitivity shows that excluding anomaly-flagged records does not materially change the central conclusion.

Base/Base model version	Modeled providers	NPR	Outpatient exposure	Repricing pressure	UC/liquidi ty offset	Stabilization	Net impact	Impact % of NPR
<i>All modeled providers</i>	886	~\$264.92B	~\$135.15B	-\$13.52B	+\$1.91B	+\$0.30B	-\$11.30B	-4.3%
<i>Excluding exposure anomalies</i>	807	~\$261.88B	~\$133.51B	-\$13.35B	+\$1.89B	+\$0.29B	-\$11.17B	-4.3%

The Table 7F sensitivity preview shows that the all-provider and anomaly-excluded results are nearly identical as a percentage of net patient revenue. This validation is important because it indicates that the base/base result is not being driven by a small number of anomalous exposure records.

<i>Script</i>	<i>Main function</i>	<i>Data interacted with</i>	<i>Primary outputs</i>	<i>Why it exists</i>
<i>00_install_requi red_packages.R</i>	Installs and load-tests R packages	Local R package library	Console package diagnostics	Ensures reproducible software environment
<i>00_setup.R</i>	Defines project paths and FY2024 HCRIS inputs	Project folders and raw HCRIS file paths	Shared path/build variables	Prevents inconsistent hard-coding across scripts
<i>02_load_full_hc ris.R</i>	Loads and normalizes raw HCRIS files	Alpha, NMRC, RPT files	Processed .rds files and load summary	Converts raw CMS files into usable analysis objects
<i>04_create_rpt_b ase.R</i>	Builds report/provider base	Processed RPT file	RPT base .rds and CSV	Creates initial provider universe and full-year report flags
<i>05_create_provi der_master.R</i>	Builds provider master	RPT base and identity fields	Provider master	Creates central join file for all downstream variables
<i>05A_inspect_pr ovider_identity_ fields.R</i>	Inspects identity fields	Alpha/text HCRIS fields	Diagnostic identity mapping	Finds usable provider name/location fields
<i>05B_target_prov ider_identity_m apping.R</i>	Maps identity fields	Provider master and alpha fields	Provider master with identity	Supports classification validation
<i>05C_inspect_S2 00001_identity_ block.R</i>	Inspects identity block	S200001-like alpha fields	Diagnostic identity review	Confirms whether identity block is usable

<i>06_extract_beds_capacity.R</i>	Extracts beds/capacity	Numeric HCRIS/S-3 fields	Provider master with capacity	Supports scale checks and classification correction
<i>06A_inspect_S3_capacity_columns.R</i>	Inspects S-3 capacity fields	S-3 numeric fields	Diagnostic mapping	Avoids misreading capacity variables
<i>06B_validate_S3_utilization_mapping.R</i>	Validates S-3 mapping	Candidate capacity/utilization fields	Validation diagnostics	Confirms selected capacity fields behave plausibly
<i>07_extract_s10_uncompensated_care.R</i>	Extracts S-10 uncompensated care	S-10 numeric fields	Provider master with S-10	Supplies UC offset and safety-net burden variables
<i>08_extract_g2_g3_revenue_exposure.R</i>	Extracts revenue/exposure variables	G-2/G-3 numeric fields	Provider master with financials; G-2/G-3 profile	Builds net outpatient exposure base
<i>08A_diagnose_revenue_failures.R</i>	Diagnoses failed revenue extraction	Missing/unusable revenue records	Failure diagnostics	Determines whether fallback recovery is possible
<i>08B_diagnose_childrens_cah_revenue_failures.R</i>	Diagnoses children's/CAH failures	Pediatric and CAH-labeled revenue records	Failure diagnostics	Improves coverage for nonstandard provider types
<i>09_validate_provider_classification.R</i>	Cleans provider classification	Identity, beds, revenue, raw class	Classified provider master and review lists	Corrects raw HCRIS label problems
<i>10_create_stabilization_eligibility.R</i>	Creates stabilization tiers/caps	Classified provider master, UC, margin/exposure variables	Provider master with stabilization; tier summaries	Separates price discipline from targeted transition support
<i>11_create_provider_impact_scenarios.R</i>	Runs main scenario model	Provider master with stabilization	Table 7A–7D and provider-level scenario outputs	Calculates net provider impact under low/base/high cases
<i>11A_audit_provider_impact_coverage_and_exposure.R</i>	Audits coverage and exposure anomalies	Scenario outputs and provider financials	Coverage/exposure audit files	Tests data completeness and anomaly sensitivity

<i>12_format_table7_publication_outputs.R</i>	Formats publication outputs	Table 7 outputs and audits	Table 7 publication CSVs, notes, data-state summary	Freezes final publication data state
<i>13_provider_transition_protection_sensitivity.R</i>	Tests transition-protection variants	Base/base provider-level impacts	Transition/rural sensitivity outputs	Tests large-negative additions, pediatric floors, rural access guarantee, and tier enhancements
<i>13B_enhanced_rural_access_sensitivity.R</i>	Tests enhanced rural access protections	Rural/CAH/IHS records	Table 7K and 7L rural outputs	Treats rural support as permanent access-capacity protection rather than ordinary transition stabilization
<i>13C_behavioral_psychiatric_sensitivity.R</i>	Tests behavioral/psychiatric adjustments	Behavioral/specialty records	Table 7M–7P behavioral outputs	Corrects institutional behavioral repricing exposure and tests behavioral floors
<i>14_create_table7_access_protection_summary.R</i>	Summarizes preferred access package	Rural, behavioral, transition sensitivities	Table 7G access-protection summary	Combines preferred rural and behavioral protections into one policy package
<i>15_make_table7_publication_grade_tables.R</i>	Creates final publication tables	Table 7 CSVs and sensitivities	Publication-grade table files	Produces clean insertable tables
<i>16_make_table7_publication_table_images.R</i>	Creates table images	Publication-grade tables	Table image files and manifest	Supports direct insertion into white paper
<i>17_make_table7_compact_publication_figures.R</i>	Creates compact figures	Publication tables/summary files	Compact figure files and manifest	Produces visual summaries for paper layout

The provider-impact appendix supports four conclusions. First, UCC/HSE creates real provider-side repricing pressure, concentrated in routine and outpatient revenue rather than catastrophic/high-acuity care. Second, that pressure is partially offset by lower uncompensated-care exposure, improved household liquidity, and bounded transition stabilization. Third, access protection must be targeted rather than universal: rural

providers need durable access-capacity support because their fixed-cost and geographic constraints do not disappear after transition, while behavioral-health institutions need a separate repricing-exposure correction because institutional psychiatric care is not equivalent to ordinary routine outpatient shopping. Fourth, the preferred access package materially improves rural and behavioral tail risk while preserving the main provider-side price-discipline mechanism.

The appendix should not be overstated. It does not claim to predict provider closures, mergers, capital-market reactions, bargaining changes, service-line redesign, or long-run equilibrium prices. It does not replace future CBO, CMS, MedPAC, state-level, or actuarial review. Its value is that it converts the provider-impact question into a reproducible HCRIS-based scoring framework with transparent inputs, documented fallback logic, classification correction, scenario parameters, bounded stabilization, rural access protection, behavioral institutional adjustment, coverage audits, and anomaly sensitivity. That makes **Table 7** defensible as a static first-order provider-impact estimate and a publication-grade audit trail for the UCC/HSE provider-sector transition.

APPENDIX C: REFERENCES

1. Centers for Medicare & Medicaid Services, Office of the Actuary. *National health expenditure projections, 2023–2032*. Baltimore (MD): Centers for Medicare & Medicaid Services; 2024. Available from: <https://www.cms.gov/newsroom/press-releases/cms-releases-2023-2032-national-health-expenditure-projections>
2. Organisation for Economic Co-operation and Development. *Health at a glance 2023: OECD indicators*. Paris: OECD Publishing; 2023. doi:10.1787/7a7afb35-en. Available from: https://www.oecd.org/en/publications/health-at-a-glance-2023_7a7afb35-en.html
3. Congressional Budget Office. *Federal subsidies for health insurance: 2023 to 2033*. Washington (DC): Congressional Budget Office; 2023 Sep 28. Available from: <https://www.cbo.gov/publication/59273>
4. Centers for Medicare & Medicaid Services. *Data brief on Section 1332 waivers: state-based reinsurance programs*. Baltimore (MD): Centers for Medicare & Medicaid Services; 2022 Dec. Available from: <https://www.cms.gov/cciio/programs-and-initiatives/state-innovation-waivers/downloads/1332-data-brief-dec2022.pdf>
5. Whaley CM. *Health care price transparency: opportunities to improve affordability and data effectiveness*. Testimony before the Committee on Ways and Means, U.S. House of Representatives, 118th Cong., 1st sess.; 2023 May 16. Santa Monica (CA): RAND Corporation; 2023. Report No.: CT-A2767-1. Available from: <https://www.rand.org/pubs/testimonies/CTA2767-1.html>

6. Whaley CM, Briscoombe B, Kerber R, O'Neill B, Kofner A. *Prices paid to hospitals by private health plans: findings from Round 4 of an employer-led transparency initiative*. Santa Monica (CA): RAND Corporation; 2022. Report No.: RR-A1144-1. Available from: https://www.rand.org/pubs/research_reports/RR-A1144-1.html
7. Whaley CM, Dankert C, Richards M, Bravata DM. An employer-provider direct payment program is associated with lower episode costs. *Health Aff (Millwood)*. 2021;40(3):445–452. doi:10.1377/hlthaff.2020.01488. Available from: <https://www.healthaffairs.org/doi/10.1377/hlthaff.2020.01488>
8. Congressional Budget Office. *The prices that commercial health insurers and Medicare pay for hospitals' and physicians' services*. Washington (DC): Congressional Budget Office; 2022 Jan. Available from: <https://www.cbo.gov/publication/57422>
9. Papanicolaos I, Woskie LR, Jha AK. Health care spending in the United States and other high-income countries. *JAMA*. 2018;319(10):1024–1039. doi:10.1001/jama.2018.1150. Available from: <https://jamanetwork.com/journals/jama/fullarticle/2674671>
10. Gee E, Spiro T. Excess administrative costs burden the U.S. health care system. Washington (DC): Center for American Progress; 2019 Apr 8. Available from: <https://www.americanprogress.org/article/excess-administrative-costs-burden-u-s-health-care-system/>
11. Baranoff D, Baranoff EG, Sager TW, Shi B. Comparing apples with oranges: administrative expenses and finances in Medicare systems. *Am J Manag Care*. 2021;27(4):e123–e129. doi:10.37765/ajmc.2021.88532. Available from: <https://www.ajmc.com/view/comparing-apples-with-oranges-administrative-expenses-and-finances-in-medicare-systems>
12. Tseng P, Kaplan RS, Richman BD, Shah MA, Schulman KA. Administrative costs associated with physician billing and insurance-related activities at an academic health care system. *JAMA*. 2018;319(7):691–697. doi:10.1001/jama.2017.19148. Available from: <https://jamanetwork.com/journals/jama/fullarticle/2673148>
13. Cutler DM. Reducing administrative costs in U.S. health care. Washington (DC): The Hamilton Project, Brookings Institution; 2020. Available from: https://www.hamiltonproject.org/wp-content/uploads/2023/01/Cutler_OP_WEB.pdf
14. Scheinker D, Richman BD, Milstein A, Schulman KA. Reducing administrative costs in US health care: assessing single payer and its alternatives. *Health Serv Res*. 2021;56(4):615–625. doi:10.1111/1475-6773.13649. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC8313956/>
15. Government Accountability Office. *Private health insurance: state and federal oversight of provider networks varies*. GAO-23-105642. Washington (DC): U.S. Government Accountability Office; 2022. Available from: <https://www.gao.gov/products/gao-23-105642>
16. Department of the Treasury; Department of Labor; Department of Health and Human Services. Transparency in Coverage. Final rule. *Federal Register*.

2020;85(219):72158–72284. Available from:

<https://www.federalregister.gov/documents/2020/11/12/2020-24591/transparency-in-coverage>

17. Centers for Medicare & Medicaid Services. Transparency in Coverage final rule fact sheet (CMS-9915-F). Baltimore (MD): Centers for Medicare & Medicaid Services; 2020 Oct 29. Available from: <https://www.cms.gov/newsroom/fact-sheets/transparency-coverage-final-rule-fact-sheet-cms-9915-f>
18. Medicare Payment Advisory Commission. Medicare Advantage. In: *Health care spending and the Medicare program: a data book*. Washington (DC): Medicare Payment Advisory Commission; 2022 Jul. Section 9. Available from: https://www.medpac.gov/wp-content/uploads/2022/07/July2022_MedPAC_DataBook_Sec9_SEC.pdf
19. Richman BD, Kaplan RS, Kohli J, Purcell D, Shah M, Bonfrer I, Golden B, Hannam R, Mitchell W, Cehic D, Crispin G, Schulman KA. Billing and insurance-related administrative costs: a cross-national analysis. *Health Aff (Millwood)*. 2022;41(8):1098–1106. doi:10.1377/hlthaff.2022.00241. Available from: <https://www.healthaffairs.org/doi/10.1377/hlthaff.2022.00241>
20. FAIR Health. *FH Healthcare Indicators and FH Medical Price Index 2025: an annual view of place of service trends and medical pricing*. New York (NY): FAIR Health; 2025 Mar. Available from: <https://www.fairhealth.org/market-reports/healthcare-indicators>
21. Cooper Z, Craig SV, Gaynor M, Van Reenen J. *The price ain't right? Hospital prices and health spending on the privately insured*. NBER Working Paper No. 21815. Cambridge (MA): National Bureau of Economic Research; 2015 Dec, revised 2018 May. doi:10.3386/w21815. Available from: <https://www.nber.org/papers/w21815>
22. Joint Committee on Taxation. *Estimates of federal tax expenditures for fiscal years 2023–2027*. JCX-59-23. Washington (DC): Joint Committee on Taxation; 2023 Dec 7. Available from: <https://www.jct.gov/publications/2023/jcx-59-23/>
23. KFF. *2023 Employer Health Benefits Survey*. San Francisco (CA): KFF; 2023 Oct 18. Available from: <https://www.kff.org/health-costs/2023-employer-health-benefits-survey/>
24. Ministry of Health, Singapore. *MediShield Life*. Singapore: Ministry of Health; 2025 Feb 20 [cited 2026 May 18]. Available from: <https://www.moh.gov.sg/managing-expenses/schemes-and-subsidies/medishield-life/medishield-life/>
25. Organisation for Economic Co-operation and Development; World Health Organization. *Health at a glance: Asia/Pacific 2022*. Paris: OECD Publishing; 2022. doi:10.1787/c7467f62-en. Available from: https://www.oecd.org/en/publications/health-at-a-glance-asia-pacific-2022_c7467f62-en.html
26. National Health Care Institute. *The Dutch health care system*. Diemen (Netherlands): National Health Care Institute; [cited 2026 May 18]. Available from: <https://english.zorginstituutnederland.nl/about-us/healthcare-in-the-netherlands>

27. Organisation for Economic Co-operation and Development; European Observatory on Health Systems and Policies. *Netherlands: country health profile 2023*. Paris: OECD Publishing; 2023. doi:10.1787/3110840c-en. Available from: https://www.oecd.org/en/publications/2023/12/netherlands-country-health-profile-2023_33a4c54e.html
28. Organisation for Economic Co-operation and Development. *Health at a glance 2023: OECD indicators — Switzerland country note*. Paris: OECD Publishing; 2023. Available from: https://www.oecd.org/en/publications/health-at-a-glance-2023_7a7afb35-en/switzerland_12095b69-en.html
29. Centers for Medicare & Medicaid Services. *Health expenditures by age and sex*. Baltimore (MD): Centers for Medicare & Medicaid Services; 2024 Sep 10. Available from: <https://www.cms.gov/data-research/statistics-trends-and-reports/national-health-expenditure-data/age-and-sex>
30. Surgery Center of Oklahoma. *Surgery prices*. Oklahoma City (OK): Surgery Center of Oklahoma; [cited 2026 May 18]. Available from: <https://surgerycenterok.com/surgery-prices/>
31. Piniecki R. Statement of Ron Piniecki, Co-Founder and Medical Director, Wellbridge Surgical. In: U.S. House of Representatives, Committee on Ways and Means. *Hearing on health care price transparency: a patient's right to know*. 118th Cong., 1st sess.; 2023 May 16. Washington (DC): U.S. House Committee on Ways and Means; 2023. Available from: <https://waysandmeans.house.gov/wp-content/uploads/2023/09/WEBSITE-May-16-2023-Transcript.pdf>
32. Gruber J. The tax exclusion for employer-sponsored health insurance. *Natl Tax J*. 2011;64(2, Part 2):511–530. doi:10.17310/ntj.2011.2S.04. Available from: <https://www.nber.org/papers/w15766>
33. Congressional Budget Office. *The effects of the Affordable Care Act on employment-based health insurance*. Washington (DC): Congressional Budget Office; 2012 Mar 15. Available from: <https://www.cbo.gov/publication/43090>
34. Baicker K, Chandra A. The labor market effects of rising health insurance premiums. *J Labor Econ*. 2006;24(3):609–634. doi:10.1086/505049. Available from: <https://www.nber.org/papers/w11160>
35. Cooper Z, Craig SV, Gaynor M, Van Reenen J. The price ain't right? Hospital prices and health spending on the privately insured. *Q J Econ*. 2019;134(1):51–107. doi:10.1093/qje/qjy020. Available from: <https://academic.oup.com/qje/article/134/1/51/5090426>
36. Whaley CM, Schneider Chafen J, Pinkard S, Kellerman G, Bravata DM, Kocher R, Sood N. Association between availability of health service prices and payments for these services. *JAMA*. 2014;312(16):1670–1676. doi:10.1001/jama.2014.13373. Available from: <https://jamanetwork.com/journals/jama/fullarticle/1917438>
37. RAND Corporation. *40 years of the RAND Health Insurance Experiment*. Santa Monica (CA): RAND Corporation; [cited 2026 May 18]. Available from: <https://www.rand.org/health/projects/HIE-40.html>

38. Glied SA, Remler DK, Springsteen M. Health savings accounts no longer promote consumer cost-consciousness. *Health Aff (Millwood)*. 2022;41(6):798–806. doi:10.1377/hlthaff.2021.01954. Available from: <https://www.healthaffairs.org/doi/10.1377/hlthaff.2021.01954>
39. Shafer PR, Dusetzina SB, Anderson DM, Gordon SH. High-deductible health plans and use of free preventive services. *Health Serv Res*. 2023;58(3):627–636. doi:10.1111/1475-6773.14142. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC10278429/>
40. Fronstin P, Roebuck MC. The impact of expanding pre-deductible coverage in HSA-eligible health plans on medication adherence. Washington (DC): Employee Benefit Research Institute; 2024 Jun 27. Available from: <https://www.ebri.org/content/the-impact-of-expanding-pre-deductible-coverage-in-hsa-eligible-health-plans-on-medication-adherence>
41. Gidwani R, Yank V, Asch SM, Burgette L, Kofner A, Peltz A, Wagner Z. High-deductible health plans and receipt of guideline-concordant care for adults with chronic illness. *JAMA Netw Open*. 2025;8(4):e258045. doi:10.1001/jamanetworkopen.2025.8045. Available from: <https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2833325>
42. Corlette S, Monahan C, Keith K, Lucia K. *Selling health insurance across state lines: an assessment of state laws and implications for improving choice and affordability of coverage*. Robert Wood Johnson Foundation Rapid Response Paper; 2012 Oct 1. Available from: <https://ssrn.com/abstract=2160517>
43. Corlette S, Lucia K. Selling health insurance across state lines is unlikely to lower costs or improve choice. *To the Point* [Internet]. Commonwealth Fund; 2017 Apr 5. doi:10.26099/c4fp-kb46. Available from: <https://www.commonwealthfund.org/blog/2017/selling-health-insurance-across-state-lines-unlikely-lower-costs-or-improve-choice>
44. *National Federation of Independent Business v Sebelius*, 567 US 519 (2012). Commerce Clause discussion. Available from: <https://supreme.justia.com/cases/federal/us/567/519/>
45. *Kentucky Association of Health Plans, Inc v Miller*, 538 US 329 (2003). ERISA and federal pre-emption. Available from: <https://supreme.justia.com/cases/federal/us/538/329/>
46. Agency for Healthcare Research and Quality. *Medical Expenditure Panel Survey Household Component consolidated data files, 2018–2022*. Rockville (MD): Agency for Healthcare Research and Quality; [cited 2026 May 18]. Available from: https://meps.ahrq.gov/mepsweb/data_stats/download_data_files.jsp
47. Mirel LB, Carper K. *Trends in health care expenditures for children under age 18: 2001, 2006, and 2011*. Statistical Brief No. 428. Rockville (MD): Agency for Healthcare Research and Quality; 2014 Jan. Available from: https://meps.ahrq.gov/data_files/publications/st428/stat428.shtml
48. Newacheck PW, Kim SE. A national profile of health care utilization and expenditures for children with special health care needs. *Arch Pediatr Adolesc Med*.

2005;159(1):10–17. doi:10.1001/archpedi.159.1.10. Available from:
<https://pubmed.ncbi.nlm.nih.gov/15630052/>

49. Centers for Medicare & Medicaid Services, Office of the Actuary, National Health Statistics Group. *National Health Expenditure Accounts (NHEA): methodology paper, 2024: definitions, sources, and methods*. Baltimore (MD): Centers for Medicare & Medicaid Services; 2026 Jan. Available from:
<https://www.cms.gov/files/document/definitions-sources-methods.pdf>
50. Agency for Healthcare Research and Quality. Medical Expenditure Panel Survey Household Component: Child Health and Preventive Care variables, 2016–2022 public-use files. Rockville (MD): Agency for Healthcare Research and Quality; [cited 2026 May 18]. Author reconstruction from MEPS-HC public-use files. Available from: https://meps.ahrq.gov/mepsweb/data_stats/download_data_files.jsp

Avoided ER Over-utilization_2026

51. UnitedHealth Group. 18 Million Avoidable Hospital Emergency Department Visits Add \$32 Billion in Costs to the Health Care System Each Year. UnitedHealth Group; July 2019. Available from:
<https://www.unitedhealthgroup.com/content/dam/UHG/PDF/2019/UHG-Avoidable-ED-Visits.pdf>
52. Moore BJ, Liang L, Barrett ML. Costs of Treat-and-Release Emergency Department Visits in 2021. HCUP Statistical Brief #311. Agency for Healthcare Research and Quality; Sept 2024. Available from: <https://hcup-us.ahrq.gov/reports/statbriefs/sb311-ED-visit-costs-2021.pdf>

Bad DebtReduction_2026 / CharityCareTaxOffset_2026

53. American Hospital Association. *Uncompensated hospital care cost fact sheet*. Chicago (IL): American Hospital Association; 2020. (using 2018–2019 data). Available from: <https://www.aha.org/system/files/media/file/2020/01/2020-Uncompensated-Care-Fact-Sheet.pdf>
54. Medicaid and CHIP Payment and Access Commission. Chapter 3: Annual analysis of disproportionate share hospital allotments to states. In: *Report to Congress on Medicaid and CHIP*. Washington (DC): MACPAC; 2022 Mar. Available from: <https://www.macpac.gov/wp-content/uploads/2022/03/Chapter-3-Annual-Analysis-of-Disproportionate-Share-Hospital-Allotments-to-States.pdf>
55. Godwin J, Levinson Z, Hulver S. *The estimated value of tax exemption for nonprofit hospitals was about \$28 billion in 2020*. San Francisco (CA): KFF; 2023 Mar 14. Available from: <https://www.kff.org/health-costs/the-estimated-value-of-tax-exemption-for-nonprofit-hospitals-was-about-28-billion-in-2020/>
56. Plummer E, Bai G, Anderson GF. Estimation of tax benefit of US nonprofit hospitals from federal, state, and local tax exemption. *JAMA*. 2024;331(3):231–240.

doi:10.1001/jama.2023.23710. Available from:
<https://jamanetwork.com/journals/jama/fullarticle/2824116>

COOPAdminElim_2026, RiskCorridorSavings_2026, ExchangeITMaintenance_2026

57. Mach AL, Driessen GA. *Consumer Operated and Oriented Plan (CO-OP) Program: frequently asked questions*. CRS Report No. R44414. Washington (DC): Congressional Research Service; 2016 Jul 13. Available from: <https://www.everycrsreport.com/reports/R44414.html>
58. *Maine Community Health Options v United States*, 590 US ___, 140 S Ct 1308 (2020). Available from: <https://supreme.justia.com/cases/federal/us/590/18-1023/>
59. Centers for Medicare & Medicaid Services. *Health insurance exchanges 2024 open enrollment report*. Baltimore (MD): CMS; 2024. Available from: <https://www.cms.gov/files/document/health-insurance-exchanges-2024-open-enrollment-report-final.pdf>

PreventiveCareOffset_2026

60. Cohen JT, Neumann PJ, Weinstein MC. Does preventive care save money? Health economics and the presidential candidates. *N Engl J Med*. 2008;358(7):661–663. doi:10.1056/NEJMp0708558. Available from: <https://www.nejm.org/doi/full/10.1056/NEJMp0708558>
61. Baicker K, Chandra A. Can prevention save money? *JAMA Health Forum*. 2025;6(4):e251464. doi:10.1001/jamahealthforum.2025.1464. Available from: <https://jamanetwork.com/journals/jama-health-forum/fullarticle/2832497>
62. Newhouse JP. *An ounce of prevention*. NBER Working Paper No. 27553. Cambridge (MA): National Bureau of Economic Research; 2020 Jul. doi:10.3386/w27553. Available from: <https://www.nber.org/papers/w27553/>

EndOfLifeIntensitySavings_2026

63. French E, Kelly E. Medical spending around the developed world. *Fiscal Studies*. 2016;37(3–4):327–344. doi:10.1111/j.1475-5890.2016.12127. Available from: <https://onlinelibrary.wiley.com/doi/10.1111/j.1475-5890.2016.12127>
64. De Nardi M, French E, Jones JB, McCauley J. Medical spending of the US elderly. *Fiscal Studies*. 2016;37(3–4):717–747. doi:10.1111/j.1475-5890.2016.12106. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC6680320/>
65. French E, Jones JB, Kelly E, McCauley J. End-of-life medical expenses. In: Ferraro KF, Carr D, editors. *Handbook of aging and the social sciences*. 9th ed. London: Academic Press; 2021. p. 393–410. doi:10.1016/B978-0-12-815970-5.00024-3. Available from: <https://www.sciencedirect.com/science/article/pii/B9780128159705000243>

MedicationAdherenceSavings_2026

66. Bosworth HB, Granger BB, Mendys P, Brindis R, Burkholder R, Czajkowski SM, et al. Medication adherence: a call for action. *Am Heart J*. 2011;162(3):412–424. doi:10.1016/j.ahj.2011.06.007. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC3947508/>
67. Ding A, Tilson HH, Jing Y, et al. The role of integrated medical and prescription drug plans in addressing racial and ethnic disparities in medication adherence. *J Manag Care Spec Pharm*. 2022;28(3):379–390. doi:10.18553/jmcp.2022.28.3.379. Available from: <https://www.jmcp.org/doi/abs/10.18553/jmcp.2022.28.3.379>

DiagnosticDuplicationSavings_2026

68. Kamat AB, Midgley S, Kimbrell K. Duplication of radiology imaging studies in the emergency department: what is the cost? *Emerg Med J*. 2015;32(2):144–148. doi:10.1136/emered-2013-203136. Available from: <https://emj.bmj.com/content/32/2/144>
69. Jung HY, Vest JR, Unruh MA, Kern LM, Kaushal R. Use of health information exchange and repeat imaging costs. *Health Serv Res*. 2015;50(1):219–237. doi:10.1111/1475-6773.12218. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC4722040/>
70. Bailey JE, Pope RA, Elliott EC, Wan JY, Waters TM, Frisse ME. Health information exchange reduces repeated diagnostic imaging for back pain. *Ann Emerg Med*. 2013;62(1):16–24. doi:10.1016/j.annemergmed.2013.01.006. Available from: <https://pubmed.ncbi.nlm.nih.gov/23465552/>
71. Gottlieb M, et al. Cross-sectional analysis of avoidable emergency department visits before and during the COVID-19 pandemic. *BMJ Open*. 2023. Available from: <https://pmc.ncbi.nlm.nih.gov/articles/PMC9883066/>
72. Moran K. *How public programs affect non-profit hospital charity care*. Working paper; 2024 Jun. Available from: https://kelseymoran.github.io/files/moran_public_private_charity.pdf

TaxableWageIncrease_2026

73. Newell P. *Easing the stigma of public coverage: workers with health coverage at their jobs get significant government aid too*. New York (NY): United Hospital Fund; 2019 Sep 26. Available from: <https://uhfnyc.org/publications/publication/easing-stigma-of-public-coverage/>
74. Congressional Budget Office. *Reduce tax subsidies for employment-based health insurance*. In: *Options for reducing the deficit: 2025 to 2034*. Washington (DC): Congressional Budget Office; 2024 Dec 12. Available from: <https://www.cbo.gov/budget-options/60953>

75. Blumberg LJ, Holahan J, Mermin GB. *The ACA's "Cadillac" tax versus a cap on the tax exclusion of employer-based health benefits: is this a battle worth fighting?* Washington (DC): Urban Institute; 2015. Available from: <https://www.urban.org/sites/default/files/publication/76821/2000482-The-ACAs-Cadillac-Tax-Versus-a-Cap-on-the-Tax-Exclusion-of-Employer-Based-Health-Benefits.pdf>

DisabilityClaimReduction_2026

76. Baicker K, Finkelstein A, Song J, Taubman S. The impact of Medicaid on labor market activity and program participation: evidence from the Oregon Health Insurance Experiment. *Am Econ Rev*. 2014;104(5):322–328. doi:10.1257/aer.104.5.322. Available from: <https://www.aeaweb.org/articles?id=10.1257/aer.104.5.322>
77. KFF. *The connection between Social Security disability benefits and health coverage through Medicaid and Medicare*. San Francisco (CA): KFF; 2024 Oct 24. Available from: <https://www.kff.org/medicare/the-connection-between-social-security-disability-benefits-and-health-coverage-through-medicaid-and-medicare/>

Other Sources

78. Agency for Healthcare Research and Quality. *Medical Expenditure Panel Survey HC-251: 2023 Full-Year Consolidated Data File*. Rockville (MD): AHRQ; 2025. Available from: https://meps.ahrq.gov/data_stats/download_data/pufs/h251/h251doc.shtml
79. Centers for Medicare & Medicaid Services. *Hospital price transparency*. MLN Fact Sheet MLN7215754. Baltimore (MD): CMS; 2026 Mar. Available from: <https://www.cms.gov/files/document/mln7215754-hospital-price-transparency.pdf>
80. Cram P, Cram E, Antos J, et al. "Availability of Prices for Shoppable Services on Hospital Internet Sites." *The American Journal of Managed Care*. 2021;27(12). Available from: <https://pubmed.ncbi.nlm.nih.gov/34889585/>
81. Linde S, Egede LE. Hospital price transparency in the US: an examination of chargemaster, cash, and negotiated price variation for 14 common procedures. *Med Care*. 2022;60(10):768–774. doi:10.1097/MLR.0000000000001761. Available from: <https://pubmed.ncbi.nlm.nih.gov/35948351/>
82. Wang Y, Meiselbach MK, Cox JS, Anderson GF, Bai G. The relationships among cash prices, negotiated rates, and chargemaster prices for shoppable hospital services. *Health Aff (Millwood)*. 2023;42(4):516–525. doi:10.1377/hlthaff.2022.01087. Available from: <https://www.healthaffairs.org/doi/10.1377/hlthaff.2022.01087>
83. Blumberg LJ, Holahan J, Buettgens M, Gangopadhyaya A, Garrett B, Shartz A, et al. *From incremental to comprehensive health insurance reform: how various reform options compare on coverage and costs*. Washington (DC): Urban Institute; 2019 Oct. Available from:

https://www.urban.org/sites/default/files/2019/10/15/from_incremental_to_comprehensive_health_insurance_reform-how_various_reform_options_compare_on_coverage_and_costs.pdf

84. Liu JL, Eibner C. *National health spending estimates under Medicare for All*. Santa Monica (CA): RAND Corporation; 2019. Report No.: RR-3106. Available from: https://www.rand.org/pubs/research_reports/RR3106.html
85. Blahous C. *The costs of a national single-payer healthcare system*. Arlington (VA): Mercatus Center at George Mason University; 2018 Jul. Available from: <https://www.mercatus.org/research/working-papers/costs-national-single-payer-healthcare-system>
86. Friedman G. *Funding HR 676: the Expanded and Improved Medicare for All Act*. Amherst (MA): Political Economy Research Institute; 2013 Jul 31. Available from: https://www.pnhp.org/system/assets/drupal/Funding%20HR%20676_Friedman_7.31.13_proofed.pdf
87. Pollin R, Heintz J, Arno P, Wicks-Lim J, Ash M. *Economic analysis of Medicare for All*. Amherst (MA): Political Economy Research Institute; 2018 Nov. Available from: <https://peri.umass.edu/publication/economic-analysis-of-medicare-for-all/>
88. Committee for a Responsible Federal Budget. *Choices for financing Medicare for All*. Washington (DC): Committee for a Responsible Federal Budget; 2020 Mar 17. Available from: <https://www.crfb.org/papers/choices-financing-medicare-all>
89. Neuman T, Pollitz K, Tolbert J. *Medicare-for-All and public plan buy-in proposals: overview and key issues*. San Francisco (CA): KFF; 2018 Oct 9. Available from: <https://www.kff.org/affordable-care-act/medicare-for-all-and-public-plan-buy-in-proposals-overview-and-key-issues/>
90. KFF. 2025 Employer Health Benefits Survey. San Francisco: KFF; 2025 [cited 2026 Apr 30]. Available from: <https://www.kff.org/EHBS>
91. U.S. Bureau of Labor Statistics. *Employer Costs for Employee Compensation — December 2025*. Washington (DC): U.S. Bureau of Labor Statistics; 2026 Mar. Available from: <https://www.bls.gov/news.release/ecec.nr0.htm>
92. Centers for Medicare & Medicaid Services. *National Health Expenditures 2024 Highlights*. Baltimore (MD): CMS; 2026 [cited 2026 Apr 30]. Available from: <https://www.cms.gov/files/document/highlights.pdf>
93. Office of Management and Budget. *Analytical Perspectives, Budget of the United States Government, Fiscal Year 2027: Tax Expenditures*. Washington (DC): Executive Office of the President; 2026 Apr 3 [cited 2026 Apr 30]. Available from: <https://www.govinfo.gov/app/details/BUDGET-2027-PER/BUDGET-2027-PER-4-6>
94. U.S. Department of the Treasury. *Tax Expenditures, Fiscal Year 2027*. Washington (DC): U.S. Department of the Treasury; 2026 [cited 2026 Apr 30]. Available from: <https://home.treasury.gov/system/files/131/Tax-Expenditures-FY2027.pdf>
95. Internal Revenue Service. *S corporation compensation and medical insurance issues*. Washington (DC): Internal Revenue Service; 2026 [cited 2026 Apr 30]. Available from: <https://www.irs.gov/businesses/small-businesses-self-employed/s-corporation-compensation-and-medical-insurance-issues>

96. Centers for Medicare & Medicaid Services. Health Reimbursement Arrangements. Baltimore (MD): CMS; [cited 2026 Apr 30]. Available from: <https://www.cms.gov/marketplace/private-health-insurance/health-reimbursement-arrangements>
97. House Committee on Ways and Means. Full Committee Members. Washington (DC): U.S. House of Representatives; 2026 [cited 2026 Apr 30]. Available from: <https://waysandmeans.house.gov/members/>
98. House Committee on Energy and Commerce. Representatives. Washington (DC): U.S. House of Representatives; 2026 [cited 2026 Apr 30]. Available from: <https://energycommerce.house.gov/representatives>
99. United States Senate Committee on Finance. Membership. Washington (DC): U.S. Senate; 2026 [cited 2026 Apr 30]. Available from: <https://www.finance.senate.gov/about/membership>
100. Jiang JX, Makary MA, Bai G. Commercial negotiated prices for CMS-specified shoppable radiology services in U.S. hospitals. *Radiology*. 2022;302(3):622–624. doi:10.1148/radiol.2021211948. Available From: <https://pubs.rsna.org/doi/full/10.1148/radiol.2021211948>
101. TheStreet. Paying cash for U.S. medical care could save you a fortune. 2026. Available From: <https://www.thestreet.com/personal-finance/paying-cash-for-u-s-medical-care-could-save-you-a-fortune>

ANNEX A: Draft Section-by-Section Legislative Outline

Universal Catastrophic Coverage with Health Savings Empowerment Act

Title I — Definitions and General Provisions

Sec. 101. Definitions.

Establishes statutory definitions for terms used throughout the Act, including “catastrophic plan,” “eligible individual,” “lawfully present resident,” “universal catastrophic credit,” “routine care,” “annual HSA allocation,” “chronic-condition funding track,” “ultra-high-cost therapies,” “rural hospital,” “rural access-capacity payment,” “temporary rural implementation floor,” “catastrophic threshold,” “federal reinsurance offset,” “HSA administrator,” “supplemental coverage,” “rural household access supplement,”

“behavioral-health institutional provider,” and “provider-impact stabilization.”

Sec. 102. Rule of Construction; Preservation of Private Comprehensive Coverage.

Provides that nothing in this Act shall be construed to prohibit or restrict the offering, sale, or purchase of private comprehensive health insurance; specifies that the federal framework established by this Act pertains to financing and tax treatment of catastrophic protection and HSAs, not to the legality of broader private coverage.

Sec. 103. Hyde-Style and Conscience Protections.

Prohibits the use of federal catastrophic credits, federal HSA deposits, and federal reinsurance payments for abortion services except in cases of rape, incest, or where the life of the mother is endangered, consistent with existing federal law; affirms that no individual, employer, insurer, or health-care entity shall be required, as a condition of participation in programs under this Act, to provide, pay for, or refer for services contrary to sincerely held religious or moral convictions.

Title II — Universal Catastrophic Coverage (UCC)

Sec. 201. Establishment of Universal Catastrophic Coverage Standard.

Creates a national catastrophic coverage standard for eligible individuals, specifying minimum actuarial characteristics, benefits above the catastrophic threshold, and uniform federal rules for qualifying catastrophic plans.

Sec. 202. Covered Benefits Above the Catastrophic Threshold.

Defines covered catastrophic benefits to include hospital inpatient care, emergency and trauma services, major surgery, intensive care, and other high-severity medical events that occur above the

national catastrophic deductible, as specified by the Secretary of Health and Human Services.

Sec. 203. National Catastrophic Deductible and Indexation.

Sets the initial national catastrophic deductible and family threshold by statute and establishes a formula for annual indexation. The Secretary shall publish annual indexed amounts, apply rounding rules, and report to Congress on deductible growth, household exposure, and actuarial effects. Any material change outside the statutory indexation formula requires congressional authorization or an automatic adjustment mechanism under Title XI.

Sec. 204. Ultra-High-Cost Therapy Exclusion and Coordination.

Provides that ultra-high-cost gene therapies and orphan biologics are not financed through UCC catastrophic credits or reinsurance under this Act but remain eligible for coverage and payment under existing Medicare, Medicaid aged/blind/disabled categories, and designated orphan-drug or rare-disease carve-out mechanisms, as determined by statute or subsequent regulation.

Sec. 205. Optional State-Administered Public Catastrophic Plan.

Authorizes states to operate a state-administered catastrophic plan that adheres to the federal catastrophic standard, deductible, and reinsurance parameters; such plans may receive federal catastrophic credits on behalf of eligible enrollees but may not offer subsidized benefits that reduce the effective catastrophic threshold.

Title III — Health Savings Empowerment (HSE)

Sec. 301. Universal Eligibility for Health Savings Accounts.

Establishes eligibility for HSAs for all eligible individuals enrolled in UCC-compliant catastrophic plans, notwithstanding prior-law high-

deductible health plan requirements; designates qualified HSA administrators and custodial rules.

Sec. 302. Annual HSA Funding and Allocation Rules.

Creates a federal HSA funding structure providing base deposits, income-indexed deposits, pediatric deposits, chronic-condition supplements, and rural household access supplements where applicable. Pediatric deposits are set by age band and indexed annually to preserve real purchasing power. Exact statutory amounts are specified in the Act and reflected in the scoring tables.

Sec. 303. Definition and Treatment of Routine Care.

Defines “routine care” as services with predictable utilization and low financial variance, including primary care visits, standard laboratory testing, low-complexity imaging, routine specialist visits, preventive services, common outpatient behavioral-health services, and chronic-condition management services; specifies that routine care is ordinarily paid directly from HSAs and is not covered under the catastrophic benefit.

Sec. 304. Direct Primary Care Compatibility.

Confirms that HSA funds may be used to pay for direct primary care arrangements and that such arrangements do not constitute insurance for purposes of this Act; clarifies interaction with catastrophic coverage requirements.

Sec. 305. HSA Guardrails and Permitted Uses.

Specifies qualifying medical expenses, restrictions on non-medical withdrawals, penalties for misuse, rollover rules, and reporting obligations for HSA custodians; authorizes the Secretary of the Treasury to issue regulations governing compliance and data reporting.

Sec. 306. Rural Household Access Supplement.

Authorizes additional HSA deposits for eligible households residing in designated rural, frontier, medically underserved, or low-density areas. Funds are limited to HSE-eligible routine, chronic-care, telehealth, behavioral-health, monitoring, and access-related medical services and may not be used to purchase supplemental insurance riders, reduce the catastrophic deductible, finance comprehensive wraparound coverage, or subsidize nonmedical consumption.

Title IV — Employer Exclusion Phase-Out and Wage Adjustment**Sec. 401. Ten-Year Phase-Out of Employer Tax Exclusion for Health Benefits.**

Provides for a staged reduction of the exclusion from taxable income for employer-sponsored health benefits over ten years, with a fixed annual percentage step-down schedule, and directs additional wage income to be subject to standard income and payroll taxation.

Sec. 402. Preservation of Existing Collective Bargaining Agreements.

Allows multi-year collective bargaining agreements in effect as of the date of enactment to maintain existing health-benefit structures until the expiration of the contract term; applies the new tax treatment prospectively at renewal.

Sec. 403. Employer Contributions to HSAs During Transition.

Permits employers to make HSA contributions on a tax-favored basis during the transition period, subject to annual caps and nondiscrimination rules; clarifies interaction between employer contributions, the universal catastrophic credit, and the phase-out of the exclusion.

Title V — Federal Reinsurance Architecture

Sec. 501. Establishment of a Federal Catastrophic Reinsurance Layer.

Creates a federal reinsurance program that pays a defined share, such as 80 percent, of allowed costs above a specified attachment point, such as \$50,000 per enrollee per year, with parameters set by the Secretary based on actuarial analysis and subject to statutory budget constraints.

Sec. 502. State Coordination and Waiver Integration.

Authorizes coordination with ACA section 1332 State Innovation Waivers and SSA section 1115 Medicaid demonstration authority, subject to statutory limits, federal approval, budget-neutrality requirements where applicable, and the UCC/HSE catastrophic standard. Section 1332 pass-through funding may be coordinated with catastrophic-plan stabilization, HSA support, or state-level transition assistance where permitted by law. Section 1115 authority remains limited to Medicaid demonstrations and may be used only for Medicaid-related transition, chronic-care, rural-access, or population-specific coordination approved under existing Medicaid demonstration standards.

Sec. 503. Administrative Requirements and Reporting.

Sets reporting, auditing, and transparency requirements for reinsurance payments; mandates annual public reports on reinsurance performance, geographic distribution of claims, and fiscal impact relative to projected outlays.

Title VI — Medicaid LTSS, ABD, and Medically Complex Populations

Sec. 601. Protection of Medicaid Long-Term Services and Supports (LTSS) and Aged, Blind, and Disabled (ABD)

Populations.

States that eligibility, benefits, and financing for Medicaid LTSS and ABD populations are unaffected by this Act, except for coordination provisions explicitly authorized elsewhere; affirms that these populations remain under existing federal-state Medicaid structures.

Sec. 602. Transition of Medicaid Expansion Adults into UCC/HSE.

Provides for a structured transition of ACA Medicaid expansion adults into UCC catastrophic coverage combined with HSE accounts, with state-level implementation options and federal guidance on timing and coordination.

Sec. 603. Protection for Medically Complex Children.

Clarifies that medically complex children currently served under Medicaid waiver programs or specialized categories remain eligible for those programs and that UCC/HSE pediatric provisions and chronic-condition funding tracks are coordinated with, and do not diminish, such coverage.

Sec. 604. Pediatric HSE Coordination and Pediatric Provider Protection.

Coordinates pediatric HSA deposits with CHIP, Medicaid pediatric coverage, and medically complex children's programs. Clarifies that pediatric HSE support supplements, and does not displace, existing CHIP or Medicaid protections. Requires provider-impact monitoring for children's hospitals and pediatric specialty providers during implementation.

Title VII — Chronic-Condition and Behavioral-Health Funding Track

Sec. 701. Chronic Spending Benchmark and Classification.

Defines criteria for classification into the chronic-condition funding track based on multi-year expenditure patterns and clinical criteria, using data sources such as the CMS Chronic Conditions Warehouse; delegates methodological detail to the Secretary of Health and Human Services.

Sec. 702. Enhanced HSA Seeding for Chronic Conditions.

Sets statutory minimum HSA deposits for enrollees meeting chronic-condition criteria, calibrated to median disease-management costs and indexed to preserve real value over time.

Sec. 703. Condition-Based Care Bundles.

Authorizes the use of condition-based annual bundles for chronic care management, including diabetes, COPD, heart failure, oncology monitoring, behavioral-health continuity, addiction-treatment follow-up, and other chronic-condition categories, with fixed-price arrangements and reporting standards for quality and cost outcomes.

Sec. 704. Income-Protection Cap for Chronic Out-of-Pocket Liability.

Establishes a cap on net out-of-pocket spending for certified chronic-disease management services below the catastrophic threshold, with parameters linked to income and expenditure benchmarks; specifies that the catastrophic plan design remains uniform and that adjustments occur through financing, not benefit variation.

Sec. 705. Behavioral-Health Continuity and Institutional Stabilization.

Authorizes behavioral-health routine-care support, HSA-compatible outpatient behavioral-health services, and institutional behavioral-

health stabilization for qualifying psychiatric and behavioral-health providers identified through provider-impact analysis. Stabilization support is provider-side, bounded, and tied to access preservation, transparency, and reporting requirements.

Title VIII — Eligibility, Verification, and Program Integrity

Sec. 801. Eligibility for Catastrophic Credits and Federal HSA Deposits.

Defines eligible individuals as:

1. U.S. citizens;
2. lawful permanent residents; and
3. long-term lawfully present non-citizens who have maintained lawful presence for at least five years and filed federal income tax returns for at least three of the preceding five years.

Explicitly excludes individuals in unlawful immigration status, asylum applicants, humanitarian parolees, temporary visitors, and other categories not made eligible by statute from federal catastrophic credits, federal HSA deposits, and reinsurance-funded benefits under this Act.

Sec. 802. Eligibility Verification Using Federal Data Systems.

Requires the use of IRS, SSA, and DHS data to verify eligibility for catastrophic credits and federal HSA deposits; authorizes data-matching procedures and establishes standards for identity resolution, error correction, appeal, and administrative review. Self-attestation alone is insufficient to establish eligibility for federal catastrophic credits, federal HSA deposits, or reinsurance-funded benefits.

Sec. 803. Program Integrity, Anti-Fraud Mechanisms, and Penalties.

Provides for GAO and OIG audits of program operations, defines civil and criminal penalties for fraudulent claims or misuse of HSA funds, and authorizes the Secretary of the Treasury and the Secretary of Health and Human Services to suspend or recoup payments where fraud or abuse is identified.

Sec. 804. Emergency-Care Noninterference.

Provides that nothing in this Act alters obligations under EMTALA or other applicable federal emergency-care requirements. Ineligibility for federal catastrophic credits, federal HSA deposits, or reinsurance subsidies does not alter hospital obligations to provide emergency screening and stabilizing treatment where required under existing law.

Title IX — Rural Health Stability and Access Protection

Sec. 901. Rural Access-Capacity Payment.

Establishes a permanent rural access-capacity payment for qualifying rural, frontier, Critical Access Hospital, and other designated low-volume access providers. The payment equals 2 percent of net patient revenue, capped at \$5 million per qualifying provider, subject to eligibility, reporting, and access criteria established by the Secretary.

Sec. 902. Temporary Rural Implementation Floor.

Provides that, during the implementation period, no qualifying rural provider receiving the rural access-capacity payment shall experience a modeled UCC/HSE transition impact below -1.5 percent of net patient revenue after application of the permanent rural access-capacity payment. The implementation floor is

temporary, reviewable, and limited to transition-period access protection.

Sec. 903. Rural Eligibility, Reporting, and Guardrails.

Defines eligibility for rural access protection using objective geographic, low-volume, access-necessity, and provider-class criteria. Requires participating providers to comply with price transparency, HCRIS-compatible reporting, access monitoring, and restrictions on the use of stabilization support. Payments may not be used to reduce patient deductibles, finance supplemental insurance wraparound coverage, preserve opaque facility-fee billing, subsidize unrelated expansion, or increase commercial rates.

Sec. 904. Rural Virtual Access Network and Chronic-Care Bundle Integration.

Authorizes qualifying rural providers to participate in rural virtual access networks, telehealth-supported specialist arrangements, and HSA-purchasable chronic-care bundles. Establishes standards for local originating-site participation, care coordination, telehealth integration, advance price disclosure, and quality reporting.

Sec. 905. Rural Transformation Track.

Creates a voluntary, time-limited rural transformation pathway for qualifying providers receiving stabilization support. Eligible uses include transparent price-posting infrastructure, telehealth network integration, chronic-care bundle development, HCRIS-compatible reporting improvements, care-management redesign, workforce cross-training, diagnostic and laboratory access coordination, and service-line planning for low-volume inpatient units.

Sec. 906. Coordination with Medicare Critical Access Hospital Rules.

Provides that nothing in this Act diminishes Medicare Critical Access Hospital protections, rural hospital payment formulas, Sole Community Hospital rules, Medicare-dependent hospital provisions, or related Medicare rural payment structures. UCC/HSE rural access payments complement, and do not replace, existing Medicare rural authorities.

Sec. 907. Rural Access Monitoring and Closure Review.

Requires state and federal access monitoring for rural and frontier providers receiving stabilization support. A closure or major service-line reduction attributed to UCC/HSE implementation triggers access review when the provider serves a low-density area, is the only acute-care facility within a defined travel-time radius, or provides essential emergency, obstetric, inpatient, or diagnostic capacity.

Title X — Supplemental Coverage Guardrails

Sec. 1001. Definition of Supplemental Coverage.

Defines supplemental coverage as limited-benefit products that wrap around UCC catastrophic plans, including dental and vision benefits, defined long-term care riders, disease-specific riders, and specific drug carve-outs that do not materially alter the catastrophic structure.

Sec. 1002. Prohibition on Effective Deductible Reduction.

Prohibits any supplemental product from materially reducing the effective catastrophic deductible or replicating first-dollar comprehensive coverage; disqualifies plans that do so from receiving any federal catastrophic credit or favorable tax treatment under this Act.

Sec. 1003. Transparency and Disclosure Requirements for Supplemental Products.

Requires clear disclosure of supplemental-plan premiums, covered services, actuarial value, administrative load, and interactions with UCC catastrophic plans; mandates standardized consumer-facing forms and periodic reporting to federal regulators.

Title XI — Fiscal Caps, Cost Projections, and Automatic Correction Mechanisms

Sec. 1101. Federal Outlay Cap and Benchmark.

Establishes a statutory cap on combined federal outlays for catastrophic credits, federal HSA deposits, and reinsurance as a percentage of projected outlays under the ACA subsidy and cost-sharing framework, using CBO baseline projections as the benchmark.

Sec. 1102. Automatic Adjustment Mechanism.

Provides that if total federal outlays under this Act exceed the statutory cap beyond a specified tolerance band, automatic adjustments shall occur in the subsequent year to the catastrophic deductible index and/or HSA deposit levels, subject to defined floors and ceilings, in order to restore alignment with the cap. Automatic adjustments may not reduce statutory minimum HSA deposits for low-income households, pediatric deposits, chronic-condition supplements, rural household access supplements, behavioral-health continuity support, or rural provider access-capacity payments below protected statutory floors unless Congress affirmatively amends those floors.

Sec. 1103. Annual Budgetary and Program Performance Reporting.

Requires CBO-style and MedPAC-style annual reports to Congress

detailing federal outlays, savings relative to ACA, distributional impacts, provider-impact stabilization, rural access effects, behavioral-health institutional effects, and performance of automatic correction mechanisms.

Title XII — Transition Implementation and Exchange Wind-Down

Sec. 1201. Phase-Out of ACA Exchanges and Individual Market Subsidies (Years 1–3).

Provides for a three-year scheduled phase-out of ACA premium tax credits and cost-sharing reductions, with gradual replacement by catastrophic credits and HSE deposits; prohibits new exchange subsidy entitlements after the final transition year.

Sec. 1202. Transition of Medicaid Expansion Adults to UCC/HSE (Year 2).

Specifies the timing and process by which Medicaid expansion adults are migrated into UCC catastrophic coverage combined with HSE funding, with federal guidance for states and temporary financial support during the transition.

Sec. 1203. Employer Exclusion Phase-Out (Years 1–10).

Coordinates implementation of the employer exclusion phase-down with Titles III and IV; sets out the year-by-year schedule and reporting requirements to Congress on wage and coverage effects.

Sec. 1204. No-Lapse Coverage Guarantee (Years 1–3).

Provides that no eligible individual who had qualifying coverage under ACA exchanges, employer-sponsored insurance, or Medicaid expansion as of the date of enactment shall experience a lapse in catastrophic coverage during the first three transition years, provided that such individual complies with enrollment procedures established under this Act.

Sec. 1205. CHIP and Pediatric Continuity Clause.

Ensures that children enrolled in CHIP or in Medicaid pediatric coverage retain continuous coverage during the transition; coordinates pediatric HSA deposits with existing programs and prohibits administrative actions that create coverage gaps for children.

Sec. 1206. State Implementation Flexibilities and Regulatory Authority.

Authorizes states to sequence their own UCC/HSE implementation within federal time frames, consistent with Titles II through XI; grants the Secretaries of Health and Human Services and the Treasury authority to issue regulations, guidance, and temporary waivers necessary to effectuate an orderly transition.

U.S. CODE INTERACTION SUMMARY

Indicative Drafting Structure for the Universal Catastrophic Coverage and Health Savings Empowerment Act — 119th Congress

This Annex identifies the primary areas of federal law that would require amendment, supplementation, or cross-reference to implement the Universal Catastrophic Coverage and Health Savings Empowerment (UCC/HSE) framework. Citations refer to existing statutory authorities; final numbering and amendatory language will be assigned by the Office of Legislative Counsel.

I. Internal Revenue Code (Title 26)

A. Health Savings Accounts — 26 U.S.C. §223

Amend §223 to:

1. Extend HSA eligibility to all individuals enrolled in a federally qualified catastrophic plan.
2. Authorize federal deposits, including base, income-indexed, pediatric, chronic-condition, and rural household access supplements.
3. Establish authorized uses and penalties for misuse.
4. Standardize custodial and administrative rules across Treasury-approved institutions.

B. Employer-Sponsored Insurance Exclusion — 26 U.S.C. §106

Amend §106 to:

1. Establish the statutory schedule for phased reduction of the employer exclusion.
2. Preserve pre-existing collective bargaining agreements through expiration.
3. Clarify treatment of employer HSA contributions during the transitional period.
4. Coordinate with §§105, 125, and 162(l) as necessary for employer compliance.

C. Premium Tax Credits — 26 U.S.C. §36B

Amend §36B to:

1. Sunset ACA premium tax credits consistent with the exchange wind-down schedule.
2. Redirect eligible individuals to the universal catastrophic credit and federal HSA deposits.
3. Retire reconciliation and income-verification provisions tied to marketplace enrollment.

II. Public Health Service Act / Affordable Care Act (Title 42)

A. Abortion Funding Restrictions & Conscience Protections

- Section 1303 of the ACA — 42 U.S.C. §18023 (Special Rules)
- Church Amendments — 42 U.S.C. §300a-7

Amend to:

1. Apply Hyde-consistent restrictions to catastrophic credits, federal reinsurance payments, and federal HSA deposits.
2. Maintain that federal funds may not be used for abortion except in cases of rape, incest, or life endangerment.
3. Preserve existing conscience protections for providers, insurers, and plan administrators.

B. ACA Exchange Provisions — 42 U.S.C. §§18031, 18071 (ACA §§1311, 1402)

Amend to:

1. Establish the federal schedule for marketplace wind-down.
2. Eliminate cost-sharing reduction provisions.
3. Retire marketplace data reporting, enrollment machinery, and actuarial-value requirements.
4. Provide for the orderly transition of enrollees into UCC catastrophic coverage.

C. State Innovation Waivers — 42 U.S.C. §18052 (ACA §1332)

Clarify that:

1. States may coordinate existing §1332 pass-through funding with catastrophic-plan stabilization, HSA support, state-level transition assistance, or rural access support where permitted by law.

2. §1332 waivers remain available but must conform to UCC/HSE's federal catastrophic standard.
3. §1332 authority does not permit states to waive the uniform national catastrophic core, recreate federally subsidized comprehensive first-dollar coverage, or use pass-through funds in a manner inconsistent with UCC/HSE fiscal guardrails.

III. Social Security Act (SSA Titles XVIII, XIX, XXI)

A. Medicaid — SSA §§1902–1905 (42 U.S.C. §§1396a–1396d)

Amend to:

1. Transition ACA expansion adults into UCC/HSE within the implementation period.
2. Preserve Medicaid LTSS, ABD, medically fragile children's benefits, and HCBS waiver authorities.
3. Clarify FMAP treatment for states during the coverage transition.
4. Coordinate acute-care catastrophic payments with state Medicaid systems.

B. Medicaid Demonstrations — SSA §1115 (42 U.S.C. §1315)

Clarify that:

1. §1115 demonstration authority remains available for state-designed chronic-care bundles, Medicaid-related rural innovations, Medicaid transition supports, and population-specific coordination.
2. §1115 authority remains limited to Medicaid demonstrations and does not create general-purpose funding for non-Medicaid UCC/HSE populations.

3. UCC catastrophic standards cannot be waived except as explicitly permitted by Congress.

C. CHIP — Title XXI (42 U.S.C. §1397aa et seq.)

Specify that:

1. CHIP eligibility, benefits, and financing remain unchanged.
2. CHIP enrollment is not disrupted during ACA exchange wind-down or Medicaid redeterminations.
3. Pediatric catastrophic continuity is guaranteed.
4. Pediatric HSE deposits coordinate with, and do not replace, CHIP or Medicaid pediatric coverage.

D. Medicare — Title XVIII (SSA §§1801 et seq.)

Clarify that:

1. Ultra-high-cost therapies, including gene therapies and certain orphan biologics, remain financed under Medicare, Medicaid ABD, or orphan-drug authorities.
2. No shift of such therapies into the catastrophic pool.
3. UCC/HSE does not modify Medicare benefits, eligibility, Medicare Advantage, Part D, Medigap, or existing Medicare payment structures.

E. Rural Hospital Framework — SSA §1820 (42 U.S.C. §1395i-4)

Clarify that:

1. UCC/HSE rural stabilization measures complement, and do not replace, the Critical Access Hospital authority.
2. The permanent rural access-capacity payment and temporary implementation-period floor coordinate with existing Medicare rural payment protections.

3. UCC/HSE does not diminish Medicare Critical Access Hospital reimbursement, rural hospital flexibility authorities, Sole Community Hospital rules, Medicare-dependent hospital provisions, or other existing Medicare rural payment structures.
4. Rural access support applies to the UCC/HSE transition and non-Medicare replacement population; Medicare rural payment rules remain intact.

IV. ERISA (Title 29) — Employer Plans & Preemption

A. ERISA §514 Preemption — 29 U.S.C. §1144

Specify that:

1. Federal catastrophic-plan definitions, supplemental-coverage guardrails, and prohibitions on federally subsidized first-dollar wraparound coverage apply uniformly to employer-sponsored plans, ERISA plans, non-ERISA group plans, and individual-market products to the extent provided by federal law.
2. ERISA preemption does not displace federal definitions of catastrophic coverage, routine-care exclusions, supplemental-coverage limitations, or tax-favored HSA rules established under this Act.
3. Employers may continue to offer supplemental benefits that comply with federal actuarial, tax, and disclosure limits.
4. Employer contributions remain subject to the employer-exclusion phase-down, HSA contribution limits, and supplemental-coverage guardrails established under this Act.

V. Immigration and Nationality Act (INA) — Eligibility Verification

To implement UCC/HSE eligibility rules, cross-reference:

- 8 U.S.C. §1101 et seq. (immigration definitions)
- 8 U.S.C. §1158 (asylum)
- 8 U.S.C. §1182(d)(5) (parole)
- 8 U.S.C. §§1225, 1227 (inspection, inadmissibility, and removal-related authorities)

Statutory intent:

1. Eligible: U.S. citizens, lawful permanent residents, and long-term lawfully present residents meeting statutory residency and tax-filing requirements.
2. Ineligible: individuals without lawful status, asylum applicants, humanitarian parolees, temporary entrants, and other categories not made eligible by statute.
3. Verification through cross-matched IRS, SSA, and DHS data; no eligibility based on self-attestation alone.
4. Emergency-care obligations under EMTALA and other federal emergency-care requirements remain unchanged.

VI. New Statutory Units

Placement Determined by Legislative Counsel

A. Universal Catastrophic Coverage Standard

Create a new statutory part establishing:

- The national catastrophic deductible.
- Covered catastrophic services.
- Eligibility for the universal catastrophic credit.
- Insurer certification standards.
- Federal reinsurance structure and cost-sharing rules.
- Coordination with state-administered public catastrophic plans.

B. Health Savings Empowerment

Create authority for:

- Federal HSA deposit formulas, including base, income-indexed, pediatric, chronic-condition, and rural household access supplements.
- Coordination rules with IRS and Treasury.
- A statutory definition of “routine care” not eligible for insurance coverage under the catastrophic benefit.
- Allowable uses, rollovers, and anti-abuse provisions.
- Direct primary care compatibility.

C. Chronic-Condition and Behavioral-Health Funding Track

Create:

- Chronic-condition eligibility benchmarks.
- Enhanced HSA deposits for qualifying conditions.
- Income-based household protection caps.
- Condition-based care bundle authority.
- Behavioral-health continuity support.
- Institutional behavioral-health stabilization for qualifying psychiatric and behavioral-health providers.
- Coordination with UCC catastrophic rules.

D. Eligibility & Verification Mechanisms

Create:

- Citizenship and lawful-presence eligibility rules.
- Explicit exclusions for ineligible categories.
- Mandatory DHS/SSA/IRS verification procedures.

- Error-correction, appeal, and administrative-review processes.
- Emergency-care noninterference provisions.

E. Rural Health Stability and Access Protection

Create:

- Permanent rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider.
- Temporary implementation-period floor of -1.5 percent of net patient revenue after application of the access-capacity payment.
- Eligibility criteria for rural, frontier, Critical Access Hospital, and other low-volume access providers.
- Rural transparency, reporting, and access-monitoring requirements.
- Rural Virtual Access Network authority.
- Rural chronic-care bundle integration.
- Rural Transformation Track.
- Rural access monitoring and closure-review procedures.
- Coordination with Medicare Critical Access Hospital rules under SSA §1820.

F. Pediatric HSE and Pediatric Provider Coordination

Create:

- Pediatric HSA deposit authority by age band.
- Coordination with CHIP and Medicaid pediatric coverage.
- Protection for medically complex children.
- Pediatric provider-impact monitoring.

- Children’s hospital and pediatric specialty-provider stabilization where identified through provider-impact analysis.

G. Fiscal Guardrails & Automatic Adjustments

Create:

- Federal outlay cap benchmarked to the ACA baseline.
- Automatic adjustment mechanism for deductible indexing and HSA deposit formulas.
- Protected statutory floors for low-income HSA deposits, pediatric deposits, chronic-condition supplements, rural household access supplements, behavioral-health continuity support, and rural provider access-capacity payments.
- Annual Treasury/HHS reports, reviewed by CBO and MedPAC.

H. Transition & Exchange Sunset

Create:

- ACA exchange wind-down timeline.
- Medicaid expansion transition sequence.
- No-lapse coverage guarantee.
- CHIP and pediatric continuity clause.
- State implementation flexibilities and federal funding coordination.

VII. Committee Jurisdiction

119th Congress

House

- **Ways & Means:** IRC amendments, premium credits, HSA structure, employer exclusion, tax treatment of supplemental coverage.
- **Energy & Commerce:** Medicaid, CHIP, catastrophic standards, rural health, provider-impact stabilization, insurance market rules, public health provisions.
- **Education & Workforce:** ERISA-based employer plan issues, employer-sponsored coverage transition, workplace benefit interactions.

Senate

- **Finance:** IRC amendments, Medicare, Medicaid, CHIP, federal reinsurance architecture, HSA financing, rural access-capacity payments.
- **HELP:** PHSA provisions, insurance markets, consumer protections, provider regulation, behavioral-health access, rural telehealth and access standards.

VIII. Purpose of This Annex

This Annex serves as a non-exhaustive drafting guide for the Office of Legislative Counsel, committee counsels, CBO analysts, Treasury/HHS rule writers, and appropriations reviewers. It identifies the statutory anchors required to construct UCC/HSE without prescribing final numbering or amendatory text. Final statutory placement, cross-references, conforming amendments, and effective-date language remain subject to Legislative Counsel review.

ANNEX B: FEDERAL CATASTROPHIC CREDIT AND HSA DEPOSIT FORMULAS

This Annex provides a formal parameterization of the universal catastrophic credit and federal Health Savings Account (HSA) deposits under the Universal Catastrophic Coverage with Health Savings Empowerment Act. It is intended for modeling, distributional analysis, and fiscal scoring. Parameters may be adjusted by Congress and by the automatic correction mechanisms in Title XI.

Clarification: Benchmark Premium vs. Catastrophic Deductible

The benchmark catastrophic premium used in this framework reflects the annual market price of a standardized catastrophic insurance policy and is employed solely as the reference value for calculating federal catastrophic credits. It is not equivalent to, and should not be conflated with, the national catastrophic deductible specified in Title II. The deductible determines the point at which insurance benefits begin, whereas the benchmark premium determines the magnitude of the government's premium support obligation. These are separate constructs serving distinct policy and financial functions.

Baseline Definitions and Scoring Conventions

Baseline estimates of the tax exclusion for employment-based health insurance reflect the full tax expenditure under current law, as reported by the Congressional Budget Office. Annual revenue effects under UCC/HSE reflect only the phased statutory reduction in that exclusion, adjusted for compensation incidence and timing effects consistent with conventions used by the staff of the Joint Committee on Taxation.

Accordingly, baseline tax expenditure amounts should not be interpreted as annual savings and are used solely to scale the statutory phase-down mechanism.

Employer coverage transitions under UCC/HSE do not increase eligibility for marketplace premium tax credits due to universal catastrophic coverage availability.

Federal Catastrophic Payment Limit (FCPL).

Notwithstanding any other provision of law, federal payments made with respect to covered catastrophic health care services under this Act shall not exceed a statutory payment limit equal to the lesser of:

- (1) the maximum liability specified under the applicable benchmark catastrophic plan for such services; or
- (2) a payment amount equal to [130] percent of the Medicare allowable amount for the same or substantially similar services, as determined by the Secretary.

Such limit shall apply on a per-episode or per-service basis, shall be indexed annually to medical inflation, and shall not be construed to limit amounts that may be billed to or paid by non-federal sources.

RoutineSpendBaseline_2026 is constructed by anchoring the routine-care spending base to nationally representative utilization and spending patterns from the Medical Expenditure Panel Survey (MEPS) 2023 Full-Year Consolidated file (HC-251), which serves as the empirical basis for the workbook’s internal “H251 shorthand” distribution reconstruction **(78)**. The routine-care baseline is then expressed in national dollars and aligned with the model’s macro spending framework consistent with CMS National Health Expenditure (NHE) projections **(92)**. This structure is intentional: it keeps the “routine vs catastrophic” split grounded in MEPS microdata while ensuring the consumer-impact module remains consistent with the CMS-anchored spending totals used elsewhere in the scoring model **(78, 92)**.

RoutineSpendGrowthRate applies a forward trend to the routine-care baseline. The model’s growth assumption is benchmarked to CMS’s forward spending outlook **(92)**, with the rate set explicitly as an assumption (low / central / high) to reflect uncertainty about how quickly routine spending bends under HSE-driven price discipline. In other words, CMS provides the authoritative macro context for health spending growth, while the model transparently specifies the degree of “cost-curve bend” rather than implying CMS endorses a particular UCC/HSE reform path **(92)**.

RoutineShare_UnderDeductible represents the share of routine spending that is realistically subject to price discipline (i.e., “shoppable” or otherwise competitively priced). The empirical foundation is again MEPS HC-251 via the H251 shorthand distribution approach **(78)**. The model then applies a conservatism adjustment based on evidence that price information availability and usability remain incomplete even under federal transparency requirements, meaning it is not credible to assume that 100% of routine services can be actively shopped and repriced in practice **(79,80)**.

RoutineCashPriceIndex_R parameterizes the relative level of “cash/transparent” prices versus negotiated insurer prices for routine services. The model treats this as a bounded assumption informed by multiple strands of price-transparency evidence: (i) peer-reviewed work documenting systematic differences and variation among cash, negotiated, and chargemaster prices **(81,82)**; and (ii) evidence that cash prices are frequently lower than median negotiated rates for a meaningful share of shoppable services **(80,82)**. The index is therefore not a single published constant; it is a modeling wedge bounded by peer-reviewed comparisons drawn from transparency datasets **(80-82)**.

PassThroughToConsumers captures the fraction of any routine price reductions that actually reach consumers as lower paid amounts (rather than being absorbed by market frictions, noncompliance, billing complexity, or partial shopping). This is explicitly modeled as an assumption, but it is grounded in the documented reality that (a) CMS requires specific disclosures for shoppable services and machine-readable files (79), while (b) observed compliance and the practical accessibility of prices on hospital internet sites has been imperfect—implying pass-through is meaningfully less than 100% in the real world (80). The pass-through range therefore operationalizes the gap between “rule on the books” and “prices consumers can reliably use” (79,80).

ANNEX C: Formula Section

I. Notation

For each household h and individual i in year t :

- $P_{i,t}$ = benchmark catastrophic premium for individual i (age and rating-area adjusted).
- $P_{h,t} = \sum_{i \in h} P_{i,t}$ = household benchmark catastrophic premium.
- $Y_{h,t}$ = household modified adjusted gross income (MAGI).
- $F_{h,t}$ = federal poverty guideline for household h (by size).
- $z_{h,t} = Y_{h,t} / F_{h,t}$ = household income as a multiple of poverty.
- α = baseline federal contribution rate (share of premium for all eligible households).
- $\beta(z)$ = income-indexed supplemental contribution schedule.
- ϕ = maximum household premium fraction (income cap).
- $C_{h,t}$ = universal catastrophic credit for household h .

HSA-related:

- $H_{i,t}$ = federal HSA deposit for individual i .
- $H_{h,t} = \sum_{i \in h} H_{i,t}$ = total household federal HSA deposit.
- $H_{\text{base},i}$ = age-based base deposit for individual i , with:
 - $H_{\text{base},i} = H_{\text{adult}}$ for adults (18+).
 - $H_{\text{base},i} = H_{0-5}$ for ages 0–5.
 - $H_{\text{base},i} = H_{6-17}$ for ages 6–17.
- $c_i \in \{0, 1\}$ = chronic-condition indicator for individual i
($c_i = 1$ if statutory chronic-condition criteria are met; otherwise 0).
- H_{chronic} = fixed chronic-condition supplement (uniform amount for all qualifying individuals, income-independent).
- $\gamma(z_{h,t})$ = income-indexed HSA adjustment applied to **all** individuals in household h , based on the household income ratio $z_{h,t}$.

All parameters are defined in constant-dollar terms and indexed as described in Section IV below.

Clarifications

- The chronic-condition supplement H_{chronic} is uniform and does not vary by income. All individuals meeting statutory chronic-condition criteria receive the same chronic deposit regardless of household earnings.
- Income-indexing via $\gamma(z)$ applies only to non-chronic individuals and is used to reduce deposits for higher-income households while preserving enhanced support at lower incomes.
- The universal catastrophic credit $C_{h,t}$ is household-based and determined independently of the HSA deposit calculation, except insofar as both reference the same income ratio variable $z_{h,t}$.
- Benchmark catastrophic premiums $P_{i,t}$ reflect standardized actuarial catastrophic plans and do not vary due to individual health status, chronic designation, or prior utilization.
- The model assumes statutory eligibility for both catastrophic credits and HSA deposits for all qualified individuals, subject to citizenship and legal residency requirements defined elsewhere in the Act.

Chronic-condition status does not affect the value of the provisional catastrophic credit and is addressed separately through fixed federal HSA deposits as defined in Section III.

II. Universal Catastrophic Credit

A. Income-Indexed Contribution Schedule

The income schedule $\beta(z)$ is piecewise linear:

$$\beta(z) = \begin{cases} \beta_{\max}, & z \leq z_L \\ \beta_{\max} - k \cdot (z - z_L), & z_L < z \leq z_U \\ 0, & z > z_U \end{cases}$$

where:

- $z = Y_{h,t}/F_{h,t}$.
- z_L = lower break-point (e.g., 1.5, corresponding to 150% FPL).
- z_U = upper break-point (e.g., 4.0, corresponding to 400% FPL).
- $\beta_{\max}$ = maximum additional federal contribution (share of premium) for low-income households, independent of health status.
- $k = \beta_{\max}/(z_U - z_L)$ ensures a linear phase-out to zero at z_U .

Clarification: Separation of Premium Credit and Chronic Support

For avoidance of doubt:

- The income-indexed contribution schedule $\beta(z)$ applies solely on the basis of household income relative to the federal poverty level.
- Chronic-condition status does not modify the catastrophic premium credit and does not alter the values of α , $\beta(z)$, or $C_{h,t}$.
- All chronic-condition financial support is delivered exclusively through fixed federal HSA deposits $H_{chronic}$, as defined in Section III.

B. Provisional Credit Formula

Define the provisional credit as:

$$\hat{C}_{h,t} = (\alpha + \beta(z_{h,t})) \cdot P_{h,t}$$

This term captures:

- A **baseline contribution** $\alpha P_{h,t}$ available to all eligible households, and
- An **income-indexed supplement** $\beta(z_{h,t})P_{h,t}$ that phases down with income.

C. Income Cap and Premium Limit

To ensure affordability and preserve price sensitivity, the final household credit applies two additional constraints:

1. **Household premium cap:** net premium contribution may not exceed a share ϕ of income.
2. **Premium-coverage ceiling:** the credit may not exceed a fixed share κ of the benchmark premium (e.g., 90%).

Formally:

$$C_{h,t} = \min \left\{ \kappa P_{h,t}, \max \left[\hat{C}_{h,t}, P_{h,t} - \phi Y_{h,t}, 0 \right] \right\}$$

where:

- ϕ is the statutory household premium cap (e.g., $\phi = 0.10$).
- κ is the statutory maximum federal coverage share (e.g., $\kappa = 0.90$).

Interpretation:

- $\hat{C}_{h,t}$ is the primary credit produced by the baseline and income schedule.
- If $\hat{C}_{h,t}$ is insufficient to keep the household's net premium at or below $\phi Y_{h,t}$, the credit is increased up to $P_{h,t} - \phi Y_{h,t}$, subject to the $\kappa P_{h,t}$ ceiling.
- The credit cannot be negative.

Clarification: Chronic-Condition Separation

- The premium cap and coverage ceiling apply solely to the catastrophic premium credit.
- Chronic-condition status does not modify the application of ϕ or κ and does not alter the final calculation of $C_{h,t}$.
- All chronic-condition financial protection is delivered independently through the federal HSA deposit structure defined in Section III.

Operationally, scoring proceeds in two steps. First, a provisional federal share $s_{h,t}^{\text{prov}} = \alpha + \beta(z_{h,t})$ is computed and bounded above by the statutory coverage ceiling κ . Second, if the resulting net premium $(1 - s_{h,t}^{\text{prov}})P_{h,t}$ would exceed the household premium cap $\phi Y_{h,t}$, the federal share is increased to

$$s_{h,t}^{\text{cap}} = 1 - \frac{\phi Y_{h,t}}{P_{h,t}},$$

and the final credit share is $s_{h,t} = \min\{s_{h,t}^{\text{cap}}, \kappa\}$.

III. Federal HSA Deposits

For each individual i in household h , the federal HSA deposit takes the form:

$$H_{i,t} = H_{\text{base},i} + \gamma(z_{h,t}) + c_i H_{\text{chronic}}$$

where:

- $H_{\text{base},i}$ depends on age:
 - $H_{\text{base},i} = H_{\text{adult}}$ for adults (18+).
 - $H_{\text{base},i} = H_{0-5}$ for ages 0–5.
 - $H_{\text{base},i} = H_{6-17}$ for ages 6–17.
- $\gamma(z_{h,t})$ is an income-indexed HSA adjustment applied to all individuals in household h .
- H_{chronic} is a fixed chronic-condition supplement (uniform amount for all chronic individuals, income-independent).
- $c_i \in \{0, 1\}$ is an indicator for chronic status:
 - $c_i = 1$ if individual i meets statutory chronic-condition criteria;
 - $c_i = 0$ otherwise.

Total household deposit is:

$$H_{h,t} = \sum_{i \in h} H_{i,t}$$

Baseline numerical bands (e.g., H_{adult} , H_{0-5} , H_{6-17} , H_{chronic}) correspond to the amounts described in the main text and may be varied in scoring scenarios.

Income-Indexed Adjustment Function (Revised)

The income-indexed HSA adjustment applies to all individuals and operates supplemental to the age-based base deposit and any chronic-condition supplement.

The adjustment function is defined as:

$$\gamma(z) = \begin{cases} \gamma_{\max}, & z \leq z'_L \\ \gamma_{\max} - m(z - z'_L), & z'_L < z \leq z'_U \\ 0, & z > z'_U \end{cases}$$

where:

- $z = z_{h,t} = Y_{h,t}/F_{h,t}$ is household income as a multiple of poverty.
- z'_L is the lower income threshold for HSA targeting.
- z'_U is the upper income threshold for HSA targeting.
- $\gamma_{\max}$ is the maximum income-indexed HSA adjustment.
- $m = \gamma_{\max}/(z'_U - z'_L)$ ensures linear phase-out to zero at z'_U .

This adjustment reflects progressive HSA support for lower-income households and is applied identically to both chronic and non-chronic individuals.

Clarification: Chronic-Condition Deposits

- The chronic-condition supplement H_{chronic} is a fixed statutory amount provided uniformly to all individuals meeting chronic-condition criteria.
- H_{chronic} is **independent of household income** and is not subject to phase-out or adjustment based on $z_{h,t}$.
- Individuals qualifying for H_{chronic} **also receive** the income-indexed adjustment $\gamma(z_{h,t})$; poorer chronic individuals therefore receive larger total HSA deposits than higher-income chronic individuals, driven by the income-sensitive $\gamma(z)$, not by variation in H_{chronic} .

Total household deposit is:

$$H_{h,t} = \sum_{i \in h} H_{i,t},$$

with individual deposits defined as:

$$H_{i,t} = H_{\text{base},i} + \gamma(z_{h,t}) + c_i H_{\text{chronic}},$$

where

$$H_{\text{base},i} = \begin{cases} H_{\text{adult}}, & \text{if age} \geq 18, \\ H_{0-5}, & \text{if age } 0-5, \\ H_{6-17}, & \text{if age } 6-17. \end{cases}$$

Baseline numerical bands (e.g., H_{adult} , H_{0-5} , H_{6-17} , H_{chronic}) correspond to the amounts described in the main text and may be varied in scoring scenarios.

IV. Indexation and Fiscal Cap Interaction

Let I_t denote the annual index applied to adjustable credit and deposit parameters:

$$I_t = w_1 \cdot G_{\text{prem},t} + w_2 \cdot G_{\text{income},t}$$

where:

- $G_{\text{prem},t}$ = growth rate of the benchmark catastrophic premium.
- $G_{\text{income},t}$ = growth rate of median household income.
- $w_1 + w_2 = 1$, with statutory bounds on each weight.

Parameters such as α , β_{max} , γ_{max} , ϕ , κ , and the base non-chronic HSA deposits are indexed annually by I_t , subject to:

- The aggregate outlay cap on catastrophic credits + HSA deposits + reinsurance defined in Title XI; and
- The automatic adjustment mechanism that may adjust these parameters downward (or constrain their growth) if federal outlays exceed the cap's tolerance band.

The chronic-condition supplement H_{chronic} is indexed independently to medical price inflation and is not subject to income-sensitive adjustment, redistribution across income bands, or guardrail-triggered reallocation.

V. Use for Modeling and Scoring

The equations in this Annex:

- Translate the qualitative design described in the main text into a formal system suitable for microsimulation and CBO-style scoring.
- Allow parameter sweeps over α , $\beta_{\max}$, z_L , z_U , H_{adult} , H_{0-5} , H_{6-17} , $H_{chronic}$, and $\gamma_{\max}$, while preserving the structural distinction between fixed chronic-condition support and income-indexed non-chronic deposits.
- Link directly to the fiscal guardrails of Title XI by making those guardrails operative through constraints on the parameter set rather than through ad hoc adjustments.

Indexation and Guardrail Interaction

Parameters such as α , $\beta_{\max}$, $\gamma_{\max}$, ϕ , and the base non-chronic HSA deposits are indexed annually by I_t , subject to:

- The aggregate outlay cap on catastrophic credits + HSA deposits + reinsurance defined in Title XI; and
- The automatic adjustment mechanism that may adjust these parameters downward (or constrain their growth) if federal outlays exceed the cap's tolerance band.

The chronic-condition supplement $H_{chronic}$ is indexed independently to medical price inflation and is not subject to income-sensitive adjustment, redistribution across income bands, or guardrail-triggered reallocation.

VI. Provider-Impact Modeling Framework

This section defines the provider-impact formulas used to generate the Table 7 provider-impact outputs. The framework estimates the near-term hospital and provider revenue effects of UCC/HSE routine-care repricing, uncompensated-care relief, bad-debt relief, transition stabilization, rural access protection, and behavioral-health institutional adjustments. The model is provider-level: each hospital or provider record is evaluated using HCRIS-derived revenue, outpatient exposure, uncompensated-care, bad-debt, margin, and classification variables. Provider-level results are then aggregated by scenario, provider group, and provider class.

A. Provider-Level Notation

For each provider j , year t , and scenario s :

$NPR_{j,t}$ = net patient revenue; $GOR_{j,t}$ = gross outpatient revenue; $GTR_{j,t}$ = gross total patient revenue

$NTG_{j,t}$ = net-to-gross revenue ratio; $OPEB_{j,t}$ = net outpatient exposure base; $MRE_{j,t,s}$ = market repricing exposure

$UC_{j,t}$ = uncompensated-care cost; $BD_{j,t}$ = bad-debt cost; $CLO_{j,t,s}$ = coverage and liquidity offset

$TS_{j,t,s}$ = transition stabilization; $NBI_{j,t,s}$ = net provider impact; $PI\%_{j,t,s}$ = provider impact as a share of NPR

Scenario-specific parameters are:

ρ_s = outpatient/routine repricing parameter; ν_s = uncompensated-care relief parameter; δ_s = bad-debt relief parameter

σ_s = safety-net stabilization rate; λ_s = rural stabilization rate; χ_s = children's hospital stabilization rate

μ_s = negative-margin protection rate

The symbols ν_s , δ_s , λ_s , χ_s , and μ_s are used to avoid confusion with the household-level catastrophic-credit and HSA parameters defined earlier in this Annex.

B. HCRIS Revenue Inputs

The provider-impact model uses the following HCRIS revenue variables:

$GOR_{j,t}$ = HCRIS Worksheet G-2, line 28, column 2

$GTR_{j,t}$ = HCRIS Worksheet G-2, line 28, column 3

$NPR_{j,t}$ = HCRIS Worksheet G-3, line 3

Gross outpatient revenue is converted into a net-revenue-equivalent outpatient exposure base using each provider's net-to-gross revenue ratio.

C. Net-to-Gross Revenue Ratio

$$NTG_{j,t} = \frac{NPR_{j,t}}{GTR_{j,t}}$$

where $NPR_{j,t}$ is net patient revenue and $GTR_{j,t}$ is gross total patient revenue. This ratio estimates the share of gross charges that convert into net patient revenue.

If $GTR_{j,t}$ is missing, zero, negative, or anomalous, the model does not mechanically apply the ratio. The provider is instead handled through documented fallback rules, validation checks, or exclusion from modeled exposure if no reliable revenue basis exists. This prevents distorted gross-charge records from artificially inflating or suppressing outpatient repricing exposure.

D. Net Outpatient Exposure Base

$$OPEB_{j,t} = GOR_{j,t} \times NTG_{j,t}$$

Substituting the net-to-gross ratio:

$$OPEB_{j,t} = GOR_{j,t} \times \frac{NPR_{j,t}}{GTR_{j,t}}$$

The net outpatient exposure base estimates the portion of outpatient and routine-care revenue economically exposed to UCC/HSE price discipline. The model does not assume all hospital revenue is repriced; it isolates the outpatient/routine-care component and converts gross-charge values into a net patient-revenue-equivalent base.

E. Market Repricing Exposure

$$MRE_{j,t,s} = OPEB_{j,t} \times \rho_s$$

where $OPEB_{j,t}$ is the net outpatient exposure base and ρ_s is the scenario-specific outpatient/routine repricing parameter. Illustrative values are:

$$\rho_s = \begin{cases} 0.03, & \text{low repricing} \\ 0.05, & \text{base repricing} \\ 0.10, & \text{high repricing} \end{cases}$$

Market repricing exposure represents the modeled reduction in provider revenue attributable to routine-care price discipline under UCC/HSE. It is the principal provider-side savings mechanism and reflects the expectation that routine and outpatient services currently billed through opaque insurance channels would face greater cash-price competition, HSA-based shopping, and direct-pay discipline.

F. Uncompensated-Care Relief

$$UCR_{j,t,s} = \max(UC_{j,t}, 0) \times \nu_s$$

where $UC_{j,t}$ is HCRIS S-10 uncompensated-care cost and ν_s is the scenario-specific uncompensated-care relief parameter. Illustrative values are:

$$\nu_s = \begin{cases} 0.25, & \text{low protection} \\ 0.50, & \text{base protection} \\ 0.75, & \text{high protection} \end{cases}$$

Uncompensated-care relief reflects the reduction in provider losses expected when households receive universal catastrophic protection and greater routine-care liquidity through HSE deposits. Relief is applied only to non-negative uncompensated-care values.

G. Bad-Debt Relief

$$BDR_{j,t,s} = \max(BD_{j,t}, 0) \times \delta_s$$

where $BD_{j,t}$ is HCRIS S-10 bad-debt cost and δ_s is the scenario-specific bad-debt relief parameter. Illustrative values are:

$$\delta_s = \begin{cases} 0.25, & \text{low protection} \\ 0.50, & \text{base protection} \\ 0.75, & \text{high protection} \end{cases}$$

Bad-debt relief captures the reduction in unpaid patient obligations expected from broader catastrophic protection, HSA liquidity, and reduced exposure to large uncovered medical bills. As with uncompensated-care relief, the model applies relief only to non-negative bad-debt values.

H. Coverage and Liquidity Offset

$$CLO_{j,t,s} = UCR_{j,t,s} + BDR_{j,t,s}$$

Substituting the component formulas:

$$CLO_{j,t,s} = [\max(UC_{j,t}, 0) \times \nu_s] + [\max(BD_{j,t}, 0) \times \delta_s]$$

The coverage and liquidity offset represents the modeled provider benefit from lower uncompensated-care and bad-debt exposure. It is not a general provider subsidy; it is a modeled reduction in revenue leakage attributable to fewer uninsured catastrophic episodes and improved household liquidity for routine and semi-routine care.

I. Safety-Net Stabilization

$$SNS_{j,t,s} = NPR_{j,t} \times \sigma_s \times \mathbf{1}\{SafetyNet_j = 1\}$$

where σ_s is the safety-net stabilization rate and $\mathbf{1}\{SafetyNet_j = 1\}$ equals 1 if provider j qualifies as safety-net eligible. Illustrative values are:

$$\sigma_s = \begin{cases} 0.015, & \text{low protection} \\ 0.030, & \text{base protection} \\ 0.050, & \text{high protection} \end{cases}$$

A provider may qualify as safety-net eligible if it satisfies statutory criteria such as high uncompensated-care burden, high Medicaid or low-income patient share, public ownership, disproportionate-share status, or other designated safety-net criteria. For HCRIS-only modeling, the safety-net proxy is based on high S-10

J. Rural Stabilization

$$RS_{j,t,s} = NPR_{j,t} \times \lambda_s \times \mathbf{1}\{Rural_j = 1\}$$

where λ_s is the rural stabilization rate and $\mathbf{1}\{Rural_j = 1\}$ equals 1 if provider j qualifies as rural. Illustrative values are:

$$\lambda_s = \begin{cases} 0.010, & \text{low protection} \\ 0.020, & \text{base protection} \\ 0.035, & \text{high protection} \end{cases}$$

Rural eligibility may include rural primary-care hospitals, critical access hospitals, sole-community hospitals, Medicare-dependent rural hospitals, low-volume rural hospitals, Indian Health Service facilities, and other providers designated as essential rural-access institutions. For HCRIS-only modeling, rural eligibility is based on provider classification variables available in the provider-impact model.

K. Children's Hospital Stabilization

$$CHS_{j,t,s} = NPR_{j,t} \times \chi_s \times \mathbf{1}\{Children_j = 1\}$$

where χ_s is the children's hospital stabilization rate and $\mathbf{1}\{Children_j = 1\}$ equals 1 if provider j qualifies as a children's hospital. Illustrative values are:

$$\chi_s = \begin{cases} 0.010, & \text{low protection} \\ 0.020, & \text{base protection} \\ 0.035, & \text{high protection} \end{cases}$$

Children's hospitals are modeled separately because their patient mix, payer structure, pediatric specialty capacity, and interaction with Medicaid and CHIP differ from general acute-care hospitals. Children's stabilization is a bounded transition protection, not a permanent replacement for pediatric Medicaid or CHIP financing.

L. Negative-Margin Protection

$$NMP_{j,t,s} = NPR_{j,t} \times \mu_s \times \mathbf{1}\{Margin_{j,t} < -0.10\}$$

where μ_s is the negative-margin protection rate and $\mathbf{1}\{Margin_{j,t} < -0.10\}$ equals 1 if provider j has a patient-service margin below negative 10 percent. Illustrative values are:

$$\mu_s = \begin{cases} 0.005, & \text{low protection} \\ 0.010, & \text{base protection} \\ 0.020, & \text{high protection} \end{cases}$$

Negative-margin protection provides bounded transition support for providers already operating under severe patient-service margin pressure. It is not designed to insulate all providers from repricing; it reduces destabilizing effects among providers with materially negative baseline margins.

M. Total Transition Stabilization

$$TS_{j,t,s} = SNS_{j,t,s} + RS_{j,t,s} + CHS_{j,t,s} + NMP_{j,t,s}$$

Equivalently:

$$\begin{aligned} TS_{j,t,s} = & [NPR_{j,t} \times \sigma_s \times \mathbf{1}\{SafetyNet_j = 1\}] \\ & + [NPR_{j,t} \times \lambda_s \times \mathbf{1}\{Rural_j = 1\}] \\ & + [NPR_{j,t} \times \chi_s \times \mathbf{1}\{Children_j = 1\}] \\ & + [NPR_{j,t} \times \mu_s \times \mathbf{1}\{Margin_{j,t} < -0.10\}] \end{aligned}$$

Transition stabilization is bounded implementation-period support. It is distinct from the permanent rural access guarantee, the behavioral institutional repricing adjustment, and any long-term Medicaid, CHIP, LTSS, ABD, or public-health financing retained outside the UCC/HSE provider-impact model.

N. Net Provider Impact Before Transition Stabilization

$$NBI_{j,t,s}^{pre} = CLO_{j,t,s} - MRE_{j,t,s}$$

This measure captures the provider-level effect of routine/outpatient repricing after accounting for uncompensated-care and bad-debt relief, but before explicit transition stabilization or access-protection adjustments. It is useful for identifying the raw distributional pressure created by UCC/HSE price discipline.

O. Net Provider Impact After Transition Stabilization

$$NBI_{j,t,s} = CLO_{j,t,s} + TS_{j,t,s} - MRE_{j,t,s}$$

Substituting the primary components:

$$NBI_{j,t,s} = [UCR_{j,t,s} + BDR_{j,t,s}] + TS_{j,t,s} - [OPEB_{j,t} \times \rho_s]$$

This is the core provider-impact value used in the base Table 7 outputs. Positive values indicate a modeled net gain relative to baseline revenue exposure; negative values indicate a modeled net provider impact after offsets and transition stabilization.

P. Provider Impact as a Percentage of Net Patient Revenue

$$PI\%_{j,t,s} = \frac{NBI_{j,t,s}}{NPR_{j,t}}$$

Providers with missing, zero, negative, or anomalous $NPR_{j,t}$ values are not mechanically included in percentage-impact calculations. They are handled through documented fallback rules, audit flags, or exclusion from modeled percentage-impact outputs.

VII. Preferred Access-Protection Adjustments

The base provider-impact framework estimates the effect of UCC/HSE price discipline, coverage offsets, and transition stabilization. The preferred access-protection framework adds two targeted refinements: a permanent rural access guarantee for qualifying rural, critical-access, IHS, or low-volume providers; and a behavioral institutional repricing adjustment for qualifying psychiatric and behavioral-health providers. These adjustments are narrow, formula-based, and targeted to structural access concerns. They are not general provider bailouts.

A. Permanent Rural Access Guarantee

$$RAG_{j,t} = \min \{0.02 \times NPR_{j,t}, 5,000,000\} \times \mathbf{1}\{RuralAccess_j = 1\}$$

where $RAG_{j,t}$ is the rural access guarantee, 5,000,000 is the per-provider cap, and $\mathbf{1}\{RuralAccess_j = 1\}$ equals 1 if provider j qualifies for rural access protection.

The rural access guarantee is a permanent, capped, formula-based access-capacity payment. It recognizes that rural, critical-access, IHS, sole-community, and low-volume providers maintain essential standby capacity that cannot be fully evaluated through ordinary outpatient price discipline.

B. Rural Implementation Floor

$$RFS_{j,t,s} = \max \{0, [-0.015 \times NPR_{j,t}] - [NBI_{j,t,s} + RAG_{j,t}]\} \times \mathbf{1}\{RuralAccess_j = 1\}$$

where $RFS_{j,t,s}$ is rural floor support, $NBI_{j,t,s}$ is net provider impact after transition stabilization but before preferred rural access protection, and $-0.015 \times NPR_{j,t}$ is the negative 1.5 percent NPR implementation floor.

The floor prevents qualifying rural-access providers from experiencing modeled implementation-period losses greater than 1.5 percent of net patient revenue after application of the permanent rural access guarantee. Unlike the rural access guarantee, this floor is implementation-period support and may be time-limited, reviewed, or phased down as rural markets adjust.

C. Preferred Rural Adjusted Net Impact

For qualifying rural-access providers:

$$NBI_{j,t,s}^R = NBI_{j,t,s} + RAG_{j,t} + RFS_{j,t,s}$$

For non-qualifying providers:

$$NBI_{j,t,s}^R = NBI_{j,t,s}$$

D. Behavioral Institutional Repricing Adjustment

Qualifying psychiatric and behavioral-health providers are assigned a reduced routine/outpatient repricing exposure:

$$MRE_{j,t,s}^{BH} = \theta_{BH} \times MRE_{j,t,s}$$

The preferred value is:

$$\theta_{BH} = 0.25$$

For non-behavioral providers:

$$MRE_{j,t,s}^{adj} = MRE_{j,t,s}$$

For qualifying behavioral providers:

$$MRE_{j,t,s}^{adj} = MRE_{j,t,s}^{BH} = \theta_{BH} \times MRE_{j,t,s}$$

The behavioral institutional adjustment recognizes that many psychiatric and behavioral-health hospitals, particularly state psychiatric facilities and institutional behavioral providers, are not ordinary consumer-directed outpatient price-shopping targets. The adjustment does not create a large behavioral subsidy; it corrects the modeled exposure assumption for providers whose services are structurally less exposed to routine cash-market repricing.

E. Behavioral Pre-Floor Net Impact

For qualifying behavioral providers:

$$NBI_{j,t,s}^{BH-pre} = CLO_{j,t,s} + TS_{j,t,s} - MRE_{j,t,s}^{BH}$$

For non-behavioral providers, the standard net provider impact formula applies.

F. Behavioral Floor Support

$$BFS_{j,t,s} = \max \left\{ 0, [-0.05 \times NPR_{j,t}] - NBI_{j,t,s}^{BH-pre} \right\} \times \mathbf{1}\{Behavioral_j = 1\}$$

where $BFS_{j,t,s}$ is behavioral floor support, $NBI_{j,t,s}^{BH-pre}$ is behavioral pre-floor net impact, and $-0.05 \times NPR_{j,t}$ is the negative 5 percent NPR implementation floor.

The behavioral floor prevents qualifying psychiatric and behavioral-health providers from experiencing modeled implementation-period losses greater than 5 percent of net patient revenue after the behavioral repricing adjustment. It is designed to eliminate severe institutional behavioral-health outliers without converting behavioral providers into a broad permanent subsidy class.

G. Behavioral Adjusted Net Impact

For qualifying behavioral providers:

$$NBI_{j,t,s}^{BH} = CLO_{j,t,s} + TS_{j,t,s} - MRE_{j,t,s}^{BH} + BFS_{j,t,s}$$

For non-behavioral providers:

$$NBI_{j,t,s}^{BH} = NBI_{j,t,s}$$

H. Preferred Access-Protection Net Impact

$$NBI_{j,t,s}^{preferred} = CLO_{j,t,s} + TS_{j,t,s} - MRE_{j,t,s}^{adj} + RAG_{j,t} + RFS_{j,t,s} + BFS_{j,t,s}$$

where $MRE_{j,t,s}^{adj}$ is standard market repricing exposure for non-behavioral providers and behavioral-adjusted repricing exposure for qualifying behavioral providers. $RAG_{j,t}$ and $RFS_{j,t,s}$ apply only to qualifying rural-access providers; $BFS_{j,t,s}$ applies only to qualifying behavioral providers.

I. Ordering of Access-Protection Adjustments

If a provider qualifies for more than one access-protection category, each adjustment is applied only to the relevant component of the formula. The model does not double-count repricing relief, stabilization, or floor support. Adjustments are applied in the following order:

1. Calculate standard market repricing exposure.
2. Apply behavioral repricing adjustment if applicable.
3. Calculate coverage and liquidity offsets.
4. Apply transition stabilization.
5. Apply permanent rural access guarantee if applicable.
6. Apply rural and behavioral implementation floors if applicable.

This ordering ensures that floor support is calculated only after all other applicable offsets and protections have been applied.

VIII. Aggregation for Table 7 Outputs

Provider-level values are aggregated to generate the Table 7 outputs. For any provider group or class g , define $j \in g$ as all modeled providers belonging to group g .

A. Aggregate Net Patient Revenue

$$NPR_{g,t} = \sum_{j \in g} NPR_{j,t}$$

B. Aggregate Net Provider Impact

$$NBI_{g,t,s} = \sum_{j \in g} NBI_{j,t,s}$$

For preferred access-protection outputs:

$$NBI_{g,t,s}^{preferred} = \sum_{j \in g} NBI_{j,t,s}^{preferred}$$

C. Aggregate Impact as a Percentage of Net Patient Revenue

$$PI\%_{g,t,s} = \frac{NBI_{g,t,s}}{NPR_{g,t}}$$

For preferred access-protection outputs:

$$PI\%_{g,t,s}^{preferred} = \frac{NBI_{g,t,s}^{preferred}}{NPR_{g,t}}$$

D. Provider Counts

$$NegativeProviders_{g,t,s} = \sum_{j \in g} \mathbf{1}\{NBI_{j,t,s} < 0\}$$

$$LargeNegativeProviders_{g,t,s} = \sum_{j \in g} \mathbf{1}\{PI\%_{j,t,s} < -0.05\}$$

$$SevereNegativeProviders_{g,t,s} = \sum_{j \in g} \mathbf{1}\{PI\%_{j,t,s} < -0.10\}$$

These aggregation formulas generate the provider-group, provider-class, distributional, rural-access, behavioral-health, and preferred access-protection outputs reported in Table 7 and its appendix tables.

IX. Scenario Definitions and Sensitivity Parameters

Each modeled scenario s is defined as a tuple of repricing and protection parameters:

$$s = (\rho_s, \nu_s, \delta_s, \sigma_s, \lambda_s, \chi_s, \mu_s)$$

where ρ_s is the outpatient/routine repricing parameter; ν_s and δ_s are uncompensated-care and bad-debt relief parameters; and σ_s , λ_s , χ_s , and μ_s are stabilization parameters for safety-net providers, rural providers, children's hospitals, and negative-margin providers.

A. Repricing Scenarios

$$\rho_s = \begin{cases} 0.03, & \text{low repricing} \\ 0.05, & \text{base repricing} \\ 0.10, & \text{high repricing} \end{cases}$$

B. Protection Scenarios

$$(\nu_s, \delta_s) = \begin{cases} (0.25, 0.25), & \text{low protection} \\ (0.50, 0.50), & \text{base protection} \\ (0.75, 0.75), & \text{high protection} \end{cases}$$

$$(\sigma_s, \lambda_s, \chi_s, \mu_s) = \begin{cases} (0.015, 0.010, 0.010, 0.005), & \text{low protection} \\ (0.030, 0.020, 0.020, 0.010), & \text{base protection} \\ (0.050, 0.035, 0.035, 0.020), & \text{high protection} \end{cases}$$

C. Table 7 Scenario Matrix

The main Table 7 scenario matrix is generated by combining:

$$\rho_s \in \{0.03, 0.05, 0.10\}$$

with:

$$Protection_s \in \{Low, Base, High\}$$

This produces:

$$3 \text{ repricing scenarios} \times 3 \text{ protection scenarios} = 9 \text{ provider-impact scenarios}$$

D. Preferred Access-Protection Parameters

The preferred rural access-protection parameters are:

$$RAG_{j,t} = \min \{0.02 \times NPR_{j,t}, 5,000,000\}$$

$$RuralFloor = -0.015 \times NPR_{j,t}$$

The preferred behavioral institutional parameters are:

$$\theta_{BH} = 0.25$$

$$BehavioralFloor = -0.05 \times NPR_{j,t}$$

The scenario matrix evaluates the provider-side effect of UCC/HSE under alternative repricing and stabilization assumptions. The preferred access-protection layer is evaluated separately because it represents a policy refinement to the base model, not a change to the underlying market-repricing mechanism.

X. Data-Quality and Exposure-Anomaly Treatment

Provider-impact modeling depends on accurate revenue and exposure estimates. The model therefore includes audit and validation rules to identify missing, anomalous, or unreliable provider records. A provider may be flagged if:

$$GTR_{j,t} \leq 0$$

$$NPR_{j,t} \leq 0$$

or if:

$$NTG_{j,t}$$

falls outside reasonable validation bounds.

A provider may also be flagged if $OPEB_{j,t}$ is implausibly high or low relative to net patient revenue, provider class, or available HCRIS fallback variables.

XI. Use in Table 7 and Appendix Tables

The formulas in Sections VI through X generate the Table 7 provider-impact package. The main Table 7 outputs include:

- provider-impact scenario matrix;
- provider impact by provider group;

- provider impact by provider class;
- provider-level distribution by impact category;
- model coverage and data-quality audit;
- exposure-anomaly sensitivity; and
- access-protection policy options.

The appendix outputs include transition-protection sensitivity, enhanced rural access sensitivity, rural provider-level access protection, psychiatric/behavioral provider sensitivity, behavioral provider-level review, remaining severe behavioral effects, and the behavioral review universe.

The **Table 7** provider-impact model is not a complete provider-sector forecast. It is a structured, HCRIS-based static-impact model designed to estimate the first-order provider revenue effects of UCC/HSE routine-care repricing and targeted access protections. It should be interpreted as a provider-side scoring framework, sensitivity and distributional analysis, policy architecture for targeted stabilization, and appendix-supported audit trail. It should not be interpreted as a dynamic hospital closure model, a full general-equilibrium provider market forecast, a guarantee that all providers are held harmless, or a replacement for future CBO, CMS, MedPAC, or state-level actuarial modeling.

The provider-impact framework preserves the core UCC/HSE principle that routine-care markets should face price discipline while recognizing that not all providers are equally situated to absorb transition pressure. The model separates five concepts: market repricing exposure; coverage and liquidity offsets; transition stabilization; rural access protection; and behavioral institutional adjustment. This structure allows UCC/HSE to impose meaningful price discipline while protecting essential access in rural and behavioral-health settings.

XII. Aggregate Implementation for Federal Scoring

The equations presented in this Annex define the formal household-level structure of the Universal Catastrophic Coverage and Health Savings Empowerment framework. They describe how catastrophic premium credits and federal HSA deposits are determined for a given household as a function of income, household composition, and the statutory program parameters specified in the Act.

For purposes of federal budget scoring, the accompanying ten-year scoring workbook applies these equations using an aggregate population method rather than a full household-level microsimulation. This approach reflects standard federal budget scoring

practice when estimating the fiscal effects of large national programs whose statutory rules can be represented through population distributions and weighted averages.

Under the aggregate implementation used in the scoring workbook:

- Household variables such as $z_{h,t}$, $C_{h,t}$, and $H_{h,t}$ are represented through population-weighted averages across income bands and demographic categories.
- The catastrophic premium credit schedule defined in Section II is operationalized through a weighted federal contribution-share derived from the distribution of household incomes relative to the federal poverty level.
- The income-indexed HSA adjustment function $\gamma(z)$ is implemented as an average adjustment applied to the relevant population groups rather than calculated separately for each household.
- Chronic-condition supplements are calculated using prevalence estimates for qualifying conditions and applied to the eligible population as a distinct deposit category.
- Benchmark catastrophic premium levels are anchored to re-constructed catastrophic spending distributions derived from MEPS (HC-251) microdata and aligned with national health expenditure projections to ensure consistency with aggregate health spending trends.
- Population counts, enrollment assumptions, and demographic shares are then applied to the resulting credit and deposit schedules to produce annual federal outlay estimates.

In practical terms, the scoring workbook estimates the aggregate fiscal effect of the household-level formulas described in this Annex by applying those rules to population distributions rather than to individual household records.

The household equations therefore define the governing policy structure of the program, while the aggregate implementation provides a transparent and tractable method for estimating national fiscal effects within the conventions of federal budget scoring.

The catastrophic benchmark used in the scoring workbook is derived from reconstructed catastrophic spending distributions using MEPS (HC-251) microdata rather than directly from observed insurance premiums. This approach allows the model to isolate the portion of medical spending that exceeds the catastrophic deductible and therefore corresponds to the insured risk under UCC, ensuring that benchmark premiums reflect the underlying distribution of high-cost medical events rather than the pricing structure of comprehensive insurance products.

The reconstructed distribution is subsequently aligned with CMS National Health Expenditure projections to ensure that aggregate catastrophic spending levels remain consistent with national health spending trends.

Because MEPS microdata are known to understate aggregate national health expenditures, the reconstructed catastrophic spending distribution is scaled to align with CMS National Health Expenditure (NHE) totals for the corresponding year. This reconciliation ensures that the catastrophic risk distribution used in the scoring workbook reflects nationally consistent spending levels while preserving the empirical spending distribution observed in the microdata.

XIII. Illustrative Parameter Set (Modeling Baseline)

A. Catastrophic Credit Parameters

Baseline federal contribution rate (α): 0.50

Maximum income-indexed supplemental contribution (β_{max}): 0.35

(The maximum federal coverage share κ is reached only through application of the household premium cap (ϕ), not through formula-based contribution rates alone.)

Lower income breakpoint (z_L): 1.50 (150 percent of the federal poverty level)

Upper income breakpoint (z_U): 4.00 (400 percent of the federal poverty level)

Maximum federal coverage share (κ): 0.90 (90 percent of benchmark premium)

Household premium cap (ϕ): 0.10 (10 percent of household MAGI)

Notes:

- Households at or below 150 percent FPL receive a maximum contribution equal to $\alpha + \beta_{\text{max}} = 0.85$ (85 percent of benchmark premium), subject to the 90 percent ceiling.
- The supplemental contribution phases down linearly from 150–400 percent FPL.
- Above 400 percent FPL, households receive only the baseline federal contribution unless the premium cap binds.

B. Federal HSA Deposit Parameters

Adult baseline deposit (age 18+): \$750

Deposit for ages 0–5: \$1,300

Deposit for ages 6–17: \$1200

Chronic-condition supplement: \$2000

Maximum income-indexed HSA supplement (y_{max}): \$800

Lower income breakpoint for HSA supplement (z'_L): 2.00 (200 percent FPL)

Upper income breakpoint for HSA supplement (z'_U): 5.00 (500 percent FPL)

Notes:

- Age-banded pediatric deposits correspond to empirically observed expenditure ranges.
- Chronic-condition supplements apply per qualifying individual.
- The income-indexed HSA supplement phases down linearly between 200–500 percent FPL.
- Chronic-condition supplements are paid at a fixed rate per eligible individual and remain identical across income brackets; all progressivity is confined to the baseline deposit structure.

C. Indexation Parameters

Weight on benchmark premium growth (w_1): 0.50

Weight on median household income growth (w_2): 0.50

Notes:

- All contribution rates and HSA amounts are indexed annually using the blended index in Section IV.
- Automatic correction mechanisms in Title XI override indexation if federal outlays exceed the guardrail threshold.

D. Recession-Triggered Adjustments (not modeled)

Additional HSA deposit per adult during recession: +\$300

Additional HSA deposit per child during recession: +\$150

Maximum fiscal exposure of recession adjustments: 0.05 percent of GDP per fiscal year

Activation criteria:

- Unemployment rising by at least 1.0 percentage point above its 12-month trailing average, or
- Two consecutive quarters of negative real GDP growth

Deactivation criteria:

- Unemployment falling within 0.5 percentage points of its pre-trigger average, or
- Real GDP growth exceeding 1 percent for two consecutive quarters
- Recession-trigger adjustments are modeled as low-frequency events with average activation duration of 3–6 fiscal quarters across historical macroeconomic cycles.

E. Benchmark Premium Assumption (Modeling Anchor)

A representative benchmark catastrophic premium for modeling purposes:

\$7,500 per adult equivalent (2025 dollars).

This value may be adjusted by CBO or federal agencies for distributional or actuarial analysis.

F. Derived Federal Contribution Levels (Examples)

The following outcomes flow directly from the parameter set above:

Single adult at 150 percent FPL:

- Federal share of benchmark premium: approximately 85 percent
- Net premium: approximately 15 percent of benchmark (subject to the 10 percent income cap)

Family of four (two adults, two children under age 12) at 250 percent FPL:

- Federal share of benchmark premium: approximately 65–70 percent, assuming two adults and two children under age 12
- HSA deposits: \$750 per adult, \$1,300 for children ages 0–5, \$1200 for ages 6–17, plus income-indexed supplements

Single adult at 400 percent FPL:

- Federal share of benchmark premium: 50 percent (baseline)

G. Modeling Status

This parameter set:

- Aligns with the expenditure ranges and pediatric and chronic-care cost bands in Sections V and IX;
- Produces federal outlays consistent with the savings estimates in Section VIII;
- Integrates directly with the formal equations in Annex B.
- This parameter set is intended as an illustrative baseline for scoring purposes and does not constrain Congressional Budget Office discretion to test alternative parameter envelopes.

H. Provider-Impact Modeling Baseline

The provider-impact model is implemented separately from the household-level federal outlay model. It uses FY2024 CMS HCRIS hospital cost-report data to estimate static, first-order provider revenue effects under UCC/HSE. Full methodology, script mapping, validation logic, and sensitivity analysis are provided in **Appendix B: Reproducible HCRIS-**

Based Provider-Impact Scoring Appendix. The parameters below define the baseline Table 7 model.

Parameter	Baseline value / treatment
<i>Data source</i>	FY2024 CMS HCRIS hospital cost reports
<i>Provider backbone</i>	912 providers
<i>Modeled providers</i>	886 providers
<i>Excluded providers</i>	26 providers
<i>Exposure base</i>	Net outpatient exposure
<i>Net-to-gross ratio</i>	Net patient revenue / gross total patient revenue
<i>Outpatient exposure formula</i>	Gross outpatient revenue × net-to-gross ratio
<i>Primary offset variable</i>	HCRIS S-10 uncompensated-care cost
<i>Provider classification</i>	Cleaned analytic classes, not raw HCRIS labels alone
<i>Main provider groups</i>	General acute/academic; children's/pediatric; rural/CAH/IHS; specialty/behavioral/rehab

The baseline Table 7 scenario matrix uses the following parameter values:

Scenario dimension	Low	Base	High
<i>Routine/outpatient repricing rate</i>	5%	10%	15%
<i>UC/liquidity offset rate</i>	15%	25%	35%
<i>Stabilization multiplier</i>	0.5× bounded cap	1.0× bounded cap	1.5× bounded cap

The central provider-impact estimate is the **Base/Base** scenario:

Base/Base result	Estimate
<i>Modeled providers</i>	886
<i>Total net patient revenue</i>	~\$264.9B
<i>Total outpatient/routine exposure</i>	~\$135.2B
<i>Outpatient repricing pressure</i>	-\$13.52B
<i>UC/liquidity offset</i>	+\$1.91B
<i>Bounded stabilization support</i>	+\$0.30B
<i>Net provider impact</i>	-\$11.30B
<i>Net impact as share of NPR</i>	-4.3%

The preferred access-protection sensitivity applies targeted rural and behavioral-health protections without eliminating the main routine-care repricing mechanism:

<i>Access-protection parameter</i>	<i>Preferred treatment</i>
<i>Rural access guarantee</i>	2% of NPR, capped at \$5M per rural provider
<i>Rural implementation floor</i>	No qualifying rural provider below -1.5% of NPR
<i>Behavioral institutional repricing factor</i>	25% of standard modeled repricing pressure
<i>Behavioral implementation floor</i>	No qualifying behavioral provider below -5% of NPR
<i>Net impact after preferred rural/behavioral package</i>	Approximately -\$11.08B

Provider-impact estimates are not mechanically treated as federal budget outlays. Routine-care repricing pressure is a modeled provider-sector revenue effect. Only stabilization or access-support components become federal fiscal costs if enacted as federally financed payments. The provider-impact results are therefore reported as a separate **Table 7** distributional and access-risk analysis, not as a direct substitute for the federal outlay and savings calculations in **Annex D** and **Annex E**.

Annex D: TEN-YEAR FEDERAL SCORING FRAMEWORK

This Annex sets out the scoring framework for estimating federal outlays, savings, and net fiscal effects associated with the Universal Catastrophic Coverage and Health Savings Empowerment Act over the ten-year budget window. It specifies the categories, assumptions, and guardrail interactions that must be incorporated into baseline and alternative-scenario analyses. Numerical scoring tables appear separately. In addition to direct programmatic costs and savings, the scoring framework incorporates structural market effects arising from the transition away from comprehensive third-party payment toward consumer-directed routine care financing.

I. Federal Outlay Categories

Federal outlays under the Act consist of the following components. All amounts are scored relative to current-law baselines and incorporate annual indexation as described in Annex B and Title XI.

1. Catastrophic Credits

Federal catastrophic credits finance a defined share of the benchmark catastrophic premium for eligible households. Scoring must incorporate:

- baseline contribution rate (α),
- income-indexed supplement (β),
- federal coverage ceiling (κ),
- household income cap on net premiums (ϕ), and
- enrollment and demographic composition by rating area.

These credits replace premium tax credits under current law.

For scoring purposes, catastrophic credits are treated as refundable tax credits processed via the Internal Revenue Code.

2. Federal HSA Deposits

Federal deposits include:

- baseline adult deposits,
- pediatric deposits (0–5; 6–17),
- chronic-condition supplements,
- income-indexed HSA supplements (γ), and
- recession-triggered additions when applicable.

Deposits are treated as mandatory outlays. Chronic-condition supplements are modeled as fixed, income-independent payments; all income sensitivity applies solely to the baseline (non-chronic) deposit component.

3. Federal Reinsurance Expenditures

The reinsurance structure established in Title II generates outlays based on claims exceeding federal attachment points. Scoring shall model chronic-condition HSA funding effects consistent with the methodology specified in **Section VI**.

4. Transition and Administrative Costs

Outlays include:

- exchange wind-down activities,
- federal IT and data-matching modernization,
- transition-related marketplace closure costs,
- eligibility verification systems with IRS, SSA, and DHS,
- state implementation coordination.

These amounts are time-limited and front-loaded.

II. Federal Savings and Revenue Effects

Savings and revenue effects are scored relative to current law and generally increase over time due to projected growth in ACA subsidies and Medicaid expansion costs.

1. Elimination of Premium Tax Credits (IRC §36B)

Savings equal the projected outlays for premium tax credits under current law, phased out according to the statutory exchange sunset schedule.

2. Elimination of Cost-Sharing Reduction Subsidies (PHSA §1402)

Savings include direct CSR payments and administrative reconciliation functions.

3. Medicaid Expansion Transition

Savings reflect removal of the 90 percent FMAP for ACA expansion adults and associated administrative spending. The transition schedule is defined in statute.

4. Marketplace and Regulatory Savings

Savings result from terminating marketplace operations, call centers, plan-certification activities, and related regulatory functions superseded by the Act.

5. Employer Exclusion Phase-Down (IRC §106)

The phase-down increases taxable compensation and therefore federal revenues. JCT scores this component. Revenue effects rise over time as the exclusion diminishes.

6. Employer Contribution Reallocation Effects

Scoring shall assume that a portion of current employer premium expenditures is reallocated to employee-directed Health Savings Account (HSA) contributions or wage-equivalent compensation in response to the phase-down of the employer-sponsored insurance exclusion. While these employer payments are not scored as federal outlays, the model shall account for their impact on:

- household healthcare purchasing power,
- coverage adequacy across income tiers, and
- utilization patterns under the universal catastrophic framework.

Sensitivity scenarios shall model employer pass-through rates at high, central, and low levels to test system stability under varying employer response assumptions.

In addition to direct program elimination, long-term federal savings may arise from structural price adjustments in routine medical services as these markets transition away from third-party payment and toward transparent, consumer-directed pricing. These effects shall be evaluated using empirically grounded elasticity parameters and historical utilization response benchmarks.

III. Baseline Scoring Assumptions

Scoring must rely on the following baseline assumptions unless superseded by CBO:

1. Enrollment and Demographic Structure

- Universal eligibility for catastrophic coverage **among eligible residents as defined in Section IV.B (citizens, lawful permanent residents, and long-term lawfully present residents).**
- Medicaid expansion adults transitioned over a defined statutory period.
- CHIP enrollment unchanged.
- No changes to Medicare enrollment or benefit structure.

2. Premium Growth

- Annual benchmark catastrophic premium growth consistent with long-run private insurance projections.
- For modeling purposes, the assumed nominal growth rate is 4.5 percent absent regulatory or behavioral adjustments.

3. Income Growth

- Median household income growth consistent with long-run economic projections.
- For modeling purposes, the baseline nominal growth rate is 3.5 percent.

4. HSA Uptake and Participation

- Universal HSA eligibility is assumed.
- Deposit receipt is assumed to be nearly complete due to automatic federal contributions among individuals for whom positive federal deposits are specified under Annex B.

5. Behavioral and Market Effects

- Changes in coverage selection, employer benefit design, and insurer participation are incorporated to the extent required by CBO scoring rules.
- Dynamic macroeconomic scoring is excluded unless directed by the budget resolution.

6. Employer and Household Behavioral Response

- Scoring must reflect behavioral changes resulting from the structural shift from premium-based employer-sponsored coverage to a hybrid catastrophic-credit and HSA-financing model. This includes:

- the redirection of employer health expenditures from comprehensive insurance premiums toward employee-controlled HSA contributions and wage compensation;
- increased price-sensitivity and consumer-directed selection of routine care resulting from cash-equivalent HSA funding;
- reduced reliance on high-frequency low-acuity emergency services due to predictable funding for primary and chronic care through HSAs;
- increased continuity of coverage and reduced churn as a consequence of portable, income-indexed universal catastrophic protection.

These behavioral dynamics shall inform projected utilization rates, claim frequency, and long-term federal exposure in both baseline and alternative scoring scenarios.

These behavioral assumptions shall be applied consistently across baseline and alternative scenarios unless explicitly modified under Section V sensitivity testing.

7. Income Sensitivity and Coverage Retention Effects

Scoring shall incorporate the stabilizing effect of income-indexed catastrophic credits and HSA deposits on coverage continuity during income volatility. The model must account for:

- reduced disenrollment during periods of income fluctuation due to progressive credit and deposit schedules;
- mitigation of adverse selection through predictable exposure caps (ϕ) and universal eligibility;
- improved preventive care uptake in lower-income households due to guaranteed HSA support.

These factors shall be reflected through adjustments to enrollment persistence rates and utilization behavior across income bands.

IV. Ten-Year Scoring Structure

Scoring requires annual estimates over the full budget window for the following:

A. Federal Outlays

- Catastrophic credits
- Federal HSA deposits
- Federal reinsurance
- Transition and administrative expenditures
- Recession-triggered HSA adjustments (conditional)

B. Federal Savings

- Premium tax credit elimination
- CSR subsidy elimination
- Medicaid expansion transition savings
- Marketplace and ACA regulatory savings
- Employer exclusion phase-down revenues (JCT)

C. Net Federal Impact

- Annual and cumulative federal fiscal effect over the ten-year window
- Expressed in nominal dollars and as a share of GDP
- Compared directly to projected ACA subsidy trends

V. Sensitivity and Alternative Scenarios

Sensitivity analysis shall assess the fiscal impact of deviations in both actuarial risk variables and structural market-response dynamics across the following parameters:

1. Premium growth rate deviations
2. Income growth variations
3. Reinsurance attachment point changes
4. Prevalence and cost distribution of chronic conditions
5. Employer response to exclusion phase-down
6. HSA utilization patterns
7. Frequency of recession-trigger events
8. Enrollment elasticity across income bands
9. Routine-care price elasticity under consumer-directed financing
10. Chronic disease stabilization effects on catastrophic claim frequency and severity

Alternative scenarios must be provided in addition to the baseline score.

VI. Required Modeling Treatment of Chronic Disease Stabilization

For purposes of federal fiscal scoring, chronic-condition HSA supplements shall not be treated solely as transfer payments, but as structured investments intended to stabilize disease progression and reduce long-term system costs. Accordingly, scoring shall assume that predictable and continuous chronic-condition funding results in measurable reductions in:

- avoidable emergency department utilization,
- preventable inpatient hospital admissions,
- catastrophic claim volatility, and

- long-term federal reinsurance exposure attributable to unmanaged or deteriorating chronic conditions.

These effects shall be modeled as gradual but cumulative downward adjustments to catastrophic expenditure growth rates across the ten-year budget window. Magnitudes shall be determined using prevalence-adjusted chronic-condition cost curves, utilization elasticity parameters, and evidence-based disease trajectory assumptions consistent with actuarial best practices.

VII. Non-Coverage Erosion Risk and Coverage Stability Assumptions

Scoring shall explicitly evaluate the risk of coverage erosion during and after the transition from the Affordable Care Act framework to the Universal Catastrophic Coverage and Health Savings Empowerment system. The model must incorporate safeguards reflecting the statutory design intent to prevent loss of meaningful protection due to economic volatility, employer behavior, or individual under-enrollment.

Accordingly, scoring shall assume:

- that universal catastrophic protection remains continuously available to all eligible individuals regardless of income fluctuation;
- that income-indexed catastrophic credits and base HSA deposits materially reduce the risk of involuntary coverage lapse;
- that exposure caps and catastrophic premium limits (ϕ and κ) mitigate de facto disenrollment caused by affordability barriers; and
- that behavioral response modeling includes parameters for temporary non-enrollment or delayed enrollment during the transition period.

Where modeled coverage erosion occurs, the scorer shall distinguish between temporary administrative friction and structural access failure to avoid overstating systemic instability.

VIII. Transitional Utilization Volatility Effects

Scoring shall account for the likelihood of temporary utilization fluctuations during the transition period from comprehensive third-party payment systems to consumer-directed routine care financing. These effects may include short-term increases in:

- deferred care pent-up demand,
- diagnostic utilization normalization,

- one-time condition stabilization treatments, and
- transitional adjustments in provider billing practices.

Such utilization volatility shall be modeled as front-loaded and time-limited, with peak deviation occurring within the first three fiscal years following implementation. Long-run projections shall distinguish between transitional utilization spikes and sustained structural utilization trends, ensuring that temporary effects do not distort projected steady-state cost behavior.

IX. Relationship Between Statutory Design and CBO Baseline Methodology

This Annex recognizes that Congressional Budget Office baseline scoring conventions may diverge from the statutory intent and structural design logic of the UCC-HSE framework, particularly with respect to long-term behavioral and market dynamics.

Accordingly:

- Where baseline scoring understates second-order cost stabilization effects arising from improved chronic disease management or consumer-directed price mechanisms, such divergence shall be noted in accompanying explanatory materials;
- Alternative scenario modeling shall be used to demonstrate the full fiscal implications of statutory design features not reflected in conservative baseline scoring assumptions; and
- It is the intent of Congress that fiscal interpretation give due consideration to structural system reforms embedded in this Act, including those affecting utilization behavior, pricing transparency, employer compensation patterns, and long-term risk compression.

This clause is declarative and does not modify scoring authority, but clarifies legislative intent regarding interpretive alignment between statutory design and analytical modeling frameworks.

Nothing in this section shall be construed to alter the statutory independence of the Congressional Budget Office, but rather to clarify the interpretive intent of Congress regarding structural reform evaluation.

X. Fiscal Guardrail Integration

Title XI imposes a federal outlay guardrail on the combined cost of catastrophic credits, HSA deposits, and reinsurance. Scoring must incorporate:

- annual calculation of combined federal outlays relative to the statutory cap,
- automatic parameter adjustments when outlays exceed the tolerance band,
- indexation overrides triggered by guardrail breaches, and

- statutory limitations on discretionary increases.

All scoring must reflect the binding nature of these guardrails. Guardrail adjustments are modeled as directly influencing HSA deposit indexing and catastrophic credit parameters in the subsequent fiscal year.

Recession-trigger HSA adjustments are scored as automatic stabilizers and modeled with one-year fiscal lag consistent with CBO treatment of countercyclical transfer programs.

(including adjustment of κ (federal coverage ceiling) where necessary to enforce statutory outlay caps.)

XI. Status of Estimates

This Annex provides the required structure and methodological assumptions for 10-year scoring. Numerical estimates, cost tables, and distributional analyses appear separately and remain subject to revision based on:

- CBO and JCT modeling,
- updated economic and demographic projections,
- parameter adjustments under Annex B, and
- legislative modifications made during committee consideration.

The scoring framework explicitly recognizes that UCC-HSE represents a structural transition from actuarial risk pooling toward transparent and income-calibrated cost exposure. As such, long-term federal cost trajectories shall reflect both immediate fiscal displacement of ACA subsidy mechanisms and second-order behavioral effects associated with improved cost transparency, care prioritization, and employer compensation reallocation.

XI. Footnotes:

Technical Notes and Empirical Basis for Behavioral and Structural Savings Assumptions

The following section provides the empirical and technical foundation for the behavioral, structural, and macroeconomic savings assumptions embedded in the 10-year federal budget scoring of the Universal Catastrophic Coverage with Health Savings Empowerment (UCC-HSE) framework. It integrates narrative clarity with methodological specificity, ensuring that each assumption is both conceptually justified and technically defensible under fiscal scrutiny.

All figures are derived from peer-reviewed literature, government datasets, and nationally cited policy analyses. Savings are modeled as partial, phased, and bounded, consistent with Congressional Budget Office (CBO) scoring norms and the analytical restraint characteristic of Hoover and Manhattan Institute frameworks.

Avoided ER Overutilization (Avoided ER Over-utilization_2026)

The model's estimate of reduced emergency department utilization reflects extensive evidence that a substantial share of ED visits are clinically avoidable and driven by gaps in primary care access, insurance instability, and cost-insensitive utilization patterns. UnitedHealth Group's national analysis, based on retrospective claims-level classification using ICD diagnosis codes and standardized "avoidability" algorithms, estimates approximately 18 million emergency department visits per year are avoidable and account for roughly \$32 billion in unnecessary system-wide expenditure **(51)**. These visits predominantly involve ambulatory-sensitive conditions more appropriately treated in primary care or urgent care settings.

Baseline expenditure levels and payer distribution are corroborated by HCUP data sourced from the National Inpatient Sample and State Emergency Department Databases, which report that treat-and-release ED visits generated over \$80 billion in total costs in 2021, with Medicare and Medicaid responsible for a substantial share of spending **(52)**. These data establish a credible federal exposure baseline.

Technically, the model defines an avoidable ED cost base anchored to the \$32 billion estimate and trends it forward using national health expenditure growth assumptions. A federal expenditure share parameter is applied to isolate the portion borne by Medicare, Medicaid, and federal subsidy flows. A conservative utilization-reduction coefficient is then imposed, generally constrained to low double-digit percentages in the central case, reflecting incremental behavioral change rather than structural elimination of avoidable utilization. This ensures the model understates, rather than overstates, theoretical savings potential.

Bad Debt Reduction and Charity Care Tax Offset (BadDebtReduction_2026 / CharityCareTaxOffset_2026)

Hospitals incur significant uncompensated care burdens comprising both bad debt and charity care. The American Hospital Association, drawing on Medicare cost report data and hospital financial disclosures, estimates uncompensated care exceeding \$40 billion

annually, representing services for which no payment is received from patients or third parties **(53)**.

Empirical studies employing difference-in-differences analysis of Medicaid expansion under the ACA demonstrate statistically significant reductions in hospital bad debt, frequently ranging from 20–40 percent in expansion states relative to non-expansion states, supporting the assumption of partial and phased decline in bad debt exposure under expanded coverage conditions **(54)**.

MACPAC’s fiscal evaluations identify Disproportionate Share Hospital (DSH) payments as a mechanism through which uncompensated care is partially federalized, meaning reductions in-hospital uncompensated care translate into indirect federal savings via reduced subsidy pressure **(54)**.

The charity care component additionally informs the valuation of nonprofit hospital tax exemption. Kaiser Family Foundation estimates the federal component of hospital tax benefits at approximately \$14.4 billion annually, while subsequent JAMA analysis refines the empirical linkage between charity care levels and tax-exempt valuation derived from IRS Form 990 and cost report data **(55,56)**.

Technically, the model decomposes uncompensated care into bad debt and charity care components, applying distinct reduction coefficients derived from historical expansion experience. Bad debt reductions are treated as direct expenditure savings, while charity care reductions generate an indirect federal revenue offset constrained by conservative elasticity assumptions to avoid overstating tax expenditure recapture.

CO-OP Administrative Elimination, Risk Corridors, and Exchange IT (COOPAdminElim_2026, RiskCorridorSavings_2026, ExchangeITMaintenance_2026)

The CO-OP program was designed as a temporary capital financing initiative for nonprofit insurers, with appropriations largely disbursed between 2012 and 2015. Congressional Research Service analysis confirms that the majority of CO-OP entities failed or ceased operations by the late 2010s, eliminating any meaningful recurring administrative expenditure stream in the federal baseline **(57)**. Accordingly, the model assigns zero central savings for CO-OP administrative elimination post-2026, reflecting a disciplined distinction between sunk and recurring costs.

The ACA risk corridor program operated exclusively during the 2014–2016 marketplace stabilization period. The Supreme Court’s decision in *Maine Community Health Options v. United States* resolved retrospective liabilities but did not reinstate any forward programmatic obligations, eliminating plausible prospective savings **(58)**.

Federal Exchange IT costs, derived from CMS budget documentation, constitute a fractional subset of total marketplace administrative spending. The model therefore embeds presumed IT savings proportionally within overall administrative reductions and explicitly prevents double-counting through structured allocation constraints **(59)**.

Preventive Care Offset (PreventiveCareOffset_2026)

The literature consistently demonstrates that while preventive care improves health outcomes, only a minority of preventive interventions generate net fiscal savings. Cohen, Neumann, and Weinstein's NEJM analysis, based on cost-effectiveness and cost-utility modeling across preventive interventions, concludes that most prevention strategies are cost-effective but cost-increasing **(60)**. Subsequent regression-based policy analyses reinforce that large-scale prevention rarely produces broad budgetary relief absent highly targeted interventions **(61,62)**.

Accordingly, the model restricts preventive savings to a minimal fractional offset of federal health expenditure, representing reduced incidence of select late-stage conditions rather than system-wide spending contraction. This offset is capped and phased, ensuring alignment with empirical evidence and CBO scoring conventions.

End-of-Life Care Intensity (EndOfLifeIntensitySavings_2026)

Multiple empirical studies employing longitudinal Medicare claims analysis demonstrate that spending in the final year of life accounts for approximately 5–10 percent of total health expenditures, with concentration especially pronounced among Medicare populations **(63,64)**.

However, systematic reviews emphasize that much of this spending reflects clinically complex cases where significant cost compression is unlikely **(65)**. The model therefore applies only a modest intensity-reduction coefficient, reflecting incremental substitution toward palliative and community-based care. Savings are capped and calculated as a fraction of total end-of-life expenditure multiplied by the federal share, ensuring fiscal realism.

Medication Adherence (MedicationAdherenceSavings_2026)

Medication adherence analyses employing claims-derived medication possession ratio (MPR) and proportion-of-days-covered (PDC) metrics demonstrate that poor adherence is associated with increased avoidable hospitalization and downstream cost escalation,

particularly among chronic disease populations such as diabetes and hypertension cohorts **(66)**.

Integrated care models reducing patient cost-sharing have demonstrated improvements in adherence and modest downstream cost reductions, though these effects vary substantially by condition, socioeconomic status, and care delivery structure **(66-68)**.

The model operationalizes these findings by defining a conservative national adherence savings potential and applying a phased realization factor to account for gradual behavioral adoption and heterogeneous patient response.

Diagnostic Duplication (DiagnosticDuplicationSavings_2026)

Empirical studies employing retrospective imaging utilization analysis and chart review demonstrate that preventable diagnostic duplication constitutes a small fraction of total imaging use. Kamat et al. report that only approximately 0.4 percent of CT scans represent preventable duplicates, indicating limited aggregate savings potential **(68)**. Health information exchange studies employing comparative cohort methodologies demonstrate reduced repeat imaging but modest absolute fiscal effect **(69-72)**.

Accordingly, the model constrains duplication savings to a narrow proportion of imaging expenditures, treating such savings as secondary and structurally limited.

Taxable Wage Increase (Employer Exclusion Phase-Down) (TaxableWageIncrease_2026)

The exclusion of employer-sponsored insurance constitutes the single largest federal health tax expenditure. Empirical analyses and policy simulations confirm that phase-down of this exclusion shifts a portion of total compensation into taxable wages, increasing federal revenue without reducing aggregate compensation **(74-77)**.

Technically, the model applies a structured, phased reduction schedule to the exclusion baseline and multiplies the released compensation value by an average federal marginal tax rate, adjusted for partial wage pass-through and behavioral friction. This method avoids overstating revenue gains while preserving fiscal accuracy.

Disability Claim Reduction (DisabilityClaimReduction_2026)

The Oregon Health Insurance Experiment, employing randomized lottery assignment and quasi-experimental labor outcome tracking, demonstrates statistically insignificant impacts of expanded coverage on labor market participation and disability program

enrollment, including SSDI and SSI uptake **(76)**. Subsequent KFF analyses reinforce that disability-related savings attribution remains speculative **(77)**.

Consistent with this evidence, the model excludes disability savings from the central case and includes only minimal sensitivity adjustments, explicitly acknowledging uncertainty and preventing reliance on conjectural fiscal gains.

ANNEX E: 10-year CBO-style scoring table with historical CBO Data

Tables in this annex are reproduced from the UCC/HSE scoring workbook. The workbook contains the active formulas and source tabs; the PDF tables are static reproductions for publication. Machine-readable workbook files are available in the supplementary repository.

Supplementary Data and Analysis Repository:

This link below includes the entire CBO scoring excel workbook, scripts and outputs used to process HCRIS 2024 data, as well as the full MEPS HC-251 2023.

<https://github.com/Galtman-droid/Universal-Catastrophic-Coverage-with-Health-Savings-Empowerment-A-Structural-Replacement-for-the-ACA>

Summary Table: Outlays-Savings (2026-2056)

	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035
Total Outlays Under Act (Billions)	958.25	997.36	1038.01	1079.41	1124.66	1171.83	1221.00	1272.26	1325.70	1381.41
Total Savings Under Act (Billions)	432.9815	553.9324	684.813813	832.4944	1041.94211	1160.19864	1293.68255	1439.62151	1597.6548	1769.46945
Net Federal Impact = Outlays – Savings (Billions)	525.27	443.43	353.19	246.92	82.72	11.63	-72.69	-167.36	-271.96	-388.06
Cumulative Net Impact (Billions)	525.27	968.70	1321.90	1568.82	1651.53	1663.16	1590.48	1423.11	1151.15	763.09
10-Year Net Federal Impact (2026-2035) (Billions)	763.09									
20-Year Net Federal Impact (2026-2035) (Billions)	-7462.74									
30-Year Net Federal Impact (2026-2056) (Billions)	-32933.84									

	2036	2037	2038	2039	2040	2041	2042	2043	2044	2045
Total Outlays Under Act (Billions)	1433.18	1501.04	1539.72	1580.20	1622.55	1666.86	1713.22	1761.73	1812.48	1865.59
Total Savings Under Act (Billions)	1875.63761	1988.17587	2107.46642	2233.91441	2367.94927	2510.02623	2660.628	2820.26547	2989.4814	3168.8503
Net Federal Impact = Outlays – Savings (Billions)	-442.46	-487.14	-567.75	-653.72	-745.40	-843.17	-947.41	-1058.54	-1177.00	-1303.26
Cumulative Net Impact (Billions)	320.64	-166.50	-734.25	-1387.97	-2133.37	-2976.54	-3923.95	-4982.48	-6159.48	-7462.74

	2046	2047	2048	2049	2050	2051	2052	2053	2054	2055	2056
Total Outlays Under Act (Billions)	1921.15	1979.29	2040.12	2103.77	2170.36	2240.04	2312.94	2389.22	2469.03	2552.54	2639.91
Total Savings Under Act (Billions)	3358.9813	3560.520175	3774.15139	4000.60047	4240.6365	4495.07469	4764.77917	5050.66592	5353.70587	5674.92823	6015.42392
Net Federal Impact = Outlays – Savings (Billions)	-1437.83	-1581.23	-1734.03	-1896.83	-2070.28	-2255.04	-2451.84	-2661.45	-2884.68	-3122.39	-3375.51
Cumulative Net Impact (Billions)	-8900.57	-10481.80	-12215.83	-14112.66	-16182.94	-18437.97	-20889.81	-23551.26	-26435.93	-29558.33	-32933.84

Beta Calculation:

Band	z (normalized income)	PopShare	FPL%	rough average income	$\beta(z)$	Trigger PopSh	Federal Share (band)
Low income	1.2	0.3	<150	≤\$35–40k	0.35	0.17991244	1
Lower-middle	2.2	0.3	150-300	\$35–70k	0.072651	0.18680785	0.572651158
Upper-middle	3.2	0.25	300-600	\$70–140k	0.046917	0.24182406	0.546917092
High income	4.5	0.15	>600	> \$140k	0	0.39145564	0.5
Average Beta	0.13852462						
Average Federal Share	0.68352462						
Average Beta (trigger-weighted)	0.087886844						
Average Federal Share (trigger-weighted)	0.61487371						

Gamma Calculation:

Band	z (normalized income)	PopShare	FPL%	rough average income	$\gamma(z)$
Low income	1.2	0.3	<150	≤\$35–40k	800
Lower-middle	2.2	0.3	150-300	\$35–70k	746.666667
Upper-middle	3.2	0.25	300-600	\$70–140k	480
High income	4.5	0.15	>600	> \$140k	0
Average Gamma	584				

Outlays: UCC+HSE 2026-2056

all done with central range estimation	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035
Benchmark Premium (per enrollee)	7500	7785	8080.83	8387.902	8706.641799	9037.494187	9380.918966	9737.393887	10107.41485	10491.49662
Enrollees (persons)	2.70E+08	2.72E+08	2.74E+08	2.76E+08	2.79E+08	2.81E+08	2.83E+08	2.85E+08	2.88E+08	2.90E+08
Total Benchmark Premium Volume (billions)	791.00	827.62	865.94	906.04	947.99	991.89	1037.82	1085.87	1136.15	1188.76
Average Federal Share of Catastrophic Premium ($\alpha + \beta$ weighted)	0.61487371	0.61487371	0.614874	0.614874	0.61487371	0.61487371	0.61487371	0.61487371	0.61487371	0.61487371
Catastrophic Credits (billions)	486.36225	508.88	532.45	557.10	582.90	609.89	638.13	667.67	698.59	730.94
Adult HSA Deposits (billions)	261.20	270.8057843	280.7693	291.0993	301.8094604	312.9136341	324.4263525	336.3626469	348.7381014	361.5688736
Child 0-5 HSA Deposits (billions)	21.32	22.28002562	23.10631	23.95644	24.83784275	25.75167666	26.69913235	27.68144682	28.69990262	29.75582943
Child 6-17 HSA Deposits (billions)	42.07	43.61643837	45.22117	46.88495	48.60994296	50.39839999	52.25265792	54.17513771	56.16834938	58.23489529
Chronic Condition Supplements (billions)	121.88	126.3637942	131.013	135.8332	140.8307753	146.0122212	151.3843028	156.9540341	162.7286869	168.7158008
Total Federal HSA Deposits (billions)	446.47	463.0660425	480.1097	497.7739	516.0880214	535.0759319	554.7624456	575.1732655	596.3350403	618.2753991
Federal Reinsurance (billions)	21.42279092	22.41475184	23.45264	24.5386	25.67483089	26.86367826	28.10757402	29.40906713	30.77082457	32.19563683
Transition/Admin Costs (billions)	4	3	2	0	0	0	0	0	0	0
Total Federal Outlays (sum of all above)	958.25	997.36	1038.01	1079.41	1124.66	1171.83	1221.00	1272.26	1325.70	1381.41
10 year total outlays	11569.89									
30 year total outlays	52684.81									

	2036	2037	2038	2039	2040	2041	2042	2043	2044	2045	2046
Benchmark Premium (per enrollee)	10890.17	11304	11733.55	12179.43	12642.25	13122.65	13621.31	14138.92	14676.2	15233.9	15812.78
Enrollees (persons)	2.92E+08	2.95E+08	2.97E+08	2.99E+08	3.02E+08	3.04E+08	3.06E+08	3.09E+08	3.11E+08	3.14E+08	3.16E+08
Total Benchmark Premium Volume (billions)	1243.81	1301.40	1361.66	1424.71	1490.68	1559.70	1631.92	1707.49	1786.55	1869.28	1955.83
Average Federal Share of Catastrophic Premium ($\alpha + \beta$ weighted)	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874
Catastrophic Credits (billions)	764.78	800.20	837.25	876.02	916.58	959.02	1003.43	1049.89	1098.50	1149.37	1202.59
Adult HSA Deposits (billions)	371.1802	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408
Child 0-5 HSA Deposits (billions)	30.54681	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313
Child 6-17 HSA Deposits (billions)	59.78291	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179
Chronic Condition Supplements (billions)	173.2006	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281
Total Federal HSA Deposits (billions)	634.7105	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939
Federal Reinsurance (billions)	33.68642	35.24624	36.87828	38.58589	40.37257	42.24199	44.19796	46.2445	48.38581	50.62626	52.97046
Transition/Admin Costs (billions)	0	0	0	0	0	0	0	0	0	0	0
Total Federal Outlays (sum of all above)	1433.18	1501.04	1539.72	1580.20	1622.55	1666.86	1713.22	1761.73	1812.48	1865.59	1921.15

	2047	2048	2049	2050	2051	2052	2053	2054	2055	2056
Benchmark Premium (per enrollee)	16413.67	17037.39	17684.81	18356.83	19054.39	19778.46	20530.04	21310.18	22119.97	22960.53
Enrollees (persons)	3.19E+08	3.21E+08	3.24E+08	3.27E+08	3.29E+08	3.32E+08	3.35E+08	3.37E+08	3.40E+08	3.43E+08
Total Benchmark Premium Volume (billions)	2046.40	2141.15	2240.30	2344.03	2452.57	2566.13	2684.95	2809.28	2939.36	3075.46
Average Federal Share of Catastrophic Premium ($\alpha + \beta$ weighted)	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874	0.614874
Catastrophic Credits (billions)	1258.27	1316.54	1377.50	1441.28	1508.02	1577.85	1650.91	1727.35	1807.33	1891.02
Adult HSA Deposits (billions)	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408	389.2408
Child 0-5 HSA Deposits (billions)	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313	32.03313
Child 6-17 HSA Deposits (billions)	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179	62.69179
Chronic Condition Supplements (billions)	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281	181.6281
Total Federal HSA Deposits (billions)	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939	665.5939
Federal Reinsurance (billions)	55.42321	57.98952	60.67467	63.48415	66.42372	69.4994	72.7175	76.08461	79.60764	83.29379
Transition/Admin Costs (billions)	0	0	0	0	0	0	0	0	0	0
Total Federal Outlays (sum of all above)	1979.29	2040.12	2103.77	2170.36	2240.04	2312.94	2389.22	2469.03	2552.54	2639.91

Savings UCC+ HSE 2026-2056:

	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035
ACA Premium Tax Credits (baseline) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	82	82	85	90	93	96	102	109	115.54	122.4724
ACA CSR Payments (billions)	12.72	13.4832	14.292192	15.149724	16.058707	17.022229	18.043563	19.126177	20.273748	21.490172
Medicaid Expansion Adults baseline (federal share) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	120	129	138	149	159	171	184	198	209.88	222.4728
Marketplace Admin / Plan Management (billions)	3.18	3.3708	3.573048	3.7874309	4.0146767	4.2555573	4.5108908	4.7815442	5.0684369	5.3725431
Employer Exclusion Revenue Loss (under current law) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	451	495	523	550	578	608	641	676	716.56	759.5536
Medicaid NON-expansion baseline (2026) (billions)	424	449.44	476.4064	504.99078	535.29023	567.40764	601.4521	637.53923	675.79158	716.33908
Uncompensated care baseline (2026) (billions)	53	56.18	59.5508	63.123848	66.911279	70.925956	75.181513	79.692404	84.473948	89.542385
FULL employer exclusion baseline (2026) (billions)	371	393.26	416.8556	441.86694	468.37895	496.48169	526.72059	557.84683	591.31764	626.79669
SilverLoading (billions)	3.286	3.48316	3.6921496	3.9136786	4.1484993	4.3974092	4.6612538	4.940929	5.2373848	5.5516279
1332PassThrough (billions)	5.3	5.618	5.95508	6.3123848	6.6911279	7.0925956	7.5181513	7.9692404	8.4473948	8.9542385
ACARiskAdj. Admin (billions)	5.30E-02	5.62E-02	5.96E-02	6.31E-02	6.69E-02	7.09E-02	7.52E-02	7.97E-02	8.45E-02	8.95E-02
MedicaidChurn Admin (billions)	3.18	3.37	3.57	3.79	4.01	4.26	4.51	4.78	5.07	5.37
Avoided ER Overutilization (billions)	2.00	2.12	2.25	2.38	2.52	2.68	2.84	3.01	3.19	3.38
BadDebtReduction effect (billions)	5.96	6.32	6.70	7.10	7.53	7.98	8.46	8.97	9.50	10.07
CharityCareTaxOffset (billions)	2.00	2.12	2.25	2.38	2.52	2.68	2.84	3.01	3.19	3.38
PreventiveCareOffset (billions)	1.50	1.59	1.69	1.79	1.89	2.01	2.13	2.26	2.39	2.53
EndOfLifeIntensitySavings (billions)	2.00	2.12	2.25	2.38	2.52	2.68	2.84	3.01	3.19	3.38
MedicationAdherenceSavings (billions)	0.25	0.27	0.28	0.30	0.32	0.33	0.35	0.38	0.40	0.42
DiagnosticDuplicationSavings (billions)	0.25	0.27	0.28	0.30	0.32	0.33	0.35	0.38	0.40	0.42
TaxableWageIncrease (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Disability (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Behavioral Savings (billions) aggregate	11.96	12.68	13.44	14.25	15.10	16.01	16.97	17.99	19.07	20.21
Children in Medicaid baseline (billions) (CBO Pub. 59273 Table A-2)	57	60	62	66	69	73	79	86	91.16	96.2696
Children in Medicaid residual (special needs only) (billions) = 7.6M * \$5,000	38	40.28	42.6968	45.28608	47.974124	50.852572	53.903276	57.13795	60.566227	64.2002
Savings: PTC Eliminated (billions)	82	82	85	90	93	96	102	109	115.54	122.4724
Savings: CSR Eliminated (billions)	12.72	13.4832	14.292192	15.149724	16.058707	17.022229	18.043563	19.126177	20.273748	21.490172
Savings: Medicaid Expansion Transition (billions)	120	129	138	149	159	171	184	198	209.88	222.4728
Savings: Marketplace Admin Eliminated (billions)	3.18	3.3708	3.573048	3.7874309	4.0146767	4.2555573	4.5108908	4.7815442	5.0684369	5.3725431
Revenue Gain: Employer Exclusion Phase-Down (billions)	45.1	99	156.9	220	289	364.8	448.7	540.8	644.904	759.5536
Savings: Medicaid Non-Expansion Adults Transition	84.8	179.776	285.84384	403.99263	535.29023	667.40764	801.4521	937.53923	1075.79158	1216.33908
Savings: Uncompensated Care Reduction (billions)	42.4	44.944	47.64064	50.499078	53.529023	56.740764	60.14521	63.753923	67.579158	71.633908
Savings: SilverLoading (billions)	3.286	3.48316	3.6921496	3.9136786	4.1484993	4.3974092	4.6612538	4.940929	5.2373848	5.5516279
Savings: 1332PassThrough (billions)	5.3	5.618	5.95508	6.3123848	6.6911279	7.0925956	7.5181513	7.9692404	8.4473948	8.9542385
Savings: ACARiskAdj. Admin (billions)	5.30E-02	5.62E-02	5.96E-02	6.31E-02	6.69E-02	7.09E-02	7.52E-02	7.97E-02	8.45E-02	8.95E-02
Savings: MedicaidChurn Admin (billions)	3.18	3.37	3.57	3.79	4.01	4.26	4.51	4.78	5.07	5.37
Savings: Behavioral Savings (billions)	11.96	12.68	13.44	14.25	15.10	16.01	16.97	17.99	19.07	20.21
Savings: Children in Medicaid cut (normal kids removed; special needs retained) (billions)	19	19.72	19.3032	20.741392	21.025876	22.147428	25.096274	28.86205	30.593773	32.4294
Total Savings / Revenues (billions)	432.9815	553.9324	684.81381	832.4944	1041.9421	1160.1986	1293.6826	1439.6215	1597.6548	1769.4694
ten year Total Conservative Savings (billions)	10806.791									
thirty year Total Conservative Savings (billions)	AR1R A54									

	2036	2037	2038	2039	2040	2041	2042	2043	2044	2045
ACA Premium Tax Credits (baseline) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	129.82074	137.60999	145.86659	154.61858	163.8957	173.72944	184.15321	195.2024	206.91454	219.32942
ACA CSR Payments (billions)	22.779583	24.146358	25.595139	27.130847	28.758698	30.48422	32.313273	34.25207	36.307194	38.485626
Medicaid Expansion Adults baseline (federal share) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	235.82117	249.97044	264.96866	280.86678	297.71879	315.58192	334.51683	354.58784	375.86311	398.4149
Marketplace Admin / Plan Management (billions)	5.6948957	6.0365894	6.3987848	6.7827119	7.1896746	7.6210551	8.0783184	8.5630175	9.0767985	9.6214064
Employer Exclusion Revenue Loss (under current law) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026-2033), extrapolated thereafter	805.12682	853.43442	904.64049	958.91892	1016.4541	1077.4413	1142.0878	1210.613	1283.2498	1360.2448
Medicaid NON-expansion baseline (2026) (billions)	759.31942	804.87859	853.1713	904.36158	958.62328	1016.1407	1077.1091	1141.7357	1210.2398	1282.8542
Uncompensated care baseline (2026) (billions)	94.914928	100.60982	106.64641	113.0452	119.82791	127.01758	134.63864	142.71696	151.27998	160.35677
FULL employer exclusion baseline (2026) (billions)	664.4045	704.26877	746.52489	791.31638	838.79537	889.12309	942.47048	999.0187	1058.9598	1122.4974
SilverLoading (billions)	5.8847255	6.2378091	6.6120776	7.0088023	7.4293304	7.8750902	8.3475956	8.8484514	9.3793585	9.94212
1332PassThrough (billions)	9.4914928	10.060982	10.664641	11.30452	11.982791	12.701758	13.463864	14.271696	15.127998	16.036577
ACARiskAdj. Admin (billions)	9.49E-02	1.01E-01	1.07E-01	1.13E-01	1.20E-01	1.27E-01	1.35E-01	1.43E-01	1.51E-01	1.60E-01
MedicaidChurn Admin (billions)	5.69	6.04	6.40	6.78	7.19	7.62	8.08	8.56	9.08	9.62
Avoided ER Overutilization (billions)	3.58	3.80	4.02	4.27	4.52	4.79	5.08	5.39	5.71	6.05
BadDebtReduction effect (billions)	10.68	11.32	12.00	12.72	13.48	14.29	15.15	16.06	17.02	18.04
CharityCareTaxOffset (billions)	3.58	3.80	4.02	4.27	4.52	4.79	5.08	5.39	5.71	6.05
PreventiveCareOffset (billions)	2.69	2.85	3.02	3.20	3.39	3.59	3.81	4.04	4.28	4.54
EndOfLifeIntensitySavings (billions)	3.58	3.80	4.02	4.27	4.52	4.79	5.08	5.39	5.71	6.05
MedicationAdherenceSavings (billions)	0.45	0.47	0.50	0.53	0.57	0.60	0.64	0.67	0.71	0.76
DiagnosticDuplicationSavings (billions)	0.45	0.47	0.50	0.53	0.57	0.60	0.64	0.67	0.71	0.76
TaxableWageIncrease (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Disability (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Behavioral Savings (billions) aggregate	21.42	22.71	24.07	25.52	27.05	28.67	30.39	32.21	34.15	36.19
Children in Medicaid baseline (billions) (CBO Pub. 59273 Table A-2)	102.42738	108.57302	115.0874	121.99264	129.3122	137.07093	145.29519	154.0129	163.25368	173.0489
Children in Medicaid residual (special needs only) (billions) = 7.6M * \$5,000	68.052212	72.135345	76.463466	81.051274	85.91435	91.069211	96.533364	102.32537	108.46489	114.97278
Savings: PTC Eliminated (billions)	129.82074	137.60999	145.86659	154.61858	163.8957	173.72944	184.15321	195.2024	206.91454	219.32942
Savings: CSR Eliminated (billions)	22.779583	24.146358	25.595139	27.130847	28.758698	30.48422	32.313273	34.25207	36.307194	38.485626
Savings: Medicaid Expansion Transition (billions)	0	0	0	0	0	0	0	0	0	0
Savings: Marketplace Admin Eliminated (billions)	5.6948957	6.0365894	6.3987848	6.7827119	7.1896746	7.6210551	8.0783184	8.5630175	9.0767985	9.6214064
Revenue Gain: Employer Exclusion Phase-Down (billions)	805.12682	853.43442	904.64049	958.91892	1016.4541	1077.4413	1142.0878	1210.613	1283.2498	1360.2448
Savings: Medicaid Non-Expansion Adults Transition	759.31942	804.87859	853.1713	904.36158	958.62328	1016.1407	1077.1091	1141.7357	1210.2398	1282.8542
Savings: Uncompensated Care Reduction (billions)	75.931942	80.487859	85.31713	90.436158	95.862328	101.61407	107.71091	114.17357	121.02398	128.28542
Savings: SilverLoading (billions)	5.8847255	6.2378091	6.6120776	7.0088023	7.4293304	7.8750902	8.3475956	8.8484514	9.3793585	9.94212
Savings: 1332PassThrough (billions)	9.4914928	10.060982	10.664641	11.30452	11.982791	12.701758	13.463864	14.271696	15.127998	16.036577
Savings: ACARiskAdj. Admin (billions)	9.49E-02	1.01E-01	1.07E-01	1.13E-01	1.20E-01	1.27E-01	1.35E-01	1.43E-01	1.51E-01	1.60E-01
Savings: MedicaidChurn Admin (billions)	5.69	6.04	6.40	6.78	7.19	7.62	8.08	8.56	9.08	9.62
Savings: Behavioral Savings (billions)	21.42	22.71	24.07	25.52	27.05	28.67	30.39	32.21	34.15	36.19
Savings: Children in Medicaid cut (normal kids removed; special needs retained) (billions)	34.375164	36.437673	38.623934	40.94137	43.397852	46.001723	48.761826	51.687536	54.788788	58.076115
Total Savings / Revenues (billions)	1875.6376	1988.1759	2107.4664	2233.9144	2367.9493	2510.0262	2660.6278	2820.2655	2989.4814	3168.8503

	2046	2047	2048	2049	2050	2051	2052	2053	2054	2055	2056
ACA Premium Tax Credits (baseline) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026–2033), extrapolated thereafter	232.48918	246.43853	261.22484	276.89833	293.51223	311.12297	329.79035	349.57777	370.55243	392.78558	416.35271
ACA CSR Payments (billions)	40.794763	43.242449	45.836996	48.587216	51.502449	54.592596	57.868151	61.34024	65.020655	68.921894	73.057208
Medicaid Expansion Adults baseline (federal share) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026–2033), extrapolated th	422.3198	447.65898	474.51852	502.98963	533.16901	565.15915	599.0687	635.01282	673.11359	713.50041	756.31043
Marketplace Admin / Plan Management (billions)	10.198691	10.810612	11.459249	12.146804	12.875612	13.648149	14.467038	15.33506	16.255164	17.230474	18.264302
Employer Exclusion Revenue Loss (under current law) (billions) Baseline (CBO Pub. 59273 Table A-2; 2026–2033), extrapolated	1441.8595	1528.3711	1620.0733	1717.2777	1820.3144	1929.5333	2045.3053	2168.0236	2298.105	2435.9913	2582.1508
Medicaid NON-expansion baseline (2026) (billions)	1359.8254	1441.415	1527.8999	1619.5739	1716.7483	1819.7532	1928.9384	2044.6747	2167.3552	2297.3965	2435.2403
Uncompensated care baseline (2026) (billions)	169.97818	180.17687	190.98748	202.44673	214.59354	227.46915	241.1173	255.58433	270.91939	287.17456	304.40503
FULL employer exclusion baseline (2026) (billions)	1189.8473	1261.2381	1336.9124	1417.1271	1502.1548	1592.284	1687.8211	1789.0903	1896.4358	2010.2219	2130.8352
SilverLoading (billions)	10.538647	11.170966	11.841224	12.551697	13.304799	14.103087	14.949272	15.846229	16.797002	17.804823	18.873112
1332PassThrough (billions)	16.997818	18.017687	19.098748	20.244673	21.459354	22.746915	24.11173	25.568433	27.091939	28.717456	30.440503
ACARiskAdj. Admin (billions)	1.70E-01	1.80E-01	1.91E-01	2.02E-01	2.15E-01	2.27E-01	2.41E-01	2.56E-01	2.71E-01	2.87E-01	3.04E-01
MedicaidChurn. Admin (billions)	10.20	10.81	11.46	12.15	12.88	13.65	14.47	15.34	16.26	17.23	18.26
Avoided ER Overutilization (billions)	6.41	6.80	7.21	7.64	8.10	8.58	9.10	9.64	10.22	10.84	11.49
BadDebtReduction effect (billions)	19.12	20.27	21.49	22.78	24.14	25.59	27.13	28.75	30.48	32.31	34.25
CharityCareTaxOffset (billions)	6.41	6.80	7.21	7.64	8.10	8.58	9.10	9.64	10.22	10.84	11.49
PreventiveCareOffset (billions)	4.81	5.10	5.41	5.73	6.07	6.44	6.82	7.23	7.67	8.13	8.62
EndOfLifeIntensitySavings (billions)	6.41	6.80	7.21	7.64	8.10	8.58	9.10	9.64	10.22	10.84	11.49
MedicationAdherenceSavings (billions)	0.80	0.85	0.90	0.95	1.01	1.07	1.14	1.21	1.28	1.35	1.44
DiagnosticDuplicationSavings (billions)	0.80	0.85	0.90	0.95	1.01	1.07	1.14	1.21	1.28	1.35	1.44
TaxableWageIncrease (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Disability (billions)	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00	0.00
Behavioral Savings (billions) aggregate	38.37	40.67	43.11	45.69	48.44	51.34	54.42	57.69	61.15	64.82	68.71
Children in Medicaid baseline (billions) (CBO Pub. 59273 Table A-2)	183.43183	194.43774	206.104	218.47024	231.57846	245.47317	260.20156	275.81365	292.36247	309.90422	328.49847
Children in Medicaid residual (special needs only) (billions) = 7.6M * \$5,000	121.87115	129.18342	136.93442	145.15049	153.85952	163.09109	172.87655	183.24915	194.24409	205.89874	218.25266
Savings: PTC Eliminated (billions)	232.48918	246.43853	261.22484	276.89833	293.51223	311.12297	329.79035	349.57777	370.55243	392.78558	416.35271
Savings: CSR Eliminated (billions)	40.794763	43.242449	45.836996	48.587216	51.502449	54.592596	57.868151	61.34024	65.020655	68.921894	73.057208
Savings: Medicaid Expansion Transition (billions)	0	0	0	0	0	0	0	0	0	0	0
Savings: Marketplace Admin Eliminated (billions)	10.198691	10.810612	11.459249	12.146804	12.875612	13.648149	14.467038	15.33506	16.255164	17.230474	18.264302
Revenue Gain: Employer Exclusion Phase-Down (billions)	1441.8595	1528.3711	1620.0733	1717.2777	1820.3144	1929.5333	2045.3053	2168.0236	2298.105	2435.9913	2582.1508
Savings: Medicaid Non-Expansion Adults Transition	1359.8254	1441.415	1527.8999	1619.5739	1716.7483	1819.7532	1928.9384	2044.6747	2167.3552	2297.3965	2435.2403
Savings: Uncompensated Care Reduction (billions)	135.98254	144.1415	152.78999	161.95739	171.67483	181.97532	192.89384	204.46747	216.73552	229.73965	243.52403
Savings: SilverLoading (billions)	10.538647	11.170966	11.841224	12.551697	13.304799	14.103087	14.949272	15.846229	16.797002	17.804823	18.873112
Savings: 1332PassThrough (billions)	16.997818	18.017687	19.098748	20.244673	21.459354	22.746915	24.11173	25.568433	27.091939	28.717456	30.440503
Savings: ACARiskAdj. Admin (billions)	1.70E-01	1.80E-01	1.91E-01	2.02E-01	2.15E-01	2.27E-01	2.41E-01	2.56E-01	2.71E-01	2.87E-01	3.04E-01
Savings: MedicaidChurn. Admin (billions)	10.20	10.81	11.46	12.15	12.88	13.65	14.47	15.34	16.26	17.23	18.26
Savings: Behavioral Savings (billions)	38.37	40.67	43.11	45.69	48.44	51.34	54.42	57.69	61.15	64.82	68.71
Savings: Children in Medicaid cut (normal kids removed; special needs retained) (billions)	61.560682	65.254323	69.169583	73.319758	77.718943	82.38208	87.325005	92.564505	98.118375	104.00548	110.24581
Total Savings / Revenues (billions)	3358.9813	3560.5202	3774.1514	4000.6005	4240.6365	4495.0747	4764.7792	5050.6659	5353.7059	5674.9282	6015.4239

Consumer Savings 2026-2035:

Consumer Impact (billions unless noted)	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	Notes / status
Year											
Federal transfers to households											
Catastrophic Credits (billions)	486.36	508.88	532.45	557.1	582.9	609.89	638.13	667.67	698.59	730.9385	From Outlays: Catastrophic Credits
Federal HSA Deposits (billions)	446.47	463.07	480.11	497.77	516.09	535.08	554.76	575.17	596.34	618.28	From Outlays: Total Federal HSA Deposits
Total Federal Transfers (Credits + HSA) (billions)	932.83	971.95	1012.56	1054.87	1098.98	1144.96	1192.89	1242.85	1294.93	1349.21	
Total Benchmark Premium Volume (billions)	791.00	827.62	865.94	906.04	947.99	991.89	1037.82	1085.87	1136.15	1188.76	From Outlays: Total Benchmark Premium Volume
Average Federal Share of Catastrophic Premium (share, 0–1)	0.6149	0.6149	0.6149	0.6149	0.6149	0.6149	0.6149	0.6149	0.6149	0.614874	From Outlays: Average Federal Share
Household catastrophic premium payments (benchmark-based)	304.63	318.74	333.5	348.94	365.1	382	399.69	418.2	437.56	457.8235	Computed: benchmark volume * (1 - federal share)
Baseline Routine Spend (billions)	598.71	621.46	645.07	669.58	695.03	721.44	748.85	777.31	806.85	837.5085	
HSE Routine Spend (billions)	588.65	611.02	634.23	658.34	683.35	709.32	736.27	764.25	793.29	823.4384	
Routine Savings (billions)	10.058	10.44	10.837	11.249	11.676	12.12	12.581	13.059	13.555	14.07014	
Net household position (transfers – catastrophic premium + routine savings) (billions)	638.26	663.65	689.90	717.18	745.56	775.08	805.78	837.71	870.92	905.46	Computed: (Credits + HSA) – household catastrophic premium
Total Consumer savings (billions) ten year projection	7649.50										
UCC household out-of-pocket after HSE deposits (premium + routine – deposits) (billions)	446.81	466.69	487.62	509.50	532.36	556.25	581.20	607.28	634.52	662.99	
Net out-of-pocket spending after transfers (billions)	446.81	466.69	487.62	509.50	532.36	556.25	581.20	607.28	634.52	662.99	
Total Net out of pocket spending ten year projection	5485.22										
ACA baseline (current law) — consumer ledger											
ACA Premium Tax Credits (baseline) (billions)	82	82	85	90	93	96	102	109	115.54	122.4724	
ACA Gross Marketplace Premium Volume (implied) (billions)	129.58	129.58	134.33	142.23	146.97	151.71	161.19	172.25	182.59	193.5436	
ACA Household Net Premium Payments (implied) (billions)	47.585	47.585	49.326	52.227	53.968	55.709	59.191	63.253	67.048	71.07116	
ACA Routine Out-of-Pocket (baseline) (billions)	598.71	621.46	645.07	669.58	695.03	721.44	748.85	777.31	806.85	837.5085	
ACA Net out-of-pocket spending (premiums + OOP) (billions)	646.29	669.04	694.4	721.81	749	777.15	808.05	840.56	873.9	908.5797	
Total ACA Net out-of-pocket spending ten year projection	7688.8										
Net household out-of-pocket savings vs current law ACA (billions)	199.48	202.35	206.77	212.31	216.64	220.90	226.84	233.29	239.38	245.59	
Total net household out-of-pocket savings vs ACA ten year projection	2203.55										

Medicaid Consumer Savings:

Consumer Impact — Former Medicaid Adults (Low-income band proxy) (billions unless noted)												
Year	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	Sum	
Purpose: isolate the low-income band (<150% FPL) as a proxy for former Medicaid adults; used for distributional/hold-harmless analysis, not for headline 'consumer savings' totals.												
Low-income band PopShare (proxy)	0.26575394	0.26575	0.26575394	0.26575394	0.26575394	0.26575394	0.26575394	0.26575394	0.26575394	0.26575394		
Federal share of catastrophic premium (a + b)(z, low), capped at 100%	1	1	1	1	1	1	1	1	1	1		
Household share of catastrophic premium <150% FPL	0	0	0	0	0	0	0	0	0	0		
Benchmark premium volume allocated to low-income band (billions)	210.209701	219.943	230.1275033	240.783272	251.9325683	263.5980435	275.8036873	288.5745013	301.936655	315.9175299		
Household catastrophic premium payments — low-income band (billions)	0	0	0	0	0	0	0	0	0	0		
Federal catastrophic credits — low-income band (billions)	210.209701	219.943	230.1275033	240.783272	251.9325683	263.5980435	275.8036873	288.5745013	301.936655	315.9175299		
HSE routine spend allocated to low-income band (billions)	156.4349364	157.686	158.9479072	160.2194904	161.5012464	162.7932563	164.0956024	165.4083672	166.7316341	168.0654872		
Federal adult HSA deposits allocated to low-income band (billions)	168.77	170.123	171.4838674	172.8557383	174.2385842	175.6324929	177.0375529	178.4538533	179.8814841	181.320536		
Net household out-of-pocket after HSE deposits (premium + routine – deposits) (billions)	-12.34	-12.436	-12.53596022	-12.6362479	-12.73733788	-12.83923658	-12.94195048	-13.04548608	-13.14884997	-13.25504877		
Current Law (Medicaid) household payments baseline assumed in this tab (billions)	14.48	15.035	15.60628847	16.19932744	16.81490188	17.453986815	18.11711514	18.80556551	19.520177	20.26194373		
Remaining Household Burden UCC/HSE vs current law (billions)	2.15	2.60	3.07	3.56	4.08	4.61	5.18	5.76	6.37	7.01		
Interpretation: Medicaid households have low premium/OOP under current law; this tab treats baseline household payments as –0 to illustrate that any 'consumer improvement' here mainly reflects federal support (credits/HSE deposits), not price. If you want a non-zero Medicaid baseline (cost-sharing/premiums), replace row 20 with a sourced series and the net change line (row 21) will update automatically.												
This tab uses the model's low-income band (<150% FPL; PopShare from BetaCalc) as a proxy for former Medicaid adults; it allocates benchmark premiums, routine spend, and deposits proportionally by PopShare.												
Total HSA Deposits for each group (billions)												
Adult population (non-disabled adults)	4.14E+07											
Non-Special Needs Children	3.03E+07											
Total Medicaid population affected	7.17E+07											
Average Medicaid OOP (2014)	202.016											
Chronic Adult population	1.24E+07	44.09										
Chronic Children Population	9.09E+06											
Adult population non chronic non disabled	2.90E+07	44.92										
Children population Non chronic non disabled	2.12E+07											
Children 0-5 Non chronic non disabled	6.88E+06	14.45										
Children 6-17 Non chronic non disabled	1.43E+07	28.66										
Children 0-5 chronic	2.95E+06	12.09										
Children 6-17 chronic	6.14E+06	24.57										

Supplementary Tables and Figures:

This supplementary section reproduces selected source tables and figures used to support the UCC/HSE scoring and distributional assumptions. The tables are not independent model outputs; they are source evidence used to calibrate the federal subsidy baseline, Medicare-exclusion logic, and expenditure-concentration assumptions.

Table A-2.

CBO and JCT's Projections of Net Federal Subsidies for Health Insurance

Billions of Dollars, by Fiscal Year

	Actual, 2022 ^a	2023	2024	2025	2026	2027	2028	2029	2030	2031	2032	2033	Total, 2024– 2033
Employment-Based Coverage													
Tax exclusion for employment-based coverage ^{b,c}	n.a.	345	365	391	451	495	523	550	578	608	641	676	5,277
Income tax deduction for self-employment health insurance	n.a.	4	5	5	7	8	8	9	9	10	10	11	82
Small-employer tax credits ^c	n.a.	*	*	*	*	*	*	*	*	*	*	*	*
Gross collections of penalty payments from employers ^d	n.a.	-1	-1	-1	-1	-1	-1	-1	-1	-1	-1	-1	-11
Subtotal	n.a.	348	369	395	457	501	530	558	586	616	650	686	5,348
Medicaid and CHIP^e													
People age 65 or older in Medicaid	103	100	95	97	105	113	119	126	134	141	149	156	1,236
Blind and disabled people in Medicaid	172	160	147	151	160	170	179	189	199	211	224	236	1,866
Children in Medicaid	63	64	54	54	57	60	62	66	69	73	79	86	661
Adults made eligible for Medicaid by the ACA	118	126	116	111	120	129	138	149	159	171	184	198	1,474
Adults otherwise eligible for Medicaid	76	81	69	65	70	74	79	84	89	95	101	108	833
CHIP	17	17	17	18	18	19	20	20	21	22	20	15	189
Subtotal	548	547	497	495	530	566	597	633	671	713	756	799	6,258
Medicare ^f	759	813	862	907	972	1,041	1,112	1,188	1,270	1,359	1,454	1,572	11,738
Nongroup Coverage and the Basic Health Program													
Outlays for premium tax credits and 1332 waivers ^g	69	71	85	92	71	71	74	78	81	84	90	96	821
Revenue reductions from premium tax credits	n.a.	11	13	17	19	11	11	11	12	12	12	13	131
Outlays for the Basic Health Program	10	10	10	10	9	10	11	11	12	12	13	14	111
Collections for risk adjustment	-8	-7	-8	-9	-10	-10	-10	-10	-10	-11	-11	-12	-100
Payments for risk adjustment	7	7	7	8	9	10	10	10	10	11	12	12	98
Subtotal	n.a.	92	108	118	98	92	96	100	104	108	115	122	1,061
Other Federal Subsidies Associated With Health Benefits ^h	n.a.	43	41	42	47	51	55	58	63	67	72	78	575
Net Subsidies	n.a.	1,844	1,878	1,957	2,104	2,252	2,390	2,537	2,694	2,864	3,048	3,257	24,980
Memorandum:													
Net Subsidies as a Percentage of GDP	n.a.	7.0	6.9	6.8	7.0	7.2	7.3	7.5	7.7	7.8	8.0	8.3	n.a.

Data sources: Congressional Budget Office; staff of the Joint Committee on Taxation. See www.cbo.gov/publication/59273#data.

The table shows federal subsidies for the Social Security area population. Estimates do not include discretionary outlays (such as spending on veterans' health programs) or outlays made by the federal government in its capacity as an employer (such as spending on the Federal Employees Health Benefits program).

Positive numbers indicate an increase in the deficit, and negative numbers indicate a decrease in the deficit.

ACA = Affordable Care Act; CHIP = Children's Health Insurance Program; GDP = gross domestic product; JCT = Joint Committee on Taxation; n.a. = not available or not applicable; * = less than \$500 million.

a. Actual amounts are estimated on the basis of preliminary data and are subject to revision.

b. The estimates shown, produced by JCT, reflect the tax value of the exclusion of employment-based health insurance from federal income and payroll taxes, as well as the penalty payments by employers, but not the tax value of the exclusion associated with Medicare wraparound coverage for former employees. The tax value represents the change in tax revenues that would result if the exclusion from federal income and payroll taxes was repealed and the total compensation paid by the employer (including the employer's payroll taxes) remained constant through increases in wages. Those estimates differ from estimates of the tax expenditure for the exclusion, which represent the change in tax revenues if the amount of excluded compensation was taxed and was larger than the tax value.

c. Includes increases in outlays and reductions in revenues.

d. Excludes the associated effects on revenues of changes in taxable compensation, which are included in the estimates of the tax exclusion for employment-based insurance. If those effects were included, net revenues from penalty payments by employers would total \$8 billion over the 10-year period.

e. For Medicaid, estimates reflect spending for medical services only for those enrollees who have full Medicaid benefits.

f. Spending for Medicare beneficiaries enrolled in both Part A and Part B. Estimates include Part D benefits, are calculated net of premiums and certain other payments to the government, and have been adjusted to exclude the effects of shifts that occur in the timing of monthly payments when October 1 (the first day of the fiscal year) falls on a weekend.

g. Under section 1332 of the ACA, states may apply for waivers from some of the rules governing insurance markets or programs offering health insurance established by the ACA. To obtain a waiver, a state's proposal must be budget neutral and provide comparable levels of insurance coverage.

h. Includes federal subsidies for people with supplemental or partial coverage that, on its own, would not provide financial protection against major medical expenses and thus would not meet CBO's definition of health insurance. Includes the tax value of the exclusion associated with Medicare wraparound coverage provided to former employees, Medicare spending on enrollees who receive only Part A or Part B benefits, and Medicaid spending on enrollees who receive partial benefits, such as beneficiaries who are also eligible for Medicare for whom Medicaid pays only Medicare premiums or cost sharing.

Supplementary Table 1: 2021 10-year CBO Scoring of Net Federal Subsidies for health insurance. Used in the UCC 10-year scoring table.

Table A-1.

CBO's Projections of Health Insurance Coverage, by Source

Millions of People, by Calendar Year

	Actual, 2022 ^a	2023	2024	2025	2026	2027	2028	2029	2030	2031	2032	2033
Total Population	334.5	336.0	337.8	339.4	340.9	342.5	344.0	345.6	347.2	348.8	350.4	351.9
Insured people	309.2	311.8	310.7	311.2	311.6	312.3	314.0	315.6	317.5	319.2	320.8	322.3
Uninsured people ^b	25.4	24.3	27.1	28.2	29.4	30.2	30.0	30.0	29.7	29.6	29.6	29.6
Employment-Based Coverage ^c	162.7	161.3	161.1	161.1	162.5	163.6	164.0	164.2	164.8	165.1	165.6	166.1
Medicaid and CHIP ^d												
People age 65 or older in Medicaid	5.8	5.9	5.7	5.7	5.9	6.0	6.1	6.2	6.3	6.4	6.5	6.5
Blind and disabled people in Medicaid	8.1	7.9	7.7	7.6	7.7	7.8	7.9	7.9	7.9	8.0	8.0	8.1
Children in Medicaid	35.5	35.3	32.3	31.1	30.9	30.6	30.4	30.4	30.3	30.4	31.0	31.9
Adults made eligible for Medicaid by the ACA	17.5	17.4	14.6	13.8	13.9	14.1	14.2	14.4	14.5	14.7	14.9	15.2
Adults otherwise eligible for Medicaid	17.1	17.0	14.0	13.0	13.1	13.1	13.1	13.1	13.2	13.2	13.3	13.3
CHIP	7.0	7.0	7.0	7.0	6.9	6.9	6.9	6.9	6.9	6.7	5.7	4.5
Subtotal	91.0	90.4	81.3	78.3	78.4	78.6	78.7	78.9	79.2	79.4	79.3	79.5
Medicare ^e	59.1	60.1	61.4	62.7	64.1	65.6	67.0	68.3	69.4	70.5	71.4	72.3
Nongroup Coverage ^f												
Purchased through marketplaces												
Subsidized	12.1	14.2	16.3	16.9	12.6	11.4	11.4	11.3	11.2	11.2	11.4	11.6
Unsubsidized	1.3	1.1	1.1	1.1	2.2	1.5	1.5	1.5	1.5	1.5	1.5	1.6
Subtotal, purchased through marketplaces	13.4	15.2	17.4	18.0	14.8	12.9	12.9	12.9	12.7	12.7	12.9	13.2
Purchased outside marketplaces	3.6	3.6	3.8	4.5	5.3	5.2	5.2	5.2	5.2	5.3	5.3	5.4
Subtotal, nongroup coverage	17.0	18.9	21.3	22.4	20.1	18.2	18.2	18.1	17.9	18.0	18.2	18.6
Basic Health Program ^g	1.1	1.2	1.1	1.0	1.0	1.0	1.0	1.0	1.0	1.0	1.0	1.0
Other Coverage ^h	6.0	6.3	6.5	6.5	6.6	6.6	6.7	6.7	6.7	6.7	6.7	6.8
Memorandum:												
People With Multiple Sources of Coverage	27.7	26.5	21.9	20.9	21.2	21.4	21.5	21.6	21.6	21.6	21.5	22.0
Uninsured People With Medicare Part A or Part B Only	0.7	0.7	0.7	0.7	0.8	0.8	0.8	0.8	0.8	0.8	0.9	0.9
Share of the Population That Is Uninsured (Percent)	7.6	7.2	8.0	8.3	8.6	8.8	8.7	8.7	8.6	8.5	8.4	8.4

Data source: Congressional Budget Office. See www.cbo.gov/publication/59273#data.

The table shows coverage for the Social Security area population. Estimates reflect average monthly enrollment over the course of a year and include spouses and dependents covered under family policies. Estimates for each source of health insurance exclude people with supplemental or partial coverage that, on its own, would not provide financial protection against major medical expenses and thus would not meet CBO's definition of health insurance. For a fuller discussion, see Appendix B. The components exceed the total population because some people report multiple sources of coverage and CBO did not assign them to a primary source.

ACA = Affordable Care Act; CHIP = Children's Health Insurance Program.

- Actual amounts are estimated on the basis of preliminary data and are subject to revision.
- CBO considers people uninsured if they are not covered by an insurance plan or enrolled in a government program that provides financial protection from major medical expenses. Estimates include people enrolled only in Medicare Part A or Part B, people receiving only partial Medicaid benefits, and people enrolled in some short-term plans.
- Does not include people with Medicare wraparound coverage provided through a former employer.
- Medicaid enrollment includes only enrollees with full program benefits. Estimates have been adjusted to account for people enrolled in more than one state.
- Includes only people who are enrolled in both Medicare Part A and Part B.
- Many people can purchase subsidized health insurance coverage through the marketplaces established under the ACA, which are operated by the federal government, state governments, or partnerships between the federal and state governments. Estimates do not include enrollees in supplemental Medigap plans.
- Created under the ACA, the Basic Health Program allows states to establish a coverage program primarily for people with income between 138 percent and 200 percent of the federal poverty guidelines. The federal government provides states with funding equal to 95 percent of the subsidies for which those people would otherwise have been eligible through a marketplace. Only Minnesota and New York currently operate such a program.
- The other sources that cover the most people are student health plans (3 million) and correctional facilities (2 million). The Indian Health Service and foreign sources of coverage account for most of the remaining people in this category.

Supplementary Table 2: 2021 10-year CBO Projection of health insurance coverage. Used in the UCC+HSE 10-year scoring table.

Total Medicare Financial Projections

Table V.B3.—Medicare Enrollment

Calendar year	(In thousands)				Total ²
	HI	SMI		Private health plans ¹	
	Part A	Part B	Part D		
Historical data:					
1970	20,104	19,496	—	—	20,398
1975	24,481	23,744	—	—	24,864
1980	28,002	27,278	—	—	28,433
1985	30,621	29,869	—	1,271	31,081
1990	33,747	32,567	—	2,017	34,251
1995	37,175	35,641	—	3,467	37,594
2000	39,257	37,335	—	6,856	39,688
2005	42,233	39,752	1,841	5,794	42,606
2010	47,365	43,882	34,772	11,693	47,720
2015	55,246	50,756	41,786	17,495	55,589
2016	56,729	52,094	43,198	18,393	57,073
2017	58,344	53,446	44,475	19,817	58,683
2018	59,677	54,679	45,798	21,338	60,020
2019	61,188	56,019	47,179	22,950	61,535
2020	62,544	57,320	48,697	25,075	62,887
2021	63,643	58,393	49,976	27,547	63,980
2022	64,822	59,502	51,393	29,840	65,165
2023	66,220	60,800	52,884	32,161	66,585
2024	67,216	62,034	55,222	34,114	67,601
Intermediate estimates:					
2025	69,130	63,484	56,738	35,494	69,537
2026	70,673	65,010	58,588	37,134	71,101
2027	72,332	66,704	60,457	38,831	72,778
2028	73,937	68,313	62,132	40,407	74,402
2029	75,405	69,795	63,479	41,838	75,888
2030	76,635	71,064	64,578	43,079	77,134
2031	77,593	72,099	65,447	44,120	78,105
2032	78,420	73,015	66,209	45,018	78,945
2033	79,174	73,857	66,909	45,813	79,711
2034	79,972	74,722	67,646	46,554	80,520
2035	80,790	75,578	68,399	47,240	81,349
2040	82,862	77,932	70,530	48,877	83,456
2045	83,969	79,144	71,626	49,653	84,582
2050	85,794	80,774	73,101	³	86,426
2055	88,303	83,057	75,168	³	88,956
2060	91,291	85,884	77,726	³	91,964
2065	94,004	88,621	80,203	³	94,698
2070	97,063	91,620	82,917	³	97,782
2075	100,134	94,608	85,622	³	100,880
2080	102,111	96,681	87,497	³	102,874
2085	103,297	97,997	88,688	³	104,071
2090	103,602	98,505	89,148	³	104,375
2095	104,699	99,523	90,070	³	105,477
2099	106,166	100,850	91,271	³	106,955

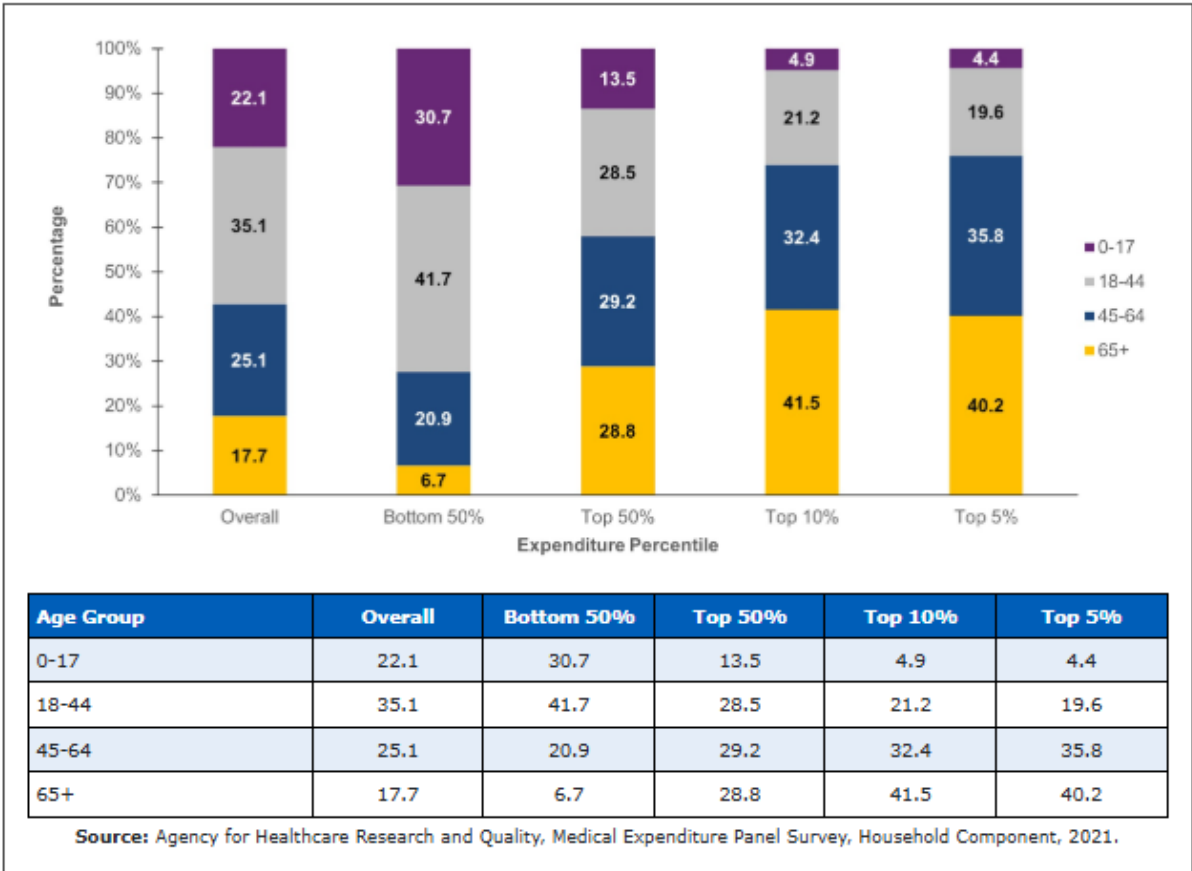
¹Of Medicare beneficiaries enrolled in private plans, about 97 percent are in Medicare Advantage plans or Part C. The remainder are in certain holdover plans reimbursed on a cost basis rather than through capitation payments, in Program of All-Inclusive Care for the Elderly (PACE) plans, or in Medicare-Medicaid Plans (MMPs).

²Number of beneficiaries with HI and/or SMI coverage.

³The Trustees do not explicitly project enrollment in private health plans beyond 2045.

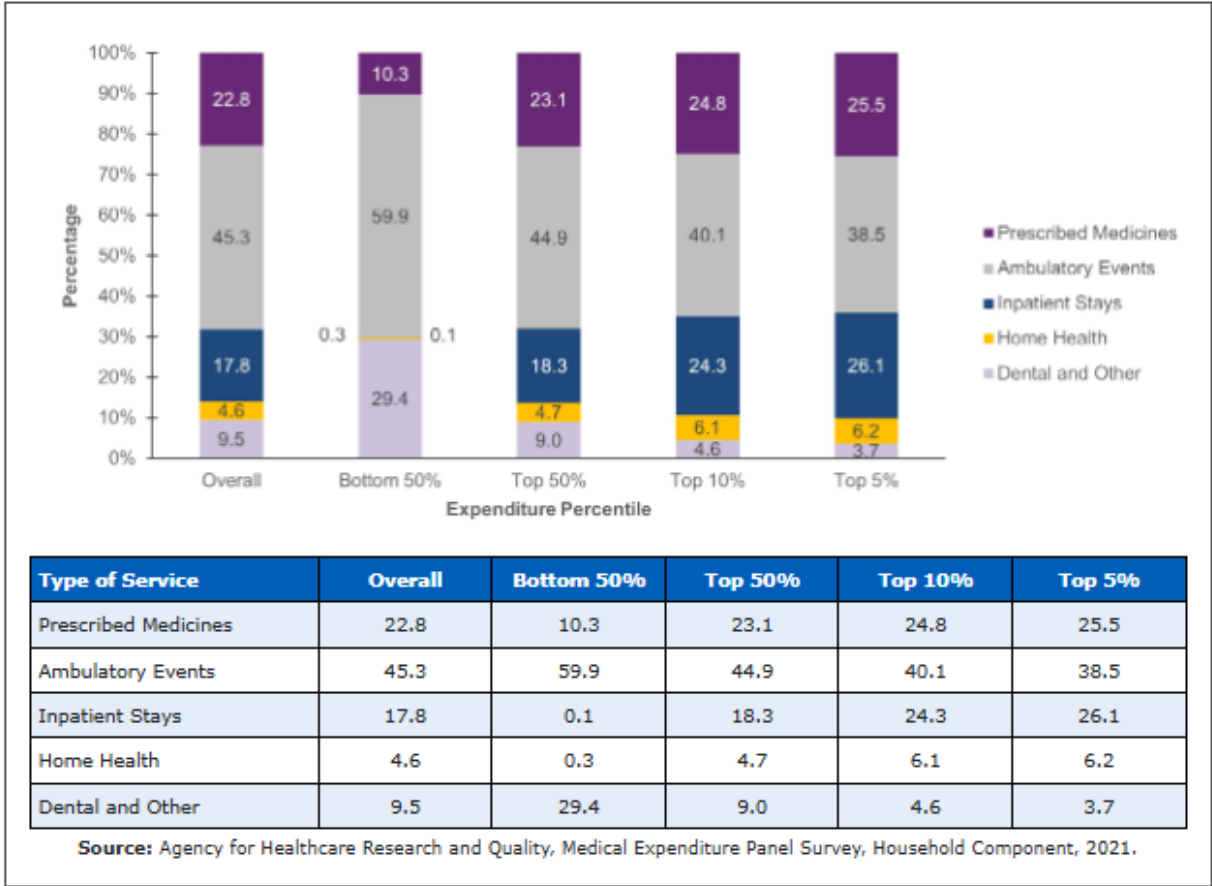
Supplementary Table 3: Future projections of Medicare Enrollment. Used in the UCC+HSE 10-year scoring table.

Figure 3. Percentage of persons in each age group by expenditure percentile, 2021



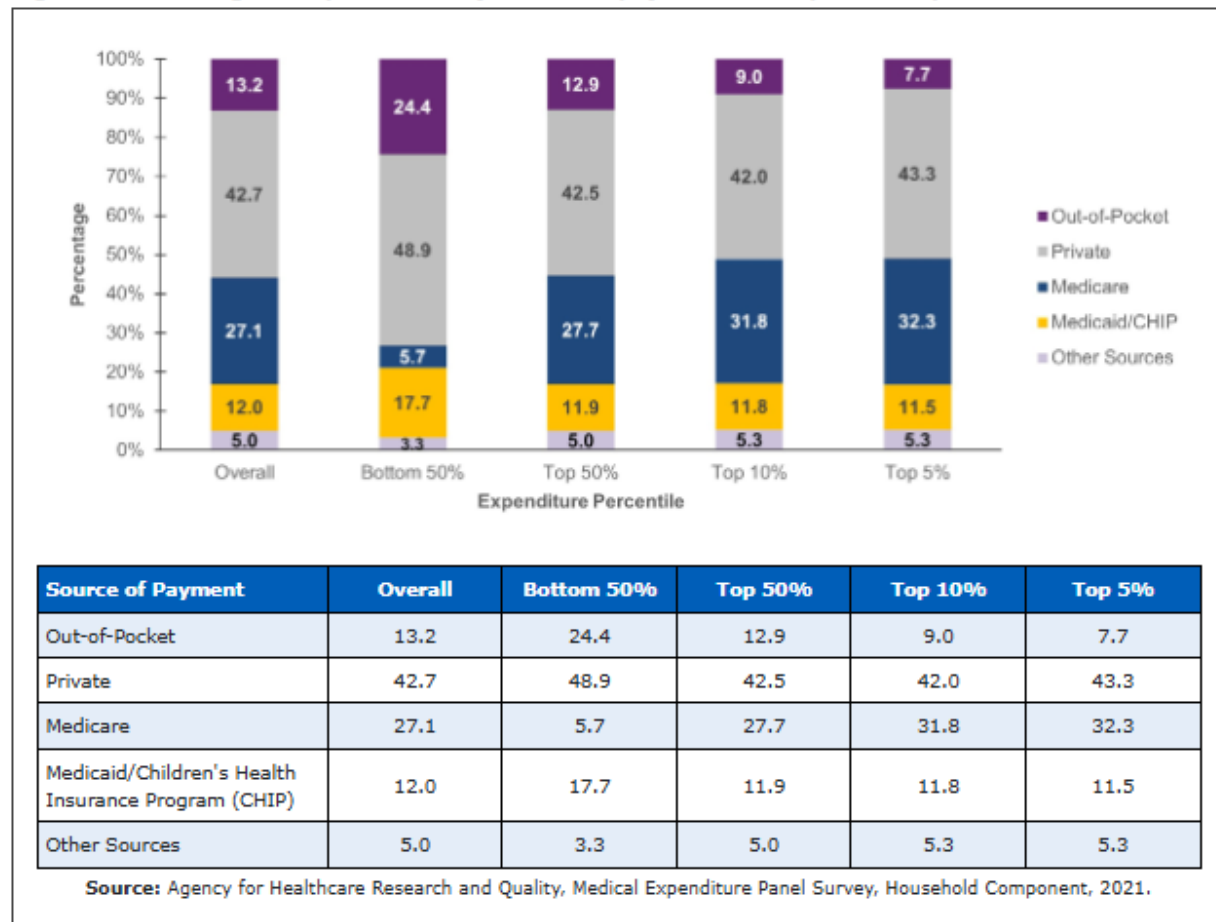
Supplementary Figure 1: The Agency for Healthcare Research and Quality (2021) suggests that most healthcare expenditures are captured within the elderly population.

Figure 5. Percentage of type of service used by expenditure percentile, 2021



Supplementary Figure 2: The Agency for Healthcare Research and Quality (2021) also suggest that the bottom 50% of expenditures rely on are ambulatory events dental and prescription medicines.

Figure 6. Percentage of expenditures by source of payment and expenditure percentile, 2021



Supplementary Figure 3: The Agency for Healthcare Research and Quality (2021) also suggest that the bottom 50% of expenditures spend the highest proportion out of pocket as well as on private insurance.